

Glaxosmithkline Pharmaceuticals Ltd. (GLAXO)

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To,

BSE LIMITED THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED Phiroze Jeejeebhoy Towers Exchange Plaza, 5th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex , Bandra (East)
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Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 23rd August 2022. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/media/analysts-meets/>.

This is for your information and record.

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN No. L24239MH1924PLC001151

Moderator: Good evening, everyone. For those of you here, we have close to 14 people in the room. We also have our Managing Director and our CFO who have joined us. And we have about 20 people who have joined us online. So, thank you all for joining. It's 4pm. And we'll start the presentation without further ado. A few housekeeping rules: One is, we will be recording. We will be recording this conversation and the presentation and the transcript, as well as the audio recording will be available on the website. And of course, you can ask us and we'd be happy to share that with you too just in case you're unable to access the website. I know it's been a while. I think it's been over two years since we had a last analyst meeting thanks to COVID. But we are back again and hopefully COVID will not. Okay, it is being recorded, perfect. Let me welcome those online as well as here. We will follow a format, which is really an overview by our Managing Director, followed by a financial overview by our CFO. And then of course, we will take questions. Before I invite our Managing Director; a few words, I mean, all of you notionally that joined us just before COVID. And Sridhar, he joined us from his role in Singapore. And he's now with GSK India for more than two years. The new addition, for those of you who haven't met Juby. He joined us very recently as our CFO. He is a chartered accountant from the Institute of Chartered Accountants

of India. And his previous experience includes roles such as finance director leader roles in South East Asia, Vietnam, Turkey, and Middle East. Just immediately prior to this, he was head of finance, global general medicines and was based in Singapore. So, welcome Jubin. And with that, I would now hand it over to our Managing Director, Mr. Sridhar Venkatesh to take you through an overview of GSK.

Sridhar Venkatesh: Okay, all right. So, great to be here. Welcome, fantastic to meet all of you face to face. I mean, we've been doing this in a small box for many, many years. And I was looking forward to do this once before my tenure ends at GSK India at some point of time, face to face. But good to be here and for all those who are online, a warm welcome to all of you. What I promised you is that we will spend most of our time today on Q&A. And we will probably spend 1 hour and 15 minutes. And we will use potentially an entire hour to try and respond to you in the Q&A session.

So I personally think for us at GSK, it's never been a more exciting time for us than where we are today. Because globally, we made a huge stand in terms of what we are going to be, what we are going to do, what therapeutic areas and I think globally, we've taken a stance around after the successful split that we have had between our pharma and our consumer team in terms of what's our clear purpose. I think the more and more you talk about what's happening across the world, it's all becoming a confluence of science, technology and talent. And that's something you need the right kind of people at the right places for us to be successful. So I personally think when I look at some of our global statements that we are making, some of the steps that we have taken, post our demerger, if I were to call out, all of them have been pointing in this direction. Some of the newer acquisitions that we made, some of them be it in the space of oncology, be it in the space of newer vaccines - all have pointed towards that.

Now, clearly, as we look at that, what we also want to do through that is to impact around 2.5 billion patients in the next 10 years and positively impact them through our portfolio. Now, where are those 2.5 billion patients going to come? I mean, you can't think about it, you can't take out India out of that equation, when we talk about 2.5 billion patients, some bit of rough calculations that we have done suggest around

and 60%

of the 2.5 billion will be out of India. So a huge part, I can say, of the global GSK purpose, will be played out of what we do in India. And you will see that in terms of our ambitions, in terms of what we are attempting to do with our newer portfolios. And clearly, this is also a company where we want everybody to be themselves, really enjoy what they want to do, thrive and do what they really love to do. So that clearly is our overall purpose and what we are attempting to create globally and what we are attempting to do in India.

Now what does that mean to India when we talk about this, right? And when we say we want to be ambitious for patients, what does that mean? We want to bring innovation into this country. What does that mean for us in India? Nucala was the first one that we talked about, we just launched Trelegy. In the last few months, we are gearing up to launch a world leading vaccine, which is just path breaking in terms of the treatment of shingles, right? If anybody knows what shingles is, it is one of the most painful diseases that anybody can get. We have 10 year data to showcase that this is one of the best vaccines. In terms of what do we mean by when we say accountable for impact, we mean - how can we maximise the portfolio that we have and I will show you some data out there, which will clearly articulate how we are being accountable for impact in terms of the portfolio that we have, like gaining share and still being relevant to a marketplace.

Of course, underpinning all of that by saying we do the right thing at all times, right? And that's been something that's been absolutely clear. We have zero tolerance and no compromises on that particular space. Let me very quickly go to what GSK India means to the global GSK. We are the second highest contributor to emerging markets. Okay, so clearly I keep telling my team that when India gets a cold, my boss gets fever, right? Because we form a huge part of our overall portfolio. We also contribute greatly to many of our key brands that we have. Augmentin, there are 17% of the emerging markets flow, Derma we are on 45-50% of emerging markets, clearly a huge portfolio. And we clearly feel that in India, we still have a long way to go with our dermatology portfolio.

Now, as I look at this portfolio for us and GSK India moving forward, clearly, the goals that we have are pretty simple. The products that we have today are very relevant, and what do we need to do with them is continue to grow faster than the market, have an evolution index greater than 100. Secondly, what Jubin is focused on and all of us are focused on as we grow, how do we continue to improve on those margins and you will see some examples of what we are doing with Jubin's presentation. Vaccines

is something we are extremely excited about. As we speak today, there is a cohort of around 24 to 25 million per year. I mean, the birth cohort in India is 24 to 25 million. And if you see one of the most penetrated paediatric vaccine in this country is a pneumococcal vaccine which gets to 1 million kids.

And I'm not talking about the National Immunization [Policy] vaccine, so don't get me wrong. So, the National Immunization vaccine gets you around 80-85% ... So forget and I'm not talking about that, but I'm talking about those vaccines, which are outside of the NIP. And clearly, some of these vaccines are real, you know, path-breaking vaccines, which can be used by many more patients. So we are very excited about that category of patients. Adult vaccination - what COVID has done is expose that space significantly. And if you look at our portfolio, starting with Shingrix, we have flu, plus some of the other ones. Globally, you might have heard about RSV. Again, that's something which is still under regulatory approval. All of those vaccines are going to open up the entire space in the adult vaccination piece.

And that's something we really want to grow for ourselves in the future. And we think that's a big space that we will grow.

Speciality, of course, there's Nucala and Trelegy. Nucala, again, is a very initial indication, but Trelegy is, doing exceptionally well, globally. We've just started that in India, we have had some very good early uptake on oncology, this is the indication that we have COPD. And we clearly have, you know, we clearly think this could be another big product for us over a period of the next couple of years. How have you been doing? You might have heard me say earlier on the three things that we want to do - build, grow and shred. Build new assets, muscles that we don't have, that's what Nucala is, that's what Trelegy is, and that's what Shingrix is going to be. Grow on some of the assets that we have, what can we do to ensure that we simplify our operations, do what is right, focus on those core brands and continue to grow that and shred some of the things that we did, you might have seen some decisions that we've taken over a period of time. Fundamentally, we want to take away the drag. And that's what we've been attempting to do. I'm not saying that we are done with that journey. That's a continuous process. But we have taken some significant steps on the shred piece. So what I'm trying to say here as we've gained market share in our key promoted brands

overall, even during COVID, we grew some of the market share. There was a fear that after COVID, we will drop all that market share, everything will go away when the other generics come back.

In fact, we've held on to that market share. And you feel our projection is by the end of the year, we'll actually gain through that because this is the key season for us from our acute portfolio and we possibly are going to see that market share kind of growth. We continue to lead in our represented markets and as we say, we are the number one dermatology player in the self-pay market. We continue to be there, we have prescription leadership for our core portfolio bet in GPs, ENTs, paediatricians, we continue to hold that prescription share position and some of our core brands, we continue to maintain that number one position in the represented market.

Vaccines, we've been doing a huge amount of activities, and now I'm not sure any of you got exposed. This is all about creating more awareness regarding vaccination. There were a lot of kids who did not get vaccinated during the period of COVID. We were right out there talking about what more can be done. But of course, a lot of awareness across the various digital forums, doctors' clinics and many other places so that people understand what vaccination is all about, and especially support them in their own vaccination journey. This is all about paediatric vaccines.

Building Speciality, I talked about Nucala. We continue to be encouraged. We launched Nucala right before the perfect storm which is when COVID hit us. And during that time, pulmonologists were, rightly, super focused on treating COVID patients. Therefore, we overall saw the uptake that we saw early on was not exactly the way we would have liked it. But again, I'm very happy to say that, post COVID, well, I can't say post-COVID right now; whatever it is right now, I'm starting to see very good acceleration. Similarly with Trelegy, we see good acceleration. It was again a unique launch - of physical and digital. And again, pulmonologist seemed excited about a device, a device which is once a day and will clearly impact many patients, including physicians and pulmonologists, especially, who have taken to this brand. Shingrix, we're getting ready to launch in the first half of 2023. That's the broad, I would say, the commercial part of what we are doing on the business front.

I'll hand it over to Juby who will quickly walk you

through some numbers, facts and

figures. And, as I promised, we will be done in the next 8-10 minutes and then we'll

open the entire hour for Q&A with all of you.

Juby: Good evening, everyone. Thank you, Sridhar. So, for the next couple of minutes, I'll take you through the high level financials and I'll also articulate some of the progress that we're making over the last couple of years. So firstly, this is a quick snapshot of 2022. If you see, our sales is 3,190 crores and 11 percent growth. Now what's really important is price as well as the volume - both are growing. And each pillar of our portfolio, promoted brands, which is growing around 25 percentage which is where we are more focused on all our brands which are promoted - all our resources are allocated into those brands. And that's demonstrating strong progress in terms of growth.

General medicines, which form part of the entire pharmaceutical brands is growing 19%. Speciality - which includes Nucala and all the new assets where we are getting new products launched - that's growing 86% almost doubling last year. Vaccines saw a decline of 9 percent. Again vaccines as you know, during COVID times there was decline in number of babies visiting the hospitals. And that's a temporary blip we could see in the market. And again, if you see the growth on some of our core brands - Calpol, Augmentin and Supacef - all are demonstrating very strong growth. Right hand side, you can see the margins, 59 percentage margin on a large base, improving our ratio by another three percentage, PBT 24 percentage, again, improving on a large basis, the ratio another two percentage, EBITDA also improved three percentage from 20, to 23. And dividend, we have consistently declared in the past.

This is a quick snapshot of the quarter results which where the reported growth was 3 percentage as you could see, but if you see the underlying growth, underlying growth here is excluding the impact of Calpol, which had a higher base last year due to COVID

is 12% underlying growth. That's coming from 6% price and 6% volume. So the underlying growth is very strong. And if you see all the pillars of our portfolio, general medicines, speciality and vaccines, all the three pillars are showing very strong growth.

And that's giving us confidence that our strategy is working. Our promoted brands, as you could see, the market share has improved. And all the three pillars, the underlying growth is very, very strong. And we are continually focusing on gross margin and

EBITDA, which you will see in the next slide also, we have demonstrated improvement.

And those who are following this closely, we are on a seasonal business, quarter two tends to be the quarter one of the financial year tends to be the lowest quarter and September ending quarter will be the highest quarter due to seasonality and this is a trend which you could see during the past two years. And just to bring you up to speed on, what are we talking about our strategy over the last two to three years, we've been doing several things that is demonstrating in our financials. If you see the three parts of our financials, revenue, gross margin and EBITDA, all the three are showing strong improvement, revenue has grown 8 percent. If you see gross margin, despite all the headwinds we had in the market, it's grown 2 percent, ratio has grown 2 percent from 57.6 to 59 and EBITDA if you see, there is a 5% improvement. So now the EBITDA is from gross margin improvement, almost 1.8 percent. Employment costs where we are finding efficiencies, we are focusing on promoted brands where it makes sense. That's giving us a 2% improvement and cost efficiencies. We're continuously working on cost efficiencies, and that's also giving us a strong uptake in our EBITDA margins.

Now, just to bring you for the last five years, what's happening on our portfolio, the left hand side

de, we have the reported numbers which numbers have got divestments excluded on the last year but the base has got some discontinued divested brands. But if you see the 5 year CAGR on the continuing portfolio, like across all the years, which is the real underlying growth, general medicines - 9% growth, vaccines - 10%, speciality, again, it's double digit growth, 100% growth - it's on a small base. And we are consistently delivering very, very strong cash flow. And that's why we could declare dividend also, year on year. So the bottom line, the story I'm trying to make here is our underlying growth, despite the divestments, despite the headwinds we have is very strong enough financials.

So lastly, this is my last slide. I just want to bring you again, to context with all these changes that we're making in the last couple of years. If you see our EBITDA margins, it's started at 16 percent on the left hand. Now as we speak in the last reported financials - is 23 percent. So a strong improvement has been made across all the years, consistently. Improvement, if you see across all the years on EBITDA margins, and that's on the backdrop of several initiatives we were doing. One is the cost efficiencies we are trying to do, the focus on promoted brands where it matters - we were reallocating our resources. We were discontinuing some brands, which doesn't make sense for us, which was articulating about new products.

And as we move forward, what we tried to do, we continue to do well, some of the things which were working, which is like growing up promoted brands, growing up focus brands, which has got better margin, WPI price increase, which will be playing out, we will have challenges in terms of NLEM price cuts, but our WPI price increases are going to help us. New products, the new launches, that is going to again drive growth. And also we'll continue to focus on cost savings. So, basically, we expect to sustain this EBITDA margins as we go forward, and try to further improve upon them. So, over to you, Sridhar. Sridhar Venkatesh: So, look, as I just want to also talk about, as we've been talking through this, we've also been very proud of how the teams have been, you can leave it there, how we've also been awarded quite a few awards across in terms of one of the best workplaces, when you can get a sense of what happened when you walk in, you put in a huge amount of effort, working women, risk awards have been overall, very proud of what we've been able to get. And at the end of the day, we never forget that we are here for patients. Every year, we touch around 200 million patients through our medicines, which is a significant number. We do around, I mean, our medicine reach around 6 million immunisation again, never forget that. That's a core of what we do. That's why we come here on a day to day basis to work for that's what keeps us going on an ongoing basis. With that, I think as I promised, we'll be done in 15 minutes. And hopefully, we have enough time for Q&A.

Moderator: Thank you, Sridhar and Juby. May I request you to kindly take the mics and also be seated. Okay. So, we will now open up for questions, the format will be as follows: those online, kindly raise your hand, and the moderator will then unmute and call out your name. And then you can choose the question that you have. Please introduce yourself, your name, which organization you are from. And specifically, if you'd like the question to be posed to either Juby or to Sridhar. Before we start, I think the first preference would then be given to people in the room because, obviously, you guys have taken, you know, the time out from your busy schedules. So those of you who would like to raise a question here, kindly come forward, because we can then give you a mic or we can pass on the mic to you.

If any of you are thinking in the meantime, let me start with a question that

came via

email from Sanjeev Vaidyanathan, Partner, Eternity Capital. He's put in a set of three questions. What is the long term outlook for growth margins and returns for the business? It would be ideal if you can split this outlook by business segment - pharma, chronic acute versus vaccine, and self-pay, sponsored. So Juby, may I request you to respond to that question.

Juby: Thank you. I think firstly, in chronic versus acute, our business is more acute. We've got only 5% chronic segment, which is only one brand, mainly Eltroxin. So majority of our business is from acute business. Now in terms of guidance, forward guidance, as you know, we are not providing forward guidance for businesses. On the past performance you would have seen the last year with growing 19% on the promoter brands and last five years, we demonstrated the growth. Do you want to add anything?

Sridhar Venkatesh: Look the question sounds like how I probably am presenting to my boss out there right, but look, fundamentally, just to give you a perspective, you saw the bets that we are making is a focused portfolio. You're growing double digits, our thinking is we can. India is a broadly underpenetrated market, we have an opportunity. So, we'll potentially go in that direction. The big growth that we will potentially see is in our new launches, especially we think of when we're launching Shingrix, we think that will be a hugely growing space. In terms of our paediatric business, as I said, we have a 26 million birth cohort, 1 million most penetrated vaccine, 4 million patients who are in the SEC category who can afford some of these private vaccines.

So look, I mean, you can do your math, but it's in my mind. It is, I think we are here for considerable future growth, they fit most of our portfolio. We will remain focused on the portfolio that we have. But we think there is a massive opportunity with what we have right now.

Moderator: Thank you Juby. Thank you, Sridhar. The next question again. So this is the second of three questions. What are the competitive advantages that you have, and continue to build on, which allows you the above long term visibility? It would be great if you can explain how these advantages compared to those of your most relevant competitors.

Sridhar Venkatesh: Look, I think the way I look at that is the biggest advantage that I have is I can have the entire GSK landscape to play the amount of innovation that is coming through GSK at a global level period oncology, dermatology, vaccines, adult vaccines, anti infective, it is huge. And I have the power of bringing all of those into

the Indian market. And to me that innovation is something that 's going to drive - number one. Number two, I think even though I have products like Augmentin and Calpol help all which incidentally, save the most lives during COVID . It was not the vaccines helped, but Calpol is the one which saved the most lives during COVID time - by a small pill . These products are still extremely relevant. And these products have become a household name. And we think there is an opportunity to continue to grow these brands, the strength of the brands, the equity that these brands command, right ? It is very significant.

We also have an opportunity to continue to generate local data on some of these molecules. So, somebody thought, Augmentin is a 50 year old brand, we still continue to bring data on antimicrobial resistance, what is happening, we invest a lot on some of those things. I think our real competitive advantages , so the strength of the brand, the relevance of the brand, the future of the portfolio, and newer areas that we think clearly are, I think , the competitive advantages that that we have in GSK India.

Moderator: Thank you, Sridhar. Last question of part three - what is the best way to understand the difference in operation of GSK pharma listed entity and GSK Asia on

listed subsidiary? In that context, why were Iodex and Ostocalcium originally sold

through the listed entity, but transferred last year to GSK Asia?

Sridhar Venkatesh: I'll leave it to the expert.

Juby: So firstly, I think all our sales are through the listed entities , 100% of our sales are through the listed entities . GSK Asia, which the question refers to, is the consumer arm of GSK . It is not our subsidiary, it is not part of our pharma business, it 's a completely different entity. And the Ostocalcium sales we have updated in the public domain in the stock exchange as part of the new GSK , when pharma and consumer split globally, the consumer brands moved to the consumer division and that 's part of this transfer of Ostocalcium to GSK Asia, which is a consumer entity. So it's a completely different business, nothing to connect with this pharma entity here.

Moderator: Okay, thank you. So we'll open up for folks here. He had raised his hand first, then I'll come to you. Just introduce your self, your name, the company, and your question posed to him.

Analyst : Thank you so much. So, I'm Aamir . The first question is on the vaccines basically. So last year was not good for us in terms of growth in the vaccine category. So if you can tell whether the post COVID , the normalization has happened already in the vaccine market? And what is the outlook for the vaccine sales in FY 23. And along with that, if you can also highlight the key products on the vaccine category which you will be focusing on, especially in the adult side. Fluarix is the vaccine which has good potential in India, and your competitor has already started doing lot of ads on radios , etc. So what we are doing to basically compete with them, and also on the outlook of Shingrix vaccine, how big that opportunity could be. Thank you.

Sridhar Venkatesh: Look, I'll attempt to tell you how the market is doing, the market is still sluggish. Okay, so it 's still in negative space. A lot of that has got to do with people either postponing vaccines, they in some of the I talked about the SEC categories, etc. We don't have those data, but I'm sure you have seen those anecdotal reports of birth rates coming down a little bit in some of these places that has all had an impact. As I said, the market is still around minus 10, minus 12 category, that is where they are, of course we are, we continue to hold our share. So while the market is declining, if you're holding our share means we are gaining share from our competitors. So which category early means we still are a key , relevant part of that.

So, in terms of future outlook, I will not be dependent upon how said there is you know, most of our vaccines are zero to one. So for that we need more kids to be out there,

you know, there is a time for kids, post all of this, there is a cycle. So we remain optimistic, not optimistic but hugely optimistic based on some of the lead indicators that we see in the hospitals, etc. So we think this is going to come back , remain absolutely committed on that particular space. You talked about Fluarix and of adult vaccine . Yes, Fluarix is clearly one and, of course, Shingrix in the future. We've been very pleased with what we have done with Fluarix , and I think IMS and IQVIA , do not do justice to what we do and what we sell. But look, I'm not here to talk about that . I mean, you've seen those numbers in our presentation, how we report . But clearly, we are what we've been able to do in the last two years has been really been significant. Probably, I need to see which channels you see, and I need to get my ads in front of you. There are a lot of those video too. Especially the ones recently you might have seen. They are not ads, let me clarify myself. We don't do ads, we never do ads. What we do is disease awareness campaigns . So they were the disease awareness campaigns, all around

and getting flu, and you would also find that the current strains also protect you. So you might have seen some of those. So yes, we do a lot of those activities, future outlook or Shingrix. Look, I'm not going to sit here and speculate. I

can give you the indication. Indication is 50 years plus who needs that shot.

We don't have the other indication yet in India, which is 18 plus immunocompromised.

I don't have that indication yet in India, but I think 50 plus people should be taking it.

Is it going to be, will everybody take it? No. I mean, I don't know how many of you have taken flu shot here in the room, or in the call, there is a little bit of an apathy about adult vaccine. And that calls for companies like us to spend more time on disease awareness. So that's something that you will see continuously.

Moderator: Great. The next question, I will pass on the mic and maybe you can introduce yourself.

Analyst: Hi, thanks for the opportunity. Nikhil Upadhyay from Securities Investment Management. I have two to three questions: one is on the vaccine side. If we look at the last two and a half years, what you mentioned is quite relevant that the entire cohort has not been there. People were not going but what we thought is that there would be a significant pent up demand whenever the things start opening up. But even in the AGM, and now also you mentioned that the market is still degrowing. So is it that something structurally has changed, because what we have also seen is that government has added a lot more vaccines in the basket of which was already there. So is there anything structural which has been seen or is it still something which you believe is temporary?

Sridhar Venkatesh: So, look, I think, yes, I think the government has taken some steps. And clearly, there are a few patients who will go into private or go into the government category. So clearly around the pneumococcal space, there is some of that that has happened. But I also think people have postponed quite a few decisions. So, some of these vaccines that we are talking about are outside of your normal vaccines that you will take. So, from an economic standpoint, people have postponed some of those decisions. There has been a pent-up demand, but it has not been as everybody has, who has not taken that vaccine has come back up. And also clearly the birth cohort, we think there is a time people have postponed.

Look, when I'm speaking to my boss, it is hard to say that marriages in India got postponed, therefore kids get postponed. And people don't understand that sometimes, and it's taken me time, for me to make people understand that. So, there is a cycle. We are seeing that cycle slowly turning around, right there are as I said, some lead indicators of other medicines that are used, which will help us tell us what's happening to the birth cohort, etc. And we have to be patient. I think it's going to take the end of the year for this to kind of bottom out. And I think slowly only next year, the markets potentially starting to come back up.

Analyst: Second, if we look at last 10 years of GSK we've seen and one clarification you mentioned at the starting before the 10 years end. So, is it something closer today?

Sridhar Venkatesh: So please, please, you know, there's nothing called everybody's 10 year end, mine will end some point of time - just making an off the cuff comment.

Analyst: So, if over the last 10 years from where you were, you were handed over Glaxo as the MD, and what we've seen is that the company at the MR productivity level or at the growth level has been a laggard. And many of the smaller companies have taken market share from and we were the leader at some point. In the early 2010s, it was like Glaxo's MR productivity was the benchmark. So, what has changed and what structural changes you have brought in over the last three to four years which gives you

the confidence that probably now the things are set for us to probably benchmark or benefit from the steps which we have taken?

Sridhar Venkatesh: A fantastic question. So look, I think 10 years ago, we were a different company, our outlook was different, what we could launch in this country was different. The regulatory environment was different, the scientific proof of what the bar that GSK has for itself, in terms of what I could launch was very different. So, clearly, over a period of time, some decisions were made over, rightfully. So we are a global company, we have to kind of work through one of the things that we have to understand is, what products we can launch and what we cannot launch. And clearly there were some decisions that were made in the last 8 to 10 years on which are the areas that we would focus on. Clearly, also, the environment externally some decisions like Zantac, Vemgal, along the way, have not helped us, but they were all done for the right reasons. I mean, I'm not going to go back into that.

So, I think it's a little bit compared to some of that. And again, I'm not challenging the growth of other companies here. They have done what is right for them, it is good from their perspective, what was good for us was, we needed to look at where do we want

to play, we made the decision in terms of where we want to play. 10 years ago was a different space and a different time. Last couple of years, and two to three, we are in this journey for three to four years. We are very clear about where we want to play. We are very clear about where we don't want to play. For example, if somebody were to say tomorrow, will you launch a product in diabetes? I can say yes, heading now, absolutely not. Do I want to be in cardiovascular? No, that's not a space that I think I have innovation, that I think I can add value. There are enough people out there. So, all the numbers that he was talking about is on those 7 or 8 therapeutic areas, key products that we have vaccines and an innovation, I think you will start to see GSK India in the future, something like what we saw 8 or 10 years ago, but with a different set of products, different set of people and a different shape of the P&L will look very different. Because the skills that I need for immunology, oncology and other vaccine is going to be very different from what I need for a mass product for which I might need another 2,000 people. So, the shape of the P&L is changing the shape of the business which is changing significantly. I think we are somewhere around 60% in that journey as we get those products and you will start to see this very differently.

Analyst: Just one last thing I'll pass on, in this process, because if you look at 2015-16 also and go through our annual reports, we used to focus on the generic medicine, vaccines and speciality. And if we look at now, the way competition is, if a product goes of generic there are 30-40 players who are competing and also probably there is a good roadmap for a player like GSK to build their speciality, Pipelines to a much larger extent create actual brands. For that to happen, how is the organisation structure been created? Like for speciality? What is the kind of MR, team? How is the team being divided and what are the KPIs on which each of them are being measured? Sridhar Venkatesh: Look, clearly, four years ago, we probably had one large team. Now, if you were to see, we not only have, of course, the general medicines team and clearly structured under that multiple teams to focus on the various brands but let's talk about vaccines. We have recently created a completely different team to focus on adult vaccines which report straight into me. So somebody who sits in there is in a business today, which has zero rupees in sales. But we know that that we need to focus on that. So there is a head of paediatric vaccine wh

o reports in to me and there

is a head of adult vaccine reports in to me. So that's a clear investment for the future, including teams under them who are trying to get ready. Speciality we brought in, as I said, we brought in somebody again, a head of speciality, whose whole goal is to create this team right now, the team under him promotes Nucala and Trelegy. But again, as we are getting closer to oncology launches in this country, we will start to see you put people six months, nine months before do with one of the things that we show, it's as I said, I have now four commercial team members reporting to me, the general medicine, paediatric vaccine, adult vaccine, speciality and there's somebody else who looks after the various emerging channels. So we are looking at all of this spectrum of portfolio, that's how we are kind of structured. Of course, KPIs are very different in terms of what needs to be done across or some it is the success of new launch for some others that is how quickly we can get a product into the market. With some others it is how do you shape the market for the new launch, so pretty different.

Analyst: Just why I stressed, sorry, but on the KPI side is like in the focus brand, we've talked about Augmentin, Calpol, Derma. But there are also products like Eltroxin, where we were the leader, the market is still pretty strong, but we've lost the leadership. So is market share defending and ability to grow the product, also an important part for the whole team?

Sridhar Venkatesh: 100%. I mean, that is that is what we live and breathe for. And as I said, I'm not saying here that we've everything is we lost, you know, we want areas where we have one share, there are a couple of products we have. Eltroxin is one. But again, I have to say in the last six months, you will see it in a year's time, I start to see data much more closely. I think we are coming back on that space very strongly.

Analyst: And from three investments on the vaccines on the pent up demand, so if you look at your portfolio, what percentage of the vaccines demand will be permanently extinguished or perish? Because, suppose the children go out of that age bracket. So, any thoughts out there that it might not be a pent up it might be perished forever.

Sridhar Venkatesh: So, they said most of the vaccines are zero to one. I think if you look at, look, then some of the vaccines are extinguished forever, I think the vaccines that we have, that we think we are not going to get back as models where people are going to the government sector to the, especially the pneumococcal space, which is

really moving over to people who can afford to are moving over to that space. On the piece of what do I think people are not going to come back for vaccines.. see, there is, we think that's going to be a pretty small portion. It's not because if you see this , as I said, market was showing a negative 17-18% . We showed a negative growth of minus 3%. So, we actually gained share in most of our key vaccines, right? So it's very difficult for me to say, how much have we given what it is? How much have we completely lost?

Look, I think it'll be around a percentage year or two, a couple of percentage points, but I really don't want to sit here and hazard a guess that how much I would have completely lost.

Analyst : All right. And the second question is, you know, just regarding pneumococcal , so Serum is competing with us directly . Is it completely playing in the government pay market or it's there in the self pay market ?

Sridhar Venkatesh: There are in both, I mean, we are competing. I mean, look, we don't play in the NIP space at all . So, we are not there. They are there in the private space too .

Analyst : Okay, so just anecdotally, the self pay market as a percentage of total vaccine market five years ago. And today, is the self pay market shrinking continuously?

Sridhar Venkatesh: Yes, absolutely. Right now, well, last five years. Look, last two years, as I said, it's been it's been an high negative.. 18-20% decline. Alright, last two years, it's been a decline. But as I said, I think this is , we think , there are enough lead indicators for us to say that this is going to come back.

Analyst : All right. One question on MR productivity . So, you know, if you look at the incentive structure in the last three years or five years, how are you directing the incentive structures to evolve and for you to get a corporate entity to where you were, let's suppose.

Sridhar Venkatesh: So, to me, I don't know if I will directly link incentive structures to MR productivity . So to me, MR productivity is a function of the relevance of my products, what I can do where I can, so I can talk separately about my incentive structure, it moved away from it was at one point of time, we had all reps, then it moved to GSK. I mean, globally, GSK moved to only a second line sales leader level from last year onwards, we have in the targets right at the rep level, so incentivised at the rep level. So, there has been a journey, there has been a different set of things. But clearly we think, our MR productivity, if you were to use that as a terminology, if I look at it within the operating space that I have, I think I'm pretty comfortable. When you look at a set of products, which is which I don't promote, which are there. That's when I think you start to see a different level of productivity. And it's a function of portfolio, my portfolio has shrunk significantly.

They said, be patient with us for a couple of years. With a lot of these new products, these numbers are gonna change significantly.

Juby: Just to add, if you look at the total employment cost, the reported numbers has declined 1% despite inflation of 8-10 percent. So, the decline is a function of productivity, where we are rightly allocating our resource to work matters, the same time providing for the inflation or employment cost has not grown over the last one year.

Analyst : And one last question, you know, if you theoretically could launch more products from the global portfolio, but you're prioritising, so what are the parameters you're prioritising a launch versus others? Thanks.

Sridhar Venkatesh: So fundamentally, the need in the Indian market , I mean, not all products, let me turn this around and say that most of the products that are coming out of the portfolio will be relevant for India. And we will launch especially in the oncology space, vaccines, immunology space right now. But if there is a very high niche kind of a product, which has it is very rare disease. And right now, I have not come across anything which I'm seeing there, which I would say I will not launch. Most

of the products that we have in the near term portfolio, we will launch and as we speak today, there are on, correct me if I'm wrong, so that either on 20-25 global clinical trials happening in India for various products , so, and the intention that we're doing those trials is potentially we will launch them in the future. We have books online.

Analyst : Yeah. So, thank you for the opportunity . Just want to understand if we look at the WPI benefit has it flowed in the one queue entirely, or we should see it in Q 2, to be more specific, and what kind of price growth should we look at for the full year.

Juby: If you see the quarter one serves its 6% price impact in the results Full year, we could expect around 8-10 percent annualised price impact, it would flow through in the coming months.

Analyst : And if we look at these price increase versus the cost inflation that we might have experienced in terms of our, you know, raw material around is there any set off

that we should expect or the number that you just mentioned, we should refer to that?
Juby: Typically what we're seeing in the

market is two types of inflation. One is our raw material, which is the API inflation, as well as the packaging material and the aluminium, all those kinds of stuff is used for packaging in the factories, that's also where the inflation is happening. So, we expect around 2 to 3% cost increase due to inflation happens. Price increase, of course, will add. As to the question to sustainable margin levels, as I explained, so it's not going to have a material dent in the margin as we speak, because it's coming around 2 to 3% cost inflation happening and a price increase is going to help us there.

Analyst: And if you can explain the volume growth, I mean, we are talking about 6% volume growth, which is significantly higher than the overall IPM volume growth. So what is it that is driving these kinds of areas, only mass or is there any contribution? Because as we were discussing around the vaccine portfolio being weak, and we expect it to come back, say only by end of this year. So how should we look at it? And with that, if we look at the EBITDA margin also, because I remember our earlier conversation, wherein EBITDA margins for vaccines are kind of much lower and at arm's length basis. So how should we know because that indicates that our EBITDA margin should be much higher. But, of course, we have seen a dent in this one quarter.

Sridhar Venkatesh: Look, I mean, we're getting our gross, as I said, there was a natural transition towards key known brands during COVID. And everybody expected once, and of course, we were able to keep our supply chain running, everything happened. And again, cheer. People thought we were going to lose share during that, it didn't happen. So, I think that momentum is helping us. We are fully but we were first to get back into the field, so all of that plus the relevance of our brand. So it's fundamentally the mass products, which is helping us, you know, get those volumes out there, now, also the vaccines coming back is, of course going to have an impact.

So I don't know, what was the second question there about? Sridhar Venkatesh: I'm not gonna comment on a future. Juby?

I think if you see the mix of pharma versus vaccine, so pharma products are around 78 and vaccine is 20 to 22% of our portfolio, and pharma has is also growing much faster than vaccines. Okay. So that margin is not going to dilute just because vaccine is going to come, vaccine is relatively low margin, but the ratio will maintain because pharma is growing on a larger base at much higher rate. So yeah, so it's a lower portion of our total sales since the margins will sustain.

Moderator: So, I will go online, just so we are not ignoring the folks who have joined online. Those of you who have a question, kindly raise your hand, the moderator will kind of allow you to ask your question. Just introduce yourself. Okay. Dheeresh Pathak, can you introduce yourself from which organization you are and your question? Please unmute the respondent.

Analyst: Yes, thank you. Am I audible?

Moderator: Yes.

Dheeresh Pathak: Yeah, thank you for the opportunity. I have two, three questions. First is on a re there any more OTC type brands like Ostocalcium, Iodex, which might transfer later on now, the portfolio is just description brands and there is no OTC.

Sridhar Venkatesh: I hope, I had and and I would make out of it. But no, there are more at this moment.

Analyst: Okay, on pneumococcal vaccine, Synflorix is that vaccine part of the NIP program me or it is yet to be part of the NIP program me?

Sridhar Venkatesh: No, Synflorix as a brand is not a part of NIP program me, I mean, pneumococcal has gotten introduced. We are not a part of that NIP program me.

Analyst: So, just to like understand that part properly. So, when they say pneumococcal is part, so, the same formulation and the strains that is in SynFlorix, that is part but you are not partic

ipating in the program me, is that what you mean?

Sridhar Venkatesh: So, there is a competing molecule from another company, which is a part of that NIP program me. They, see look, you can never say this is exactly the same, they have some strains here and there. But yes, they all have the same indication as given by the drug controller, which is for the pneumococcal space and we are not, you're right, we are not a part of the NIP program me.

Analyst: The broader question I wanted to ask there was that if our molecule if that vaccine comes under NIP in the private market, do we have to reduce prices or the prices are free in the private market, even if the vaccine is part of the NIP and we are not participating ourselves in the NIP program me. In the private market, do we have to match prices? Does it come under price control in some ways is the question.

Sridhar Venkatesh: No, no, no, I don't think these are completely different topics. So NIP is a programme run by the government. You can choose to participate in it there are some clearly laid out conditions and then you compete think of it as a tender for the country. So, we cannot compete in that tender for whatever reasons. So, we are not a part of that tender. Now, private market is a completely different market. We everybody has their own prices. MRP, is run by the rules of how much we can increase, what you can do so, these are two completely different parallel streams. They're not linked at all with each other.

Analyst: Okay, and one last question, vaccines, you said is 22% of the business then you give the promoted brands and speciality can you also give their revenue share in the total business now?

Juby: Yeah, so, vaccines is 22% of the total business. Promoted brands makes roughly 62%. Of the pharma business, the rest of the 78 percentage. Around 62% is promoted brands. So, totally including vaccines, we are talking about total portfolio, 80% is promoted.

Analyst: Understood, thank you.

Moderator: Okay. Next question. Dhaval Shah has raised his hand. Hi, Dhaval, if you can pose your question, go ahead. Dhaval?

Analyst: Yeah. Hi. Good evening. I hope I'm audible.

Moderator: Yes, you are.

Analyst: Yeah. Hi. This is Dhaval Shah from Aditya Birla Sun Life Mutual Fund. Thank you for the time. So, first question is I just wanted to understand the market potential for three speciality products that is Nucala, Trelegy and Shingrix. If you can just articulate what is the market potential that you're looking for each of these products?

Sridhar Venkatesh: Look, I think Nucala is a very niche indication, it's a biologic. Like asthma, I look, I don't know if that market doesn't exist today. The biologic's market doesn't exist. And I think broadly, you know, when I say doesn't exist, it's a very, very small market. So we're talking about, you know, again, I don't want to, I wouldn't want to sit here and talk about in crores. But we're talking about patients in thousands who could potentially take that, while you're talking about Trelegy. Trelegy is the entire COPD market, right on that, to me, is in, correct me if I'm wrong, if it's in around a few thousand crores, so it's somewhere in that particular space.

The third one was about Shingrix. Shingrix, again, the adult vaccine does not exist, market doesn't exist, but you know, anybody above 50 is at risk of this should be getting that vaccine. So, we think some, you know, if again, the apathy around vaccines goes away, and like what you see in some of the other markets across developed market across the world, you know, some potential is massive. So, again, these are markets that we are creating, the world. I mean, there is, as I said, we will be the market for some of these products.

Moderator: Okay, Dhaval, any more questions?

Analyst: Yeah, the second question that I have is on the Dermatology market, and, you

know, that has also got impacted because of COVID. And I just wanted to understand, you know, how much normalization has happened in that overall context? And how do you see that going forward?

Sridhar Venkatesh: Pretty much normalized. In fact, you know, I go by the metric of or my reps able to see the doctor walking in, or they wait for a few hours. So, if they're walking in, business is not so great, though they're meeting their doctors. So, right now, I can see those doctors extremely busy. And I think that markets coming back significantly.

Analyst: Okay, and question is Synflorix and I just wanted to understand what possibly the ratio is also asking is that do you expect the private market, you know, to grow after, you know, this getting included in an IP in that overall sense, so would you expect private market growth to continue for Synflorix, say over the next year in the next three years?

Sridhar Venkatesh: No, we see that declining, okay. And as I said, that's not a market. We are seeing that decline. There are patients who are moving over to the NIP. So, I'm not expecting that market to have that pent up demand and growth of what we saw in the past. The other vaccines which are coming back, you know, we have the meningococcal vaccine, the or the T-bact. We have, you know, the 6-in-1, but this one, I expect the private market to continue to decline.

Moderator: Thank you. Thank you, Sridhar. Thank you, Dhaval. We'll go to Gagan Thareja, who's raised his hand. Gagan, if you can. Go ahead and ask your question.

Analyst: Yeah, good evening. I hope I'm audible.

Moderator: Yes, thank you. Go ahead.

Analyst: Yeah, so the first question is around the proposed changes to the list where

three of your products can you know additionally figure in, Cefum and Rotavirus . If that materialises, you know, what could be the impact on the pricing, aggregate, basis all these three products basis the pricing methodology followed.

Sridhar Venkatesh: Look, again, I'll wait for the government to come in to tell us the pricing. I am not going to speculate. But what I can, if at all, the price gets released and we have a direct impact, I can say that we have enough experience from the past of having gone through this turmoil and ensure that we come back in terms of our, you know, in certain cases volumes or in cases other productivity improvement measures. So, look, I clearly don't want to speculate on what price they would, what would happen. But whatever be the scenario, I think we are pretty much ready to click on the button within 24 hours to do whatever needs to be done.

Analyst : Sorry, so, you were not audible for most of the comments.

Sridhar Venkatesh: Okay, let me try and sorry, am I okay, now, am I audible?

Analyst : Yes, you are audible now . Thank you.

Sridhar Venkatesh: Look as I said, I don't want to speculate in terms of how much and what would be the price impact if at all that happens. I don't know what I don't know, whatever be the methodology. But you know, but let's say, irrespective of whatever happens, we've gone through the situation a few times in the past . We are ready to manage that carefully, what needs to be done in terms of volume expansion or whatever needs to be done. We are ready with that with that thinking. But I clearly don't want to go into that space of speculating what will be the amount of price impact on these products?

Analyst : I understand that sir but could you give the salience of these three products in your portfolio today? And I mean, what proportion of sales would these three constitute for you?

Sridhar Venkatesh: Yeah, so I can tell you what Rotarix is as good as insignificant now, it's not a focus area, you're talking about Cefum. Cefum, I mean, which is an important product for us, but again, in the larger scheme of things that is less than a percent, less than even less than

a percent. And the other one is T-bact, which is again, two to three percentage points.

Analyst : Last year, you also indicated that the API for paracetamol had seen a very sharp increase while you know the volumes of Calpol would have taken some sort of an impact, because year on year, last year, you know, you had the second COVID wave. In terms of input pricing, if things look better this quarter, and also you would have taken some price increases on the final product as well.

Sridhar Venkatesh: Yeah, taken.

Juby: So, I think, firstly, I think most of the raw materials, particularly paracetamol, has gone up roughly 20 to 25 percentage in terms of cost inflation. Now, the good thing is we have long term contracts with the vendors, we have signed long term contracts and most of the APIs, so we got a certain level of, you know, visibility, in terms of how the cost is evolving, that's one thing. So that is why and also the price increase, both are kind of cushioning us. So you're not seeing that impact, the reported quarter also, the last quarter, also on the margins, we still see the margin improvement. So, we continue to have this kind of cost pressure, but we will be carefully managing that.

Sridhar Venkatesh: I mean, I would say the margin pressures remain. Okay, so let's see, the geopolitical situation around us is not improving for the short term. So, we continue to watch it closely. Again, that's the strength of being a global multinational. We have scale and size to kind of globally look at some of these, especially some of our vendors. So, that helps us a little bit, but Juby is bang on to say, right now, the cost has helped us a little bit. I mean, the price increases as well plus a little bit more pressures remain.

Moderator: So, we'll go to the room.

Analyst : Hi, good evening, sir. I'm Satish Bhatt . I'm tracking your company since 1992. So, it's more than I think three decades. And during this phase, I think some of our past speakers are told that we were once leader and now we are not there to be seen in the, not to be recognized as a leader, you know, we become more of acute company than a chronic company, you know, and the presentation which you made did you want to focus on the speciality specialty products like Nucala and Trelegly, and whatever oncology portfolio you have, you haven't mentioned how fast you're going to introduce your product. Because already I think, your product is out in the market for last two and a half years we are still not yet launched in India. So why it is so?. You still haven't launched in India so, and sir you told that I think what I track your speciality is less than the hardly 1% of sales . Maybe Trelegly and Nucala put together if I can share that to your forthcoming products, which are going to literally change the structure of the company in terms of the product portfolio which you have presented, you know .

So I just wanted to know, sir, I think two years back in your annual report, you returned it? It's written in your annual report. And today, we're talking about something only 1000 patients, you know, so what type of investments do we have to require to get those type of patients you know, because these are the products which are biologics, maybe you will have to develop the market.

So whatever frontend investments, you may have to need to make, 50-100 million products need to really move the needle for a company like Glaxo now, because the size is as big as 3000 crores, to make a needle you have to make at least 10% of sales, you know, so, just wanted to know, your thoughts on those lines?

Sridhar Venkatesh: Happy to. By thousand, I didn't mean 1000. I meant thousands.

Sridhar Venkatesh: So, in the few five figures, 1000s. Okay, so that's not the five figure. So, there is that potential, but it's not millions, as what you see in the others, look, some of these products are, as I said, e

ither first in class or best in class they

need. So, the investment that we are making you can.. let's talk about July, yes, we will be, you know, it's been launched globally.. we had to do a clinical trial. And we are filing and so the process is on, committed to bring that in.

Let's talk about Shingrix. A new asset globally, a billion dollar product already. But in India, we are creating, you're talking about investments, we are creating a team with a set of people. Product is going to be larger than second half, first half. And the first half, we are putting investments today to ensure that we understand the market worked with the key stakeholders in the market and ensure that when we launch, people understand what this medicine is for. For speciality medicines, I think unlike

launching an antibiotic, unlike launching another painkiller, which I'm not demeaning that they have their own space, these products will require a significant investment way before launches... 9 months, 10 months, 12 months and I'm not talking something that the industry does not do. Every other company also has the same methodology.

We are also in that space, they said we will be focusing on some of these are merely around 8, 9 months prior to launch. And that's the journey we are in. So, yes, that's

the kind of money and investment that we will need. Will these products be for everybody? I don't think so, they will be for certain category of people, certain set of people and for them they will definitely make it available.

Analyst: Sir, basically, what type of investments we require to make a 300-400 crore brand, you know? Maybe you are initially first you spend 50 crores on that, because maybe that's why you have to come back and we have to create the market. Is you being the leader I have to decide what I have to make the market, if I know I have a product potential of becoming a 500 crore, are you going to be shy of investing 50 crore in year one. Because your balance sheet supports any type of investments you know, and definitely you would be willing to put.

Sridhar Venkatesh: Yeah, absolutely. And the answer to that question is yes. If I can make this product to 300-400 crores, we will. The first few years definitely, you invest much more than what you would get when you're not looking at the P&L at that point of time. Look, as I said, it all depends upon the nature and the kind of the product that we are talking about. A product like Shingrix, we are making that investment I'm telling you right now we are seeing it right now today, right? I'm sure next time around when I call you, you will say you made so much investment on Shingrix. You don't have the sales yet what so I'm sure you will do that. But we are making that investment right now. So for a product which is now if I had to launch something else, which is in the general medicine space, I may not invest that level so the products that I'm looking at in the future will all be as I said invest well before 8 to 9, 10 months before launch, create that market.

I don't think I'm going to talk about will I spend 50 crores, will I spend 100 crores it will be for an oncology product which is run by maybe 50 key centres. For 200 doctors I may not need 50 crores.

Analyst: Sir, let's look at Nucala specifically.

Sridhar Venkatesh: Yeah, Nucala again what I may not what for a product like Shingrix which can potentially be it is a vaccine, nobody knows about it. So, pathbreaking vaccine can be prescribed by potentially every GP. And we have a 10 year data protection as of now, which is fantastic. There is an apathy for vaccines, I will have to invest. So those, and again, I'm not going to talk about numbers, those investments are going to be significant. So, it will depend upon the nature of the product and portfolio.

Juby: If you see the last three years, new product launches, which you had

done, so

we've done both on vaccines as well as in pharma space, products. So around 5.2%

of our total sales is from these new products . We got Fluarix , we got Nucala, we got now Trelegy. So three, four products, we have around 5% of a base of the last three years is from this new product. So, it's not that it's small not to move the needle. 5% is significantly big. And the investment is completely dependent on what product , like Sridhar said. And we will try to, as much as reallocate from the P&L. It's not all to be incremental, we'll try to find opportunities as well as incremental , both will be coming through. So, it's a commercial decision depending on the potential of that product in the market.

Analyst : Sir, one thing I've seen your peak margins, maybe nearly of 32%, maybe some 10 years back , 10 to 15 years back. So, do we see we will ever reach or that's , we can pick out at 25 to 27%, nothing wrong in growing top line being at 25%.

Juby: I don't want to comment on the future margins. But you know, if you bring in perspective, there were NLEM waves we were going through. So we had significant waves of NLEM , which we had to experience from then what you're talking about 32 percent. From there, we have gone down to a kind of 16, 15 percent. And we've gone a long way from there. So, we are continuously working on that. Whatever happens, we have to bounce back to that's where it is in.

Analyst : Hi, this is Gokul . So, my question just on the clarification you mentioned. The promoted brands are 62% of the overall sales and vaccines is another 22. So, 84 is promoted, the rest would be the non-promoted brands , is it? What is the remaining 16%? Is this that is something to be shed is which you mentioned?

Sridhar Venkatesh: No, not exactly. Look, I wouldn't call them shed. Shed is, we had to make some decisions, big ones , most of it is out. But these are products which have a life of their own. These are products which have a long, sorry, long carryover effect. Yeah, thank you. And, you know, we don't think by investing in any of that I'm going to grow that significantly. So, we made this conscious decision to focus on these , on this 80%. But they are profitable brands. They don't , they're not dragging me at all.

Gokul: Should we assume that this 16% of the portfolio would be largely inflation driven growth and not much of volume growth over there?

Sridhar Venkatesh: In some cases, you do get volume growth, some cases very slightly 1%, 2%. But the other ones it is more or less inflation.

Juby: Yeah, it's more inflation driven, very low single digit volume growth.

Analyst : And what would be the aspiration on the promoter drug brands .

Sridhar Venkatesh: So far as of now, it's been a double digit growth.

Analyst : Okay. Secondly, on the parent has vertically divided the business into three segments. One is the general medicine, speciality , vaccines, we are more of a general medicines business Very little of speciality and 20 odd percent from vaccines. So, the parent 's focus is to increase margins on the general medicine? . While you may want to actually grow that business, so is there a misalignment over there where the R&D focus at the parent level may not be there and hence, you are constrained by new brands or something which you want to grow these brands in India?

Sridhar Venkatesh: Look, so first of all, I think I'm actually pretty much aligned with what the global teams are doing. So what's my strength? My strength is general medicine. What is it that I don't have? That is speciality, I'm getting that from the global portfolio. And vaccines is a space that I can grow and I'm getting some of the new assets. From the general medicine space to me, I personally think given more digitalisation that is happening in this country, more online pharmacies, more kind of doctors . All of that , the penetration is g

oing to increase. And these are the most relevant brands that I have with that penetration increase. As we speak today, what is it around 35, 38, 40% penetration in India for healthcare. And if that increases by another 5, 7, 10%, the impact that we could have with some of our current general medicine portfolio is huge.

So, whatever may be the group's aspirations at a global level, because global level we are driven by innovation and some of the other markets . Our aspirations , emerging markets aspirations , for us and as I said, I'm a big contributor to emerging markets - is continuing to grow significantly in the general medicines portfolio. I'm not the only country in that space, most of the emerging markets country are in a very similar space, and the focus is continuing to grow double digits.

Analyst : Thank you.

Analyst : Hi, this is Ravi Pur ohit.. Sir, one clarification on Ceftum I think in the DP CO. Is it Ceftum injectable or is it the ceftum ? The tablet form is also included? Because if it's the tablet form, which has been included then the...

Sridhar Venkatesh: No, it's not been

Analyst : Because when you mentioned it contributes to 1% of your revenue, I assume it's the injectable, the tablet form would be much bigger. Right?

Sridhar Venkatesh: Around 3%, 2 to 3% tablet, but yeah, again, none of that is clarified. I didn't, as I said, I don't want to sit here and speculate. And I have no insider knowledge of what's going to happen.

Analyst : I just wanted to clarify that. Second question is, at any point of time, how much inventory sits in the system in with the chemist shops, in the sense of in the chemist's shop. So for example, when I do a channel check, I realized that the entire dermatology portfolio of ours, the pricing has already changed from first of April, right? So, already 8 to 10% or 7-8%. I think a lot of creams have, when I see the Calpol's and the Augmentins, I don't see any price changes, so maybe the inventories from the previous fiscal. So at any point of time, you know, what is the duration?

Sridhar Venkatesh: I won't be able to tell you exact chemist shops, but in the market, it's anywhere between 35 to 45 days is what we have, depending upon the nature of the portfolio. Right?

Analyst : And have you taken price increase on Augmentin and Calpol or we have not? If you could confirm it?

Sridhar Venkatesh: What do you mean pricing? You mean, normal NLEM? Those price increases. They're yet to hit the market? I mean, we haven't enough, hopefully. Look, quarter three is generally the biggest quarter for us. When I say quarter three, I meant for the whole year. So this quarter, and you will see some of those new prices starting to hit the market this quarter around.

Analyst : The last question. So, if we historically see in it, just like some of the other participants have mentioned that we were industry leading 10 years back. Today, we are like not even in the top 5 in terms of growth. Same happened with the parent also right, the pharmaceutical business, guys like AstraZeneca, Abbott, lot of companies outgrew GSK at the global level as well. And which is where this need to break up the company and sharpen the focus, consumer division and former division. And now that the division has happened, does the focus sharpen in India as well? And therefore, the question is, are two-three new product launches enough? Whereas we've seen a lot of your MNC peers in India launching 10, 15, 20 products every year. So, is that something that's going to change for us?

Sridhar Venkatesh: Look, I don't know who's launched 15, 20, I mean, I'm not too sure which would that be? But I mean, look, I'm not going to talk about 20 products of those. But look what I think I will bring in as the innovation assets that are coming out of that. So in terms of what we were 10 years ago to what we are now, as I said, it's not something

about situation was different, thinking was different today. I am, I want to be leader in the markets that I play, and that I can play, right? I'm not in, yes. If you put in the entire diabetes and cardiovascular market, I'm not even in the top 10. So that does have an impact. But I'm pretty convinced that with portfolio that I have, I can continue to grow my top line I can continue to as Juby was saying focus on my margins and then I think I have space there.

I have two phenomenal areas where my muscle I can increase significantly, or that vaccine portfolio, new assets. New assets - will I launch 10 a year? Absolutely not. I can tell you, sitting here, I don't think I have the bandwidth or the capability or I can do well with that, will I have at least a few launches every year, based on what I would see? So, I think the focus has been, be selective of what you want to launch when you want to launch but do that well. And you will see that with Teregly, you will see that with Shingrix, you will see us getting ready for launches 8-10 months before, so that when the launch happens, it's not the slow, we take off much faster. So that's how I would state the focus on India. I personally think again, I'm not going to comment about the group.

But I can say for me in India, this is the best possible thing that has happened for me because they said, India is one of the few markets where I have scale and size. I have trust, I have equity, and I walk in. So that is a big positive for me from general medicines. What I need to build is my speciality portfolio without diluting my current string that I have, and my vaccine. So I'm pretty clear we are on that path. Will I become a number one? Look, honestly, I'm not even thinking about that. I don't think about that at all. I want to be relevant in the spaces that I am in, and I want to grow in the spaces that I am in. And I think if I can do that, that's a win for me.

Moderator: Thank you Shridhar, we have last two questions. If you permit, we'll just extend beyond the allocated time. We have a question from Sameer Baisiwal.

Sameer, hi, if you can pose your question, either to Sridhar or to Juby. Over to you, Sameer.

Analyst : Thank you so much. Sir, the quick question is for the represented market that you are in, which is accurate, which is antibiotics, derma. What sort of market growth do these categories have? When is it fair to say that you are a definition in the

low growth areas?

Sridhar Venka tesh: I think , yeah. On that base, if the market is growing by 8 to 9%, I don't know. I mean, it's a pretty , it's though markets are, because again, given the under penetration, I still think these markets will continue to grow at 8 to 9%. The derma market is growing much faster though.

Analyst : Okay, and if you can share your thoughts on, you know, the bigger towns versus smaller towns. So, you know, how do you think about these two markets?

What's the sort of penetration that you have? Are you deploying more field force, more doctors in the smaller towns?

Sridhar Venkatesh: No, I think , look at in terms of our people. I think we've gone through an exercise, looked at it very closely. We think there is more for us to gain greater productivity of where we are operating in. Look, we cover I think, correct me if I'm wrong. Around 160, 170 towns across the country easily. And, I think maybe more, a little bit more. But I think we are not adding any more customer s, we are adding any more feet on the ground, we are we are looking at enhancing our productivity with our existing customers.

In terms of our growth, obviously, we are getting a higher growth from smaller cities compared to places like some other some ot her bigger cities, but if you look at cities like Delhi, Bombay, Bangalore, Chennai, given the corporatization that I see in these places, these places have a normal tendency to get a higher

set of growth, especially

for the kind of products that I have, w hich is fundamentally in the acute space or the antibiotics.

Analyst : Thanks, one final, if I may, with your permission, what's the sort of , how much of your cost is really imported? And I have already felt the impact of the depreciating rupee.

Juby: Yeah, I think we got we got roughly 800 crores of important cost base. And we have, roughly , if you see the FY' 22 financials we got around 5 crore impact in that full year on depreciating US Dollar versus Rupee. So, it's not material from the scheme of things it' s roughly 5 crore i mpact we have in the supported financials.

Analyst : Thank you so much.

Moderator: Thank you, Sameer. Thank you, Juby , last question that was actually typed online. And after that, we will close with closing remarks from Sridhar . Mitesh from Nirmal B ang. He says , post-COVID , multiple companies have additional capacity in vaccines and are looking to launch different vaccines . Can we see competition intensified?

Sridhar Venkatesh: Yeah, I do think that, you know, vaccines are space, which is exciting, everybody. So yes. Do we expect vaccines to come in? Yes. I think what GSK brings to the table is the freshness of its portfolio. The kind of newer assets that are happening globally, the acquisitions that we're making globally, I think, for us, it's going to be how do we continue to bring some of the newer assets of vaccines into this country? So yes, you will see more vaccines coming to the market, I think it's a good thing. There is more noise in the market, there are more people who understand vaccines, so I'm not averse to more competition coming in. I personally think it's a good thing. More of them come and talk about adult vaccination will be fantastic. But yes, I think I am in a very strong space as GSK given the newness of my portfolio, and then the pipeline that I'm coming and on the pipeline, I can be pretty sure on vaccines, I potentially will get most of those into this country.

Moderator: Thank you, Sridhar . With that, before we close, any closing remarks ?

Sridhar Venkatesh: Thank you for coming. Really wonderful to speak to all of you face to face and you get this opportunity a few times. Thank you for coming. And again, from a G SK standpoint, I only , you know, my only message or my only focus is going to be do a few things , do them well , focus where you can, you know, change what you can change and, you know, ensure that at the end of the day, what ever you do, makes value for the patients. That' s our key mantra. Thank you for all of you for coming.

Moderator: Thank you all for joining online and thank you for all those who came here.

Appreciate it. Thank you very much. We will now disconnect the call . Please disconnect the call. Thank you.

Call: Mar 2023

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16th March 2023

To,

BSE LIMITED THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED Phiroze Jeejeebhoy Towers Exchange Plaza, 5th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
Mumbai - 400001 Mumbai - 400051

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 16th March 2023. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/investors/analyst-meets/>.

Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

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Moderator: Good evening, and a warm welcome to those of you in person and, of course, to those of you online. We will, of course, get to see your faces when you ask a question online, but really nice of you, to all of those who've come to the office - a warm welcome. As you can see, it's really warm outside. Without further ado, I am really pleased to have our new Managing Director, Bhushan. Bhushan is a veteran with over 27 years of experience and has been with GSK now for over twelve years. He has done roles in India, the Middle East, Russia, CIS and African regions. And before that, Bhushan had been with Janssen for 15 years. So, in those roles, Bhushan has worked in India, South Korea and Belgium. So, he's seen it all. He's been in India, gone there and now come back and in two different companies. So, Bhushan, it's really nice to have you leading us at this point in time. Of course, we have another new person. Probably some of you have met Juby or have not met him before. Juby has had a successful track record with GSK for over 14 years. He comes to us from his most recent role that he had in Singapore, but before that he's been in roles in South Asia, Vietnam, Turkey and the Middle East. With that, I'd like to hand it over to Bhushan, our Managing Director, who will brief you for the analyst meet.

Bhushan, over to you.

Bhushan: So just remind me-I'm not supposed to cross this, all right. So that I'm still visible to them.

So very good evening to everybody in this room as well as everybody online. Thank you very much for your interest and your time this evening. The way I'll structure the next 20-25 minutes, at best, is to just lay out the context of the industry where we operate as GSK. Then we'll zero in on specifically GSK India and what we are doing and where we intend to be in the next three to five years. We'll briefly touch upon the financial overview, and then we'll open it up for the most important part of this evening session, which is the Q&A.

One inflection point in our evolution as a corporation globally was in June 2022, when we split the pharmaceutical organisation from the consumer business. And we now remain completely focused on what we call as 'biopharma'. I think that has allowed us to further sharpen our focus on doing our best in terms of the businesses for which we are known, namely vaccines and pharmaceuticals. But I think, more importantly, it's allowed us to sharpen our purpose, which is all about uniting science, technology and talent to get ahead of disease together.

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I think every word there has a meaning. And in the next 20 minutes, I'll try and lay out how this global purpose also reflects in our ways of working in India. As many of you may be aware, the pharmaceutical industry in India is just short of about 200,000 crore. If you see the revenue in a calendar year, of course, the data points are from IQVIA for 2022, just short of about 190,000 crore. And if you focus on the right-hand side of the graph, the tables there really outline how the industry is structured in terms of, say, if you talked about the diseases on the previous slide in terms of therapy areas, how is all that laid out? And the reason for putting that out there is that, as we've tried to sharpen our business operating model, we're making some conscious choices about where we want to operate, where we want to play, and how we want to win. And as you can imagine, some of the growth ends of the therapy areas remain in the therapy areas, which are coloured in orange. And that's exactly where we've decided to remain focused and anchored. So anti-infectives, vi

tamins, pain, analgesics and most importantly, the derma business. These are the few strategic choices that we've made in terms of where we want to put our energy and remain focused. The reason is twofold. One, if you look at the last five-year CAGR as well as what we see over the next three to five years, the compounded growth continues to be strong double digits, or at least high single digits. And that allows us to play to our strengths. So clearly, the choices that we are making are aligned with our strengths. And that's exactly what we will play out.

Just a couple of minutes on how the past twelve months have been just, again, to drive home the point that at GSK we will not probably launch everything under the sun in terms of branded generics, because we all know this is a non-patented branded market in India. But we make some strategic choices that are very aligned with our strengths. So, although you might see the pharma industry, generally speaking, as having new introductions as one lever that's been conspicuously missing for us for the last few years, but I think what's heartening is that we have volume growth in a market that's not really grown in volumes. GSK as a company has really used the volume growth driver as one of the key levers. If you were to zero in on that overall market and see, where we are making our strategic choices, if you split that further down at the TC-4 level, which is a therapy, plus at the molecular level, you will see it's clearly evident that the choices that we made in terms of where we play are strongly aligned to strong market leadership positions. So, just to give an example, if you look at anti-infectives, and if you look at all the molecules where we operate, or where we have our research molecules, or where we have our strengths in place, if that therapy area has been growing by 9%, our growth has been better than that therapy area. So, if you just look at the bottom side, it correlates to having almost a 25% share, which is a significant share, and growing in strong double digits, not just on a year-on-

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year basis but even on a compounded basis. So, if you were to zero in, anti-infectives, derma remain the most durable as well as strong areas for GSK as an organisation. Pain, where we operate with a couple of brands, mainly Calpol, is where we operate.

Now, the therapy area has really seen, especially if you see the last two-three years, what we've seen with the pandemic, that therapy area has really shown an upswing. And even there, if you look at our CAGR, it's ahead of the market; it's just in line -15% CAGR.

Where we have not done well is obviously in the vaccine market. But if you juxtapose that with the external reflections in a market that's actually declining, we've been adding share. So, although our decline is not as sharp or as significant in a market that's overall declined by 18-19%, we've still managed to hold share. So that's exactly where we operate. We made our strategic choices in terms of remaining focused on our represented market, which is roughly about 20,000 crore of the 1PM. What really makes us relevant in the Indian market even today is that even today, we are the second biggest multinational organisation in India with a very significant footprint in a highly fragmented industry, we still have a value share in excess of 2.6%. But in volumes, given the sheer number of lives that we are able to touch with our mass products, we have a volume share just in excess of 4%. What makes this even more enduring is the people-the 3700 plus employees who work for us across the value chain, but most importantly, our manufacturing footprint, which has remained intact. So, just like this facility, where we've operated right from independence, we still continue to hav

e our factory in Nashik, which has definitely been upgraded in the last several years with all the measures that we've taken. The state-of-the-art manufacturing unit that we still continue to have for India, where we still manufacture for India. Apart from that, we still have about two thirds of our products manufactured by contract manufacturing organisations. I think one of the data points that I want to share with you is that in a population of 1.4 billion where we still don't know, if you go to different sources, you'll get different numbers. But access to medicines remains a critical area. Globally, we are always proud to see that GSK continues to figure as the number one company in the access to medicines index. But even closer to home, although we may not have a similar index, given the nature of our brands, given the nature of our products, we clearly touch in excess of 200 million patients every year. If you have to see the number of prescriptions that we generate for our brands like Calpol and Augmentin, it is in excess of 300 million prescriptions. And that clearly tells you the wide footprint that we have.

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Similarly, we continue to be the number one company in the self-pay private vaccine market. In spite of the decline that we've seen for the last couple of years, we are still the number one company in private vaccines. And in a typical year, we typically see about 6 million immunisation sessions. So that's the real footprint that we have in India. And as I said, one of the things that's remained at the core of what we've done over the last several decades is our footprint in manufacturing. Here's a short video that summarises what we continue to do in Nashik with our manufacturing footprint.

Thanks for your patience. As you can see, not just the manufacturing footprint, but as a part of our ESG commitment, we're doing a lot there to remain relevant and also complete our formalities. So specifically, now zeroing in on our global strategy and how GSK India plugs in post the split from consumer globally, you would see a similar nomenclature. We remain rooted in three areas: general medicines, vaccines and speciality. And that's exactly how we play out our strategy, even in India. Within the general medicines business, this is the business that really touches millions of lives. As I said before, the strategic choices that we've made to remain relevant in anti-infectives, derma, vitamins and pain analgesics with those select few promoted brands are what shape our strategy.

We've just launched a couple of brands in speciality, but in the coming years, given the fact that we also have several clinical trials ongoing, even in India, the idea is to build that additional growth lever. We remain the leaders in private, self-pay vaccines. But I think what you will see at the top of that slide is probably something I'll spend a few minutes on in the subsequent slides, which is the new launch of Shingrix happening in exactly less than a few weeks, a couple of weeks from now. And that will really open up a completely new category in the form of adult vaccination for GSK India. As I said, we continue to make inroads in spite of the declining market that we have seen for at least the last five quarters now. Things are turning around, but it's too early to say we've turned the corner. But whether it's products like Infanrix hexa, the-6-in-1 vaccines, Synflorix, we still continue to hold a significant stake. So, if you were to anchor all these three businesses and see where it is that we really play, it's really the focus that we have in terms of defining sharp strategies for each of those assets. Our responsive go-to-market strategies in terms of our agility. And last but not the least, given the fact that we have an extreme lev

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commoditisation and fragmentation in this industry, how do you remain relevant by differentiating yourself with science, and which continues to allow us to differentiate versus the

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other competitors. The only reason for that being that the proof of the pudding is in the eating. So even after several decades, we are very proud to see the strong portfolio playing out in the form of recognition, not just in the form of sales, but even for the calendar year 2022 - Augmentin, continued to be the number one brand in the pharma industry overall.

There's a lot of science that still goes behind each of these brands. So, a clear reflection of the trust that healthcare practitioners in general have in our brands is borne out of the fact that four of our brands continue to be in the top 50 brands in the Indian pharma market. We also, as I said, are ahead of the market in terms of our growth. Therefore, most of our promoted brands have an evolution index in excess of 100, which means we are growing better than the market. Just to give you some reference in terms of the sheer long shadow that we cast with many of our brands, with these brands no longer remaining trademarks, they are really trust marks in terms of the trust that patients and healthcare practitioners in general have. And just to give you a quick sense of the sheer volumes that we sell in India, over the last couple of years, we've been trying to also reshape our strategy in terms of using ways to really improve our footprint, not just the physical footprint, but to use ways of reaching out digitally to healthcare practitioners. So, as much as we are well represented on the ground with more than 2,700 reps comparatively in terms of the share of voice, so you see on the left-hand side all the data points, which clearly tell us why we remain relevant as an organisation. We cover in excess of 10,000 distinct towns and cities. We have our reps based in 400 rep headquartered towns. So that allows us to really reach out to almost 300,000 HCPs on a daily basis in terms of the details of the promotion.

But most importantly, apart from those physical touch points, in terms of science-based promotion, we also make ourselves relevant. I think in the last couple of years, we really accelerated our digital strategy by opening up new channels of meeting out and reaching out to customers where they wanted us to be seen, be it emailers, be it channels like WhatsApp or be it doctor networking platforms. So, this balance continues, I think, as we continue to shape our journey in the next few quarters. We'll see more of us being present where HCPs want us to be seen, and not just the face-to-face interactions that remain relevant and will continue to be the mainstay of our operations.

So, if I were to summarise our strategic focus in the short-to medium-term, it's clearly about growing our promoted brands. The brands that I just shared with you in those three pillars, in gen-med, vaccines and speciality, really improve the competitiveness of each of those assets

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profitably. And second, to defend many of those brands with some optimisation. We call them performance brands. And last but most importantly, launch new assets and have some exceptional new category creation, starting with Shingrix next month.

All this is possible only with a culture where we really believe that people can thrive, develop and do the right thing. One of the mainstays of our portfolio continues to be the general medicine portfolio. We have sizable brands, significant in terms of the lives that we touch, but I think these are just pulling out two examples

to show you that innovation is not just about launching new molecules. It's also contemporising the portfolio and really putting energy into lifecycle management. And just to give an example of the number one brand, Augmentin, we also launched Augmentin ES. Obviously, we want to address this molecule, which continues to be one of the most susceptible in terms of effectiveness, and therefore, to further help specialists, we launched Augmentin ES, targeted specifically at certain appropriate patients and promoted only to specialists, which are ENTs and paediatricians.

It's really helped reshape some of the thinking around how to treat these difficult to treat infections. So that's one example. The second example is about Calpol. This brand continues to save lives quietly, apart from all the medicines that saved lives during COVID. I think paracetamol, led by Calpol, continues to be at the top end. But even in a commoditised category like paracetamol, we have invested in new technology. We launched Optizorb technology, which essentially accelerates the disintegration of the tablets, potentially leading to a faster effect. So, Calpol 650 is not just trusted; it's also trusted and advanced now. So, these are just two examples showing how we are doing a lot more in terms of unlocking value in terms of lifecycle management.

Ah, the vaccine market -we've clearly seen a struggle for the last several quarters. The market is yet to regain the momentum that it had before COVID. But as you see, in the last couple of months, there's been the negative declines have now started evening out. So probably in the forthcoming quarters, we'll see some reflection. But I think most important is that even in a

declining market where the market declined by an excess of 18-19%, we've grown our share. So, in this private market, our share grew from 34% to almost 37.9% on the left-hand side. One of the questions that keeps getting asked is what our focus is. So obviously we have a portfolio aligned to the interests of all those birth cohorts, all those babies in the first and second years. So, in spite of many of the antigens going into the national immunisation programme, I think our portfolio remains relevant and a case in point is Infanrix Hexa, which continues to grow

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well for us even in the first two months of this quarter. We will also have some supplies restored in the coming months in our vaccine portfolio. So that should also strengthen and help accelerate some of those growth numbers. So clearly, on the private vaccine market, our endeavour is to sustain the momentum and go back to the growth numbers that we had.

I think the big area where we remain focused is the disease awareness campaigns that we have, especially to build the footfalls to encourage all those parents who may have missed vaccination because one of the themes that was predominant in the last 12 to 18 months was that many parents were missing their vaccination during the COVID period. And as many of you may have seen, last year we ran a very successful 6-in-1 campaign with celebrities like M.S. Dhoni. This year we've just launched a new campaign with some more brand ambassadors under an overall umbrella such as a campaign called 'Faisla Sahi, Zindagi Sahi', and I want to give you a quick example of what we are trying to do, given also what we are seeing around the flu vaccine right now. So, as I said, this is an umbrella campaign that we've launched. The example that I just showed you was just about flu, given the topical nature of many of us navigating through this challenging time where flu is quite rampant right now. But we have a similar campaign for many other diseases, including, for example, meningococcal disease. So, there's a similar one. We're using social medi

a extensively. There's a whole media plan that we have for the next few quarters. I will not show this. You probably get the drift of what I'm sharing, but that was for another area, I think, where I want to spend the next three or four minutes specifically: opening up a completely new category for us, which is the launch of Shingrix. As some of you may have tracked, our global corporation may be aware. This is right now one of the biggest products and the biggest asset for us, delivering almost 3 billion on top line growth of 72% for the last calendar year, 2022. It was launched in nine markets last year. So right now, as we speak, it's available in almost 26 countries and we're very proud and privileged to have Shingrix being launched in India in the next few weeks.

Clearly, this is a massive opportunity. Almost 90% of adults older than 50 years old. This is all scientific data where we have the data points around epidemiology and prevalence available. They are always infected with the virus that actually causes Herpes Zoster or shingles. One in three people, it is estimated, will have this reactivation from the varicella-zoster vaccine that remains in our cerebral spinal fluid when we have chickenpox in our childhood. I think most importantly, this vaccine helps protect adults older than 50 from painful, extremely painful and debilitating conditions like post-herpetic neuralgia or Herpes Zoster Ophthalmicus, which are quite dangerous. And I think this is where we are opening up a completely new category. I think a

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lot of the organisational effort will be spent in ensuring that we spend our energy in ensuring that all available subjects get to know about this disease, but more importantly, have the potential opportunity to get vaccinated.

Why is this vaccine so special? As I said, this is the first time we have a non-live vaccine for Herpes Zoster in India. So, it's really a first in the class, in its category. It's approved for everybody who's 50 years of age and older. It's innovative to the extent that this is one of the first vaccines with an efficacy of 97%. Not just that; if once you've taken those two jabs, you have protection. We have proven protection data for ten years, which adds up to 90%, and that's really at the deep end of efficacy, which you rarely get to see with innovative medicines. So, this is exactly where we will be launching. We've just created a whole new adult vaccine team in the last few months, and we are going live with the launch in April. So that's about Shingrix. As I said, this will open up the beachhead for us in the creation of the adult vaccine category market in India. Obviously, there are a lot of follow-on vaccines, including vaccines like RSV, for which trials are happening. We'll see which of those innovative medicines and vaccines really make sense for India. But this really is the category for which the time has come, and that's where you will see a new platform of growth for us in the Indian context.

I think one of the main questions that we will address, but I want to get that upfront, is, yes, the NLEM, which is announced once every five years, was announced, as we all know, in late 2022. And this time, we have Ceftum and T-Bact, which are the two assets that we have that were new inclusions. Just to give you a quick sense, if you follow the pie charts, till last year, we had almost one third of our portfolio that was NLEM impacted. That's moving up by about nine percentage points. And now we have about 42% of our portfolio which is included in the NLEM. Obviously, to mitigate this exposure, this short-term exposure, we will use the Shingrix launch effectively to ensure that we are able to not only create the category, but also build this additional growth lever in the immediate

term. We also are quite optimistic about stabilizing our paediatric vaccine business in the coming couple of quarters. But most importantly, as we've seen in the past, this is the third time we have had the NLEM impact. And with every single impact, I think there's always a short-term impact, but we've always seen that we've grown the volumes, 2x, 3x for some of our products, and that's exactly what we've seen in detail working and we have the plans in place now in terms of specifically zeroing in on what kind of volume upsides we will have for each of the assets that are now included, apart from, of course, the tailwind that we may potentially get from WPI, and of course, cost optimisation, that continues to be part of our ways of working.

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So that's the summary of the NLEM impact. I think if you were to ask me, how does it directly correlate with the EBITDA and margins? Yes, in the last three or four years, we've definitely improved our margins. And as you would see in the immediate near term, we don't see an impact for the financial year, but we certainly see in the financial year 23-24, we will see some erosion there. But as I said, it's a short-term erosion, which we are confident in given the volume growth that we will drive. Plus, the launch of new assets will help us tide over. So that we are back to the EBITDA and the margin expectations in the near term. If you were to look at the five-year ambition that we have at GSK India, obviously, it all begins with touching the lives of as many Indians as we can. As I said, at any given point in time, we touch about 250 million to 300 million lives annually. The question is, how do we touch, continue to touch, a billion lives in India over this planned period or even more? And the three pillars, therefore, on which we will base our strategies. One is driving all innovative launches to fuel this growth machine. Most importantly, deliver and continue to deliver competitive performance with profitable growth for each of those assets in the base business.

Both general medicines and paediatric vaccines, above all else, remain rooted in our cultural pillars of being ambitious for patients, being accountable for impact, doing everything that's right for our patients and for the organisation, and evolving a culture where our people can really develop.

So that's the substance of what I had to share. I also wanted to share with you that we are incredibly privileged and honoured that in November 2023 we will start our centenary year of celebration of GSK's operations in India. We are entering our centenary year. So, November 24 is when we actually complete 100 years. But our centenary year begins, and that's only because of the trust. So, we continue to do all that you saw in the previous slides, and our endeavour will be to build on the growth story and continue to build trust, not just for the assets that we have, but for everything we do to serve the needs of patients and consumers at large in India for the next hundred years. So, with that, this is my last slide. I think I and Juby can open up the question and-answer session.

Moderator: No, thank you. Thank you, Bhushan. May I request you, Juby, to come? So, I think the first preference for questions would be the people here, because you've already taken the time to come here. So, maybe we'll start with Alka; maybe just introduce yourself first so that everyone

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knows who you are and your questions. And if you have a preference for a response from Bhushan or Juby, just take that.

Alka: Hi, this is Alka. Sir, if you could explain a little bit more on the Shingrix

market size in India in

order to, you know, help us understand how big the opportunity is going to be for us. I'm a bit confused because I was referring to a few articles. So, the cases in India for Shingrix that I have received have come from multiple sources. So, some sources say it's like, you know, annual cases

in India are about 1 million, some say 3000, or something like that. So, if you can share some broader-level prospects on that.

Bhushan: Sure, that would be helpful. So, just to give you again some context, remember that one of the things that's unique at GSK is that we probably are one of the few companies that still continues to be broadly diversified in terms of operating at both ends of the spectrum - prevention and treatment. So even if you look at the treatment of Herpes Zoster, what you talked about is the actual treatment. And obviously, Shingrix is to prevent. So, the numbers, depending on what sources you see, are different. But even there, for example, Acyclovir, is another research molecule from GSK, which continues to be the main line of treatment for Herpes Zoster. Now, when you look at who all potentially can benefit, the label that we have is every single person over 50 years old. Given the fact that it's well documented that the immunity and immunogenic responses go down after 50, plus, coupled with some of the concomitant illnesses that people may have that may also trigger immunogenic responses and therefore lead to reactivation, because most of us are infected with this virus and it remains in our cerebrospinal fluid.

As I said, when we have chickenpox in our childhood, this gets reactivated and then manifests itself in the form of Herpes Zoster. So, if you look at the scoping, clearly, everybody over 50 years is part of that cohort, which is very significant in our country. We are talking about millions of people who can potentially benefit. Now, obviously, it's a vaccine. As I said, it's a new category. So, we are mindful of using all appropriate filters in terms of who can benefit, who can access and who can afford these kinds of preventive medicines and vaccines. So that's, you know, prima facie. The cohort that we are talking about is in excess of 70 million people to begin with. And that's the kind of change you can bring about with a vaccine like Shingrix.

Alka: Thank you. Talking about affordability. So, if you could explain, this is also self-pay. Okay. So, when do we expect the sales to start coming in? Because in the speciality portfolio we are

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already seeing Nucala and Trelegy at roughly 1% or 2% of the overall sales. So how do you see the ramp up in Shingrix's post-launch?

Bhushan: So I just want to contextualise probably Nucala continues to be a niche product. As you're aware, it is indicated for severe eosinophilic asthma, which is really at the top of the pyramid in terms of being an incredible product-it's a biologic-but it's indicated for a select few groups of patients, unlike the base of the pyramid, which continues to be mild to moderate or even severe asthma. This is really severe asthma in patients who really need a biologic. So, to begin with, the comparison might be a little incorrect. But coming back to Shingrix, if you look at most of the markets where it has been launched, the trajectory has been significant in the first six to twelve months itself because suddenly you have now an innovative vaccine that has 97% efficacy. Most importantly, it helps avoid serious complications like post-herpetic neuralgia, which is extremely painful. We will start with the stocks that have already arrived in the country. They are undergoing testing, and we estimate that we should be able to offer this vaccine to the consumers at large starting next month, in April.

Alka: That's

answered my question.

Moderator: Okay? We passed the mic. Okay, go ahead, just introduce yourself.

Gokul: Gokul here. So just two questions: one is that you break up your business in terms of promoted brands, others and vaccines. So, in the last nine months' results, what would have been the growth rates between the three broader segments of the business?

Bhushan: The way we categorise general medicines, private vaccines, and of course, speciality. Now, given the size of the specialty business, it's still relatively small because it's a muscle that we are building for the future launches. A bulk of our business, so almost 80% of our business still comes from general medicines portfolio and that part of the business has been growing in high single digits.

Juby: So general medicines is growing by 10%. So general medicines make up almost 80% of our total business, okay? And vaccines make up 20% of our total business, which is deg rowing roughly 34% in the last quarter, 35% in the last quarter. But we expect that is going to stabilise. Now inside general medicines, which make up 80% of our business, so, the promoted brands grow 10 to 13%, depending on the last two quarters. 02 was 13%, and the last quarter was 10%.

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So broadly, double digit growth across most of the quarters. You could see it's a very strong portfolio, and we expect the same might be continuing as well.

Gokul: Just to clarify, you mentioned vaccines decline of 34%.

Juby: So, vaccine, if you see Q3 of FY23, which is the last quarter, in this quarter there has been a 35 percent decline year-on-year, okay? The quarter before it was also around 30-32% decline. Now, what is interesting is that the absolute value of the last two quarters is almost stable. We are seeing stability come through in the vaccine portfolio. So, this decline is mainly driven by the market decline as well as the NIP business, which is this decline from the National Immunization Programme. Some of the antigens are getting there, but the good news is that we started seeing growth in some of our key assets. That's one part. Second, the overall portfolio started stabilising in the last two quarters.

Gokul: So essentially, in FY24, you should see growth in the vaccines portfolio.

Juby: That's right. That's what we are expecting.

Bhushan: Absolutely.

Gokul: Okay.

Bhushan: Just to add to that point, if you recall, there were enough data points available to actually show that the birth cohorts were indeed suppressed for at least three to four quarters. And when you look at some of the surrogate markers in terms of the pregnancy test or some of the products used for pregnancy, those have gone up since September. So, that means birth cohorts will start normalising as we speak and that's another indicator for us to tell us that we will definitely see stabilisation in the paediatric vaccine market.

Gokul: Okay. And secondly, WPI last year allowed us-I mean, the price hike in the NLEM was 10% because WPI was that number, you know, in the NLEM portfolio. Did we pass the 10% price hike across?

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Juby: Yes, we have taken on all the portfolio -full 10% price increase. But that said, it is a little bit phased out. Some are coming after April to May. So, there's a bit of a phasing there, but the full annualised impact on that for FY24 will be completely 10%, which fully answers your question.

Gokul: So, the impact of this will be felt going forward.

Juby: That's fine. That's fine.

Gokul: Okay. And lastly, just on the slides, which you mentioned, that high single digit growth aspiration that was for the next three to five years, is that right?

B

hushan: Yes, in the next couple of years, given the fact that we will definitely have-so we don't foresee much of an impact on the overall financial year ending March 2023. But in 2023-24, we definitely see with all that we are doing, there will be some marginal impact. Both top line, bottom line. And that's why I said single high digit. Obviously, when I scope it over the five-year period, the aim is to grow by double digits.

Gokul: Okay, great. Thank you.

Moderator: You can just pass it on. Just pass the question. Okay, you're right. We'll come to you. Just introduce yourself.

Satish: Yeah, I'm Satish. Sir, I have seen Glaxo from maybe 1990s. So literally a span of 35 years. You had a good phase till maybe 2000. Then you had a very bad phase till maybe the last ten years; I think maybe it was very erratic, maybe because of any NLEM coming in and those things. So, in the last five to ten years, we have seen that the entire market is moving more towards a chronic area. That way, how Indian things are also getting changed. You can see from your portfolio -cardiovascular, which is just 5% maybe years back, now it is 30-40% of the market. And your portfolio is now more confined to vaccines and respiratory, which we feel is one of the specialties of GSK, which is putting money into the R&D side. You said that with Shingrix, you're targeting a market of one crore people, maybe the maximum in Tier I and II cities, where they can understand such products. So just if you do some mathematical work and launch at Rs. 5000, which may be a decent thing, so that's roughly Rs. 5,000 crore product. So, I just want to know what type of thinking is there in terms of management to make, if not 5, maybe 500 to 1000 crore worth of product. Because that is really going to move the needle for Glaxo. For a

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company whose size is 3,500 crore, you need some good products, really. You can make 500 or 1000 crore and you are the only one to have such a vaccine. So, what type of differentiated marketing investments you want to make? Maybe, you would have to spend 100-200 crore on a product. Is the Board or the Management thinking along those lines because you have some really good products. So, what is the management thinking about those? Then I'll say we'll talk

on Nucala also, sir.

Bhushan: Sure. So first of all, thank you very much. It's a great question in terms of scoping out the size and potential. We would love to have this kind of aspiration translated. But as you would also agree, in a market where not just paediatric but also adult vaccines are self-paid, there are definitely filters for access and affordability. But that's exactly the reason why we are playing at the deep end now in terms of placing these bets in areas where we think we have the advantage. We know and we are confident that we have the differential advantage in terms of innovation, but more importantly, we have the ability to build that market and that category. So that's exactly the reason why we are spending our money and our energy there. So, the point that you make, I think, is a very important one. If you remember one of the points from my slides, we are one of the few companies that still has four brands among the top 50. That's almost 10% of the top 50 brands from GS K's stables. And that's one of the reasons why, as an organisation, you will not see us launching products that make just 5 crore or 10 crore. It doesn't make sense to put our energy and focus there. It's better to invest in areas where we know we have a differential advantage, where we know we can make a significant impact and solve a critical medical need. And so, you'll see that most of the subsequent launches that we have or the work that's happening is all in areas where we believe we have the abi

lity to build such mega brands just by

following up. So, the question that we ask internally is, which is the next Augmentin for us, which is the next Cal pol for us? And that's the intent with which obviously, we want to build, spend and create that new category for Shingrix. So, the answer would definitely align with what you're thinking.

Satish: Sir, the vaccine is basically preventive. The next is on Nucala. I think two or three years back, in your annual report, you told the eosinophilic-based asthma patients that we have around nine lakh patients in the country, and maybe the cost of treatment is something like, one and a half lakh rupees for the entire four or five doses that you take. That itself is another big market. So, this is a product, and the market is ready. So, what is preventing you from making 500 crore? What is preventing you? What have you got to do differently that Glaxo has not done till date to make it something?

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Bhushan: Sure, yes. We truly believe Nucala is a great asset and as I said, it continues to offer benefits to patients. Now obviously, we have aspirations in terms of taking it to the next level. We've just created a new structure now to support our entry into the institutional segment, build key accounts, and manage management capabilities. The brand has done I think, for 2022, we did cross 1,520 crore. The question is, what will it take to make it 50 crore or 100 crore in the coming time? And I think that's where we are spending.

Satish: But you have a market; you have nine lakh patients already there.

Bhushan: But also, you have to remember it's a biologic. And I think, like any new category, physicians will have to be ready to start choosing biologics. And I think that's an area where we have invested in the last twelve months, and we'll continue to invest even in the coming time. So that's exactly where we are putting our resources and energy in terms of category creation. When categories get created like this, they will not follow a linear curve. They follow an exponential curve at some point. So, although the build-up might seem like a slow burn at a certain point, it really creates a different exponential trajectory. And that's exactly what we're aiming for.

Satish: Are you ready to spend 5,000 crore on that particular brand in a particular year for the next three years? You can spend 150 crore. Maybe the payback will be a fact. Maybe after three years you may see the top line suddenly becoming exponential, like 100, 300 or 500 crore. But that's already a USD 3 billion drug for you globally. And that's a product that I think India needs; I think education might be the science provider for the doctors. So, I still think, why is Glaxo still not making it a killing? So, what is preventing that?

Bhushan: No, I think there's nothing preventing it. As I said, even if you look at all the biologics that are used in this space, it is a significant new entrant. The reason I say that new entrant is because we started getting into some of the key accounts because these kinds of products also need to get into the right institutional account. So that's what's happened in the last few months and I don't think anything is stopping it. It's just a question of finding the right time for us to swing that needle. Because, as I said, category creational biologics do not definitely form a linear curve; it's definitely an exponential curve.

Satish: Okay, thanks.

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Moderator: Thank you. Thank you for your question. Can we pass? Introduce yourself.

Anuj: Yeah, hi. This is Anuj from N3 Investments. I want to drill down on vaccines a little bit

. So

first, on the short term, industry after industry has seen pent-up demand show up. And if I see the parents' portfolio, the vaccines have grown by 20% post-COVID. It's interesting that India continues to show degrowth and it's not a normal scenario. So, you said the cohort is showing up, but isn't it too delayed? So why is India's scenario being different than the global? That's question number one.

Bhushan: So, two things happened, as I said. One is that we did see suppressed birth cohorts. But again, in a population like our country, where you have an average birth cohort of 26 million annually, should it really be material enough to start denting? It is a question we can ask together. But we did see that the footfall in the clinics of paediatricians had definitely gone down. The other reason that Juby just alluded to was also the introduction of some of the newer antigens in the National Immunization Programme. So, you had PCV or the pneumococcal conjugated vaccine, that got into the National Immunization Programme and that was probably the biggest. If you look at the immunisation rates in our country, even in the targeted cohorts, the highest immunisation rates are in some of these categories. And when these get into the NIP, a large part of the population also starts moving to places where this is given for free. And I think that was also one of the reasons why the vaccination for other antigens started getting mixed up in the private chambers. Again, these are anecdotal. This might seem anecdotal, but in terms of connecting the dots with all the data points, this is exactly what we know was happening on the ground. As I said, some of the surrogate measures that we've seen in the last few months tell us clearly that you've seen that it's turned the corner. We are stabilising in terms of both volumes as well as value. And the growth will only start coming in the next few quarters.

Anuj: Sure. Thanks. My second question is: if I again look at the parents' portfolio, the growth for vaccines, it seems, has been 10% CAGR for the past 22 years, maybe 23. And if I take the India data, it seems that it's nowhere close. So again, there are three parts to it. One is the NIP programme, one is the competition, and coming to the third, can you just give some more colour as to why, as you explain, some part of the loss is structural because of the NIP programme, so can you share some more details regarding how we are losing in terms of the NIP expansion every year?

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Bhushan: Sure. So, if you look at the vaccine market over the last two decades, if you look at GSK, 20 years ago, our biggest vaccine brand was a brand called Engerix-B. Now that is no longer even available in most of the countries. So as vaccines evolve, where you have higher valencies, where you have tetravalent, where you have pentavalent, where you have six diseases covered in one single shot, obviously the nature of your portfolio evolves. And I think that's the nature of the vaccine business today. If you look at the pentavalent and the hexavalent, these are the ones that are mostly there, so if you look at these two categories, they're really driving the growth. And therefore, if you look even internally within GSK, our own hexavalent brand, Infanrix Hexa, is not following the same curve as the PCV, which is going to the NIP. It's growing. So compared to the minus 35% decline that we see in the market, this brand is still growing, maybe in single digits, but it's never seen a decline. So, you see, the market is shifting to vaccines that really make sense for physicians in terms of requiring fewer jabs and lesser pricks for children. I think, that's the nature of the vaccine business.

Anuj: Okay. On the specialty portfolio, I think the last time you communica

ted that it was 4 to 5%

of the revenues and you had aspirations of 10%, where is that heading up? And also, the portfolio, which could be transferred from the parents, in let's suppose for the next three to five years. Throw some light on that area, please.

Bhushan: I don't know about 4%, but we've just launched, so we have only two products in the specialty portfolio right now. We have Nucala, which is, as I said, a biologic. And we just launched the triple-therapy SIIT, which is an inhaler called Trelegy, but again, that's indicated for COPD, not for asthma. So, these are the only two products we have right now. We'll see, as I said, that there are more than 20 clinical and global trials happening even in India for many of our assets. We'll see in terms of the access strategy, in terms of what makes sense for us in terms of unmet needs, what to tap into from that portfolio, but that's something for the future. As we talk, the idea is right now to build a speciality muscle with the two assets that we have in the forms of Nucala and Trelegy.

Anuj: Okay. And one last question: I think the earlier gentleman was talking about investment. So, could you talk about the structure or the sales for which you are aligning to this segment so that it will give us some thought flow to how it is being built?

Bhushan: It's a brilliant question. In fact, both were brilliant questions in terms of the appetite that we have to really fuel the growth at the growth end. And just to give you an example with

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Shingrix, we've created a completely new team for adult vaccination, especially for Shingrix, with almost 200 people on the ground with specialised capabilities and experience in creating new categories. So that should also tell you, plus, these kinds of new launches will also depend on how we are able to resource all areas around disease awareness, around creating consumer campaigns, really creating patient assistance programmes. So, I think those are all the levers that we are unlocking and that is something that you will see in a very significant way, beginning with Shingrix.

Viraj: Hi, my name is Viraj. Just a couple of questions. First, you talked about a high single-digit growth rate in the next three to five years. And this is, I'm assuming, it's after the impact of NLEM. Now, if I look at FY24 based on the current prices of the brands we have, what will be roughly the impact of NLEM and after the WPI increase, what will be the net impact? So that is question one, if you can answer it.

Bhushan: Sure. So I'll hand it over to Juby. But before that, look, the NLEM prices are coming at different stages. So even as we speak today, a lot of these draft prices are out. Some of the draft prices are still not out. The final ceiling prices are coming, I think, in a staggered fashion. So, it really depends on which quarter it falls in and therefore which year it gets accounted for. And that's why I made that statement that for the financial year ending this year, March 2023, we don't see a significant impact. But for the coming year, definitely, we will have exposure. And that's why we have lined up all the initiatives that I just shared in terms of how we intend to mitigate primarily through new launches like Shingrix, but also by learning from all the lessons that we've had in the past, where, as I said, Augmentin has had an NLEM impact twice. And today I just showed you that it still continues to be the number one brand in the industry by way of revenue, in terms of value. So that gives us confidence to mitigate a large part of that exposure. But I'll hand it over to Juby if you want to add something.

Juby: Speaking on NLEM, I think first there are two or three new assets that got added to this right, which are Ceftum and T-Bact. These are

big additions to the NLEM. Now, the total annualised impact for next year, FY24, could be in the range of 8-9%. We're talking about 240 to 270 crore. That's massive. Now, we should not see this impact as a discrete price impact because most of these price impacts are so massive or significant that the price volume gains will kick in. So, we've been thinking about how we can mitigate it. There are several things we are doing to mitigate. One is the volume that we were talking about on the slide. Second are new launches, which are like the product lifecycle extensions that we are seeing in Cal pol Plus; third is cost

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optimisation, fourth is expense optimisation. All these things will help us net what we are talking about, which is roughly a good line of sight of 70% profitability mitigated, as we speak. Okay, so 89% of the top-line impact flows into the bottom line. We will be seeing 70% of those mitigated impacts on the things we are still exploring, as we did in 2014. We will subsequently see a similar level of impact, but since we've been mitigating it, perhaps the same will be happening this year also.

Viraj: So, this 8-9% is pre-WPI pricing.

Bhushan: Pre-WPI, WPI will be further helping us to mitigate this one.

Viraj: Okay, got it. The second question is on the vaccine part and what you were answering to one of the earlier participants. How the mix has changed over, say, a 20-year period. So, there are two or three questions on vaccines versus if I have to look at the last five or seven years, can you just probably give some more colour in terms of what the contribution is? So, what were the top five vaccines contributing to the portfolio? How has that changed, say, versus the last five years? And now, what are those? And if you want us to understand the market segmentation further, can you provide a little more perspective on which segments you have seen more sharper degrowth in and which segments are still growing and, in that sense, how is overall competition in each of those segments?

Bhushan: There were multiple questions in that, but I'll try to peel back those layers and try to

answer the broad themes. But, yeah, I think, if you look at the biggest launch that we had in the private vaccine business, it was almost a decade back when we launched Synflorix, which was our brand of pneumococcal conjugated vaccine, indicated for, obviously, the prevention of pneumonia. And in fact, this was the first vaccine in the industry to cross Rs. 200 crore in annual revenues. And today, this is exactly the vaccine that is in the NIP. So, in the private segment, obviously, will this demand evaporate completely? Absolutely not, because there will still be parents who will want to vaccinate their kids in a private setting. And that's why we still believe that at any given point in time, for a company like ours, we will still have access to vaccinating about 25,000 kids every month in the paediatricians' chambers with this vaccine. So that's one part of our plan, from a criticality standpoint, with this vaccine. So, to the you know, broader question, if you look at our vaccine portfolio today and look at what's in the NIP and what's our exposure, because we've already mitigated that, is now less than 6-7%. So, for example, Rotarix, which is our other brand for rotavirus and is also in the NIP or now Synflorix, both of these are

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accounting for less than 6% of our top line. So, I think that globe is already factored. Where we are now playing is the hexavalent that I said it's the largest vaccine now. It's almost 165 crore in top line value, it is growing. So that's where

we're investing all our energy. We have flu, for which

we have Fluarix Tetra, which is not just during COVID time, those two years, but even today, as you can see, it comes in both southern hemisphere and other northern hemisphere two variants given the seasonal changes. So that's another vaccine that's significant for us. And of course, we continue to have leading brands like T-Bact, Boostrix and Menveo. So clearly our portfolio is now very well aligned as opposed to in the past where one brand was pulling up the whole portfolio. Today, it's much more balanced in terms of the contribution and also the growth end.

Viraj: Okay, so when we're expecting the growth for this portfolio vaccine to grow next year of Shingrix, what kind of growth do we see starting to come for us because of the effects of another immunisation and other elements that are kind of going that way now? So how should one look at the vaccine portfolio as it is behaving for us? And what kind of team is supporting this?

Bhushan: No. So, it's a very good question. Again, look, from a massive slide that we saw for the last 12 to 18 months consistently in the paediatric vaccine segment, as I said, in the last couple of months, which means two months of the last quarter of this financial year, we are seeing the business stabilising as opposed to the 19-20% growth.

We have low single-digit, 3-4% growth, which means we've turned the corner, at least in terms of holding. The idea is as we enter the new financial year, is to get back to double-digit growth. Of course, as I said, we also have the advantage of some of the vaccines that were stock constrained, like we have a vaccine which has been out of stock for more than a year. So that's coming back, which is for chickenpox, basically. So that's coming back this year. So, with all of that, we should be able to get back to our intent, which is drive double-digit growth. Again, Juby, do you want to add something?

Juby: I think you completed it. We are expecting double-digit growth, particularly with the vaccine business stabilising, some supplier assumption exchange is also required.

Viraj: Okay.

Juby: And also, just to add one more point, the NIP part was having some impact. It's so miniscule at this point in time. So that's not going to have a big impact in terms of decline. Okay,

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just one more question was on the MR team, if you can provide more perspective in terms of how the digitalisation, has affected the speciality vaccine and other segments, what kind of MR team is focused on each of these? Just to get a perspective on how we are going, about where we are putting more resources in the future.

Bhushan: So, as we watch, as I said on one of my slides, our strategy is very clearly aligned in terms of therapy areas and the customers that we serve. And that's how the strategy is dictating this current structure. So, as we speak right now, we still have a long way to go if you were to look at this as a continuum. At one end of the spectrum, you have the mass teams, which are largely the general practitioner-led paediatric teams, which promote products like Augmentin, Calpol, and are more about reaching people and getting their voices heard. Then we have the dermatological business team, which has a very specialised focus. Then we have the paediatric vaccine team and then we have the Shingrix team. So that's how we are structured. On top of that, I think we will continue to provide a lot of access to science.

We still have a rural team that is based in Tier III and IV towns. We have 175 of them who are based in the interiors of India. And they cover the entire portfolio because largely the practitioners in these areas also become specialists. So, this is how we are structured currently. And that's exactly to serve

the strategy that I showed you in the earlier slide. So, this is how we are structured. What's changed in the last three months is that we've carved out a completely new Shingrix team and we have a speciality team that we've just started reinvesting in, similar to what you heard. We've just expanded that team to build some of the new capabilities. So, this is how we are structured as an organisation.

Vi raj: Okay, just two more questions. One is on the employee base part. I think we downsized around 125 people in the sales and distribution functions. And suppose that with this change in employee base, we're doing something between a 100 and 4150 crore quarterly run rate in terms of expenses. So, should this kind of thing continue, or do we have other options, you know, it is resizing.

Juby: So, the resizing thing, if you see from the 2022 financial exit to now, it's almost 200 headcounts. We have resized down. Okay, so this is the trade team. So, the inflationary pressure is what we are seeing. What you said about the employment cost is correct; this resizing is

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assisting us in managing that. Other than Shingrix, we are not investing in headcounts in other areas because we are already resizing them. We have already been categorised into these three categories, and we are put in them. Shingrix is again a new headcount, and we are positioning 200 headcounts into the business.

Vi raj: Just one last question for you. If you look at our own listed entity, for the last ten years, the shareholder returns have been below FDA returns, to put it in that sense. If I have to look at the top management, if you can provide some perspective, how are they incentivised? Do we have a formal ESAP policy, which might bring more focus to the growth part of the business? And how is the parent organisation looking at the listed entity in India?

Bhushan: Well, we continue to be listed here. As I said, we have been listed here for more than 50 years now. So that should tell you that we continue to do so, and we have a board of independent directors who are absolutely focused on rewing up the growth engines of this company. So that's the top line. Yes, I think the global organisation is such that we do have visitors in India, including our global leadership. Obviously, given the geopolitics, India continues to be now an even more important market for us as an organisation. So I think yes, if you look at the last ten years, the TSR has definitely left a sour taste in everybody's mouth, including ours. What are we doing about it? I think we've, as with any organisation, placed some bets. Unfortunately, as with the nature of business development or the bets that you place, some of them worked, some of them did not work at all and have set us back, including, for example, what happened with our plant and some of the key assets that fell off the radar for us. So obviously, the idea is to be forward looking and see what we can do. And that's why I said my last slide was clearly focused on what it will take for us to deliver 10% growth as an enterprise. If you look at the last couple of years, there has been double-digit growth in our parts of the business. If you look at general medicine, which is 80% of business, that's grown by 10%. So hypothetically, if a vaccine business had delivered, if it were not for these externalities that were completely out of the syllabus, so to say, you would have probably been able to solve that growth story and answer some of your questions. But that's the intent moving forward. I think the whole organisation, both globally here, as well as the listed entity, is focused singularly on the growth agenda right now.

Janelle: Yeah, hi. This is Janelle. I have only one question. The global reorganisation that we had between the consumer and pharma businesses, does it have any implications for India in terms

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of the focus on pharma from the point that you've been mentioning? So does that really have any focus in that sense?

Bhushan: From an Indian standpoint? Not really, because there was no umbilical cord connecting the consumer and the pharma business in India because we were two separate, legally listed entities here. So, to that extent, on the Indian market, it hasn't really made any difference. I think the only difference is that globally, obviously, it sharpened the focus in terms of putting energy into biopharma. And that plays to our advantage because the whole

organisation is now focused on how we can increase the growth trajectory within the pharma business, of which India is continuing to be the largest business within the emerging markets' business. So, we are the largest business within the EM group. Therefore, the focus even further improves moving forward. Juby, do you want to add something to that?

Juby: No, I think you said it. It's a completely different business now. Two things. One is that we become more relevant in the emerging markets group. The second is globally, as they're completely different entities and the focus on research, development and finding new entities, all those things will go up globally. Eventually, that will flow into India. Right? Directly, once they find something. So that's the simple connection we'll have. Other than that, logically, at this point in the ground, there is no difference. We have completely two different management teams. Everything is separate. No sharing of expenses, nothing of the sort. So, it's business as usual for us at this point in time. Okay. Thank you. Good luck.

Moderator: So, we have two folks who've raised their hands online. We have about 58 people at a point in time who joined, but two of them have raised hands. Melroy, if you can unmute one of them and if you can introduce yourself and then shoot the question to Juby or Bhushan. Melroy?

Melroy: Yes, I've asked them to unmute.

Moderator: Yes. If the name is visible, maybe you can call them out.

Melroy: Mr. Gagan?

Gagan: Yeah. Am I audible?

Melroy: Yes, you are. Hi, Gagan.

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Gagan: Yeah. Good evening. So, the first question is more of a clarification. You indicated a 250-crore impact of the revised NLEM on T-Bact and Ceftum on the top line. And you said you'd be able to offset 70%. Does that mean the operating level EBITDA impact would be around Rs. 75 crore because of this?

Juby: So firstly, Rs. 250 crore is not for Ceftum and T-Bact. It's the total impact across all the NLEM portfolios. Okay, so we got around 34 stockkeeping units under NLEM, so it's a total impact on all the portfolios. So, we'll see 2 to 3% impact on the bottom line for the first year at least. But we are expecting to mitigate in the outer years as we speak.

Gagan: Right. And in the last few months, I think the prices of paracetamol API and also some other APIs have sort of come down and softened a bit. So, as you enter the year on the NLEM piece you probably get an 8% to 10% increase. You also get the input prices corrected. And the full impact of the last year's 10.7% increase. Would all of this not in some measure offset the impact of the revised prices? Or are you saying that after considering these positives, the net impact of 2 to 3% is what you estimate?

Juby: Yes. So, taking everything into account, we're talking about a 70-75% mitigation at this point in time, because you can see the magnitude of the impact. Right? So, 89% has a massive impact on the top line. So, the mitig

ation covers all of these things, including the cost savings, the WPI that will be introduced, and some of the manufacturing changes that we are implementing. For example, when you are seeing that video of Nashik, there are some packaging changes we are making on some products. So, these are all going to help us with the cost savings. Now, just to add, we've been working on this for a long time. If you think about 2014 or 2015, our EBITDA used to be 14 to 15 percent. It used to be higher. It has dropped to 14-15% at some point in time. Now that it has gone up, several initiatives have gone through. So, we are working on several initiatives like that. And as we speak, we are talking about 70% mitigated. 70% to 75% will be mitigated.

Gagan: Right. And you also indicated that, in 2024, you are aspiring for a high single-digit top line growth. If the impact on your top-line is of the order of 8 to 9% negative from these 2-3 products or whatever more SKUs you've talked of, it means the balanced portfolio has to grow very sharply, probably closer to 20%, for you to be able to create that sort of top line growth.

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Could you elaborate a little more? I mean, is the new product that you're going to launch plus the baseline growth on other products going to be able to give you that much?

Juby: Yeah, that's right. I think what we are seeing is two to three dynamics playing out in FY24. One is the Shingrix launch. It is a big launch for us. That is one piece. The second is the volume increase. Some of these price reductions, particularly T-Bact and Ceftum, you will see a massive volume increase. That's what we are expecting. So, with the volume increase, with the new

launch, with some of the supply coming back, we are expecting that kind of a double-digit growth. So, the net we are talking about is high single-digit growth.

Gagan: Yeah. And final question from my side: one of your slides showed the operating margins initially coming down and then over the next three to four years, you aspired to take them up to 25 odd percent, if I could eyeball it correctly. So, what are the levers for you to be able to take your margins up once you've sort of digested and encountered the impact of the revised NLEM?

Juby: So, two, three things. One is the NLEM is once in a five-year impact, right? Then you'll keep having this WPI as well as the normal price increase. So that is one major factor that will help us improve our margin. Second is a promoted category, which is relatively high margin business because that's going to grow high double digit or double-digit growth. That's also helping us improve the margin. Last but not least, the cost-saving initiatives that we are working on are also going to help us improve our margins.

Gagan: Final question. Why is it that you have two SKUs in paracetamol, which are Crocin and Cal pol, and Crocin is with your other entity, which is consumer health? Essentially, they are both the same product. It could have been captured very easily by the listed entity. What's the difference between Crocin and Calpol for you? These are the two questions.

Bhushan: I think I'll take that question. First and foremost, as I said earlier, we are two separate legally listed entities. So, there is absolutely no connection between Cal pol and Crocin. Crocin continues to be a trademark owned by Haleon, as that company is now called. So, there is no correlation. But if you were to look at the IQVIA data, I won't hazard a guess in terms of their size, but we continue to be the number one paracetamol brand when it comes to not just paracetamol, but if you look at all the prescriptions that are written in India across all categories and across all brands. The number one prescribed brand in India is Calpol.

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Gagan: Sorry to have interrupted. My question is: what was the rationale behind putting one paracetamol brand in a consumer entity and one paracetamol brand in a listed entity? I mean, in this entity, which is an Rx entity? Logically speaking, either it is a consumer brand, or it is a Rx brand. Which means that either the whole package should be with you or the whole package should be with them. Why is it split up?

Bhushan: Yeah, it's a good question, but I think we have to go back to history because this was a brand originally owned by a company called Duphar, as some of you might know, and it was divested by that company and then picked up by Smith Kline and then it went to the consumer business. There's a long history, but that's the reason why it stayed there, whereas Cal pol remained anchored in ethical promotion. So, it's sort of brand that you will see in the public media being advertised. So, we are clearly rooted around ethical prescriptions, whereas the route that I think that company took was more leaning towards the OTC space. But again, as I said, you'll have to go back and see the history of how that brand had moved from Duphar.

Gagan: And likewise, what was the rationale for that. Absolutely. Yeah, that's true. Okay, thank you.

Moderator: Hey, Gagan, I hope we've answered your questions. I don't know whether it's comprehensively as far as your mind goes, but I think you should tell us. You've been tracking GSK for a longer time, so we'll connect with you for answers sometime too. On the history of Cal pol and Crocin. There is one more hand raised, Melroy, if you could just announce the name and unmute.

Melroy: Nikhil?

Nikhil: Yeah, hi.

Moderator: Nikhil. Hey, Nikhil.

Nikhil: Hi. Good evening. Am I audible?

Moderator: Yes. Hi. Yeah.

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Nikhil: Hi, I'm Nikhil Upadhyay. Just three questions. One is a clarification for Juby. When you set this to 50 crores of NLEM impact, and if we build in the WPI price increase and volume increases, what I understand is that the impact -does that water down significantly, and considering the growth and the cost initiatives, the impact on the margins would have been much more normalised. So, when you say this 2-3%, do you consider the WPI price increase from April 23, which we will get, just a clarification?

Juby: Yeah, we counted that as well.

Nikhil: Okay, fine. Now, two questions: One is, if we look at, like in your presentation, you mentioned that from 23 to 25 or 26, you would like to grow in high single digits. But if we break down GSK's portfolio, there are three buckets. One is vaccines, one is speciality medicine, and one is general medicine. Now, both for vaccines and speciality products, if we look at your launches, these are more focused on meeting the unmet needs or to enhance penetration where there is a lower penetration of these products. So, in a way, intuitively, this part of the portfolio can grow at a higher single-digit, and it used to grow in the past before COVID on the general medicines. If we look at it, we've got brands where we've got significant market share, like, on an average in our focus brands, our average market share is beyond 30-40%. And that part of the portfolio is also grown in high single digit or high double digit. So, when you say over three to five years, the Company's growth rate of a high single digit, and if I look at the portfolio breakup, which looks like intuitively, can grow at a higher double digit, what part of the business is pulling this growth down?

Juby: Like someone asked you? So, the whole NLEM is sitting in the general medicine portfolio, right? So that is approximately 10% of the gen

eral medicine portfolio. Now, to grow in double digits or high single digits, the business has to grow in high double digits. Okay. So that is the pricing -the whole pricing element is pulling it down. There's nothing else we're expecting from our promoter brands other than to continue to grow by double digits. The pricing element will be offsetting it, bringing it to high single digit.

Nikhil: Okay. The end markets are still so if we look at the end market, say, for Augmentin or Ceftum or CCM, the addressable markets, are they still seeing healthy mid-single digit or high single digit growth? Because we've got a significant market share. So maybe increasing market share beyond some of the brands looks very difficult. So how do you see the end markets

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growing? And do you see how that can sustain healthy growth for us in the general medicine portfolio?

Juby: So let me just give you a flavour of what I'm talking about. So, if we talk about Ceftum, which is one of the biggest impacted NLEM products, the price cut we are going to see is roughly 60%. The price is still at 60%, which means our price is going to be so low that the price volume play will play. So, what will happen is that there are a lot of patients or a lot of doctors who are not able to prescribe because of the high price of that product will now start prescribing. So, we mapped it out very clearly how to play this one. So, we believe some of the factors, particularly when the price becomes much more accessible, will help access the lower end of the pyramid. And that is what we are seeing in other products, also like Augmentin. So, it's the same analogue which we have used as we were building up all this forecast and we have seen that in the past also when the NLEM impact has been significant. So, the volume will definitely be there. There's a market share gain that will be there, as well as for the doctors who are using generics. There will be a shift. Some part of that will be from that, and some part will be from the market building that also will be happening. The category will also be expanding as the price dynamics change in the category. So, it's a function of all those things, but we'll be expecting to see that.

Nikhil: Okay, fine. And just one last question. Now, what we've seen is that there is a constant change in the management at the MD level. Like in the last ten years, there's been a fourth change that's happened. And as an outsider, what we see is that the new incoming is always trying to clear out the problems of the previous and by the time the problems get cleared, the management again changes. So, there is no continuity in terms of, say, a focus on how we have to grow and probably make the company stronger. And that's clearly visible in the earnings also of the companies that we are flat where we were. So, how does the parent look at this three to four years of change in the management? How is it driving the focus in terms of making the Company longer or larger over 5-10 years? Who is driving this whole idea of the Company over next five to ten years, where we want to be and how we have to be better than the market?

Bhushan: So probably I'll take a stab at that. It's a relevant question, a good question indeed. But as a listed company, as you must be aware, we are also having an NRC committee. We have a Board that's actively involved in terms of ensuring that there is leadership, and more importantly, leadership continuity in shaping and reshaping the organisation so that we can continue to leverage this incredible equity that GSK has enjoyed for the last century. Now, again, in a global organisation like ours, it is also quite likely that talent moves from geographies. I

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mean, both of us have spent 12 years. But probably for Juby, this is the first job in India, although he's been with GSK for more than 14 years.

And for me, in these 12-13 years that I've been here, I've also had a chance to work outside for almost 5 years. So, the point I'm making is that it's also a function of when you operate in a global organisation, some of these choices of who has to move where are also driven by how the organisation is seeing the individual talent move.

Having said that, leadership continuity is absolutely critical. And that's why I think even moving forward, I can't predict, I don't have a crystal ball to gaze into and tell you. But clearly, in terms of our appointments also, I think the Board has been quite clear about ensuring that type of leadership continuity. More importantly, I think even when I look at my own example, I've been here for the last two years. I was a commercial head for the general medicines business. So, one thing that you will not see happening is me cleaning anything because I was very much a part of the team running the pharma business before I got this job three months ago. But I think I don't have an answer which can directly tell you whether this is right or wrong. But I think in global organisations, this is part and parcel. But I think in terms of ensuring and showing leadership continuity, the organisation is fully aware of why we need to have that continuity moving forward. That's all I can say right now.

Nikhil: Thank you.

Moderator: Yeah, I think the Company secretary has a word to say.

Ajay: Just to add, the earlier one was for 2 years. Your appointment itself is for four years, if that's any indication.

Bhushan: I thought they wouldn't be knowing.

Moderator: So, we have Ajay, our Company Secretary, in the room. So, he says that the contract with Bhushan is for four years. If that's any indication of how things are, you know. Thank you,

Ajay. We have two more hands raised. We'll take those two questions. One is from Rohan Samant, and the other is from Mr. Sameer Baisiwala. So, Rohan, your question.

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Rohan: Yes, thanks for the opportunity. So, on the NLEM portfolio, where the price reduction would be effective mostly in FY24, the WPI-led price increase would be allowed in FY25. Is my understanding correct?

Juby: Yeah, I think WPI starts from April. It depends on government notification. Right. So NLEM will start. Some of the products started got implemented from January this year, some will start from next quarter. So, depending on the timing of implementation, this works. Okay, so theoretically WPI starts from April every year.

Rohan: Right. But let's say a product. Ah, that the price cap is implemented from, let's say, April 23 or May, the WPI would be applicable from April 24 onwards. Right? For those specific ones.

Juby: So, it depends on how the government is going to announce that price. Right? So, we're still waiting for the final pricing to come out. It could be how the government is going to notify that. It could be with WPI. Also, they could be notifying it. We don't have clarity at this point in time, but we have to wait and see how the prices are going to be notified.

Rohan: Sure. Okay, thanks.

Moderator: So, we go to Mr. Sameer Baisiwala. Hi, Sameer. Your question?

Sameer: Yes, thanks so much for a very insightful discussion. So, the question here is, how are you thinking about Shingrix pricing here in India?

Bhushan: Well, I mean, if you look at the global pricing, I will say in India, it's priceless. But on a serious note, obviously we will not. So, if you see what the pricing is in the global setting, it's significantly different. But coming to India,

the reason I'm not giving you the price is because the brand will be launched next month, and let me say it is comparative, given the fact that it's an innovative vaccine that has 97% efficacy and is the first of its class non-live vaccine for Herpes Zoster, and it's fairly competitive.

Sameer: So, I asked this question

Bhushan: Just wait. For a few more weeks.

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Sameer: Okay. I asked this question because I think it's priced at about USD 200 in the US -per dose, and two doses need to be taken. So that seems to be very prohibitive, even at a 30%

pricing here for India. So therefore, you may not be able to realise a lot of value that we were discussing for last hour or so.

Bhushan: Yeah, it's a fair point, and I can assure you it's not the US price. I mean, in that sense, we will definitely have a comparative India price for our Indian subjects.

Sameer: Okay, great. And just one final one. You mentioned taking a 10% price increase through fiscal 2023, and I think the industry in general took 6 to 8%. So, what has been the response from the doctors in such a price-sensitive market? And what's the outlook for fiscal 2024? Because WPI seems to be in the 3 to 4% range. So, are we looking at a much lower price increase next fiscal year?

Juby: So, for WPI products, we can only take what the government notifies. So, we are not going to decide what price we're going to accept. Right? So last year, the government notified us it had a 10.7 or odd percentage. So we are taking it that it's a completely price-controlled portfolio, and it's completely up to the government to notify.

Bhushan: But on the non-control, obviously, in a calendar year, we have the opportunity.

Juby: We can take it. But that is, again, depending on the competitiveness of the product in the market.

Sameer: Thanks. Just going back on this, how has been the response by the doctors when you did take a 10% price increase through fiscal year 2023 and based on your estimation for next year's price controlled and non-controlled, the government notification of WPI is also based on what WPI has seen through these months, isn't it? So, just for the outlook of fiscal year 2024.

Bhushan: No, I think so. I mean, look, over the last 8-10 years, when you look at our portfolio, and all the assets that I had outlined in the general medicines portfolio, except for Ceftum, which was probably a statistical outlier, most of our brands have always been in a certain band. So, it's not that we have been comparatively so expensive that we have been out of the range. So, when we took the price hike also last year, I don't think it really made a material difference in terms of the elasticity. Physicians do understand. I'll give an example of Cal pol, a pill that costs Rs. 2 per

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tablet. I mean, when you look at the cost of the raw material of paracetamol having tripled over the last 12-18 months, the price was still under Rs. 2 per pill. So, to that extent, even if you were to increase it by 10%, the materiality doesn't exist. So to that extent, I don't think to physicians really, given our portfolio, given the nature of our portfolio, it didn't really make a difference. And that's exactly the reason that Juby was saying earlier. We are absolutely confident that this cut that's going to happen on Ceftum is going to help us reshape the whole market. And Ceftum is a trust mark, so we are very confident that that's going to play out favourably for us in the coming fiscal and driving volumes.

Sameer: Okay, great. Thank you so much.

Moderator: Thank you, Sameer. Thank you. Bhushan, we have one last question from Bezaad. Bezaad? If

you can ask your question, that's the last question for the day.

Bezaad: Yeah. Am I audible?

Moderator: Yes, you are.

Bezaad: So, three questions from my end. So, on the 6-in-1 vaccine which you've launched, do you have competition in the market?

Bhushan: So, Bezaad. Yes, it's a fairly competitive segment. We do have competition, both from multinationals as well as Indian companies.

Bezaad: Okay. And on the vaccine front, Synflorix is quite a meaningful product for you. And now, since it's in the NIP, how do you see the private market shaping up? And since you'll be facing competition. Do you expect this product to sustain?

Bhushan: So, look, very good questions here. And I'll just probably answer a part of your first question. So, in spite of that competition that I talked for six-in-one, we still have market leadership position in the hexavalent category. So, we still have more than 50% market share, actually 60% in hexavalent, as we speak.

Coming to the PCV category. Yes, over the last 12 years, we've retained our leadership. As you rightly said, part of that business has started going towards NIP, now we have new players even

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in the private segment. But I think that's exactly the reason why companies like ours are continuing to invest. So, if you're following our vaccine strategy globally, we just acquired a

company called Affinivax, globally.

So that's going to be the first 24 valent vaccine for pneumonia. And pneumonia will continue to be one of the most important vaccine preventable diseases for consideration. So as a strategic imperative, this antigen continues to be important for us. This area of pneumonia continues to be important for us, even moving forward.

Moderator: Okay, great. So, thank you, Beza, you have one more question. Beza? Hi, Beza?

Beza: Yes, I do. And what percentage of your raw materials are imported, and in what currency do you trade in?

Judy: So, raw materials-the whole vaccine portfolio-are important because the vaccine portfolio 20% of our business is in the vaccine portfolio. The whole vaccine portfolio is imported. Other than that, hardly anything is imported. Less than 5% are imported products. So, 97% of our pharma business is from local sources.

Beza: Okay, thank you very much and thanks for the opportunity.

Moderator: Thank you. So, with that, we call it a close. Thank you, everyone, for joining online, and especially all of you who came here. Do join us for snacks. For those of you present here, thank you very much. We can now close the call, Melroy. Thank you.

Bhushan: Thank you very much, everyone. Thank you.

Call: Feb 2024

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THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED
Exchange Plaza, 5th Floor, Plot No. C/1, G Block
Bandra -Kurla Complex, Bandra (East)
Mumbai - 400051 21st February 2024

To,
BSE LIMITED
Phiroze Jeejeebhoy
Towers Dalal Street
Mumbai - 400001

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 15th February 2024. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/investors/analyst-meets/>.

Thanking you,

Yours faithfully

For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni

Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239M

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“GlaxoSmithKline Pharmaceuticals Limited Analysts
Call”

February 16, 2024

M
MANAGEMENT : MR. BHUSHAN AKSHIKAR - MANAGING DIRECTOR ,
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED
MR. JUBY CHANDY – CHIEF FINANCIAL OFFICER ,
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED

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Moderator: Good evening, everyone. We are on the dot of four. For the benefit of others, we will give them another minute or so, and we will start at 4:01, since some of them have registered, but possibly they will take some time to join. In the room, I have Bhushan and Juby, who will be speaking to you. We will start at 4:01. So, kindly bear with us. Thank you.

Good evening, everyone, and a warm welcome from GlaxoSmithKline Pharmaceuticals Limited. We are speaking to you here from our Worli office. And in the room, we have our Managing Director, Bhushan and our CFO, Juby.

So, before we begin, and before I hand over to Bhushan, I just wanted to kind of give a bit of a short introduction.

Bhushan has been with GSK now for over 13 years in key leadership roles across India, Middle East, Russia, CIS and the Africa region. Before joining GlaxoSmithKline, Bhushan has spent 15 years with Janssen, Johnson & Johnson in local and regional positions in India, South Korea and Belgium.

Also in the room, I have Juby. Juby has a successful track record of over 16 years in key leadership roles in GSK across India, Singapore, Southeast Asia, Vietnam, Turkey and the Middle East.

With that, I will now hand it over to Bhushan.

Bhushan Akshikar: Thank you very much, Rans om. Very, very good afternoon. Good evening to everyone. So, the intent of this meeting is twofold. One, not sure how many of the investor community was available the last time Juby and I had investor meetings. But, there are a couple of objectives for this meeting.

One is we have just announced the results, but clearly stemming from what we have announced, it's clearly one of the most important quarters that we have seen behind us. And I wanted to set the context of what we have undertaken as an organization. But more importantly, reset and re-contextualize where we operate as a company.

So, for many of you who may be aware in the pharmaceutical space, GSK continues to be among the foremost multinational pharmaceutical companies, still among the top 12, top 13 companies in the Indian pharma market, one of the few broadly diversified healthcare companies straddling across prevention in the form of vaccines. Of course, having a large presence in primary care, what we call a general medicines area. And now getting into newer areas like adult immunization.

The reason, I said that Q3 was one of the most significant quarter for us is because of the transformation that we are bringing about in our general medicines business unit. As probably you are aware, it's one of the most enduring, one of the most endearing parts of our business with large brands, brands that are synonymous with what we do as GlaxoSmithKline in India.

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But most importantly, given the changing landscape, given the need to unlock value, we have really relooked at our current operating model to have six to seven teams operating with different set of brands and portfolios. We moved to an objective prescriber centric sales model. The whole intent is to reduce the overlap, but yet not compromise on the reach so that we are able to reach and connect with the healthcare practitioners.

Most importantly, we are trying to redraw the strategy in terms of our reach and coverage for healthcare practitioners based out of tier two to tier four markets. So, we used to have three and four reps going to the same physician, carrying different set of products. We are trying to bring leverage some synergy there.

Essentially, what this unlock means is, we are trying to

accelerate the digital transformation

within this large business unit using more of digital touchpoints, using more of the omni-channel ways of working as we had announced in our press release as well earlier.

What it means is we have now put together a very clearly articulated vision around what that digital transformation will mean so that we don't compromise on our healthcare practitioner coverage. We are able to expand and reach out to physicians and practitioners beyond what our reps do. But most importantly, really integrate seamlessly our face-to-face interactions with all the digital ways of connecting with these healthcare practitioners.

So, over the last couple of years, we have been working seriously in terms of creating personas, connecting the channel preferences of individual N is equal to 1 HCPs for their need to be connected through either emailers or WhatsApp channels. So, different channels and different touch points will get seamlessly integrated with the face-to-face interaction.

So, that's the massive unlock that we have. The idea is to continue to enhance our engagement with our healthcare practitioners, but more importantly, unlock value so that we can focus on our growth areas. What it will help us do is resource our brands, which are more growth oriented, better than what we have done in the past and more importantly, improve productivity.

What this will also allow us to do is focus and release some resources to set up and scale up our

presence in respiratory. As many of you may be aware, we have two innovative assets, Trelegy and Nucala.

So, clearly, given the fact that the future portfolio is also aligned in this area, how do we ensure that we deepen our coverage and continue to build our presence in the respiratory space. So, that's the major unlock that we have had, which has also made us take some decisions around releasing people and announcing the VRS that we did in Q3. So, that's one part of the business. Last time when I spoke, I talked about the need to really build the new adult vaccination space. As many of you may be aware, globally for GSK, we just announced our results for the past year. And the number one brand, the number one asset for GSK globally is Shingrix, one of the GlaxoSmithKline Pharmaceuticals Limited
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most innovative vaccines and that's something that we launched last year in the month of April /May. And the idea is to really create an adult vaccination ecosystem.

As you would imagine, this is really an endurance sport in the sense of the kind of unlock that can happen, given the fact that we have more than 60 million Indians in our population who can potentially benefit. But even if you were to screen the number of potential target addressable segments, we are talking of almost 10 to 12 million Indians who can benefit with this extremely innovative vaccine.

Just to give you a quick sense of where we are with the progress on creating this adult immunization space with the prevention of shingles, with this innovative vaccine, we are working very hard with the HCP segments, essentially creating a whole vaccine ecosystem, which currently doesn't exist for adults especially. So, whether it is connecting with individual HCPs or creating ecosystems within hospitals or with partners, really having that adult vaccination space built is a major task.

The other one is, as you must have seen, we are also continuing to invest in educating our consumers around driving awareness around the pain and the debilitating pain that is caused by shingles. So, I think, from our zero base, we almost have an awareness of 22-23% where consumers are now aware about

what shingles is and what the resultant pain.

But most importantly, I think we have been building partnerships with significant healthcare players to develop this category. As you would imagine, the future portfolio of GSK globally is also aligned in terms of the next adult vaccines, which we have in the global pipeline. So, this is where the next unlock is in terms of really creating this adult vaccination category. Can we move Ambuja, to the next slide ...

A quick snapshot of what that performance was, so before we come to the financials, something that I want to restate is the Q3 was externally a very competitive and favorable quarter for us. Although the numbers don't reflect that, I think when you look at the comparative figures from IQVIA, which is the syndicated research agency, you look at our evolution index both on volumes, we are significantly ahead of the market for all our key assets that you can see on the right side of the chart.

So, we maintain our leadership. Not just that we actually improved our market share for all our key brands across the category. It's just that the base effect from Q3 of last year, which was 2022, is something that has not allowed us to deliver the kind of growth that we were expecting. So, really, there's a base effect playing there. But more importantly, the General Medicines portfolio as well as the Pediatric Vaccines portfolio, as you will see in the financial slides, we have actually got a 10% growth on the vaccines business for Q3 led by most of the assets. Of course, Shingrix continues to grow and do well month after month. But our base business of Pediatric Vaccines led by some of the assets that you see on that chart have helped us consolidate
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and defend our market leadership is almost 24% market share in the private self-paid Pediatric Vaccine market.

So, those are some of the top line highlights for us for Q3 and I will allow Juby to spend some time on the financials before we open up the session for questions.

Juby Chandy: So, I would like to take two slides and provide some context to the financials we reported. So, we reported 1% growth, 805 crores for the quarter. And if you see that 1% growth, earlier this year, if you remember, we have updated you that there is a huge NLEM impact on our top line, which is 8% top line impact. And as we speak, we have mitigated almost 90% through price increases from WPI, efficiencies and our volume strategy.

All our key brands are growing with top brands like Ceftum, T-Bact, and Calpol growing 7% on volume and that's helping to offset that massive 8% NLEM impact. Vaccines have returned to growth almost 12% growth backed by Shingrix as well as other base vaccines. Now 1% quarter

growth it's on a flat represented external market, our volume growth is helping us to offset all this NLEM impact.

That's on the top line. That's the flavor of the top line. And if you see the EBITDA margins, it's 27% and all these NLEM would have a big impact on the EBITDA margins. As we speak, we have mitigated almost all the impact on that EBITDA. We are talking about 150 basis point year-on-year gap and we expect that also will be mitigated in the coming quarter.

So, our initiatives on SG&A, our margin improvement initiatives, cost savings, the VRS you have seen which is going to help in the coming quarters as well as other restructuring has helped us to mitigate all this impact and as we speak, we are having a very healthy cash flow and working capital improvement we have seen in the quarter.

Profit after tax 45 crore and if you see that profit after tax, there is a one-off e

xceptional item of

VRS of 163 crore reported during the quarter. This is on account of the people who have left the company on VRS initiatives, which we have rolled out in the quarter.

So, this is the year-to-date results. 3% growth. Very similar trends. You can see the 8% NLEM impact, which has been offset by the volume increase on all our top brands. Vaccines, again, is growing 7% led by the base vaccines and Shingrix. All our top brands are maintaining market share and leadership in the market.

EBITDA margins flat despite the NLEM impact. We have mitigated almost all the impact from the NLEM for the year-to-date. 2% growth versus last year on the margin improvements mainly from the efficiencies and all the new investments we are reallocating from the old investments.

So, continued investment in the new assets will be reallocated from the existing resources.

We are very careful on our EBITDA margins. We are working very hard to maintain the EBITDA margins as well as improve driving efficiency. So, our cash flow and working capital
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remains very healthy. For year-to-date also the same one-off VRS impact is flowing through in our year-to-date.

So, that's the broad summary of our financials. I covered the top line EBITDA impact as well as the one-off. So, over to you, Bhushan.

Bhushan Akshikar: So, again, this is not a forward-looking guidance, but as I said in the last two investor meetings, for a company that will be celebrating 100 years of operations in India, 1924 was the year when we set up GSK's presence and operations in India, so 2024 is a year of our Centenary celebrations of 100 years of operations in India.

So, when I look at the next 3 to 5 years for our organization, the ambition is clearly to deliver double digit growth and that doesn't change in the short to medium term. That's something that we have remained cued to and the idea is some of our brands really touch the lives of millions. So, some of our brands which really play a big role in the primary care setting, so the idea is to continue to grow double digits and more importantly touch the lives of a billion people in India, billion Indians over the next 3 to 5 years.

And how are we going to do that? As I said, Shingrix continues to be one of the most important platforms of growth for us. So, that will become the frame of reference in terms of the innovative assets that we will continue to launch in India. But most importantly, continue to build and defend our competitive performance profitably in the core business which is both Gen Meds and our base vaccine business. And of course, all else have the right people who will continue to help us do this and deliver value for our patients, healthcare practitioners and of course shareholders in this country.

So, that's the quick summary of what we wanted to share this afternoon. So, I think we will open it up for questions.

Moderator: Yes. Thank you, Bhushan, and thank you, Juby. So, everyone on the call, we are now opening up for questions and the format is that you will need to raise your hand in case you need to ask a question. And the operator will then allow you to speak and ask your question. You can also feel free to put your video on. If not, just your audio is fine. For those of you who don't want to ask a question but would like to make a comment, kindly do so in the chat section, which is also available. A request, kindly keep your question brief. So, we have a question coming from Anupam Agarwal. How many people did we lay off in our current MR count? Okay. So, that's the first question. Maybe we can wait for somebody to ask a questio

n online first and then go to

that. Anyone wants to ask a question online? Yes. So, we have Mr. Ravi Purohit. Hi, Ravi. Team, can you kindly unmute Ravi. Go ahead. Can you unmute Ravi?

Ravi Purohit: Thanks a lot for doing this Analysts' Meet. So, Mr. Bhushan, I have a couple of questions. I think in your annual report, the first message as the MD of this company, you have mentioned about this double-digit sales growth target. You have again reiterated that today in the presentation. If

you look at our company's history over the last 10 years, there has not been a single year when
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we have experienced double -digit sales growth. So, you know, it's great to hear this, right? And we are already like shareholders of our company.

But if you could just elaborate a little bit, what is the path that will result in us kind of getting this double -digit sales growth? What are the things, what are the steps that we are doing? And we already had brands, but we had these brands earlier as well, right? So, what is changing now? And what are the changes that you are bringing in? Is it going to be new product launches? Is it going to be existing products? Is it going to be existing products reaching a wider customer base? So, if you could add some more color to this ambition of growing double digit, right? Because Glaxo has not done that in a long, long time when practically all investors have forgotten that Glaxo could actually grow, right? So, it would be great if you could change that perspective and give us some insight into that.

And my second question is on the vaccines that I think you have mentioned a lot in our presentation as well as in our annual report about vaccines. But vaccines essentially is a traded good for us, right? We import from Europe. How does a faster growing vaccines portfolio affect our margins in the long term? Because we also mentioned about maintaining or improving our margins. So, can the two coexist? Those are my two questions. Thank you.

Bhushan Akshikar: Thank you very much, Mr. Purohit. As always, I love those questions, both of them. The first question, yes, I think that it's not only a statement of intent. And I think when you look at the three building blocks as to why we believe we can deliver this double -digit growth as a team, it's clearly hinging around two or three things.

First and foremost, I think if you look at the choices, the strategic choices that we made about the therapy areas where we will invest and continue to remain relevant, those have been clear than ever before. And the reason I say that, if you look at the last two or three quarters and you compare the data with externally reported data points from syndicated research, all the brands that I just talked of, they have more than double -digit volume growth.

So, as you saw in Juby's financial deck, in spite of a significant NLEM impact that we had, and this was probably the most significant one, because if you see what got covered in the last NLEM were two assets which were hitherto never covered, namely, i.e. Ceftum and T-Bact.

Now, moving forward, if you look at the next five years, we have a range of brands now with a certain portfolio which is covered under price control. We don't see these kind of seismic events happening every two years, every four years. So, I think the impact has now been mitigated over the next four years when we see, we are pretty much in a range.

So, I think what we have done now, unlocking value with the omni -channel allows us to help unlock some value on the SG&A, especially the wage bill. So, that partly answers your second ques

tion about how are we confident of remaining focused on delivering that margin. And that's one of the reasons why we are trying to do more through the digital transformation route.

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But your first question about the growth strategy and the intent, I think first is based in this country, we cannot wish away the importance of the enduring nature of our general medicines and our vaccines portfolio. Yes, it's a traded business in terms of the margins, but when you look at the kind of lives and the volumes that we can impact, I think there's a significant untapped opportunity for us. So, those two coupled with the fact that you now have a slew of innovative assets kicking off with Shingrix, we are looking at ongoing trials happening for our next group of assets from the innovation portfolio which will help us unlock that value.

So, the answer is clearly doing a lot more competitively in our base business and we have seen that in the last two or three quarters. And more importantly, helping us drive this completely new space in the form of adult immunization with a new asset, innovative asset like Shingrix.

So, those would really help us deliver this double -digit growth. I don't know if Juby, if you want to add something on the second point of margins.

Juby Chandy: So, I think the only other point I want to add on the second point of margin is vaccine is almost 18% of our total business of our entity. And vaccine growing doesn't dilute the total margin because it is still very small portion of the business, and we are growing other business, which is general medicines also in line with or above vaccines growth. So, our intention is not to have a lower growth in general medicines. General medicines also will grow equally at the same pace of vaccines and the ratio might slightly change, but overall it's not going to be diluted.

And again, on the margin front, all these initiatives what we are doing, we are trying to improve the efficiencies inside the organization, whether it is the medical rep contribution per medical

rep or the efficiencies per medical rep as well as spend, the marketing mix, everything we are looking into. So, as long as we grow our General Medicines with a profitable growth pattern, right investments, the margins will not be getting diluted.

And if you are tracking GSK for the last 5 years or 6 years, we have gone from 18%, 19% EBITDA margins to 20%, 26% margin. And despite such a massive NLEM, we are holding to that even this quarter, right? So, margin improvement remains a core priority for us. We are looking into that very closely and right allocation, improvement of the efficiencies of the field force, as well as growing both the pillars of the business right sized is not going to be diluting us.

Moderator: Thank you, Juby. We will move on to the next question. Anybody wants to ask a question, kindly raise your hand?

Juby Chandy: Mr. Agarwal had asked that question around how many people did we let go of?

Bhushan Akshikar: So, I can quantify that in a range. We let go of about 12%, 13%. Remember, we are also reinvesting. So, as I said, Trelegy and Nucala are two brands which really reflect the future in the respiratory business. If you see the kind of growth that we are seeing in single inhalation triple therapy, which is the SITT category, is the fastest growing category. Almost on a monthly basis that category is adding 10 crores.

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So, clearly, that's an area where we want to continue investing because globally if you see, Trelegy is a £2 billion asset for us.

It's one of the biggest brands, some of the top three brands of GSK globally. And given that innovation, we have now resourced that team even more. So, we have redeployed some of our resources so that we can do much more in these innovative assets. As I said, some of the newer assets that we have are also in the respiratory area in the coming years. So, it's important that we remain committed, and we continue to have that significant foothold and presence in the respiratory area.

Last but not the least is, if you have seen our global results, we have launched an extremely innovative asset called Arexvy (RSV) in some parts of the world, especially in the U.S. last year and it's one of the most innovative and the fastest billion-pound asset for us globally. I think in a matter of a few months, it's clocked a billion pounds in the U.S. and that again plays out in the area of respiratory. So, clearly, we want to build that beachhead and that's the reason why we redeployed. But Mr. Agarwal, to answer your question very specifically, it's about 12% of our total headcount in sales which took up the VRS.

Moderator: Do we have any other questions coming in?

Operator: Hello, Mr. Viraj. You can proceed with your question.

Viraj: Sir, I would like to have your opinion on the RSV vaccine and the parent is launching a lot of new drugs also.

Bhushan Akshikar: Sorry, I didn't get the question.

Moderator: The ending was not very clear, Mr. Viraj.

Juby Chandy: He is saying RSV vaccine, and the parent is launching a lot of new products.

Bhushan Akshikar: So, as you know, our regulatory requirements require clinical trials. So, obviously, as a global corporation, we have almost 18 clinical trials happening for our global assets in India. We are pursuing each of these opportunities one by one. So, the intent is to crunch the timelines and accelerate our regulatory pathways in India so that we can have faster launches. But the only difference is in spite of many of our products being available globally, we still have to do clinical trials given the nature of the regulatory requirements. So, the idea is to get access to all those innovative assets even in India.

Viraj: Sir, which one would be faster we launch which are in the phase of trials especially for Indian market?

Bhushan Akshikar: So, in terms of an access strategy, we still have the oncology portfolio. We still have RSV. We have some new assets in the area of hepatology. So, those are a few ones where we clearly have, India is on the global clinical map. So, the idea is to conduct those clinical trials in India and of course use the data for registration. So, with that intent, spread over the next two-and-a-half,

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three years, we see many of these innovative assets also being included on the Indian access strategy.

Viraj: And sir would you like to give guidance for next year?

Bhushan Akshikar: I am sorry. I would love to have answered that question, but I wouldn't. I think we will have this similar meeting after our Q4 results.

Moderator: So, any more questions, kindly raise your hand and the administrator will permit you to ask the

question. Please go ahead.

Operator: Hello, Mr. Nikhil.

Nikhil: Continuing with the question on double -digit growth, what I wanted to understand is if we look at, if we divide our portfolio, there are three parts. One is the new businesses or the new products which we will be launching , and in many of these products we have to do the category creation work , like in adult vaccines or in the respiratory, the new products which we have launched. So, maybe they can grow it on a

small base faster, but a large part of the growth has to come from the existing top 20 focus brands .

And from our presentation itself , over last three years, what we have seen is that these brands on a volume term have grown at 6% to 8% range. So, when we talk about growing double digit, where do you see a large potential? Like, something has to grow at 15%, 20% which is meaningful. So, is it like vaccines you believe can grow at 15%, 20%? Or do you believe the top 20 products where volume growth has been around 7%, 8%, we can accelerate the growth there? And in order to do that, how does this omni -channel strategy work there?

Bhushan Akshikar: Absolutely, it's a brilliant question. Thank you, Mr. Nikhil. So, even if you were to look at the last three quarters right from Q1 to Q3, barring the aberration that we saw in Q3 because of the base effect , but if you look at the sheer volumes that we have driven for many of the assets that you talked of, both in general medicines and in our base vaccine business, it's been in double digits.

So, if you see a volume growth even for a brand like Augmentin , it's been in double digits for the first three quarters. If you look at a brand like Ceftum, which was impacted on the pricing front by almost , you know , 40%, 50%, our volume growth is in excess of 90%.

So, the whole idea is to ensure that the omni -channel strategy allows us to unlock value and I think a large chunk of our efforts are to ensure that healthcare practitioners who are not, you know, they are equally connected and affected in the form of what's happening in the digital transformation world , how can GSK be at the forefront in really integrating all those touch points that we have ? Because we have the science. We have the connections . We have the foundation of good science. How do you ensure that a large part of this is further able to fuel, you know , the kind of work that we are doing ?

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To give an example, we are doing some work along with healthcare organizations in creating the first ever India Infection Index , which kind of tells every healthcare practitioner in his or her zip code or pin code , what is the prevalence of particular diseases. More importantly , which antibiotics are susceptible , so that he or she can make the right choice. And this is all tech led, and that's exactly what we are delivering to the healthcare practitioners at n is equal to 1. And that's what is helping us drive the volume.

So, to answer going back to your question , this double -digit ambition is coming from primarily unlocking value through some scientific activities that we have thought through , but more importantly , leading it with a strong volume emphasis. So, that's something that is clearly articulated over the next three years.

Nikhil: Just two more questions. One is on vaccines. Just while our presentation says that the underlying growth has improved , but they only reported sales to sales. Year -on-year, are we flat for the nine months or is there still a de-growth?

Bhushan Akshikar: So, vaccines, for the nine months, it's a 9% growth is the reported growth. Also got one vaccine which is perhaps discontinued Infanrix plain vaccine, not the Infanrix -hexa vaccine. There are two Infanrix. One is plain and Hexa . Hexa is one of the biggest one which is still there. There is a plain vaccine which has been phased out. So, excluding that, it is 12%. And otherwise , reported growth is 9%.

Nikhil: Last question. See, I think for last three years , what we have seen is that, at the, so I think if we look at our first presentation , we had mentioned two goals . One was margin improvem

ent and

other was growth. On the margin side, I think we should congratulate you that we have been able to do a good job that even though NLEM impact and everything was there, margins have sustained. Incrementally, the efforts which we have taken and in our previous presentation, Juby had given a trajectory that the margins can keep going up year-on-year. But in order to grow or achieve that double -digit growth, would you say that are you okay with compensating some of the margins and pushing more on the growth side? How are you thinking about this over the next three years in terms of growth versus improving margin or sustain ? Sustenance is possible but improving margin versus growth?

Juby Chandy: So, like on margin side, as you rightly said, we are tracking fine particularly on year-to-date, and a couple of our initiatives are going to play out more also in the coming quarters . Like the VRS

which we instituted in the last month of the quarter, December, it will further improve a little bit margins in the coming quarter as well.

Now the way we are seeing it is, if you see the last slide or the slide which is on the screen, we want to drive growth with profitable growth. It is not either way. We want to grow as well as we would want to maintain the profitability. So, it is not either way. So, wherever there are opportunities, we are prioritizing that. We will reallocate our investments. We have many assets to reallocate investments, and that's how we are doing that. So, we have mechanism to closely track the return on investments on our assets which we are doing on a regular basis, and

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whenever there is a need to invest in new assets like RSV, like Shingrix, like any other assets coming, we will try to prioritize where it matters in terms of growth and margin. So, yes, it's a bit of a balance we are doing. It's not diluting the margins and driving the growth. It's balancing out properly here.

Gopal: Yes, thank you so much. So, just two questions. One is on the Ceftum and T-Bact. If you could just explain us the impact that you have taken if the NLEM price cut the sales to 100, what was the impact on the prices? And what has the corresponding offset by volumes? And where would the 100 stand today?

Juby Chandy: I think Ceftum, to be precise, had a 57% drop on the price. So, it is one of the biggest NLEM-impacted product after a long time. So, its prices used to be higher compared with the Generics and it is one of the biggest impacted product. T-Bact has got two SKUs, but on an average, it is roughly 30% price cut. Ceftum volumes have almost doubled, close to 85%, 90%. T-Bact, I think that decrease in price is not to that extent to double the volume, but T-Bact also has got close to 10% volume growth in the quarter.

Now, I think one thing we need to be clear on this quarter is last year same time, Bhushan has touched upon that, last year same quarter had an infection outbreak. So, external market has been flat or negative 1% growth in the previous slide, if you have observed that. So, this growth on this quarter is from a very large skewed higher base last year quarter.

So, growth has been slightly softened in the reported basis because of the last year base is quite higher because of the infection. And remember most of our products, bigger products are on antibiotics and when infection spikes in one year and next year if the infection normalizes, we will see that base impact playing out. So, that is also playing out there.

But to answer your question, both Ceftum and T-Bact had a massive price decrease on NLEM. Partially, our majority has been offset by the

volume increases. That is why we are not seeing the value decrease in a negative territory despite a flat quarter, despite such a huge NLEM impact we had taken.

Gopal: So, with the value proposition you expect like in FY '25 also that we will be seeing strong volume growth in these two brands given price is much attractive for the customer?

Juby Chandy: So, the base has got corrected in the last two quarters. It might still continue to grow but not like 80% growth which we have seen in the last two quarters quarter-on-quarter, but the base has corrected a lot. It is still we are seeing volume growth on Ceftum and T-Bact both. So, the trajectory will continue for one or two more quarters. Then the base will get corrected completely. And that is one area.

Also, if you have seen in the Slide 1, we were trying to reallocate a field force into tier 2, tier 3.

So, where we have no coverage or where we have no productivity, we are working on that to expand our coverage not only through field force, but also through omni-channel. So, omni-

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channel also will play a role to expand our coverage and reach to get the best of these medicines to the patients who need it.

Gopal: My second question was, would you give a broader break-up of the business you used to give like general medicines and vaccines, and within general medicines you have promoted the other products for the 9 months?

Juby Chandy: So, we normally don't share the breakup by product in any of our previous things. So, I will still try to maintain that format, but 80% of our products are promoted.

Gopal: And lastly, are there any non-core assets in the business which you would capitalize for in the next or monetize in the forthcoming period?

Juby Chandy: So, we have some assets, tail assets which are not promoted, but it is having no different treatment than any other assets, so which we will only do omni-channel approach, nothing else.

It is not held for sale or nothing of that kind of classification. It is a normal asset. It is just that it is not promoted with field force in the ground. So, we are just using omni-channel and doing it.

As of now, nothing is earmarked for sale or nothing of that sort. It is business as usual for us.

Moderator: So, we have a question on the size of the pool for Nucala and Trelegy for you. The size of the pool.

Bhushan Akshikar: Yes, I saw that. So, again remember something Nucala and Trelegy do not operate in the same sphere. One is promoted for severe, so Nucala is promoted and indicated for severe eosinophilic asthma, whereas Trelegy is indicated for chronic obstructive pulmonary disease or COPD. So, they do not necessarily operate in the same space. That's point number one.

But more importantly, both the therapies especially Nucala being a monoclonal antibody, it's one of the most remarkable assets for treatment of severe asthma. And if you see how patients progress, if they are not controlled, you know, from oral corticosteroids to inhalers to then moving on to monoclonal antibodies, there is a certain customer journey or patient journey that a patient undertakes. So, it would be improper or incorrect to take a look at the entire oral corticosteroid market or inhaler market and say that this is the target potential market for us. But there are patients who remain uncontrolled. So, as we speak, every month we are adding 100, 150 new patients. Remember this is an expensive treatment. This is not a reimbursement category. So, every year we are adding about 1,000 patients who stabilize and get treated with Nucala.

So, I wouldn't hazard a guess in terms of the exact numbers, but clearly

, asthma being a large

segment for us, the patients who remain uncontrolled and therefore progress to requiring the higher treatments like Nucala is only increasing. Given the air quality index, given the pollution levels, we are certainly seeing increasing patterns for the usage and number of more patients coming on for treatment with innovative treatments like Nucala.

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Trelegy is, again, a similar example. Although it is indicated for COPD, the patient pool remains the same in terms of patients who first start off with oral corticosteroids. When they remain uncontrolled, they move to inhalers, and they use multiple inhalers. In Trelegy or the class of treatment that Trelegy belongs to, which is called Single Inhalation Triple Therapy, SITT, this oblates the need to use multiple inhalers or multiple oral and inhaler treatment, combination of both. So, this is where the maximum growth has been seen in the respiratory.

So, again, to answer your question, it is difficult to quantify and give you exact number because both operate in different segments. But increasingly over the next 3 years, this is where most of the growth will be in terms of therapy areas.

Operator: Mr. Agarwal, can you hear us?

Agarwal: Sir, can you give a timeline of your trials that you mentioned about your assets in the Indian market, your tentative timelines for trials and launches?

Bhushan Akshikar: Again, remember this is the regulatory trials. I can't really put it because there are steps, there are Subject Expert Committee meetings that happen, which therefore give us the green flag in terms of the next step. But I think spread over the next 18 months, we have trials ongoing for, as I said, for RSV, for our assets in hepatology. We also have, of course, both the trials on our oncology assets are already done. But these are the three broad areas where we currently have trials which have visibility. Of course, most of the data is available on the public domain. So, if you look at the regulatory databases, you will find most of the data published there.

Agarwal: So, next two years if I have to see your launches, how many products are we looking to launch?

Bhushan Akshikar: As I said, I won't hazard a guess and give you the exact number. But clearly, the whole idea is, when you see that first point on that slide, the idea is we have already launched Shingrix. How do we ensure that most of our innovative assets are available in this country on the back of what we have done with Shingrix?

Agarwal: Right. A second question on Shingrix. It's been about 10, 11 months now to launch. How has been the traction month-on-month? And what are we clocking right now?

Bhushan Akshikar: So, just a minor correction. It's about, so we launched it in the month of May. May was the first month available to us for making Shingrix available in the market. And obviously, as I said, it's an endurance sport in a country where the vaccine ecosystem is still being developed by companies like ours. Month after month, we have seen a progression. So, again, when you look at the number of people who have taken both the jabs, probably, I wouldn't hazard a guess, but it would be in the range of, Juby, 30,000?

Juby Chandy: Yes.

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Bhushan Akshikar: 30,000 patients who have taken both the jabs. But month after month we are seeing progression. That's how I would answer it.

Agarwal: On revenue basis how much are we doing, how much have we done in January or December?

Bhushan Akshikar: So, as I said, every month we have an improvement

because remember, assets like Shingrix don't follow a linear curve. They follow typically an exponential curve and you could see that everywhere where we have launched it. So, if you are selling 2,000, 2,000 becomes 4,000 doses. 4,000 becomes 6,000 doses. So, now with every passing month we are talking about 7,000 to 8,000 doses being sold every month. So, that's how I would put it.

Agarwal: And as a thought process, when you had initially launched, are we on track in terms of delivery? Or are we...?

Bhushan Akshikar: So, as I said, when we do the annual meeting post our Q4, we will definitely share the full year numbers because that's exactly the time frame that we would have the annual sales. But I think, given a new category launch, given the fact that we are creating a new category with this launch, we are pretty much stacking in line with what we had set out. But as I said earlier, the ambition is huge because we have almost 12 million Indians who have the potential disposable income, who have the wherewithal in terms of access. So, every month, every quarter is only an upside as we look forward.

Agarwal: And just a data point. So, we have about 3,000 MRs right now?

Bhushan Akshikar: Across the company, no, I think that's exactly why I said we reduce our headcount by about 12%, 12 to 14%.

Agarwal: So, about 2500.

Bhushan Akshikar: Juby?

Juby Chandy: Yes, somewhere close to that.

Agarwal: And with the new portfolio coming in, in terms of oncology and respiratory and other assets, how much addition are we looking at?

Bhushan Akshikar: I mean, these are all super specialty portfolios, right? I mean, given the fact that a country of our size doesn't even have more than 500 medical oncologists. These are all lean teams, extremely efficient in terms of the specialty -selling model itself. So, I don't expect these teams to be large. They would still be under 40 to 50 headcount.

Agarwal: Just lastly, on your productivity per man, how much are we doing right now? And what is the headroom left for us to grow in terms of productivity?

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Juby Chandy: So, let me take it. So, firstly, before answering that question, one clarification on the pipeline.

We are doing clinical trials, but not all the products may not come to the market because at the end of the day, if the market, once we do the clinical trial, if it doesn't make sense for the market to get it into the market, either because of the profitable launch or if there is not enough patients to take that or there is no potential in this country to take that because it is different from developed countries versus some of the assets profile in the country. So, depending on all those things, the management and the Board will take that decision. That's how it happens.

Now, on your productivity, it used to be 8.5 lakhs per medical rep. It would go close to 20%, 25% up once we, you know, that is the kind of upside we are trying to see as we implement all this restructuring, VRS reallocation, new assets kicks in. That's the kind of peak productivity uptick we could see eventually over the period of 2, 3 years.

Agarwal: Lastly, have we broken even on Shingrix on EBITDA level?

Juby Chandy: Yes, it is a traded product. So, we don't make a loss at any time.

Bhushan Akshikar: Thank you very much, Mr. Agarwal. So, probably, I will take a couple of questions here. So, Julie has asked a question. Can you throw some light on the vaccine segment? We face significant challenges in terms of the volume and slow down due to NIP. How are you working on mitigating such business risks and volatility? What is the target revenue

mix between genmed and vaccine in the coming times?

So, just to give you a quick flavor, we still have about 80% of our business coming from our General Medicines portfolio, whereas 20% is from vaccines. So, to that extent, we remain, you know, we can absorb those shocks given what we saw in volatility. But that doesn't take away the fact that we worked extremely hard as a team to come back to this double-digit growth on the vaccines front. We have revamped the whole strategy. So, yes, NIP did impact us for those few quarters that you saw the slowdown.

But if you look at each of our assets in the Pediatric Vaccine portfolio, they have a strong double-digit growth. More importantly, we have gained share. And that's largely because of what we have done, not just in the first year, but also we have just launched a major consumer awareness campaign for immunization in the second year, which is the secondary vaccination.

So, we are opening up new segments where, you know, to mitigate some of the potential losses that we may have had in the past, given the NIP, we are seeing the comeback with the secondary

vaccination campaigns that we have just launched. So, that's how we remain committed to ensuring that we defend and protect every single baby who can be vaccinated and from preventable diseases. So, that's the general sense.

I think I already answered Mr. Mohamud Patel's question around contribution of GenMed and vaccines. Mr. Samir Palod has asked me, can you provide us any color on the size of the potential market in India for some of the new drug launches?

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So, as I said, as Juby talked about, so we have trials ongoing, as I said. Again, we have not reached a stage where I can quickly tell you. But, for example, one of the global assets is a molecule called Bepirovirsen, which is a potential cure for chronic Hep B.

Now, you can imagine in a country of our size, that can be a complete game changer. Now, obviously, we have not reached a stage where we have got all the green flags to say that we are good to go, but potentially those are the kind of assets that we are looking at. I wouldn't probably give you a size in terms of the market as of now.

For the oncology assets, the approvals that we have are limited. They are largely in ovarian and endometrial cancer. But again, you know, meaningful impact of what these assets can do. So, globally, we have three assets. We have Zejula, Jemperli and Blenrep, three assets in the oncology area. So, we are seeing how and what potential these assets may have in India.

Yes, Mr. Samir, I did talk of a functional cure for chronic Hep B.

Moderator: Any more questions?

Bhushan Akshikar: There is a question from Mr. Vishal Manchanda. Has Synflorix returned to growth? Or is it still declining? Well, it declined significantly for the first couple of quarters. The third quarter we have stabilized that decline. So, as I said, we have now opened up new segments in terms of secondary vaccination. So, clearly, we have arrested that decline, and as we close out the annual performance, we will definitely share with you the individual antigen results.

Juby Chandy: Now, I think Synflorix has declined quite a lot. It is now a very small portion of the total portfolio. So, it is still on declining territory, but it is not material at this point in time.

Moderator: Any more questions, Abbas? Anyone else who has raised their hand?

Operator: Yes. Hello, Mr. Kimani, can you hear us?

Kimani: Yes. Good evening, sir. Thank you for guidance, and I think great to see that Glaxo is getting into growth mode. Just a couple of

questions. If you could share that as you grow in double digit,

could it be also significantly margin accretive like Glaxo at one point in time was in the 31%, 32% kind of profit margin range? So, do you see as you grow significantly higher on volume, that trajectory also changing in couple of years' time? Number one.

And secondly, on the General Medicine side, you have spoken about, you know, so what are the other, I mean, is this a part of the general strategy to have more and more product coming through the Indian market from your global portfolio? Is it going to continue for in the times to come and what you give some more color on that?

Bhushan Akshikar: Should I take that, Juby or you want to take?

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Juby Chandy: No, I can take it. I think you are talking about the 30% margin many years back because after that two or three rounds of NLEM came, and as we speak our portfolio, around 40% of our portfolio is pharma portfolio is on NLEM. So, which the price increase plays a big role in the margin, improvement in the margin. So, that is pretty much dependent on the WPA increase as you guys know.

The remaining 60% of the pharma portfolio, what we normally do is wherever there is an opportunity to price it rightly, optimize the pricing, we review it every year and take the right pricing to manage it. So, I don't want to comment on a forward-looking margin, but we are closely observing what's happening on the NLEM, where is the play for us to dial up the price. If there is an opportunity, we always take it. So, we have a system to do that every year and we are doing that.

So, that is one of the key aspects which we are doing in the last many years. We had 34% margin, but then we moved to around 16%, 17% margin. Now we come back to 26% or 27% margin, and that's where we are tracking, but wherever there is an opportunity to play on price, bring new assets at a higher price, we are doing that. That's one of the important levers to play on the margin.

Bhushan Akshikar: There were two parts to the question.

Juby Chandy: What was the second question, Vikas? Can you repeat?

Vikas: That I was asking what are the other major on the general medicine side from a global portfolio

to bring to Indian markets and generally outlook for...

Bhushan Akshikar: So, Mr. Vikas, you know, one of the things that I keep saying is that there is a lot of innovation even in our General Medicines portfolio. So, for example, if you look at Augmentin as a classic case, we launched Augmentin ES which is a new formulation in the liquid side which has quickly notched up almost 10% of the overall revenue. I mean, if you think about a standing start and creating a 50 crore SKU on an annual basis, it is still substantial given the high margins that we have for the GenMeds portfolio.

Another example is Calpol . For a category which is as commoditized as paracetamol , we not only continue to be remaining the number one brand in the pharmaceutical industry in prescriptions. So, Calpol is the number one prescribed brand in India across all categories. We have gone ahead, innovated and launched new versions with new technology, which allows faster dissolution . Both for Calpol 650 Plus as well as Calpol 500 Plus and launched those new versions. So, I think those kind of innovations will continue , and that's why I said the GenMed portfolio has enough and more to do. You will see a lot more line extensions and investments in some of those key brands in the coming 18, 24 months.

Vikas: There is one more add-on that when you talk about double -digit growth, this you talk about more in value terms o

r volume terms?

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Bhushan Akshikar: I am talking value, clearly. That's why, as I said, the NLEM was an aberration for us and as many of you are aware, NLEM doesn't happen annually, thankfully. So, at least for the next four years we can clearly see. And now that we were most impacted, if you look at all the three versions of the NLEM, this was probably the worst. And I think somebody has asked, have you mitigated? We have mitigated almost 90% of the NLEM impact in spite of what happened in terms of the sheer size of the impact , and we don't see that happening every year now. So, that's why I said over the next three to five years, the intent will remain to grow by double digit in value, not volume, volume alone.

Mr. Purohit had asked one more question. Are you getting back into oncology? So, you had got out of oncology. You are absolutely right. It was a global decision in 2014. We had an asset swap arrangement with Novartis where we traded our onco portfolio with what we got in terms of consumer and some vaccines. Over the last three years, we have been rebuilding our muscles and getting back into oncology globally. So, most of these trials are happening in India as well in terms of registration. So, as I said, there are three assets globally , which are being launched in different parts of the world, Zejula, Jemperli and Blenrep. And obviously, trials are ongoing . We have to also see what's the fit between the types of cancers that we have here, the data, and of course the price point . But the answer is yes, oncology remains an area where we are getting back into.

Operator: Hello, Mr. Vishal . Can you ask?

Moderator: I think Mr. Vishal has posted a question.

Bhushan Akshikar: What is the volume growth you have seen in Nucala during the year?

Juby Chandy: Only Nucala is on a low base. Nucala has doubled the volume, 112% volume growth.

Bhushan Akshikar: I remember in one of the investor meetings somebody had asked me last time, why can't Nucala be a 50 crore brand? And I think we are exactly going in that direction. We should be, you know , that's the ambition is to start creating Centers of Excellence in terms of the specialized brands as well. So, that's why the growth is more than 100% , but on a small base. But as we close out the year and for our Q4, we will probably give you the exact value , but it's we are almost doubling these brands.

Vishal: Sir, this is Vishal. So, sir, on Nucala, a follow up. Can you share the annual cost of treatment ? Just trying to understand the affordability challenge here.

Bhushan Akshikar: So, if you look at severe eosinophilic asthma, which is an extremely debilitating condition, the treatment cost for a patient is roughly around 60,000 rupees ...

Juby Chandy: Per dose.

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Bhushan Akshikar: Yes, per dose. It typically has the cost of treatment for a month and patients go on. So, we have patients who are also on treatment for 12 months . So, typically , HCPs give a break till the patient is remission free and then comes back . So, I can't really hazard a guess and say that you know this is the total cost of treatment for a lifetime because given the fact that most of the patients start responding, they do come back. They get off treatment maybe after 4 cycles, 6 cycles till they have another breakthrough and then they come back.

Vishal: And does it compete with drugs like Lebrikizumab and Omalizumab ?

Bhushan Akshikar: Absolutely, yes. Yes, Mr. Vishal.

Vishal: Is it competitively priced to those assets?

Bhushan Akshikar: Yes, it is. It is competitively

priced.

Vishal: And just a question on the traded portfolio. Would you be able to share some color on the margins there? How different it is from the manufactured products that you do?

Juby Chandy: So, we don't share margins by products. So, I don't want to start that a new practice here.

Vishal: The entire traded basket. So, just as a basket, not by a product level, but at a basket level.

Juby Chandy: We are not sharing product by margins in the past. So, we just maintain like that.

Bhushan Akshikar: Mr. Mohat Patel has asked, we understand that Shingrix would drive adult vaccination growth.

Which key products would drive pediatric vaccination growth going forward? So, there are three or four products where we clearly see a strong double-digit potential and that's where we are putting all our energies. One is Infanrix Hexa, which is the hexavalent vaccine that prevents six preventable diseases. So, that's the mainstay of our pediatric portfolio. Flu continues to be one of the major assets for us. So, we continue to invest in flu as a vaccine. The third one is the Tdap vaccine, which is Boostrix. So, these are three vaccines which are right at the top.

We also are probably ahead because we are one of the companies that has created the category for maternal immunization. When expecting mothers have to be given in the third trimester, this is one of the safest vaccines. So, these are the three or four ones where we clearly see enough growth potential over the next 12 months, 12 to 18 months.

Juby Chandy: Maybe one question more and then we can wrap.

Bhushan Akshikar: Yes, sure. How big can Shingles be from a three to five-year perspective? What's the strategy to drive that growth? That's from Bhavesh Sangvi. Now, clearly, as you can imagine, it's the biggest asset for us globally. It's a £3 billion brand globally out of that. So, it's almost 10% of our top-line revenues as a corporation. In all the countries that we have launched, be it regardless of the reimbursed or the non-reimbursed category, the fact is that it's one of the most innovative

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vaccines can prevent and is efficacious to the extent of almost 94%. So, as I said earlier, assets like Shingrix will not follow a linear curve. So, clearly, given the fact that we are talking of a target addressable market of almost 12 million Indians to begin with, with each year cohorts getting added, which means everybody who is above 50 years can potentially qualify and be eligible, it's a significantly big market. So, clearly, in Shingrix, you have the potential opportunity to see a brand which can potentially be the number one brand, not just within the vaccine category, but even within the basket over the next 3-year period. So, the opportunity is massive.

Moderator: Great. So, with that, we have come to an end. We have taken most of the questions. For those possibly who we have not answered, we will come back to you as much as we can. Maybe a little closing remarks from you.

Bhushan Akshikar: No, I think, thanks for your interest and continued commitment. I think as investors who remain in a company for the last 100 years that has been in this country for the last 100 years, I think we appreciate your time. As I said, the objective of this meeting was because we have just undergone a massive transformation and obviously this VRS is a one-time exceptional item. And therefore, I and Juby thought it's appropriate for us given the journey that we have undertaken from that 18%, 19% margin to stabilizing it in the range of 26%. The idea is to ensure that all your questions and concerns are addressed. More i

mportantly, continue to share the progress,

and we will do this regular feature. As and when we close our fourth quarter and announce our fourth quarter results, we will see you again. Thank you very much for your time. Once again, really appreciate it.

Moderator: Thank you, Bhushan. Thank you, Juby. Thank you everyone for joining. As Bhushan said, we will have a call again at the end of our full-year results. So, stay tuned. We will, of course, upload the presentation and the audio file on our website. As always, if you need anything, please do reach out to me. Thank you very much. Have a great evening.

Bhushan Akshikar: Thank you very much to everyone. Thank you.

Call: May 2024

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THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED

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Bandra -Kurla Complex, Bandra (East)
Mumbai - 400051 21st May 2024

To,
BSE LIMITED
Phiroze Jeejeebhoy
Towers Dalal Street
Mumbai - 400001

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 17th May 2024. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/investors/analyst-meets/>.

Thanking you,

Yours faithfully

For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni

Vice President – Administration, Real Estate

& Company Secretary

CIN: L24239MH

1924PLC001151

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"GSK India

Analyst Meet Call"

May 17, 2024

MANAGEMENT : BHUSHAN AKSHIKAR – MANAGING DIRECTOR –
GLAXO SMITHKLINE PHARMACEUTICALS LIMITED

MR. JUBY CHANDY – CHIEF FINANCIAL OFFICER --
GLAXO SMITHKLINE PHARMACEUTICALS LIMITED

Ransom D'Souza : Thank you for joining , on behalf of GlaxoSmith Kline Pharmaceuticals India, warm welcome. As you know, we've just announced our results and we thought that we could share the results with you in person. And for that, we have our Managing Director , Bhushan Akshikar.

Some of you have met Bhushan or at least interacted with Bhushan in the previous calls. As you know, Bhushan

has experience of over 14 years in key leadership roles in GSK across India, Middle East, Russia, CIS and Africa region. Before joining GSK, Bhushan has spent 15 years with Johnson & Johnson in local and regional positions in India, South Korea and Belgium.

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So Bhushan will speak. We also have our Chief Financial Officer, Juby Chandy. Juby has a successful track record of over 17 years in key leadership roles in GSK across India, Singapore, Southeast Asia, Vietnam, Turkey and the Middle East. Without further ado, I'd like to invite our Managing Director Bhushan on stage to address you. And we will take questions at the end. Preference for those who are in the room. So that's our intent. Thank you Bhushan, over to you.

Bhushan Akshikar: Thank you very much Ransom. So at the outset, thank you very much to everybody who's here in this room. I know there are two rallies going on in Bombay, so really appreciate the investment of your time. And to everybody on the line as well, thanks a lot for joining us. We've just announced our results for Q4 and the full year, 23-24. I don't intend to spend much time taking you through PowerPoint deck, but I'll just spend between me and Juby, we'll just spend about 15 minutes max, 10-15 minutes to contextualise and frame the results, and then open it up for Q&A, which is I think the more important part.

So for those who are here, those who have already been to what we call, we have just unveiled with our board, what we call as the Heritage Gallery. We are in our 100th year of our operations. In the next few months, we will complete a century of our operations in India. And if you look at the results that we've announced today for the full financial year of INR3,400 crores, broadly diversified between General Medicines, which is the primary care business, the vaccines business, and the specialty business that we're rebuilding.

And if you were to characterize each of the businesses around the anchors that really help us take the business forward, it really is about the focus. Over the last three, four years, we've really optimised our portfolio. We put energy into identifying where is it that we need to put all the resources and doing that in the most innovative manners, which means doing much more with digital acceleration, doing much more with the right medical support. And last but not the least is really be agile in the form of building those new platforms of growth. So those are the three real variables that underpin our operations on the commercial side.

Jumping straight into our business performance, I'll just frame the external reflections as reported by the IQVIA. If you recall, I and my CFO, Juby, we met up with all of you when we announced our results for Q3 and we had said that we'll come back to you on the progress that we're making at the end of Q4 as well as the financial year.

And one of the biggest things that we did at the end of Q3 was transform our general medicines portfolio and our operating model, doing a lot more with digital acceleration. So I just want to give you a sense of how that has translated in the quarter that has gone by. So if you look at our evolution index, clearly two percentage points ahead of the market, so which means we've done well compared to the represented market and I'm talking only volumes. So obviously the closest unit that represents the health

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of the business is the prescriptions, and prescriptions are the closest to the volume. So clearly I want to highlight that as a fact.

Within our represented market for general medicines, which is almost 80% of our total business, we have actually gained 0.6% volume share, which is another data point which I want to share.

If you recall a year ago, when we announced the results, we were greatly impacted with the NLEM and I think the two assets that we talked about, which is the most impacted were Ceftum and T-Bact, which were hitherto not included in the essential medicines list and just to report, when we closed the quarter and the financial year, we have delivered 56%

growth in volumes for Ceftum. That has really allowed us to mitigate the significant impact. And the volumes on T-Bact have also grown by 10%. You will hear in the financials, when Juby comes and presents how this all stacks up in terms of our top line and bottom line, but I'm giving you those two as illustrative

examples of how we've been able to really come back and mitigate the significant impact. And on the right hand side, you'll see, it's been a good report card in terms of the greens that you see there. So every single brand, we always say that at GSK, we still have seven brands in the top 100 of the Indian pharma market. And if you look at that list, each of those brands has actually gained share. So if you see right from Augmentin to Supacef, every single brand has added market share, as well as in volumes, we've been ahead of the represented market.

The question is really what has helped us do this? For a portfolio that continues to reinvent itself or a company that continues to reinvent its ways of working, but remaining rooted in the purpose for which we exist. I just want to spend probably less than a minute to give you a sense of what we did in the last quarter.

[Video]

Well, again, that was just an illustrative example of showing if we have to keep reinventing ourselves, how do you really bring more value to the millions of patients who depend on a category which is as commoditised as paracetamol. And yet, if you look at the equity that the brand commands, it's a trust mark and that's why we've done. So, this was launched, and clearly if you saw the market share gain, that I've shown in the previous slide, it's attributed to some of the innovative work that we did. The other bit that we did in the general medicines business in November, December, if you recall the announcements where we shared the digital transformation that we've done. So, we've put in a significant investment in the form of a dedicated team, an omnichannel team, which really helps us create a seamless omnichannel experience for all the healthcare practitioners that we cover, as well as the ones that we don't cover on a face-to-face basis.

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What do I mean by that? Clearly, for all the healthcare practitioners who have consented to allow us to reach out to them in the form of different channels, including WhatsApp, emailers, digital, we are doing that in a seamless manner and ensuring that the reach and awareness that we create for our brands is only getting 2x and 2.5x in a short time, and not just depending on the face-to-face transactions in terms of the communication alone. This is really helping us deliver some best-in-class experience. So, in spite of having a significant reduction in the field force in Q3, if you remember, we've still been able to maintain our competitive performance and that's because we are also supplementing our frontline sales teams with a lot of digital work at the backend. We've already enabled nine of the key brands that you see in general medicines to go through a significant omnichannel experience. So, as I said, we opened up and enabled new channels for engagement with healthcare practitioners. Coming to the real turnaround this quarter, it's been a rock-solid performance on the paediatric vaccines portfolio. If you recall the last two quarters, I've been sharing that we are stabilizing and now turning the corner in terms of growth. This quarter really represented a strong delivery by the vaccines team.

So, overall, in the periodic vaccines business, we continue to maintain the leadership in the self-pay market with almost 23.5% market share. But again, the same story, if you've seen strong volume growth, our EIs are ahead of 100, plus the value growth of 19%. All enabled by some significant work that we are doing in the consumer awareness area, reaching out to consumers and par

ents, but also putting in some robust plans with the healthcare organisations, especially the Paediatric Association as well as the Gynaecologist Association.

Right-hand side, the table really shows, again, the competitive performance across the portfolio. We've done well, both in terms of the growths as well as the shares. So, on a quarterly basis, we are back to about selling a million doses of periodic vaccines across this portfolio. And again, we've retained our leadership across the board.

[Video]

So, if you saw in that campaign, clearly it is saying that the birth cohort, which is one year plus. So, apart from defending our leadership position in the zero-to-one year category, we're really ensuring that many of the babies that miss out vaccinations in the one-to-two year, get covered. So, this is being run right now across all social and digital channels.

Coming to the most important launch that we've had in the last 12 months, we launched a vaccine to prevent shingles, herpes zoster. And if you recall what I talked to you in the last two meetings, we have a very clearly defined roadmap in terms of

winning with healthcare practitioners, winning with consumers, because especially after COVID, there's one thing that changed. Adult vaccines definitely are understood
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compared to where we were probably five years ago. But we'll have to continue to do a lot of work to educate consumers and build that disease awareness, especially around shingles.

And the last is creating the adult vaccination ecosystem, be it with healthcare organisations, be it with hospitals, be it with assisted living places and communities, be it with diagnostic labs. So, there's a lot of work happening across all of these three elements and for the last quarter, as well as the first full year that we've had from May till March, we sold almost 55,000 doses, translating to roughly about INR50 crores worth of shingles in the first year of launch.

Again, a very strong program enabled to really create an awareness around shingles in the consumer community. This is just a 30-second video, but really shows a glimpse of the kind of work that's happening at the back end to really bring science and also a sense of ownership around preventing many of these diseases, including shingles.

[Video]

So this is an example to show the kind of work that we are doing is transformational in nature. Obviously, when you're creating a new category, that too in a country where adult immunization is almost embryonic or even sometimes non-existent, there's a lot of heavy weightlifting that organisations like us have to do in terms of bringing the message around prevention. So, we've run two very successful campaigns in the last quarter around both shingles awareness as well as the Immunization Week. So, that's just an example of how we are doing, which is helping us in the outcome that I talked earlier.

Just a slide on how we are doing on the Innovation Index. One of the things that I had talked to all of you, I think, six months ago was for an organisation like ours, how do we set up the Freshness Index will be important in the next two to three years? And this is just an example of showing that three assets were not a part of our portfolio maybe three years ago, which contributed almost nothing, are contributing almost INR100 crores as a group in the last financial year.

So, this is the way forward apart from defending the competitive performance and building on the significant assets that we have. How do we really create the new growth platforms for us is a singular objective for me and the leadership team here. And those three things that you see on the slide, those three assets will continue to be the growth drivers for the organisation in the next two to three years.

So, I think I'm going to now invite Juby, who will just quickly take

us through the

results. We already announced and shared the results with the Stock Exchange, but we'll just spend a few minutes and then we'll open it up for question and answers.

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Juby Chandy: Thank you, Bhushan and good evening, everyone. I'll not take so much time on the financials because you would have seen the results by now, but I would like to connect the progress which Bhushan has articulated on the commercial side to the financials. So, you've seen the headline growth of 16%. This is the quarter results. And what's interesting is the quality of the growth.

Okay, so what do I mean by that is both pharma, as well as vaccines, both delivering double-digit growth. Remember, there is an NLEM impact in the same quarter last year, which is impacting the base of last year. Now, if I restate that base, we are talking about roughly 11% growth, okay, so on the turnover.

EBITDA aside, if you remember when we met you last time, one of the major headwinds we had was the NLEM impact. And that time we flagged to you that, I'll come to that in the full year, when we were talking about the full year, we had a full year impact of roughly 8% on the top line, close to INR200 crores. And we committed that time that we'll be mitigating most of that, perhaps around 60%, 70% of that will be mitigated.

Again, in the quarter, I think quarter two, we came and said we will be mitigating

around 80% of that. Now, as we speak, when we close the year, we have mitigated almost close to 100% of that. So, which you will see in the full year EBITDA margins. So EBITDA margins for the quarter it has improved 670 basis point, this is through sustained margin improvements. So we have margin improvements on several products.

We have the product mix of our promotion which has also changed, as well as cost saving initiatives. If you remember last quarter, we have optimized the headcount that's also flowing through in the results. Our scalable omnichannel, the digital engagements are also driving the upside because we could scale and reach out to more doctors with much lesser cost.

So, we'll be continuing the focus on the cost engagement, cost management, and you will see the EBITDA will be sustainably perhaps maintaining or improving from here on. PAT is having the same impact of the EBITDA, it's showing the same behaviour of EBITDA.

Moving on to the full year, if you see the full year, it's 6% growth. Remember, there is an NLEM impact of top line 8%. So that is mitigated to a larger extent and 6% is the reported growth. NLEM impact has been mitigated through a series of activities, right from WPI price increase.

We have taken price optimization at many places, including our discounts at many places. Cost efficiencies, volume, the focus on volume, which the commercial excellence on driving volume like Cefum and T - Bact, that is being fully leveraged in the last year. As well as the new launches vaccines, both Varilrix as well as Shingrix, GSK India

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both had an excellent offtake last year, and that is seeing in the top line results of vaccines at 35%.

Close to one third of that growth is coming from this uptake of the new products. All our brands are gaining market share, which is perfectly connecting to the financials. Margin - the NLEM impact has been fully mitigated. That's why you're seeing the margin above last year, despite the fact that we had a significant NLEM price cut impact. Our productivity has improved. If you see above, we have reduced around 450 people through the optimization exercise we had done.

And our per head productivity of the field has improved close to a north of 20% plus. So that's also the, and volume focus was there throughout the year. Right side on the last box, PAT, it's a decline of 3%. If you see that's a headline reported decline, that's primarily

ly because of one of VRS costs, which we have reported in the previous quarter, not in this quarter. Return on capital employed has improved to 53% from last year, 48%. Our cash flow is very strong.

As we speak, we exited with 100% of our profits converted to cash. We had better working capital management also this year, lower inventory and lower receivables. Our working capital has improved and our cash flow is close to 1,800 crores plus cash we are holding.

Earnings per share, excluding that one-off adjustments of VRS, we are talking about 40.83, which is 14% growth year on year. That's basically the summary of financials. Most of the efficiencies in the commercial is playing out. The volumes are coming. The margin improvements are happening. Cost efficiencies have been dialled in as well as the cash focus and return on capital employed, all are playing out in the full year.

Bhushan Akshikar: This is a slide that I had shown a year ago. And I think I'm just pulling this slide back to reaffirm our commitment to the double-digit growth that is a clear objective for this organisation over the next three to five years. Not too many companies can take this pride and say that we've been operating in this country for 100 years, but I think we are making it even more fulfilling by saying that our simple objective, apart from delivering growth, is to touch the lives of a billion Indians.

Given the nature of our brands, given the nature of the portfolio, where we have the remarkable opportunity to play in the prevention space, in the general medicine space, which really is the ability to touch the lives of 300, 400 million Indians in one stroke with brands like Calpol or Augmentin. But we really are setting up new platforms of growth for us in the coming time.

And that's exactly how our mission remains the same. We continue to remain focused. So the idea is to build on what we've done with Shingrix with the next few slew of launches. The focus will be on the relentless improvement of our top line and

bottom line by delivering a competitive performance on our base business, which is what we just talked of the paediatric vaccines.

And of course, to do all of that, have the right culture. This is an incredible moment for us because you can see on that right -hand side, 100 years of trust, the access and the equity that we have to healthcare practitioners in this country, when we say that we represent GlaxoSmithKline, still remains intact. And I think the last 30 seconds, we'll just share a video.

[Video]

Thank you very much for your attention. I think this is what symbolises the purpose, the larger purpose that unites all of us across the value chain in the organisation. So on that note, I think I will open it up for Q&A. Request, Juby, both of us.

Ransom D'Souza: Thank you, Bhushan and Juby. So, we have in the room close to 30 people and on the line, we have close to over 50 people. And, we have limited time. The way I'm going to do it is, please raise your hand. We'll have a mic that will come to you.

Please be brief, introduce yourself, your name, where you are from, your brief question and to whom.

So the first preference would be for people in the room. And for those who are online, to make it more optimal, we have a chat section. So kindly put in your question in the chat section. We will then kind of read out with your name because I think that will become more optimal rather than go back and forth. So first question in the room, can we have the mic here?

Mitul Mehta: First of all, congratulations on the excellent performance and the effort that you've laid out over the last two, three years to come here and build a base from your own. In your remarks, you sort of didn't mention about the growth going forward to sort of maintain double-digit. One is, on the volume side, if you see for the year,

we've sort of

grown at about 6%, 7%, 8%.

And for Q4, we grew at about 12%, 13%. I'm talking about the general med portfolio, which is a larger part of the portfolio. Going into FY '25 and FY '26, do you believe that you can sustain the Q4 volume number for the entire year? Or there is some element of seasonality also in the general med portfolio, if you could just tell me. Yeah, my name is Mitul Mehta. I'm from Lucky Investment Managers.

Bhushan Akshikar: Thank you very much, Mitul, for that question. Absolutely, yes, as you rightly picked up, the general medicines portfolio is really the foundation on which a large part of our business rests. And if you look at the last three years, regardless of the NLEM impact, we've had a strong volume evolution every single year.

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Last three years in a row, if you look at all the top nine brands that we have, we've not only been in the top in the pharma industry, but even in terms of volumes and the represented categories that each of these assets belong to, we've had a strong performance. So I think when I scope out the next 24 to 36 months, we definitely see the continued momentum. If you see the volume growth that we will talk about, each of the assets has delivered single high digit or at least a double digit growth in the last 12 months.

And we see every reason to continue doing that. If you remember my first slide, I talked about some innovative ways of reaching out. And we've really unlocked some value with the digital acceleration that we've seen, which has really helped us remain ahead of most of the competitors in each of the categories. So the answer is an emphatic yes, in terms of the volume growth. Yes, the seasonality is inbuilt to a large extent. I think some of the brands we've always seen, associated between a few quarters, we always see a spike.

But regardless of that, I think the volumes will remain. Yeah, the second point is, if you've seen the last NLEM, that was the most significant one for us because hitherto we didn't have Cefum and T - Bact included, which were the biggest brands in terms of the sheer erosion that happened on the top line, bottom line. For the next three years, thankfully, we know there is no NLEM coming around, but we're not going to depend only on that.

Is this the real time to build the continued momentum? So the answer is yes, we see the same momentum continuing over the next three years for General Medicines. So given the, always there are spikes as we know, but typically given the nature of our portfolio, for example, the dermatology portfolio has one of the best performances in Q3, which gets evened out in the summers, whereas anti-infectives and pain category really pick up in those next two quarters. So I think as a portfolio, a blended growth anywhere in 9% to 12% is what we will see continuing to be delivered.

Pritesh: This is Pritesh from Lucky Investments. So the question is around the specialty portfolio side. It's been three years that we are trying to create a significant business there. The business achieved is about INR100 crores. We have been traditionally a general medicine selling company. What changes have been brought in to make sure that the specialty medicines portfolio ramps up because the skillset there is slightly different?

So what changes have been brought in? My guess is there must be some challenge by which in three years you have reached 100 because usually brands start with saying that I want to reach 100 immediately and we have three products and three products combined to reach 100. So what were the challenges? And incrementally, you have an aspiration to introduce the respiratory and the cardiac medicine from the global portfolio. What are the stages of those 18 or 20 drugs or 18 or 20 options and GSK India

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what is the progress there? So around

the specialty and this 3% of sale, if you have a five-year double-digit sales growth target, this 3% of sale should be what in the next five years?

Bhushan Akshikar: Probably I'll spend an hour answering those 2 questions that you asked. You can always give it. But I'll take all the questions. I'll try and ensure that I don't lose out the sequence. Well, the first thing is I just want to remind that we did have that muscle. So it's not that we didn't ever sell specialty. We did have a thriving oncology business when the asset swap deal happened a decade ago globally for which we had to hand over the oncology portfolio to Novartis, if you recall. So that was, again, INR150 crores business that we had built from scratch.

It was a standing start for us. So just wanted to remind that there is some muscle memory there. To the question that you asked about the portfolio, I just want to remind, Shingrix is just 12 months old. We launched exactly, May was the month that we got last year, which is, as I said, we already touched, we already vaccinated about 55,000. We've had 55,000 doses already as a part of the first remit. The second is, if you look at each of the assets that I talked of on that slide, each is representing a massive opportunity for us.

Take the example of Trelegy, which operates in the broncho inhalation space, large market, INR4,500 crores in the respiratory. But if you look at where the growth is coming from for the last 12 to 18 months, it's all on this category called closed single inhalation triple therapy, the SITT. And as you must have picked up in our global results also, Trelegy is one of the biggest, apart from the vaccines that we sell, it's one of the biggest assets that we have.

It's more than GBP 2.5 billion for us globally. And that's the kind of grounds well that we are already seeing. Somebody had asked me, in the last meeting, why can't some of these new assets be INR50 crores and INR100 crores each?

And I think that's the ambition that we are chasing. If you've seen the last two quarters, each of these assets is growing in excess of, the EI is in excess of 145%, 150%. So obviously that's the kind of incremental growth that we are doing. You talked about skill sets. Yes, we have now, apart from the digital acceleration, we have access teams. We have redefined some of the work that we do in the medical area to really change the trajectory on the uptake and upswing of the assets.

So the last question that you had asked is moving forward, what should that number be? Again, as you know, we don't give forward-looking guidance, but as a corporation, I think my personal belief is that that freshness index has to be at least 10% to 15% of the top line so that you continue to have your base, which delivers sustained performance, but you're bolting on the new platforms of growth, as I keep saying, and that's exactly where we will play. We also have, you asked a question about trials. We

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have over 19 global clinical trials ongoing right now in areas like oncology, in areas like hepatology.

We probably have the first functional cure for hep B, and a trial is ongoing even in India as a part of the global remit. So that's a clear area of Specialty medicines which will get bolted on. So it'll continue to allow us to be most broadly diversified from prevention in the form of vaccines, everything in the middle with general medicines and Specialty on the other side.

Pritesh: Anything coming this year, or next year in that?

Bhushan Akshikar : We have the marketing authorization for our oncology assets and right now we are looking at the go-to-market strategy for these assets. So as you know we have the marketing authorization for two assets indicated in the ovarian and endometrial cancer, but we're still looking at what is the best go-to-market strategy.

Pritesh: Okay, my last question is,

at the GSK board global what are the changes with respect to India in their discussions or in their thought processes about GSK India? And the other usual thing with MNC and obviously GSK has been a slightly frequent change in the last year. So on that side, any comments you want to make in terms of continuity or how do they look at GSK India?

Bhushan Akshikar: Well, somebody asked me a question last week on our commitment to ESG and sustainability. And I said, look, our organisation has sustained for 100 years in India. So, on the lighter side, the fact that we continue to be a relevant organisation, if you look at our top line we just announced our results. We still are one of the biggest stakeholders as well as contributors in the emerging markets remit.

And the idea is to definitely look at continue to look at India as a source for not only delivering business results, but as a source for talent. As many of you may be aware, this is outside the listed entity, but we also have a global capability centre that GSK has invested with creation of almost 4,000 jobs in India and Bangalore in early R&D, in manufacturing, in procurement. So a large part of the global jobs are a part of this GCC. So really India continues to be a shining example of renewed commitment apart from the century that we have already got here.

Gaurav: So a couple of questions from my side. One is this 12% growth for the quarter that we have shown. This was not shown as per IQBI. So is there some difference between the primary sales or secondary sales which would have led to this? Second, post VRS so where are we on the employee strength or the MR strength? And will this quarter employee run rate be a fair representation to be extrapolated for the next year? And third, if you can also help us understand where is the - how much of the general medicine continues to be in NLEM as per FY24?

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Bhushan Akshikar: So I'll take a couple of those questions and I'll ask Juby to chime in. So if you recall last time I did talk about it, I think 6 months ago I did when I interacted with the investor community. I said that we have about 40% of our General Medicines portfolio now in the NLEM. So that's the answer to your last question. On the second question, I think Juby articulated the number there, but we had only one quarter in terms of digging in the upside with the reduced headcount. So I think the full benefits will accrue over the period of the next 12 months and this financial year and we had said that with this, we certainly see anywhere between 1.5% to 2% margin benefit, but I don't know if Juby, you want to add something there?

Juby Chandy: So I think the headcount piece which you are asking, so it's pretty much stabilized.

We have completed the VRS initiatives. It is the whole quarter is full completed and it's up and running with the full strength basically. So there's no immediate changes we are expecting on that front. On the sales gap versus IQVIA, typically there is some part of hospital business, but IQVIA is not a full representation in terms of what the reported numbers is, but remember last year there was an NLEM impact.

So that could also be some part of the difference. That's what I said, 16% is our total sales growth. Last year, same quarter if you compare quarter-to-quarter, there is what you said the credit notes we issue for the NLEM to the distributors has been there in the base. So the base has been a bit understated to that extent. So if I exclude that, means if I add back that credit note which I have issued back, so which is the leg-to-leg comparison, we're talking about 11% growth on the quarter.

Gaurav: Just last one question if I may squeeze. If you can help us understand the margin

breakup between the vaccine and the General Medicines portfolio to the extent you can help us understand or at the gross margin level

or the EBITDA margin whichever.

Juby Chandy: Yes, I think we don't give, we have never given the strict guidance on margins, but all the vaccines are imported products and all the pharma products are locally manufactured and 98% of the pharma products are locally manufactured. And so margins have huge variance. And again, between the products, we have large number of products, between the products also there's huge margin variance, but since we continuously promote the same product in the same mix, for example, if you have 100 people or 1,000 people promoting augmenting, they will continue to promote the same way.

So the mix always maintains. So for a margin guidance, you could assume the same margins could continue unless there is any structural changes happens in the market which we are not seeing in the foreseeable future at this point in time.

Vijay Bhatia: I had a very generic question that I took Shingrix vaccine last week and the cost is kind of very prohibitive, 11,000 per dose and the second dose also is around 11,000.

So for a person who's aged more than 50 INR22,000 for a dose. What is your target GSK India

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audience that you think will be the blue sky scenario in this? And do you plan to reduce these prices going ahead? Because how do you address the market?

Bhushan Akshikar: Thanks for the question. So if you recall, I've highlighted my slide at the top as well. It's there in the industrial presentation that we loaded last time. If you look at a population of our country and if you look at all the filters in terms of geographies, the economics, we're still talking of about 60 million Indians spread over the top 15 cities which can be the potential target at addressable market for this category.

You asked about price. This is by the way an India-centric price because if you were to go to the US or UK, it's almost GBP 3.50 worth of the price to the consumers. We also have to remember that for an innovation like this where it's providing protection for now we have data for 12 years, which means once you've taken these two shots, you're protected for 12 years.

Earlier when we launched this last year, we had data for the first 10 years. We just got released data now which says that for 12 years, you will be protected against herpes. And if you were to prorate that over a 12-year period this is probably cheaper than a flu influenza vaccine on an annualized basis because flu, as you know, has to be taken, the seasons are changing, you have to take actually both.

So you're talking of at least INR2,000 to INR3,000 doses per year for a flu vaccine. So on an annualized basis spread over the decade that 12 years of protection that you get, it is at a price point. I'm conscious that yes, for the average Indian consumer, this might seem a higher amount, but it's largely driven by the innovation, the kind of innovation that is a part of this category. At this stage, no, I think we have just focused on ensuring that as many people benefit with this innovative vaccine as possible.

Jay: So, sir, we've got three verticals, General Medicines, Specialty and Vaccines and we've got activation happening in all of those verticals. Now, within the organisation what do you think is going to be the next growth driver if you were to see amongst these three divisions, the faster growing divisions?

Bhushan Akshikar: So I think, look, if you see the global organisation where GSK as a biopharma organisation is reinventing and repurposing itself and if you've seen our investor presentations the last couple of years we have clearly stated our intent to do work in adult immunization. So if you look at our global results, one of the best launches globally has been in the area of RSV which is another vaccine. It's one of the fastest, in I think 4 months it became a billion pound vaccine in the US alone.

So there are trials going on for

this vaccine across the globe, including in India. And I think if you look at the next few years the ability to take charge of your health, the whole sense of well-being and to prevent will be an area. So I think the first question that you asked is around what does it take to really help create those ecosystems for enabling consumers at large as well as healthcare practitioners who are so focused GSK India

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only on treatment, but also to change the narrative and say, we want to be as focused on prevention along with treatment? So that requires a different capability and that's where we'll continue to build our muscle.

The second one is around general medicines. So what we did in the last quarter in terms of the transformation that we just talked of, I had that one slide on what we've done in terms of omni-channel. So today we have elements of machine learning or AI where if I'm a rep detailing to a doctor in say Worli area of Bombay, I can actually in real time show to that doctor which are the infections prevalent and which anti-infectives are the most susceptible and therefore the choice that he can or she can make. And that's all real time.

So we're investing our energy here because every physician is interested only in one thing, improving his or her treatment outcomes. And that's an area where we are making some significant investments. So how to really digitally accelerate so that those decisions that the HCPs make are in the best interest and the fastest interest of the patient. So those two would stand out.

The third one is, of course, vaccines and you saw clearly what we had seen during COVID, the periodic vaccination is gradually coming back to the growth space. So continue to do work there to help enable paediatricians. So I think broadly those three will remain. And the first question that was asked, on top of this bolt-on are new growth platforms which includes the Specialty portfolio. So that's going to be a clear call out for us as an organisation in the next three to five years.

Juby Chandy: But just to add, if you see the India industry as a whole, General Medicines still has a lot of scope, whether it's antibiotics, it is the number one, Augmentin is the number one product in the industry. So even a small initiative in Augmentin or Calpol, that's still going to be a major growth driver for us. So unlike the global GSKs where other countries where we have to definitely have a big shaping happening in the Specialty or oncology space, here General Medicines is still relevant.

Our organisation is fully focused on delivering general medicines business, including Augmentin, Calpol, Eitroxin all these brands despite four or five decades in the country is still growing these assets. So we are conscious of the value of these assets. We are continuing to invest in these assets. Our largest field force are also lined up behind these assets. So I think we are not taking our eyes off from these assets.

Bhushan Akshikar: Absolutely. I just want to add to -- regardless of what happens on June 4th, clearly one of the things that the Health Ministry has stated explicitly is the OTC bill. And I think that's going to be one of the big levers of growth, especially from a General Medicines standpoint, because we shouldn't forget that we are still the number one paracetamol brand in the country. And if there's anyone that has to do work there and really ensure that we maximize the value and benefit to consumers and patients at GSK India

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large, it's the brands which have equity like the ones that we have. So that's going to be one of the big opening, you know, areas for us in the next 12 to 18 months.

Jay: And the second was, like you mentioned that the vaccine portfolio is largely imported for us. Is there a possibility to locally source these products?

Bhushan Akshikar:

As of now, no.

Jay: And when is the expected launch for onco portfolio?

Bhushan Akshikar: We are looking -- as I said, we are right now exploring the best go-to-market model, because obviously we want to come up to speed. So that's where the whole energy is.

I won't hazard a guess in terms of the time, but sooner than later.

Dhaval Shah: Dhaval Shah from Birla Mutual Fund. I have three questions. The first one is on Shingrix. It's been one year of launch, 55,000 doses. Three to five-year outlook initially was somewhere between 3 lakh to 5 lakh doses is what we could typically scale up to, given the potential size of what we're trying to look at. Would you still maintain that, or would you want to reduce the overall doses or increase it in terms of the outlook, given your interactions with HCPs?

Bhushan Akshikar: Sure, it's a brilliant question. No, we would not. We still remain absolutely optimistic, because remember, in a country where absolutely no vaccination ecosystem exists, so whatever you have sold is pure play in the HCPs clinics. Today, you don't have -- so if you were to go to different countries, like Brazil, for example, there are vaccination points. You go to Vietnam, there are vaccination centers where patients actually go and take vaccines. So I think it's just the beginning of this whole opening

up of this new area.

If you think about our population, almost 8% of our population right now is more than 60 years old, and that's a significant number when you extrapolate that over INR140 crores or INR145 crores. So I think from a forecasting standpoint, we're still firmly committed to that. We still don't have any accounts opening up in the institutional or the care accounts. So in the coming two years, that's going to be another, with the propriety and the patented medicines, and the patent that we have on Shingrix, clearly that's one area we will continue to build. So the answer is we remain committed to the numbers that we have given.

Dhaval Shah: That's very encouraging. Second is on Synflorix. There was this pain of volume decline, which has happened because of migration also, in terms of the government contracts. How do we see that now incrementally, as that bottomed out, as it started growing, and how should we assess that on the overall?

Bhushan Akshikar: Absolutely. So if you saw the campaign that we have right now, which is, it's really about getting parents back, because if you remember, many of the antigens went into GSK India

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a national immunization program, and some antigens like Synflorix did get affected. If you look at the last two quarters, we've stabilized. But the last month or so, we are clearly seeing, so it's still too early, but I think when we look at the next 12 months, we certainly see Synflorix coming back to growth, because we are certainly seeing the groundswell.

The other bit is GSK globally has already stated its intent with the successor to Synflorix. So we have -- we acquired a company called Affinivax, which has now a 23-valent, if I recall correctly, 23-valent successor in the area of PCV. So that's an area which will remain important to us, because we should all remember this was one of the biggest vaccines we had, and therefore, we continue to see the growth coming back, and we definitely would love to continue to remain invested, because we have a successor to Synflorix in the pipeline.

Juby Chandy: But for this, just to add, for this purpose, it is bottomed out to question, because vaccines make 20% of our portfolio, and Synflorix, perhaps, the NIP portfolio just makes 8% of our vaccine business, so which means it is, for an entity, legal entity, listed entity purpose, it is less than 2% of our portfolios in NIP overall, right? So it is bottomed out, and it's not material at this point in time, the NIP portfolio.

Bhushan Akshikar: Anything that comes from

here on is a bonus.

Juby Chandy: Yes, absolutely.

Dhaval Shah: Sure, and the 23-valent one can come in the next 12, 18 months?

Juby Chandy: No, trials are still ongoing, so it's still further down.

Dhaval Shah: The last one is on the cost savings portion. Job, given the fact that NLEM had a INR200 crores kind of head, from where we are today, are there further ability to optimize on the cost front over the next 24 months, in terms of the other expenses, or any other costs that we can further optimize on?

Juby Chandy: So I think, pretty much, the low-hanging fruits have been addressed through, but we are on a continuous journey, monitoring the cost and optimizing it. The good news this year is, you might be following all the Pharma company results. The raw material prices started softening a bit, quarter on quarter.

It's a sequential quarter, rather. It's almost 2% softened, the raw material prices. Year on year, you're seeing a 5% to 6% softening on the raw material prices. So that is one area which, you know, logically it has to flow through into the results, pretty much across the industry. And we also will be having a fair share of that, what you're seeing. Other than that, we are carefully optimizing wherever possible, the cost.

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It's going to be a journey. It's not like a one-off exercise we have done. So that's why the cost will be growing much slower than the revenue growth. That's the kind of direction we are taking inside the company. And that you could see even this year results, in which our format, you slice and dice the results. The cost is growing much slower than the sales growth. And that's something which we are, the culture we are embedding in the organisation across the piece, because that's where you will see

that.

Ransom D'souza: Okay. Sorry. So it's been one hour since we've done the call. So there are people online. So just to be fair, I'll take two questions online. One is from Gokul Maheshwari. Two questions from him. You mentioned that vaccines achieved 1 million doses in the last quarter. Could you give a sense of the doses for full year 23 and 24? And second is the employee cost. Given the VRS has happened, what inflation should we build in employee cost in FY'25 and FY'26?

Juby Chandy: So employee cost may be fairly straightforward. We normally budget a 7% to 9% inflationary increase on the employee cost. We're not expecting headcount increases. These are the normal salary increases we are expecting. Wherever there's a need, we'll be trying to reallocate the resources. If there's a new launch or something coming, we'll be trying to reallocate the resources. That's on the employee cost. On the doses.

Bhushan Akshikar: So on the doses clearly, as I said, for a year we said the vaccines business is stabilizing. And we've now come clearly in the growth territory. So the answer is that we want to maintain the same volume growths, the same number of doses. So I think the million doses will only go up. Obviously it depends on who comes first for some of the categories like flu, who's the first to market. And that remains the focus for us in the coming quarters as well.

So the answer is we will pretty much remain in the growth range that you saw for the last quarter for the financial year. But again, no forward-looking guidance on that one. But on a volume standpoint, we remain committed to the same growth.

Ransom D'souza: Thank you. Bhushan. The other question is from Nikhil from Simple. While we did rationalization of employees last quarter, but still employee cost was higher. So is there some one-off and what is the sustained run rate?

Juby Chandy: So the employee cost, the benefit has not fully flown through, right? Because we have completed the VRS in December end. So we are talking about a quarter. So when you see the full year, it has not been, the full actualization has not happened. So let's wait

for a couple of quarters to come in on that.

Ransom D'souza: Okay. In continuation, in the base, when NLEM hit us last quarter, was there some benefit of lower volume in base quarter? In the base, when NLEM hit us last quarter, was there some benefit of lower volume?

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Juby Chandy: No, it's other way around, right? So when NLEM hit, the price went down and we got higher volumes in Ceftum, T-Backed, including augmenting. So the volumes have been pretty much sensitive to the pricing and NLEM has been a price reduction happened. So it's other way around.

Bhushan Akshikar: If you just take an illustrative example, our prices for Ceftum went down by 56% and for the full year, not the quarter, for the full year, we've grown volumes by 60%. So it's the other way around.

Ransom D'souza: Okay. In specialty portfolio, how has been our coverage led growth and existing RX led growth? In the Specialty portfolio, how has been our coverage led growth?

Bhushan Akshikar: I don't understand the question. Coverage led growth.

Ransom D'souza: In terms of, he's looking at overall coverage in terms of the coverage that we've been doing, have we increased? [inaudible 78:12]. The doctors as well.

Bhushan Akshikar: Yes, but I think to begin with, specialists are not, the teams are very nimble-footed. They're very agile. So it's not a cast of thousands, like in General Medicines, where you have to really, have a large net. It's largely restricted to the, but one change that we've done is for the Specialty portfolio, we've expanded.

So as much as we optimize the headcount in some of the teams, we've actually expanded in our respiratory team. And that's why some of the ambition that we're putting around our assets in respiratory beat, Nucala or Trelegy, will pan out in the coming quarters. So yes, we have expanded our reach and coverage by almost 40% in this last quarter, but it's still early days. We just had one quarter behind us. The benefits will start accruing in the coming quarters.

Juby Chandy: But again, just to add the, like he was asking the question, over the three years, we've gone to INR100 crores, but it's not a linear forecasting because we first launched Nucala, which is for severe eosinophils asthma, which has got a very small patient pool. Then came Shingrix, which has got a little bit more, higher patient pool. Trelegy is a very different product.

It's kind of a general medicine product. It has got a mass appeal in terms of reaching

out to more patients. So that is where these are not exactly comparable. We are, depending on the product, we are reaching out to more doctors. It's actually depending on the product type, your coverage is covering.

Ransom D'souza: Okay, and the last basket of questions from Nikhil is, can you give sales of top three vaccines other than Shingrix?

Bhushan Akshikar: So Infanrix Hexa is one of our biggest vaccines. Fluarix -Tetra is the second one in terms of share value. So if you look at a vaccine's portfolio, which is roughly about

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INR600 crores annually, those two would be right at the top. Apart from Viralix, which has now come back, after almost two years, supplies were resumed and we've got Viralix back. So those three in that sequence would be the top three.

Ransom D'Souza: Thank you, thank you Bhushan. So there is a similar question from Gokul Maheshwari and Abdul Qadir Puranwala. Gokul asks, break up of sales for general medicines, vaccines and other products in FY'24 and '23. And Abdul Qadir is saying, please share FY'24 growth of general medicines, specialty and vaccines.

Juby Chandy: So, General Medicines and vaccines are 80 -20 split. General Medicines is 80% and vaccine is 20%. This year, FY'24, it is 18%. It will move only plus minus one percentage because the growth

of both the businesses are exactly in the same time it is progressing. So 80 -20 is the split, specialty is fairly small at this point in time, it's 3%.

Ransom D'Souza: Okay, thank you. Last question online for now. Ravi Purohit says, Glaxo Globally had discontinued sales targets for their MRs a decade ago. It has that changed, that has changed I assume. At the same, we have also reduced our headcount. How do we see the overall path towards employee productivity?

Bhushan Akshikar: So yes, we got our budgets back globally as well as in India in 2022. So it's been two years that we've got sales targets back in the commercial functions. So yes, that's the first answer to the first question. I think productivity, I think Juby just talked about the improvement that you've seen the productivity. So the focus will, tying back to what was asked earlier, all that we are doing in terms of the investment in the digital side is really to unlock value. So I think we are very clear in our market, the role of the rep is never going to go away completely.

How do you supplement and complement that rep's visit? So the question that we are asking is, why does someone have to be met two times, three times? Can that be done? Can the first visit be complemented by many other things? Not maybe at the general practitioner level, but many of the other specialists prefer that you reach out to other channels and that's where we're investing in the content, the relevant content medically, which makes sense for them. So productivity with that headcount that we now have, will only, the idea is to sustain that productivity now and improve it in the next 12 months.

Ransom D'Souza: So we'll come here. So we have, raise your hand, introduce yourself and the question.

Deepak: Yes, hi. This is Deepak from Carnelian Capital. So when I see in the last two years, so GSK is in the process of transformation with your coming in, giving the guidance for 12% or double digit growth in the volume, in the top line. And when I see that almost 19 products from the parent is coming to India, the clinical trials are going on. Is it like the company is redefining itself from a generic focus to an innovation led company? So what is the vision behind this working on and what is the next five year kind of plan or the milestone which you want to achieve to get to that vision?

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Bhushan Akshikar: Sure, so it's a great question because you will, if you've not seen what we call as the heritage gallery that we just created, one thing is common and will remain constant in GSK's evolution, it's reinventing ourselves. So if you see the portfolio has evolved several times over in the last two, three decades. Just a small correction.

I don't think we are a generic company. I always say in this non-patented branded market, we are, it probably mimics the FMCG industry. So if you look at why somebody chooses an augment in all the other brands of Amoxiclav and vice versa, why somebody chooses another brand versus the brands that we have is because in India, brands tend to have a long tail in their life cycle.

They're not woven around patterns. So to that extent, I think it's a completely branded

market. We're not operating in that branded generic space, so to say. But I think the question that you asked, and I think Juby answered part of it earlier, if you look at the population that we have, if you look at the healthcare needs, our general medicines portfolio will still remain relevant. The strategic choices that we've made in terms of therapy areas, be it anti-infectives, be it the terms business, be it pain vitamins, those will remain relevant over the next 5 to 10 years. I don't see anything drying up in terms of the sheer need for any of these assets.

So we will continue to build. So this double-digit objective that I keep talking of is still hanging around success

ess of this GENERAL MEDICINES portfolio. And the question is how do you therefore bolt on the newer therapy areas or the newer areas in which GSK Global is getting into as well as we have these trials happening in India. So the idea is it will be a blended growth on both sides. I don't see the relevance ever going away in this market for the brands that we have in GENERAL MEDICINES, or even for that matter in the periodic vaccine business.

Deepak: When these 19 clinical trials with the products which is going on, is it safe to assume that in the next 3 to 5 years, all these will get launched in India?

Bhushan Akshikar: I mean, 19 clinical trials are not necessarily 19 assets because there are some assets where we're having two or three trials also ongoing. So, I mean, I wouldn't give you a number and say there are seven assets, but clearly there are areas where we are putting energy. So vaccines will continue to be an area.

The second one is, as I said, hepatology is one area where we are having trials coming up for a couple of assets. And then of course, we have respiratory which will continue to be one of the area for us to work on. Apart from the successes that I talked of for some of our earlier vaccines.

Juby Chandy: I think it's very clear. We are not going to get distracted with 19 or 20 assets. So where there's an opportunity only we'll be launching. So we'll be carefully analyzing there's a process in the company. When the new product comes, we'll be carefully GSK India

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analyzing the opportunity, the potential. And if it makes commercial sense and medical sense, we'll be launching. So it's not all the things we'll be launching. There is a phased manner in which it will go. But again, it is the general medicines will be the key growth driver. These are all the bolt-ons inside the product.

Bhushan Akshikar: I mean, example is Calpol 650 plus that you just saw there. I mean, for a brand that is almost 60 years young in this country, there is still need to reinvent and contemporize the offerings. And that's exactly what we've done. And I see opportunities like that continuing. Two years ago, we launched Augmentin ES which is a higher dose strength in the liquids formulations for a specific indication for resistant cases. And I think wherever there is a need, we will continue to extend the life cycle or for each of the assets in GENERAL MEDICINES provided there is a patient benefit at the end of the day.

Deepak: And last question from my side. So with cutting down on the cost, on the employee cost and rationalizing the cost and launching of these new innovative products where margins are generally higher than the generic ones. So can the profitability go back to your 10 years or 15 years back when you used to make 32%, 33% kind of margins? Can it go back to that kind of profitability in some time in the future?

Bhushan Akshikar: Well, I can't crystal ball guess, but I think our objective as a team, I and Juby keep saying that our EBITDA margins, what we've delivered now for the quarter and the year is how do we sustain those margins? I think relentless focus will be on the top line growth. That will be the focus of the company but and maintaining the margins. I don't know, Juby, if you want to add anything.

Deepak: Thank you.

Ransom D'Souza: So we have time for 2 or 3 more questions. Okay.

Satish Bhatt: Hello, hi, this is Satish Bhatt from Envil family office. I've been tracking Glaxo from 1990s, I think. One thing I want to know in your specialty portfolio, especially Nucala, it's a very specialty product where I think it's a biologic where the efficacy is far better than the older ones, niche one, and you have only 400 or 500 pulmonologists we have to track, basically map around the country. So what is preventing from making it a INR500 crores or INR1,000 crores brand? I still can't understand.

nd. So what

management is thinking in a different way to make it a INR1,000 crores brand?

So that's my thing. Trelegy is where INR4,500 crores market inhalers. So these are mass market where I think Glaxo is a phenomenal brand equity. If Glaxo introduced something in mass market, I think it has to become INR100 crores in day one. Gone are the days, I think we talk about 90s and 2000s, INR5 crores, INR10 crores, INR50 crores. See now Shingrix first term is INR50 crores.

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So what is preventing you or what is, what differentiator Glaxo is reinventing the market dynamics itself, to overtake well-established players, where you have a better science delivering to the patients. And you have only 400 or 500 pulmonologists who are going to write it. So just wanted to know, I can't understand why Glaxo is talking only 5%, 10% after five years. Why it can be 15% in five years? So what differentiator think you are, I think management is focusing on that. That's my only question.

Bhushan Akshikar: Sure, it's a good challenge to have. Just to contextualize, we have about 75 million COPD patients in the country. And we have about 45 million patients suffering from asthma. Now, if you recall what Juby said earlier, asthma is a continuum. Right from oral corticosteroids to inhalation and then to monoclonal antibodies. We have to all remember that Nucala is indicated for severe eosinophilic asthma and not for every single type of asthma. So when everything else is irrelevant, then that kind of patient really gets the opportunity to benefit now.

That's why we have to be a little mindful about ensuring that only those patients that really require Nucala are prescribed and therefore they benefit. Because it can't really be an opportunity like Trelegy. I agree with you, that Trelegy can easily be a significant opportunity. If you look at where this closed SITT therapy is going, in a matter of 12 months, this therapy has become INR170 crores. It's the fastest growing area in the inhalation space.

If you look at the next five years, if there is one category which will be the biggest one, it will be this closed triple inhalation because you don't have to carry two, three inhalers then. And I think the device that we have, the combination that we have at Trelegy really sets us apart. So the answer to your question actually lies in the opportunity that we are seeing equally, as you say, for Trelegy more than Nucala. But Nucala will always remain a highly, I mean, it's a brand which has, you can't paint it with a broad brush. There are only appropriate patient types that can really benefit. So the idea is really to provide access with this incredible medicine so that patients benefit. But that pool is very limited. Yes. But again, then you'll have to have filters of, at what stage do they get diagnosed and will they really benefit? And then again, the economics around it. So that's where we are. But yes.

Ritesh: Yes. Hi, Ritesh from Nippon Mutual Fund. In your overall headcount of MR, how would you split on general medicines, vaccine and specialty? What are the changes you have made on that new number in last three years of IS? And particularly on the general medicine, have you taken help of third party MR outsourcing provider like Coaptive? And I presume your VRS would have some linkage to that. So if you can provide, how this model of third party outsourcing going to a provider like Coaptive and scaling your products in tier three to tier five towns work?

Bhushan Akshikar: So just to reset, we were one of the first companies that had a team set out in the town. So I think almost a decade back, we put up a rural team that was based in class

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two to class six type of cities. And these teams were, these reps were promoting everything. So as a part of the optimization exercise,

cise, we will also optimize in the

metros. And because we've seen the last 18, 24 months, our extra urban growth is at least a couple of percentage points more than what we see in the metros. And that's where we have rebalanced and redeployed some of our resources.

So we actually expanded this team that we have, which allows us to access many of these cities and towns in the interiors. And the rep is also able to be the one single point of contact for that general practitioner, because the nature of practice in many of these towns is of a general nature. You have very few specialists.

And that's where we expanded actually. So we have expanded in that geography and optimized in some of the metro cities. So that's the first part of your question. Yes, we

are looking at alternative ways to really access , go to market, and really be, how do you open up? We have some partnerships in place, but I think not specifically, as I said, we put up our own team here in terms of really putting the content, the channel activation. So we have some internal teams now, which are well -placed to really harness what we can do apart from just calling face -to-face.

So that's my answer. I think the first question that you asked in terms of the headcount, obviously, we remain, he gave you a number, we optimised about almost 500 headcount in the last quarter. And that's made us competitive in terms of the share of voice that we have for our General Medicines business. Obviously, we are doing much more with digital ways, supplementing that face -to-face. So we've also rebalanced some of the headcount into our respiratory team that we've just started, apart from, of course, the vaccines team. So that's how I would answer.

Juby Chandy: Just to add one point, we have optimised the headcount, but the coverage has not reduced. We're still maintaining the coverage with the doctors. And there are many ways we are doing that. One is omni -channel. One is the same team might be covering more doctors, maybe reduce the frequency. Different methodology we are doing that, but the coverage of number of target doctors we are reaching before and after, it still either has gone up or it has come down.

Ritesh: Just to your 3,000 or 3,500 MR, how would you split between the three divisions broadly?

Bhushan Akshikar: So that headcount is not the MR headcount. So when you look at the total number of employees, there's also a factory. So there are employees there. So our total headcount number in sales is significantly lower than that. It's no longer in the range of 3,000 and 3,500. So I think the last head count is in the range of 2,200 that we have across all teams.

Ritesh: And we have not leveraged the third party?

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Bhushan Akshikar: Not as yet.

Ritesh: Not yet. And just one more question on your NLEM price hike, which you would have taken WPI linked. So this year, given the WPI number would be hardly anything, would the non -NLEM portfolio price hike would be different than what you would have taken last year? You will try to take an average of what?

Bhushan Akshikar: Generally, we look at ensuring that we are taking the relevant and appropriate price hikes for all our non -NLEM assets in the calendar year as permissible. So I don't think we'll continue to do what we've done in terms of not leaving any money on the table.

Ritesh: But we are not anchored to a NLEM price hike. For non -NLEM , there is no implicit anchoring then?

Bhushan Akshikar: No, no.

Juby Chandy: Not at all, because I think NLEM , you cannot take any pricing risk. Non -NLEM , we are carefully reviewing at SKU level. Where are there opportunities? Where is a huge competitive advantage we have if we still maintain the price? So that is where we are taking it. So we'll be dialing up prices in few places, but not all the places, depending on the competitiveness.

Ritesh: And usually it's spread out over the years. Like it comes with that 12 -month kind of thing. So it's not bunched up the way NLEM is bunched up.

Juby Chandy: Yes, because you need to, it depends on the last time when you took the price.

Ritesh: In the calendar year, right?

Ransom D'Souza: In the calendar year, you can take probably one time. So different times it has been taken, right? It will be spread over the year.

Ritesh: Thank you.

Puneet: Yes, thanks for taking my question. I 'm Puneet from Helios Capital. So, Bhushan, if I tie your commentary with the commentary by Juby on employee cost, it seems that in FY '25, there's no plan of launching J emperli and Zejula . Is that correct way to understand?

Bhushan Akshikar: Well, I didn' t say that. I said that we will, remember oncology is not a cost of thousands. For a country of our size, we hardly have 500 medical oncologists, probably lesser than that in terms of hematologists. So as someone who's also run the oncology business in the past, I'm conscious that you don't require a large team. We made significant investments in the medical team because assets like this need a lot of unlock on the access area, on the medical side in terms of evidence generation. So those are the areas which are unlocking.

And in any case, even if you're launching the oncology assets, it won't be a large team because the biggest oncology teams in India are not, they're still in a few, not even hundreds actually. You don't require a large team. So the answer is, we have been looking at accelerated launches, but as I said, we are really evaluating the go -to-market at this stage. Probably, hopefully we can have an answer for you next time around.

Puneet: Sure. And could you update on where are we in terms of bl end reps, progression, trials or regulatory?

Bhushan Akshikar: Yes, so the DREAM, we have a trials called DREAM 7, DREAM 8 and DREAM 14. India is a part of this global clinical trials as well. And belantoma, which is the molecule of blend rep is indicated for multiple myeloma. As you've seen in some parts of the world, we've just got the second line approval and that's going to be a significant change. So that's why I said, we now have a portfolio in terms of coming back in oncology with just, not just with Zejula Jemperli, but even blend rep. So the global organisation is continuing to remain focused on ensuring that it pursues the right indications because eventually it is as good as the standard of care is what the last readout was in second line. So we will continue to focus on that as it as well. But I think the first stage is for the clinical trials to get over.

Puneet: Sure. And last one is now that there's a plan of having a well -rounded oncology portfolio, any plans of reintroducing cervical cancer vaccine in India?

Bhushan Akshikar: Well, I mean, look, the HPV is already a part of the NIP now. So, and again, unlike the PCV where I just answered him, we have a very clear plan of having a successor to our existing antigen. That's an area where we've not really made an investment. So to come in an area where one, we already have enough players in the market. Plus the second thing is it's now a part of the National Immunisation Program. We don't see real market attractiveness or even the kind of value that we can create both for the investors as well as for healthcare practitioners.

Puneet: Sure. Thanks. All the best.

Ransom D'Souza: Great. With that, we've come to a close. Okay. Sorry. Okay. Since you've raised your hand.

Analyst: Thanks, Ransom, for this . Sir, I have a few questions, two or three. So please bear with me. One is you touched upon the regulations on the OTC side. Would like to hear your thoughts on how the dynamics will change. I'm sure that it's still under discussion phase and all those things, but will be interesti

ng to hear the dynamics on that.

Second related a little bit on the OTC side is how do you think about the brand extensions? Particularly you talked about Calpol being a 60 year young. So do you think that there are opportunities around brand extensions of some of those successful brands that you have?

Third on the vaccines, is there a critical size that lends itself to manufacturing in India? I mean, I don't know, GBP50 million, a GBP100 million. I don't know. Is there a critical number? And lastly, it's more of an assurance rather than a question that given our strength, I would assume that on the specialty side, we will not opt for an out license and we will still like to do it on our own.

Bhushan Akshikar: All four brilliant questions. I'll take the last question first and go in that sequence. So as I said, the whole idea is to maximize value. The whole idea is to ensure that we not only maximize value for patients and healthcare practitioners, but also to investors so that we don't leave any money on the table. And that's why I said, when it comes to oncology, we want to be very thoughtful about what go -to-market strategy we follow. I wouldn't hazard a guess and say that, we're going to not look at out licensing or in licensing , but everything is on the table right now.

And I think that's where I would, as I said to him, hopefully I will be able to answer you after the next quarter results on that last question. On the first question that you asked on OTC. So apart from the MD of this entity, I also work very closely with industry associations. So I've just taken a charge as the President -Elect of the OPPI. And we do work along with OPPI and the IPA because there's a lot to collaborate instead of

compete these days. And I think that's one area which I think we are in discussions with both the Department of Pharmaceuticals and Department of Health to see where all this evolves.

I think we'll have to just wait for a few more weeks. Like many other things in our country, I think June month will probably help us accelerate what happens in the next two months post that. But this is going to be one significant one coming our way, especially for the healthcare industry. I think this is a big one.

The second one that you asked was about potential to, so line exchanges are always up. I think I don't see any reason why. I mean, the fact that you saw 650 plus as well as 500 plus, there are many such opportunities that we see across our portfolio. We have a brand called Physiogel, which plays out in the area of cosmetic dermatology. Again, a INR50 crores brand for us.

We just launched, we used to always have moisturizer. Now we have a face wash. So the idea is what can we do more so that we don't leave any money on the table and really expand the portfolio. I think the third, last question you asked about vaccines manufacturing. It's not really a threshold of value sale. It's really about the quality standards because if you look at the vaccine footprint that GSK has, even today, we manufacture vaccines in a handful of locations in the world because the manufacturing process is pretty complex.

You're dealing with living organisms or at best attenuated viruses. So obviously the line of supply in terms of the controls that we expect as a company are of a different standard. Not that we don't have those in India. Definitely I think we have big global GSK India

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players, big local players operating here even at global scale. But I think there are choices that the corporation has made in terms of where to operate. As of now, we don't see local manufacturing for vaccines really opening up in the immediate here and now.

Analyst: Great, thanks a lot for all the answers. Thanks.

Ransom D'Souza: Thank you. On that, we come to an end. Thank you all for joining online . I know that

you may have had questions. I can see that there were certain questions that were posed but in the interest of time, we are unable to answer and we'll possibly see how we can respond to you one -on-one. Thank you all for coming.

Before you go , please do visit our heritage gallery. We have the architect of the heritage gallery, our company secretary, Mr. Nadkarni, who is very passionately...

Bhushan Akshikar: Out of the hundred years that we have completed in this country, Ajay has completed 25 with GSK. So you can imagine the passion, the love that he has - So he's done that. What we call as this heritage gallery. So I would urge each one of you to step in and take a look. And I think many of the questions that you asked and I answered would probably you can relate to the wall that you will see there in terms of how we are trying to evolve as a corporation.

Ransom D'Souza: And we have the privilege of Ajay being the personal tour guide for you.

Bhushan Akshikar: So everybody who's not here today or on the line, that's one extra motivation for you to be here next time around.

Ransom D'Souza: The next time around.

Bhushan Akshikar: Thank you very much.

Ransom D'Souza: Thank you all. Thank you, Bhushan. Thank you, Juby. And on behalf of GSK, thank you for coming. We also have snacks there apart from the heritage gallery. Thank you and have a lovely weekend. Cheers.

Call: Aug 2024

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8th August 2024

To,
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th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
Mumbai - 400001 Mumbai - 400051

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 5th August 2024. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/investors/analyst-meets/>.
Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

GSK India
5th Aug 2024

"GSK India
Analyst Meet Call"
August 5, 2024

MANAGEMENT : BHUSHAN AKSHIKAR – MANAGING DIRECTOR – GLAXOSMITHKLINE
PHARMACEUTICALS LIMITED

M
R. JUBY CHANDY – CHIEF FINANCIAL OFFICER ■■■
GLAXOSMITHKLINE PHARMACEUTICALS LIMITED

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Ransom Dsouza: Good evening, everyone. And thank you for joining the call. In the room, we have our managing director Mr. Bhushan Akshikar. Some of you have interacted. Some of you have interacted with Bhushan earlier. Also, we have our CFO Juby Chandy. Without further ado, I'd hand it over to Bhushan, who will give you an introduction and also after that, followed by the financial slide presentation by Juby and post that we will open up for questions. In the meantime, if some of you have any questions, kindly pose your questions in the chat box, and we will respond to them at the end. Thank you so much. Over to you.

Mr. Bhushan Akshikar: Thank you very much, Ransom. So, first of all any thanks to each one of you for taking our time to be on this call. As you must have seen in the results that were announced to the state on Friday last week, this is concluded Q1. But as we use the segue to talk about the financial numbers in a few minutes, I'll probably frame the context in the next five to six minutes. Just to let you know, what is it that has happened in Q1, which has allowed us to deliver the performance and the results that you already see? As I said in the last call and the investor meetings that we had prior to this, on the left-hand side, if you can see on the slide GSK India continues to be one of the most broadly diversified healthcare companies amongst multinationals and even otherwise operating at both ends of the spectrum in the form of preventive medicines and vaccines in the prevention area. And more importantly with the general medicine's portfolio. We continue to operate, as I said, at both ends with general medicines straddling anti-infectives, dermatology products which are used in the therapy areas that train vitamins and are renewed efforts in the respiratory. In the vaccines business, apart from the mainstay that we have in the GSK India
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self-pay pediatric vaccine market, we've also made strides. So Q1 was the first full year that we completed. That's for our foreign adult vaccination with the launch of shingles prevention. So that's where the portfolio is. If you look at where we remain anchored in terms of our strategic intent, it's all about growing and growing with a sharp focus. Clearly having strategies which are allowing us to make our big brands even bigger and to be having to deliver that competitive performance for all those identified assets, both in general medicines as well as our vaccines portfolio.

Continue to innovate, so some of our brands are as I say, as young as 30 and 40 years, but we continue to bring innovations both in terms of lifecycle management as well as the kind of science that healthcare practitioners expect from a company like us. So that continues to be one of the other anchors in innovation. And the third one is really having agility to not only test and learn but to really, you know, kickstart activities, especially given the fact that we are launched and a new category in the form of federal vaccination, really learn a lot along the way. Make those changes in a nimble-footed manner to further improve the offerings. Underpinning all of that is a culture were aligned with our global values, we do everything possible to have a culture where our people can thrive.

Moving straight to the external performance and in terms of seeing how our competitiveness has played out for

Q1 this is all data for Q1 as you would see at the bottom. So, for the general medicine's portfolio, we've delivered in unit terms you know, in line with the markets, our evolution index in units, which is the closest that you would see to the metric of descriptions in terms of relation is at a GSK India
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hundred. But I think what's more reassuring is if you look at across the board, we not only have volume EIS, but even after the impact that we saw exactly a year ago on our portfolio mitigated to a large extent. And that's that will be taken up in terms of the, how it played out in our financial performance. But just to give you a quick sense of

the key brands that we have in our portfolio, across the portfolio, you would see healthy Evolution Index in excess of hundred.

What is earmarked as 90 is one of our key brands, which was the most impacted last year, but even on that one, the volume EI have already gone up to almost 131, which tells you how significantly better we've done compared to the market. The same story continues for our P ■ vaccine business over the last four successive quarters. Now, we've not only stabilized, but grown that business to double digit growth. And that's why if you look at the pediatric vaccines business our unit evolution index is ahead of the market at 103. One of the significant things that we had done in Q4 of the last financial year was the transformation, including putting up a new omnichannel strategy. And just to give you a flavour of what has really happened in the first quarter we've clearly supplemented and complimented our face■to■ face interactions with our healthcare practitioners, with digital touch points. And just to give you a sense of the kind of needle that have almost had a 20% jump in terms of the touch points that we have with healthcare practitioners, apart from the face■to■face interactions. So clearly, we are opening up several channels apart from the face■to■face channel that a rep goes and meets a healthcare practitioner using digital ways and accelerating that digital adoption in terms of our interactions with these healthcare practitioners. At an
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overall level, if you look at the number of healthcare practitioners that we've touched, we almost automated five digital channels including channels like Emailers, WhatsApp to reach out to all those consented physicians who are consented to be allowed to be reached by allowing us to have almost more than 7.6 million digital touch points.

Just a quick sense of what has changed in the first quarter for our pediatric vaccine business is a script two■minute video that we want to show.

Video plays

Mr. Bhushan Akshikar: This was just an example of showing how we continue to innovate to be already the leader in the periodic vaccination, self■pay private market. But we are opening up a whole new segment in the second year of vaccination with that campaign building on top of our foundation of periodic vaccination, we continue to build new energy in creating the adult vaccination ecosystem. If you recall had talked in the last industry meeting, the need to really build this category given the demographic dividend that we have in this nation,

a target

addressable market continues to be in excess of almost 12 million Indians spread across those 60 cities who can potentially benefit with shingles prevention. So, at all three arms, which means building a great ecosystem with healthcare practitioners getting prevention as an equal part to treatment in the healthcare practitioners routine with patients is an area. So clearly, we've had a 70% growth versus the last quarter. And more importantly, more and more healthcare practitioners continue to believe that vaccine shingles prevention and vaccination, therefore is an important part across specialty, we
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continue to build on the consumer awareness campaigns through the digital awareness campaigns that we launched last year. And we almost have a reach of 55 million spread across all social, digital, social platforms.

The real metric is to have how many consumers have those conversations with the healthcare practitioners. And now from almost a zero base, we have almost 10% of those consumers based on two panels that we have in place, having that conversation with their healthcare practitioners, asking about shingles prevention and the next steps towards vaccination. As I said in the earlier two calls of

building an ecosystem for other vaccination is a strategic and a long term you know, approach. And with every month, every quarter, we are seeing progress being made there. Last, but not the least, is in a country where we have almost non-existent adult vaccination channels. We are also putting up partnership programs with the right partners, including hospitals corporate chain clinics with detailed joint business plans. So that's a big update on the Shingrix launch over the last one year, as we close out Q1. We continue to use different platforms throughout the year, really bring about the importance of why it is necessary to prevent a painful condition like shingles. So, this, just to give you a couple of examples in terms of how we've stayed relevant in the last quarter. With that, I think I will hand over all the financials to Juby.

Mr. Juby Chandy: Thank you, Bhushan. Okay. I'd just like to start by highlighting some of the quality of our growth in the revenue profile. As Bhushan highlighted in the last couple of quarters, you could see that there is an improved focus on where to plan, how to plan that. You could see in across the lines of
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our sales growth profile. So overall growth around 10 percentage. And the key brands where we put almost 80, 90% of our sources. That's also growing around 10 percentage. Specialty business, it's a small base. We have Nucala enterology in that specialty business. These are the two new assets, which is growing almost 57 percentage. So, it's growing much faster than the bigger base business. Vaccine, as you have seen some of the new innovative marketing methods, we are trying to reach out to the doctors and to the patients with increased consumer awareness or patient awareness. Vaccine business is growing around eight percentage, and Shingrix again, focus on how to drive immunization and the benefits of that through different GTM strategies. That's also helping us.

Now I'll spend a bit more time on EBITDA margins. EBITDA margins is something which we've been talking

about in the last two quarters. As we speak this quarter, we are talking about 229 for our EBITDA, which is 62% growth and 28% margin ratio. And this margin is almost 920 basis point. That is nine percentage points improved versus last year. And last year, as you remember, we had the biggest impact on NLEM around the same time, and we have been impacted almost nine to 10 percentage on our top line in terms of margins. And that was the quarter number lasted. And over the last one year, we have taken many initiatives to improve upon that margin. So, over the last two, three quarters, we have been seeing consistent growth on the margins. This is on the backdrop of optimizing a price, negotiating on better raw material cost. And pricing productivity has improved, which means ahead of medical rep that are blocking more sales and optimizing our field force and that is resulting in savings on this genre and base. So overall, with multiple levers playing out, our margin profile has improved to 28%, which is nine percentage points improved
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over last year. And the same trend you could see on PAT, profit after tax. And that's also growing close to 39 percentage. The other key highlights of this quarter are almost a hundred percent of our profits are converted to cash. We have a very healthy cash position. The cash position working capital also has improved consistently over the last few quarters.

Let's move to the next slide. So, this slide gives you the flavor of how we have been progressing on our EBITDA margins over the last couple of quarters, we were lowest same time last year, at 19 percentage, there was some one-offs of roughly two percentage. So underlying EBITDA margin, same time last year was around 20-21 percentage. From there, we have moved to 28 percentage, and we are seeing a stabilization in our margin profile. And complete impact on NLEM, which we have seen earlier last year, has been mitigated. As I highlighted, there are many initiatives we were rolling out during the last year, including our digital volume focus, cost containment, all are playing out. So as we speak, we

are at a very stabilized stage in terms of EBITDA margin, and most of the low-hanging fruits have been addressed and we would see continuing to maintain this level of margin as we go ahead in the next part of the, so with that, maybe Bhushan, we can close the meeting and we can take questions.

Mr. Bhushan Akshikar: Sure. The last slide, so just to summarize this was largely to talk about our Q1 performance in terms of how we have set out for the financial year, but more importantly, building on the momentum that we already created over the last three quarters. And to summarize therefore in the medium to long term, our ambition continues to be able to touch the lives of a billion Indians. And that's something that we are focused on. As we celebrate our hundred years of operations
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this year in November, the idea is really to ensure that we continue to deliver what we've said in terms of not only touching the lives of a billion Indians by growing in double digit, but more importantly having new platforms of growth to drive that growth and most importantly defend the competitive performance and continue to grow base business profitably. So that's, I think that's the summary of what we had to share

on this call, but I think the whole intent is to have you know, answer your questions and have a two-way conversation. So, I think if we can open up the floor.

Ransom Dsouza: Thank you, Bhushan. Moderator will now take questions for those of you on the call. Kindly raise your hand and the moderator will unmute you so that you can ask a question. The other option is for those of you who would not like to ask an audio question, you can also type your question in the Q&A chat box, and we'll pick that up so we can have a first name. Moderator over to you.

Ransom Dsouza: Can we take the first question?

Nikhil: Yeah. Hi good evening. Nikhil here from Simple. Just two questions. This is one is on to Juby. Earlier there was a difference between the standalone and consolidated numbers of around 30 crores, which is not visible this year. So, is there a change in the subsidiary as a result that differentiation is not there, or is there some alignment? If you can just explain, because otherwise year on year numbers don't seem to be comparable.

Mr. Juby Chandy: Thank you. Thank you for that question. I think there's not change in the subsidiary. So, the subsidiary has got only one main business, which is we sell AI Penta sole to WHO organization, roughly around 50 to 60 crore per annum sales we make to WHO on AI Penta sole business. Okay. So now
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there is a phasing impact. So, this total number doesn't change over the full year, but this quarter it's been lower building. So standalone may be the right benchmark to see for the quarter, but end of the year, towards the end of the year, you will see that full numbers coming through. So, this quarter cell lower building, that's where you're seeing that two, three percentage deltas when you see the consolidated growth profile. But that's only a phasing impact. That's just going to happen in the coming quarters.

Nikhil: Sure. second question was on two of our specialty products globally. We've seen a label extension on Shingrix and Jemperli. Now what I understand, Shingrix, we've already launched, and we were planning to launch Jemperli and Zejula in India. If we have to launch with the extended label extension, do we need to do the whole trials and everything in from zero or how does it work for us? And any timelines when we are looking at the launches?

Mr. Bhushan Akshikar: So, thank you very much for that question, Mr. Nikhil. Yes, we've already launched in weeks, so for us, it doesn't really in fact, from the label that we already have for temporally, yes, we have the mark. As I said in the last call, we have the marketing modernization, and as we speak, we are in the final stages of what that go to market plan will look like in terms of commercializing Jemperli. We have data on only one indication, which is second line. Currently we are in talks with the regulatory agency in terms of

what it would take, but just to let you know, we have an ongoing clinical trial already. So, India is a part of many global clinical trials. So, some of that data might be able to use if you were to push for the first line. So that's where we stand as of now, but that doesn't have any bearing on our commercial ambition at least to begin with in terms of the launch phase.

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Nikhil: Okay, sure. I'll come back in the thanks.

Mr. Bhushan Akshikar: Thank you.

Ransom Dsouza: We have time for any more q

uestions. Kindly raise your hand so that we can then unmute you. The moderator can unmute you. Anyone would like to ask a question. Kindly raise your hand.

Mr. Bhushan Akshikar: Can also check whether there are questions in the chat box.

Ransom Dsouza: If you have a question in the chat box, please type that. Please raise your hand if you'd like to ask a question. Anyone else would like to ask a question?

Kindly raise your hand. Bhushan and Juby will be happy to take questions.

Yeah, there's a question from Deepak. Can you please outline the new launches for FY 2055?

Mr. Bhushan Akshikar: Sure. Thank you very much, Mr. Deepak. So first and foremost, we continue to contemporize our portfolio just as we launched the new extensions for some of our big assets like Calpol. Last year, if you remember the last financial year, I had shown a couple of videos. Also, we launched the new versions of CALPOL Six 50 plus CALPOL 500 plus with a new optizorb technology. So, we continue to scout for opportunities to extend the lifecycle with new launches for our existing assets. And that includes the gen meds portfolio. In terms of pulling in from our innovation assets, I talked about it, the oncology assets, there are two oncology assets, namely Zejula and Jemperli. Currently, the indications are gynecological malignancies, both ovarian and endometrial cancer. So those two would go off in this financial year.

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Ransom Dsouza: Thank you, Bhushan. Any other questions? Please type it in the chat box, or please raise your hand. Can we expect the employee cost run? So, there is a question from Vishal, will the cost base remain the same? Now is a question from Vishal Manchanda. Okay. The answer is yes, but I think I'll ask Juby to give some comment.

Mr. Juby Chandy: Yes. So, I think the answer is yes, we are not having any new increased headcount having in the rest of the year. So, which means the headcount run rate will remain same as a percentage of sales, it'll remain same.

Ransom Dsouza: Okay. The follow-up question from Vishal. Can you give more color on Shingrix vaccine in terms of birth unit price, growth prospects and dementia trial?

Mr. Bhushan Akshikar: Sure. Okay. Thank you very much for the question. Mr. Viraj, yes. So as aligned with the global strategies, but more importantly, customized to the local needs, we have an India specific pricing as I mentioned in India earlier calls as well. So, we have an India specific pricing for Shingrix. But more importantly you know, these two doses of Shingrix vaccine provides shingles prevention. And now we have data for almost 11, 12 years, which means if you have these two shots you know, protected for the next 11, 12 years in terms of not having this painful condition of herpes roster. In terms of giving you color, I think performance wise, we are improving every single month. Even within the quarter, we have seen improvements. So typically, now we are touching the lives of almost 7,000 patients every month in terms of preventing vaccines. Clearly with every passing month, they're only gaining confidence as well as improving the ecosystem to prevent you know, this kind of preventable disease in that adult population. The third part of your question is about, I think, yes, there is a new clinical data available, which potentially you know, makes a point

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about preventing

dementia in a certain section subsection of, again, these are not GSK led clinical trials. So, you know, if you want more details, I can always ask our clinical team, our medical team, or our communications team to respond appropriately. But yes, there's been a couple of reports, including one that was published in the Nature Journal about potential prevention of dementia in population that got the Shingrix vaccine.

Ransom Dsouza: Thank you, Bhushan. The next question is from Gokul Maheshwari. Can you indicate the average price hikes taken in the non NLEM portfolio X of vaccines?

Mr. Juby Chandy: So, the average price hike depends on the timing of the pricing we've taken in the previous years. So roughly it is four to five percentage on the non NLEM portfolio.

Ransom Dsouza: Okay, thank you, Juby.

Mr. Bhushan Akshikar: But again, in a calendar year of 12 months, so obviously it's a blended average that is giving, there might be some, which might be lower.

Ransom Dsouza: That's right. Great. The next one is from Tushar. Can you share the price volume data for mass medicines?

Mr. Juby Chandy: So, what exactly is price? How much is price?

Mr. Bhushan Akshikar: I think how much is volume is worth?

Mr. Juby Chandy: Yeah, so roughly the whole, like I said, four to five percentage, 4.5 percentage, if you take it as a price remaining is volume. So, we are talking about 7% volume and four percentage, 4.5 percentage price, and a

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0.5% mix decrease. That's, how it is, the blended growth of 10 percentage.

Ransom Dsouza: The next question is from Viraj Mithani, colour on RXV about launch in India. And what are the requirements for launch in India?

Mr. Bhushan Akshikar: Thank you very much, Viraj. So, as you would expect all the products that are approved in the US and the European Medicines Evaluation Agency, which is the EMEA we still have to do clinical trials. The good news is that India for within GSK is an equally important for location for doing clinical trials. So, most of our assets now, we have almost 19 clinical trials happening from the global landscape in India. So, the process remains the same, for RXV, we will still have to do a clinical trial unless of course, regulations change otherwise but we've already got the clearance from the subject expert committee. The data is available in the public domain on the website of regulatory agencies. And we should be kicking off the clinical trial for this asset.

Ransom Dsouza: This follow on question is, when are you planning to launch Arexvy in India?

Mr. Bhushan Akshikar: So, it is a function of a clinical trial. So, as I said, the trials have just got approved. The estimate is, as you see with, in any case, it takes about typically 18 months at least from the time we initiate the trial still launch, we'll keep the regulators engaged and in informed on the progress so that we can get this innovative asset sooner than later.

Ransom Dsouza: Thank you, Bhushan. One more question. Is any guidance on FI 25 in terms of top line and bottom line margin, top line growth and bottom line margins?

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Mr. Juby Chandy: So, we don't typically give guidance for the full year, but we are expecting the same trend to continue for the rest of the quarter. Also, in terms both on margins as well as sales growth.

Mr. Bhushan Akshikar: I just give some more color to that. I think if y

ou've seen our portfolio,

we have you know, this was Q1. Q1 is not typically the highest quarter for us, even the dependence of some of our portfolio on the seasonality. So, we expect to continue the same momentum in Q2 and beyond. So that's probably aligned with what you just said.

Ransom Dsouza: Okay. Anyone else wants to raise your hand, ask a question? We have about

10 minutes or so. Okay. Vishal again, can you share a timeline around the launch of a bepirovirsen hepatitis b drug and gepotidacin in uncomplicated UTI?

Mr. Bhushan Akshikar: Thank you very much for that question. So, the good news is, all the assets that you mentioned, we are undergoing clinical trials in India. So gepotidacin is currently undergoing the clinical trials in India with several centers, including some of the leading liver treatment centers in India. Now potentially, as we all know, this can be the first functional cure for chronic Hep B. So obviously there is a, being a part of the global clinical trial, this has to concurrently get approved elsewhere. And that's why we are engaging with the regulators. Also, given the nature of this asset, we will ensure that we don't lose our time. And the good news, as I said, this trial is a part of global clinical trial. So as, and when it is approved in the US or other regulatory agencies, we should be in a position to launch sooner, but the trial will get over the end of this year. Gepo, I mean, not all clinical trials will be relevant. So Gepo, yes, we potentially have the approval, if you wish but clearly given where we sit in the current treatment

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modalities with some of, including what we already have in our portfolio with, for example, with assets like Cefuroxime you might not find the best fit in terms of what assets we want to bring into India. So gepotidacin is a commercial decision whether we will actually go ahead and launch it at the end of the day. But the other ones, whatever assets we think are relevant and fit for purpose in terms of bringing and serving the needs of Indian population, those are the ones that we will definitely accelerate.

Ransom Dsouza: 24 valid pneumococcal vaccine in India.

Mr. Bhushan Akshikar: Yeah. So, yes even the trials for what is called the M.A.P.S. is something on our radar, which is the something, you know, because if you recall we had some great success. We continue to have our PCV asset available to prevent pneumonia. So, the M.A.P.S. are the next one, which is it's not 24, it's 30 valid, actually. So that's something that we are also ensuring that India is on that roadmap. But as I said, given the nature of the clinical trials, given the engagement with the regulators, that's at least 18 months away.

Ransom Dsouza: Mr. Abdul Kader has two questions. Please provide some color on the performance of your key therapies in pharma segment, and then I'll take the next step.

Mr. Bhushan Akshikar: Thank you, Mr. Abdul Kader. So, the first one, if you see the external reflections, the pharmaceutical market for Q1 grew by about 8.6%, 8.5 percent. And if you were to split that between acute and chronic, acute grew lower, the acute growth acute segments grew up about 6% and if you look at our split, our portfolio is largely you know, acute and in spite of a 6% growth, externally, we've delivered a growth with agile medicines business of about double digit. And this

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is not a peak season quarter. So clearly as we move ahead, we expect to continue the same momentum. So that's the first response to your question. I think if I were to characterize the growth pattern of what we've delivered versus competition, as well as the therapies where we operate, be it anti-infective is bit gums if you saw the data points in my slide evolution index is more than a hundred across the board. So, we've delivered a better performance for Q1 compared to others in all therapy areas.

Ransom Dsouza: His following question is, what's the contribution of vaccines in the quarter and the underlying growth in Infanrix hexa, averages and value?

Mr. Bhushan Akshikar: Sure. So, the first question was underlying growth or...

Ransom Dsouza: Its contribution of vaccines in the quarter.

Mr. Bhushan Akshikar: You want to take that Juby?

Mr. Juby Chandy: So, contribution of vaccine continues remain same, roughly 21 percentage of the total number is vaccines. What's the next question?

Mr. Bhushan Akshikar: The underlying growth of Havrix, Infanrix, I had it on my slide, but if you

can just quickly recall have, I think Havrix boost some of these assets have grown in significantly high double digit. Infanrix hexa has been a single digit, and that's something that the team is working on to ensure that we get back to a double digit growth there as well.

Ransom Dsouza: Viraj Mithani says, colour on export, in terms of target markets, especially USA, Europe growth prospects.

Mr. Bhushan Akshikar: Our Indian business is largely made in India, for India. So, as you know, we have our own manufacturing facility in Nashik. Plus, we have GSK India

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more than 20 CMOs whom we work very closely. Clearly this, the model that we have is for India but that's what we will follow. As of now, I don't see us really going after external markets and using India as a sourcing base, at least not in this financial year.

Ransom Dsouza: Okay. The follow-on question is, which is the best quarter for us?

Mr. Juby Chandy: So, the best quarter typically for all pharmaceutical companies are the monsoon season, which is the second quarter of the financial year, and followed by the third quarter of the financial year. So, second and third quarters are the best quarters.

Ransom Dsouza: Next one is from Nikhil.

Ransom Dsouza: For Ceftum and T▯bact. What would be our volume share pre NLEM and now?

Mr. Bhushan Akshikar: So, for Ceftum, for example, we have gained, if you look at Q1 performance, we have gained almost 8.5% volume share. So, you can imagine the kind of impact that we have nullified in spite of almost a 57% drop in the prices. Our for▯value EI are almost close to 95% now.

So that's the share gain that we're talking of. I think even for the Q1, our volumes have grown in excess of 45% for Ceftum. Similarly, for T▯bact, we continue to maintain our leadership position in the not just the Mupirocin market, but the wider topical anti▯infective market.

And even there, we have gained 2% market share, and we have completely mitigated the impact of NLEM and even the value is now in excess of hundred.

Ransom Dsouza: Thank you, Bhushan. Next one is from Gokul Maheshwari. Will your growth be higher in FY'25, given the low base of NLEM drugs into vaccines ceftum in GSK India

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FY'24? The impact of lower price in FY'24 will be offset by higher volumes

in

FY'25?

Mr. Juby Chandy: Yes, that's right. So, the FY'24 and lower growth because of NLEM. So, this year we, for the last two quarters, if you are observing a number, it's striking close to double digit growth. So, we'll see the same trend continuing, but last year it was a single digit growth.

Ransom Dsouza: Next question is from Vishal Manchanda. Can Jemperli be used off▯label in first line setting, and would you have a number as to how many cases of advanced endometrial cancer are diagnosed in India?

Mr. Bhushan Akshikar: So, first and foremost, we would not know as an ethical company, obviously, we recommend only a full indication for all our assets, and that includes Jemperli. As I said, the marketing authorization that we have got is only for second line to begin with. We are seeing ways and means to see how we can engage with the regulators for the first line. Endometrial cancer incidence data depending on what sources you see. But there are typically at any given point of time, about 30,000 patients who come into the system. So, the idea is to ensure that we provide the best▯in▯class treatment and with the data point that's in terms of the readouts that we already have.

Ransom Dsouza: Next question is from Virat Mithani, would we have any impact of barren Zantac case? And if so, how are we preparing ourselves?

Mr. Bhushan Akshikar: So, when you say impact that clearly, we don't have Zantac available in India. So, I think that's something that we've already moved away from for the last few years now. As you must be following cases, the case is GSK India

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already being taken up. So, the legal team globally is involved, in ensuring the answer is we don't see any impact locally here for India. And just to let you know, the molecule continues to be sold by Indian companies. So, in India, that molecule is still sold and widely available. So, the regulators have not done anything as of now regarding the availability of this product.

Ransom Dsouza: Great. and okay, Tushar, can you please share the timeline for completing the clinical trials for Gepo?

Mr. Bhushan Akshikar: I think it's already done, if I'm not mistaken. The clinical trials are done. The commercial assessment is something that we are evaluating given the price point at where it sits. Remember, it's an in licensed asset. So, as I said earlier, the asset has to make sense in terms of the cost of treatment. But the trial is done. It's about really assessing whether this makes financial sense as well as it is bringing real value to the patients in India. So, as I said earlier, not all trials we will look at with the same energy. I think there are some assets which will prioritize Gepo clearly undergoing a commercial value.

Ransom Dsouza: Thank you, Bhushan. Nikhil, you raised your hand, or is that an earlier hand?

Nikhil: Yeah, I have one question.

Ransom Dsouza: Yes. Go ahead.

Nikhil: See we have a good specialty of Janumet segment and the vaccines. And in the specialty, we are focusing on launching molecules from the parent pipeline. But if we do a mapping of the therapies, we focus on versus what the parent is focusing on, would it be a hundred percent mapping? And if not, would you look at acquisition as a possibility to GSK India

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build up that pipeline or bad debt therapy mix in India? Otherwise, what is the point of keeping so much cash at the compa

ny level?

Mr. Bhushan Akshikar: It's a very good question, Mr. Nikhil, and I think I'll probably answer your question with the help of one data point, which is the launch of Trelegy. It's a global asset. It's one of the largest products for us in the non-vaccine, in the medicines category. And we launched this 18 months ago. We really put some energy. So, we have expanded our field force. We put up robust medical marketing plans now, and even within the fast moving, fast growing triple category, you know, this the single inhalation triple treatment category, which means you just need one device. You don't have to have multiple devices. We are one of the fastest growing brands. We are already got a market share of almost 6% in the last six months since we've long, since we've put some energy. And guidance, if you are able to get at the growth end of the market assets like Trelegy, it makes immense sense, yes to the question that you asked. Are we open to acquisitions, and you know, inorganic growth? We've always looked at opportunities, but those have to make sense in terms of the complementarity that those assets might potentially have with what we bring to the market. So not always we find the best fit, both from a governance compliance standpoint as well as the novelty that some of these assets might bring in the future. So, it's a mix of both, I would say. I know it's not the best answer, but clearly, we would love question, take our strengths more than just launching something where we are the 50th one without bringing any material value to both physicians as well as patients. You are asking something, Mr. Nikhil.

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Nikhil: Yeah. just a continuation. When we go about marketing or promoting Trelegy is it helping us with our older respiratory products as well in terms of increasing market share or better growth for them? Or is it like Trelegy is the only product in the respiratory which we are focusing on, and others have lost relevance? How should one understand versus our old?

Mr. Bhushan Akshikar: So, if you see the combination that Trelegy is today, umeclidinium, which is the only one available in Trelegy, we still have a pattern

there. So clearly there are competitors, but not with the same combination that Trelegy offers. So clearly the superiority of the device combined with the inclusion of umeclidinium, which no one else has in the market. So those are the two variables which are really setting us apart. So, although you have generic competitors available for different combinations, not the one that we have it's really helping us set us apart from the competition. So, the other question that you asked, yes, it's definitely having an impact on Nucala. So, for example, we currently have almost 800 to thousand patients who have benefited and who continue to benefit. So, every month although severe eosinophilic asthma where Nucala is indicated being a monoclonal antibody is still a niche area, I think, the ability for physicians to make right decisions, identify patients earlier on and put them on treatment like Nucala has also. So, we see the last six months, clearly good impact on both these assets because they are both together in the same team. I hope, I answered your question Mr. Nikhil.

Nikhil: Thank you.
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Ransom Dsouza: Vishal Manchanda, on Trelegy. Are there any generic competitors in India?

Mr. Bhushan Akshikar: I just answered that question.

Ransom Dsouza: So, I just wanted to close that. Vishal, we have responded to that. He says, if yes, how is Trelegy priced versus generic competitors?

Mr. Bhushan Akshikar: Well, as I said, there is no generic direct competitor for this, because umeclidinium is one of the unique patented molecules that we have in this triple combination. None of the other companies have this and if you look at the price point, we are, it's a band. Maybe we are, it's depending on whom you're comparing with, but there's a certain band in which we are operating and that's where Trelegy is right now. It's roughly about two and a half, 3000 per month.

Ransom Dsouza: Mr. Deepak, your question on the launch of RSV vaccine in India has already been responded, so that has been taken care of. We moved to the last question.

Mr. Bhushan Akshikar: Sorry. Mr. Nikhil has his hand up? Is that a new hand or?

Ransom Dsouza: No. That is an old hand. Last question is from Viraj. He says, colour on the AIDS portfolio in terms of growth and launching of new products, AIDS portfolio?

Mr. Bhushan Akshikar: No, we don't have HIV initially. I think many years ago we have given away our licenses. So, we had an arrangement with several Indian local companies where assets to our view portfolio mean licenses to our, we assets were given out. So, there's no intent to launch the HIV portfolio in India.

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Ransom Dsouza: Great. With that, we've come to the close of this meeting. Sir, closing remarks.

Mr. Bhushan Akshikar: No, no remarks. I think thanks a lot for your time as we've committed in the last investor meeting also although we don't give guidance of any kind at least for the full financial year, what we've presented to you, we still remain committed to the sense of direction and more importantly, the holding the margins at what we have, what Juby has just shared. So that's something that we remain committed to.

Ransom Dsouza: Great. Thank you all for joining in and see you next time.

Mr. Bhushan Akshikar: Thank you very much everyone.

Call: Oct 2024

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30th October 2024

To,
BSE LIMITED THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED
Phiroze Jeejeebhoy Towers Exchange Plaza, 5
th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
Mumbai - 400001 Mumbai - 400051

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of the Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 29th October 2024. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link: <https://india-pharma.gsk.com/en-in/investors/analyst-meets/>.
Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

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Call: Feb 2025

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17th February 2025

To,
BSE LIMITED THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED
Phiroze Jeejeebhoy Towers Exchange Plaza, 5
th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
Mumbai - 400001 Mumbai - 400051

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the transcript of the Analyst / Institutional Meeting held on 14th February 2025. The said transcript is also uploaded on the website of the Corporation and can be accessed through the link:
<https://india-pharma.gsk.com/en-in/investors/analyst-meets/> .
Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

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“GlaxoSmithKline Pharmaceuticals Limited Q3
Earnings Analysts Conference Call ”

February 14 , 202 5

MANAGEMENT : MR. BHUSHAN AKSHIKAR - MANAGING DIRECTOR ,
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED

AND

MR. JUBY CHANDY – CHIEF FINANCIAL OFFICER ,
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED

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February 14, 2025

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Moderator: Hi. Good evening, everyone. This is Dorwin Dias, your moderator from Chorus Call. Welcome to the GlaxoSmithKline Pharmaceuticals Limited Q3 FY2025 Earnings Call. From the management at GlaxoSmithKline Pharmaceuticals Limited, we have Mr. Bhushan Akshikar – Managing Director, GlaxoSmithKline Pharmaceuticals Limited; and Mr. Juby Chandy – Chief Financial Officer, GlaxoSmithKline Pharmaceuticals Limited.

By participating in this event, you consent to the recording, distribution and publication of this event. Kindly note that this call is meant for investors and analysts only.

All participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation from the management concludes.

I now hand the conference over to Mr. Bhushan Akshikar. Thank you, and over to you, Sir.

Bhushan Akshikar: Thank you very much, Dorwin. A very good afternoon and a warm welcome to the Q3 Earnings Call. From the last time we met, end of October, we have had some significant milestones in our journey. And some of you may have seen in the video that was running in the background before we started this call. We definitely had one of the most significant milestones which was on the 12th of November celebrated aptly at the Bombay Stock Exchange to reaffirm our commitment to all stakeholders, especially the industrial community. Apart from many things that we have done throughout the year, this was really symbolic of the long-standing commitment that GSK continues to have as we step now in the second century of our operations.

If you recall, we set out a very clear strategic direction where we said [that] as a biopharma healthcare company, we operate at two ends of the spectrum: one with general medicines spanning several therapy areas, then being present in the vaccine business both pediatric and adult, and at the deep end of specialty medicines with launches that you see on the screen here.

One of the focal areas for us as an organization has been to remain focused on delivering a competitive performance and therefore delivering ahead of market performances. As you can see on the left side, most of the brands that we have in our general medicines portfolio delivered a strong Evolution Index, clearly recording a GlaxoSmithKline Pharmaceuticals Limited February 14, 2025

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higher than market delivery of our market growth. Even in categories where we didn't see the traction externally, we kind of held our market shares, as you can see, across the board. So, our general medicines portfolio has clearly driven some of the momentum in Q3 led across the board by the brands that you see.

On the pediatric vaccines business side, we continue to hold leadership in the self-paid private pediatric vaccine market, and again here too, it's been a consistent story. Over the last four quarters, we have been sharing results about not only stabilizing but growing the business and getting back to market share gains, and that's something that we have seen across the portfolio.

Clearly if you look at some of the volume growth that we have delivered both in the general medicines and the pediatric vaccine-based business, we have had some strong volume growth across the board and that's really been the highlight of Q3.

On the new products, we talked extensively to you over the last few quarters about the renewed commitment that GSK has in bringing some of these innovative assets into India. Nucala continues to be one of the key brands indicated for severe eosinophilic asthma. And of course, we have the other asset in the single inhalation triple therapy, which is the fastest growing in the Broncho inhalation space. So, both of these continue to grow from strength-to-strength, month after month, and Q3 has been specifically highlighted in terms of the new patients that we have been able to offer these innovative assets to.

So, if you go to the next slide, clearly, we believe that apart from maintaining and sustaining the growth momentum in our base business, it's new growth platforms, namely adult vaccination led by Shingles awareness. We have done some innovative things in Q3. As you can see in that screen, the whole focus has been to improve awareness, to educate more and more consumers, to understand the risk factors associated with Shingles and spread awareness around Shingles prevention. And we have done some innovative campaigns in Q3, which I am sure many consumers must have seen, including the industry community on both TVCs as well as social digital platforms.

So, as we step into the second century, we remain focused as a management team on delivering top-line growth and delivering, of course, returns to our shareholders and

that will remain the commitment on behalf of management. We will spend a few minutes looking at the financials which we just released and then we will open it up for questions. Juby?

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Juby Chandy: Thank you, Bhushan. So, we have got a very good set of results to share. If you see the numbers which were released to the stock exchanges, both on revenue as well as on EBITDA side, margin side, we have seen significant progress over the last quarter. Revenue has been growing 18%. If you see what are the drivers of this revenue, all the business segments, general medicine business segment, vaccines, as well as the specialty portfolio, all have grown double digits. Strong performance in general medicines, which has grown 11% underlying volume. Specialty portfolio has been growing close to 37%, and pediatric vaccines has been stabilized with a growth of 15%. So, you could see all the segments in our business have been growing double digit. Continued focused on Shingles awareness is helping us to maintain our disease awareness campaigns in the market. EBITDA is 290 crore, growth of 33% with a margin profile of 30.7 %, which is an improvement of 370 basis points from previous year. As well you would have seen from the results the SG&A ratio, the expense ratio has improved by almost 2% of sales on continuing to dial up the efficiencies which we have in the business which we have started close to two years back and we have been working on that the last many quarters. Profit After Tax is maintaining the same trend with 350 basis point improvement, and this quarter we are sitting with a very healthy cash flow and our cash position is very strong with 100% conversion of our profit into cash. And as you know, we are a debt free company, and we are holding a significant amount of cash as on the balance sheet date.

Year-to-date December [is] also tracking the very similar trend. Growth of 10%, profit growth of 30%, margin profile of 30.4 with a margin improvement of 450 basis points. Profit After Tax is a growth of 31% and a margin profile of 24%. That is also margin profile improvement of 380 basis points. Across the business we have been growing with strong efficiency management on the expense side, field productivity improved and cash flows maintained.

With that, perhaps we can open for the questions. You would have seen the results are all up on the stock exchanges and we can open up the floor for questions.

Moderator: Thank you. We will now begin the question-and-answer session. You can choose to ask your questions in two ways, on video or by typing it in the chat box provided below. In

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case you would like to ask a video question, please press the 'Ask a Video Question' tab and follow the instructions to join the queue. By clicking on as 'Join as Attendee', you will be on audio only

y. By 'Joining as Panelist', you will be on audio and video.

Before asking the question to the management, please introduce yourself, providing your name and your organization name. Please limit yourself to a maximum of two questions so we can accommodate as many as possible. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

Once again, ladies and gentlemen, please press the 'Ask a Video Question' tab and follow the instructions to join the queue. You can also join by clicking on 'Join as Attendee'. You will be joined and on audio only. By 'Joining as Panelist', you will be joined and on audio and video.

Mr. Jay Modi, please accept the prompt on your screen and go ahead with your question.

Jay Modi: Yes. So, congratulations on a great set of numbers. See, I have a question around GenMed. So, our volume growth for the quarter was around 11 % and nine months would be around 9 -10%. So, have you taken any price hike in this segment or is there any element of pricing impact for the quarter and nine months in the Gen Med segment?

Bhushan Akshikar: There is close to 3% blended price increase in that segment. So, yes. So, there is a 3% price increase.

Jay Modi: And see, the employee cost for the quarter was lower by around 25 crores right now. How should we look at employee cost going forward? Is this the base or was there any sort of one-off?

Bhushan Akshikar: There is a one-off in the employment cost close to 9 crores. That is because some of the incentives were lower for some brands. That is why, but it is, there is a one-off of 9 crore in that one.

Jay Modi: And the reason for other expense being higher?

Bhushan Akshikar: Sorry, reason for the expense?

Jay Modi: Being higher for the quarter?

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Bhushan Akshikar: That's because of the phasing of the advertisement and promotional spend, as well as there is certain element of CSR phasing also. That's why. But year- to-date numbers tracking the trend for the expense.

Jay Modi: Okay, got it, got it. And, Sir , secondly, you mentioned that vaccines had a growth of around 15%. So, is this for the vaccine's portfolio, including of Shingrix or it's only the pediatric vaccine?

Bhushan Akshikar: It's including Shingrix 15%, total vaccines portfolio.

Jay Modi: Okay. And what would be this growth for 9 months?

Bhushan Akshikar: So, growth for 9 months, just hold on.

Juby Chandy: It's in the same range.

Bhushan Akshikar: Yes.

Jay Modi: Okay. Thank you.

Bhushan Akshikar: So, growth for 9 months is 14%.

Jay Modi: Okay, got it. Thank you, Sir .

Moderator: Thank you. We have the next question from PM. We request you to please accept the prompt on your screen. Unmute your audio and video and proceed. PM, we request you to please unmute your audio.

PM: Sir, how do you see the next year in terms of going ahead and how do you in terms of growth in terms of Nucala as well as the other drugs going forward in terms of oncology, if you can give the guidance?

Bhushan Akshikar: Sure. So, first of all, as I have said in every call after our quarterly results, we remain focused on, one, delivering sustained performance both top -line and our EBITDA. So, that remains consistent even in the year that we step into after we close our Q4.

Yes, we will have definitely the momentum in the form of some of these additional growth levers or growth platforms as we call them, both on the specialty side including oncology. So, first and foremost, if you look at our country, we have more than 35 million patients both of COPD and a sthma each.

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And if you look at where one of our assets strategy operates, globally, it continues to be one of the biggest and for us biggest asset for us in our general medicines portfolio. And

if you look at even in the Indian market context, this is the fastest growing therapy within the Bronco Inhalation space.

If you look at the 12- month basis or even on a quarterly basis for Q3 , we have growth in excess of 80%. In a short time, this category is almost touching 100 crores now. And that's where a brand like Trelegy , which has already got a 5, 5.5% market share, we are confident of building on that platform.

Similarly, for Nucala, which operates in one third of the asthma market, because it's indicated for severe eosinophilic asthma, we are still talking significant numbers here.

So, both these brands remain critical to our, you know, ' 25-26 performance.

On top of that, as I said in the past, we have got marketing authori sation for two of our assets which we intend to bring to market in the coming couple of quarters latest, and definitely these two assets, currently the indications that we have are in gynecological malignancies. One of the assets is indicated for endometrial cancer and the other one is for ovarian.

So, those will be two assets which will definitely get factored in, which will help us drive our growth ambition because something that we have always said is our relentless focus on delivering top -line growth. So, that's how I would look at it here.

PM: And Sir, just any timeline on when the products will be launched?

Bhushan Akshikar: We will definitely launch it in either end of Q1 of the next year or earliest of Q1 of the next financial year or latest Q2.

PM: So, both the products you mean, right?

Bhushan Akshikar: That's right.

PM: Thank you, Sir . I will come back in the queue.

Moderator: Thank you. The next question is from Ravi Purohit. We request you to please accept the prompt on your screen, unmute your audio and video and proceed with your question.

Ravi Purohit: Yes, hi. First of all, congratulations, Mr. Akshikar. I think you have been one of the few MDs of this company, who when you joined the company, I think, put out in your

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communication to investors in the annual report that you will be targeting double -digit sales growth. And the first nine months is kind of an indication of that, I think, and hopefully we will be able to kind of, you know, maintain this growth momentum.

So, you know, Sir, if you could kind of give us some sense about what other areas are we looking at to kind of drive double -digit growth over the medium term, right? You mentioned about Nucala and Trelegy, of course. But are there more products in the pipeline?

And of course, you also mentioned about these two cancer drugs, but is there more in the pipeline that we are looking at working with or are we also at the same time looking at inorganic opportunities or in-licensing opportunities? So, if you could just kind of provide a medium-term path as to how you visualize, you know, growth is coming for our company.

And also if you could spend some time on, you know, things or initiatives that we might have taken on the general medicines side, because that has also shown pretty strong volume growth. Is it because of Ceftum or some of the products which had taken severe price knocks last year and then NLEM, they have started kind of showing volume growth.

So, if you could kind of, you know, highlight each of these aspects in a slightly more detailed manner, it would be helpful for us. We get like four times in a year to kind of hear you.

Bhushan Akshikar: Thank you very much, Mr. Purohit. I am glad you remember the context that I had set out 2.5 years back. Yes, we remain consistently focused on that objective. As I said, in every meeting, we don't give guidance. But I think in terms of top-line growth ambition, we remain absolutely laser focused.

I will answer your last question first in terms of what really helped us in this quarter. And if

you see, as I said, it's volume growth. Juby just talked about the strong double -digit volume growth that we had in our general medicine business.

So, clearly, in spite of the fact that this is not price-led, this is largely volume-led, is because of some of the initiatives that we had put in, in the last three to four quarters. One of the things that I have been talking about is digital acceleration or ability to really create touch points beyond the face-to-face interactions that our sales teams have with healthcare practitioners.

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So, in fact, in spite of optimization of our field force, we have actually gone ahead and increased the touch points in the last two quarters with consented healthcare practitioners in terms of offering messaging, in terms of offering medical education through channels of their choice and their preference at the timings that they want. So, that's one area which is definitely unlocking value for us across some of the key assets that you talked of. That's one for sure.

Second is some of the science that we have got in our activities. So, we have something called as the India Infection Index which really helps us talk extensively about appropriate usage of antibiotics, both oral and topical, essentially to again improve clinical outcomes but also give the decision of choosing the right antibacterial and therefore also contributing to prevent antimicrobial resistance. So, I think there are very specific actions that have contributed. All of them have got together to deliver this strong volume-led performance.

Your question around the future, so clearly, as I said, I have said several times before that we have about 16 global clinical trials happening in India and just to give an example of one of the assets, now Jemperli as we know, Dostarlimab, it's a PD-1 inhibitor, now that's indicated currently for endometrial cancer, second line, which is among the top three gynecological malignancies in this country.

But at the same time, there are trials that will happen in several different cancers, including head and neck, including non-small cell lung cancer, including colon. So, each of these indications will open up different revenue streams. But again, that's over a period of time. That's not for the immediate here and now.

In the next 12 to 18 months, we are completely focused and continue to invest in building our adult vaccination category. As I said, this is an endurance sport in a country where adult vaccination ecosystems do not exist. Our efforts are clearly focused on building that ecosystem so that adults above 50 years can benefit from some of these vaccines to prevent painful and dreadful conditions like Shingles and continue to lead better

quality of life. So, the strategy remains consistent with what I had shared a year ago as well as two years ago when I joined as an MD .

So, I think in terms of the future pathways, the strategic choices that we have made to build and sustain the momentum in our general medicine business, continue to stay invested and consolidate our leadership in pediatric vaccines and build new growth platforms, be it adult vaccination, be it areas like oncology or specialty respiratory. So, those are the bets that we will continue to place.

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Ravi Purohit: Great, thanks a lot. And Sir , another thing is, if we look at the overall pharmacy market in India, right, so various estimates put out the number of pharmacies in the country between, let's say, 6 lakhs to 9 lakhs, right. In that sense, is there any opportunity for us to kind of improve our penetration in that network yet? Or would you say that we are well penetrated across outside of metros and tier 1 and tier 2 and penetration across our brand portfolio is fairly saturated or would

you think there is opportunity for us to kind of expand there?

Bhushan Akshikar: So, Mr. Purohit, if you remember 12 months ago we repurposed our General Medicines team as an optimization, but also we redeployed some of our sales team. So, as we speak, we cover more than 12,000 distinct towns in the country with our feet on the street, with our sales teams. Plus we look at different channels including making availability and access for some of our iconic brands through partners, through super distributors.

So, I think, as of now, we are well penetrated given the categories that we operate in, be it the pain category or some of the anti-infective categories or the derms business where we continue to be leaders, I think we are sufficiently penetrated.

Ravi Purohit: Okay. And one bookkeeping question. We had in one of our AGMs about 3 years back or maybe 5 years back, had mentioned about a couple of land parcels that we had, I think in Hyderabad or Bangalore, if I am not wrong. And I think Juby had mentioned in one of the calls or in one of the discussions that, you know, the company is looking to monetize those land parcels and at appropriate time. So, any update that you could share? Are those on the block or they are not on the block or what's the status like? Those were fairly significant and valuable parcels of land I believe.

Bhushan Akshikar: Absolutely right, Mr. Purohit . We still continue to hold those land parcels including the one that you are alluding to which is the site in Bangalore. As of now, there is no intention to monetize that asset immediately. So, but that continues to be on the books.

Participant: Thank you so much. I will get back in the queue.

Bhushan Akshikar: Thank you, Mr. Purohit.

Moderator: Thank you. The next question is from Abdul Kader Puranwala. We request you to please accept the prompt on your screen, unmute your audio and video and proceed.

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Abdul Kader Puranwala: Sir, just first question, if I look at your quarterly revenue numbers, so while this quarter has been quite fantastic in terms of growth, but just want to have a better sense of your quarterly growth in the last 2 years, Sir , I mean, there seems to be some volatility. And if you could help us understand what causes this volatility and how should we look at this in the quarters ahead?

Bhushan Akshikar: Thank you very much, Mr. Puranwala. So, if you see, there were two big events that if you go back and see the last 3-3.5 years, one was we were significantly affected with the inclusion of some of our assets in the last round of the NLEM in terms of the drug pricing control. So, where almost 40% of our portfolio got included in the price control, and that was the one significant event. Apart from that , if you recall the second one was the periodic vaccines portfolio coming out of COVID where we had seen lesser foot falls in the periodic clinics . Those were the two ones, and I think in the last 2-2.5 years , we have been consistent about not only ensuring that we stabilize both these, but move forward with strong growth delivery and that is exactly where we are putting all our energy and focus. I think when you look at the inclusion of our brands now in the NLEM list, we are some of the brands which either two are not included are already there now. And I think we are operating the same price range . So, next time around, whenever the pricing orders come in place, we won't have those kinds of surprises. So, everything is contextual. And again, in the past, if you remember some of our assets have got withdrawn from the market. So, I think if you look at the last 3 to 5 years, there

have been some twist and turns that we have seen, but that is something that we have gone and stated to the investor community about where we want to put our energy and that is exactly what we are seeing

playing out over the last 4 quarters now. And
Juby, if you want to add something.

Juby Chandy: No, I think you kind of explained well as well there is a seasonality when you compare between quarters because monsoon season is heavy for us. So, you typically see growth tapering off on a sequential basis post -monsoon that's one thing. And last but not the least if we had to compare the margins also, we have made significant progress in terms of margin evolution mitigating most of this risk from the NL EM also, which also we would have been sharing with you past 18 quarters before right. So, that's something which we have been doing actively to mitigate those risks from the NL EM. So, all those things are playing out in the growth profiles both on revenue as well as on the EBITDA margins.

Abdul Kader Puranwala: Thanks for the detailed explanation, this is now well understood. Sir, just on two more questions from my side then. Firstly, on your MR count, so I refer to your PPT where you
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talked about 27% improvement in your MR productivity, so in the 9 months of fiscal 25. So, wanted to know first of all, I mean is there a further rationalisation which has happened in your fieldforce here or this is more because of the digital uptake what you have been talking for quite some time now?

Bhushan Akshikar: So, there is no further optimisation. The last time we came and shared with you was in Q3 of the last financial year. So, the last 12 months we have not optimised. As I said, we redeployed, repurposed our sales teams. And on top of that, unlock significant value with the digital acceleration for the healthcare universe in this country. So, that's what I would, it's clearly led by, the productivity is led by those two events.

Juby Chandy: But on year-on-year, if you remember last year, we had an optimisation that's driving this productivity improvement.

Bhushan Akshikar: After that we have not had.

Juby Chandy: Yes, after that we didn't have.

Abdul Kader Puranwala: Got it Sir. Thank you.

Moderator: Thank you. The next question is from Nikhil Upadhyay. Please accept the prompt on your screen, unmute your audio and video, and proceed with your questions.

Nikhil Upadhyay: Hi, good evening. Just one question. Two questions in fact, one was a clarification. Juby, you mentioned Rs. 9 crores of one-off in the employee cost. So, is it lower or is it a negative provision which has resulted in a lower employee cost or is it a higher provision which we have taken? Just one clarification for this quarter number?

Juby Chandy: So, sequential quarter, it is lower. From September to December quarter, it is lower.

Nikhil Upadhyay: Okay and secondly on the other expenses, now as we are preparing for these new launches and also building the market for Shingrix. One on Shingrix, incrementally when we say the growth is around 15% for the total portfolio, are we finding a little difficult in terms of bringing conversion for patients on the Shingrix platform or which is resulting in a higher spending year-on-year or is it incremental investment for preparing for the new product launches which we are planning?

Bhushan Akshikar: So, first of all, clearly when you are creating a new category which is almost non-

existent in our country like adult vaccination, there have been some investments that have gone. We remain focused as I said we remain focused on creating those adult
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vaccination ecosystems both in the healthcare practitioner clinic setup as well as the hospital or the HCO setup, and that is that something where our energy is. To the question that you asked, yes, we spent some energy

in creating awareness because in

a country where Herpes is prevalent, the risk factors are sometimes unknown especially for individuals who may have concomitant diseases like diabetes or cardiovascular. So, that is where we are creating some energy for awareness campaigns. To the point that you asked, yes, there are some -- there have been some liquid leakages in terms of the patients or the consumers who see the awareness campaigns and the ones who actually get vaccinated and that is exactly the reason why we are spending time and

energy in creating that vaccination ecosystem because unlike pediatric vaccination which again we had established 2-3 decades back, the ecosystem is almost non-existent. So, that is the way we are approaching it. With every quarter, there are new wins that we see. So, to give an example, there are 2500 healthcare practitioners who had never vaccinated adults either two who are now vaccinating consistently and that is taking a while. And as you can expect when you are creating new categories, it will require some patience and investment.

Nikhil Upadhyay: Just extension, Bhushan here. I appreciate the effort which we spent and in pediatric, we spent that effort and finally we could bring in over next 10-15 years bring in a basket of 10 to 15 products and utilize that efforts. How should one understand about the adult vaccines? And it's not about next 2 years, 3 years, but over next 5 years do you think

that we can have a basket of 4-5 products which can compensate for the costs or the investments or the platform which we are creating today?

Bhushan Akshikar: Mr. Upadhyay, that's a very good question. In fact, preventive healthcare is a significant area which is getting unlocked across the spectrum and within that space of preventive healthcare, vaccination is one such sub element and if you look at state level or even at the national level as healthcare policies get rolled out, this will be one cornerstone. So, as you rightly pointed out, this is not a 100-meter sprint. It is clearly an endurance sport. It's a long-distance endurance sport. And with the commitment that GSK has, including some of the trials that will happen that are happening right now in the country for areas in the respiratory, for example, we certainly see a bouquet of vaccines that will get into the adult vaccination space. So, it is not just around, anchored around one antigen for Shingles prevention, but there will definitely be a bouquet of antigens there.

Nikhil Upadhyay: Just one final question here. See what we see in the market over the last 4-5 years is that the efforts of creating a platform in time in kind of the pediatric and I am taking GlaxoSmithKline Pharmaceuticals Limited
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one example of the P neumococcal vaccine we created through Synflorix and then Prevenar and there were effectively two products for a long period of time, and the education and the platform creation, the hard work was done by us but eventually competition came in and those products became marginalized for us. What are the learnings which you think can be taken so as to we don't have to go through the same cycle when eventually others also come into these products? Is there a way where we can protect our investments over next 5 years, 10 years and not be the way we have seen since Synflorix happening?

Bhushan Akshikar: That is a great question, Mr. Upadhyay. Clearly as categories get commoditized with the examples that you just mentioned, the idea will definitely be to keep up scaling the portfolio looking at. So, as we speak, we have a successor for our PCV vaccine, which is undergoing, which is the MAPS vaccine undergoing clinical trials. So, clearly there are areas which we have identified where we will see still stay relevant by moving from volume and remaining focus on value. So, I think that is the only way

because as you

rightly pointed out as categories get commoditized, we will have to step up the game and look at which are the next categories which we will continue to build.

Nikhil Upadhyay: But is there a way that we can extend the lifecycle, or we can extend monetizing this investment over a longer lifecycle and where I am coming from is that in pediatrics, we got a room for 20 years to monetize. Now the way the Indian pharma market has, and the domestic companies have scaled up, do you think that leeway would be 10-15 years or that period will get shortened?

Bhushan Akshikar: The periods will get shorter, but if you look at the periodic space also, you have a 23-valent PCV vaccine that is undergoing trials globally right now. You have a Pentavalent Meningococcal Vaccine that is undergoing trials. So, the idea is obviously to stay ahead and continue to invest in research so that some of these innovative assets can be launched much earlier. You reduce the launch lag by getting these innovative assets as early as you can so that you are able to blunt some of that shortened window that you just talked of.

Nikhil Upadhyay: Okay and just one last one to Juby. So, these other expenses which we have looked at for this quarter, should this be the sustained run rate, which secular run rate, at which we will continue, or can this increase or how should we understand this in terms of along with the sales growth, as a percentage will remain same or how?

Juby Chandy: As a percentage year to date, the cumulative of three quarters remain same as a percentage of sales. Seasonality impacts, for example last quarter and this quarter, GlaxoSmithKline Pharmaceuticals Limited

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these are seasonal quarters . So, there is a phasing difference between the quarters , but year -to-date basis it will be stable.

Nikhil Upadhyay: Sure , thanks a lot . I will come back in the queue .

Moderator: Thank you. The next question is from Nitin. Please accept the prompt on your screen, unmute your audio and video and proceed. Nitin , we request you to please unmute your audio and video and proceed.

Nitin: Sir, on the super specialty portfolio, Nuc ala, Shin grix, and the T relegy . As a proportion, where do you see this portfolio reaching for us as a business over the next 3 years or 5 years? I mean, is there an aspirational n umber that you're working with?

Bhushan Akshikar: Obviously, it is a function of every category. First of all thank you very much for that question Nitin because we do talk of a freshness index in terms of what should these new growth platforms contribute as a part of the freshness index. And that is why I said the advantage for a company like ours is we have a rock -solid general medicines and pediatric vaccine business which will continue to sustain , how do you bolt on these growth platforms on top. So, each category is different. So, as I said once you start unlocking value with new indications for each of the assets that you just talked of including the ones in oncology, these should start contributing to signi■cant number.

But I mean if you have to put a number , there were these three assets that you took that you named namely Nucala, T relegy and Shingrix , they did not exist for a company, but in a in a 12 -month ■nancial year , they sho uld start creating at least 150 -200 crores in the immediate here and now. So, w hen that happens, it starts moving the needle for us for innovation and that's the intent to continue to cont ribute to that freshness index.

Nitin: Thank you. And the second question is in the market, there has been concerns around the overall IPM about the slowdown in the volume growth and some of that seems to be becoming a little more endemic versus the assumption that it's a short -term

transient situation. Given the fact that your presence across length and breadth of the

business, what is your own sense on is there any speci■c reason because of which there has been an IPM sort of a broad slowdown in the IPM and what implications does it have for businesses like us which have a la rge sort of a legacy portfolio?

Bhushan Akshikar: So, if you see the 2,50,000 crore Indian pharma market annually and break it down quarter wise also, you are talking 60,000 crores almost on a quarterly basis and the split is almost 61 % , 60%- 61%, a few and the balance is chronic. Now, obviously, the growth end is chronic, but if you look at the strategic choices that we have made as a GlaxoSmithKline Pharmaceuticals Limited

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company given the assets that we have , the sizable iconic brands that we have in our portfolio , we have at least 12 brands which are more than Rs. 100 crores each and some of them right at the top of the Indian pharma market. So, our strategic choices have been very focused on anti -infectives, dermatology , and pain . Areas which are still seeing if not double digits , high single digit growths and those are the choices that we made. We continue to contemporize the portfolio by launching line extensions for ma ny of these assets. So, in the short to medium term , we still remain focused on delivering this double -digit growth that we talked of in terms of driving the volumes and that is something that is played out well for us in the last 3 to 4 quarters. So, yes , there has de■nitely been slow down on the overall IPM, but as you seen the last 2 quarters , our growths are completely volume led.

Nitin: Thank you so much.

Moderator: Thank you. The next question is from Viraj Mithani . We request you to please accept the prompt on your screen, unmute your audio and video, and proceed. Viraj Mithani , you are not clearly audible.

Viraj Mithani: My question is answered.

Moderator: Thank you. We will move to the next question, which will be from Cogito A dvisors, we request you to please unmute your audio and video and proceed with your question.

Cogito Advisors : So, my question was how do we actually look at your business in a slightly longer term over the next 3 years? So, what I can see is that you have your older brand s, which continue to be to do reasonably well. They're growing in line of faster than the market.

They're all well -established brands. And then there's a whole slew of new products which are getting added. Do you see this as becoming bigger? And is there some thing

which is, you know, is there an underlying factor here that is the nature of the market itself changing as the country gets richer and there's a bit of a greater willingness to

pay for some of the drugs or vaccines which were not available in the country. Could you just give us a sense of as to how

you see the market itself evolving?

Bhushan Akshikar: Sure, I am sorry I didn't get your name, but clearly as I said in the beginning of the call, we continue to be one of the most broadly diversified pharmaceutical companies and the reason I say that is we do straddle in the primary care setting including having the biggest paracetamol brand in this country. So, our ability to touch almost 30 -35 crore Indians on an annual basis with brands like molecules like paracetamol or some of the dermatology or anti -infective products because of the science that we still continue to

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dole out in terms of our engagement remains at the core. When I scope that over the next 3 to 5 years, I do not see any of these molecules going out of relevance because if you look at the life stages for anyone born in India right from infancy till the geriatric population, everybody will require some of these fundamental molecules and the acids that we have there. So, I think that is the unique vantage position that we have in terms of having these big brand

s, which will only become bigger as we see moving forward.

On top of that how do we therefore, create energy and get the sharp focus for the successful launch of many of the innovative assets that I just talked about in the earlier questions as well. So, that's the real 3 to 5 years journey that we see. We will continue to have the groundswell. We continue to invest for a sustained performance of our base business. But how do you get to speed with some of the new assets that we will have coming up in the next 18 to 24 months? I talked about oncology. We do have assets in hematological malignancies as well which globally we are pursuing in a very significant way. We are also doing trials in hepatology. So, these are some therapy areas and the advantage of having many of these trials happening in India as a part of the global clinical trials only means that we can have access to these assets in a much faster way than you have seen in the past. So, that is how I would scope out and have a prognosis over the next 3 to 5 years.

Cogito Advisors: And what will happen to your margins? Because we have seen margins improved this year and is that likely to continue and therefore how much scope is there for margins to go up in future?

Bhushan Akshikar: I won't really forecast growing up, but I think our effort will be to sustain the margins because being a significant distance that we have travelled and I think where we are today, our entire energy will be to sustain these margins as we go forward.

Juby Chandy: No, I completely agree. I think our effort is to sustain we are on the top quartile of the pharma companies in terms of margin as we speak. So, many of the low -hanging fruits as well as many of the issues particularly on the raw material prices, the inefficiencies we have sorted out. So, the margins are stabilized if you see over the last couple of quarters. So, our effort is to sustain this going forward.

Cogito Advisors: Alright. And we have seen you introduce a lot of new products from your parent over the last couple of years, and you're talking about more such introductions. Has there been a change in the nature of the market which gives you the confidence to do this, which you had not done to the same extent in the past?

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Bhushan Akshikar: That is not completely correct because we used to have, if you remember we used to have a sizable oncology business in 2014 when there was a global asset deal where we sold off our assets, an asset swap deal with another multinational company globally and that was a standing start again when we divested that business a decade back, it was a range of 150 crores. So, it is just that we are rebuilding those muscles, getting the relevance of many of the innovative assets in India. To the point that you made, clearly we have populations that can afford, we have target addressable segments within our population of 1.4 billion which are increasingly focused on better quality of life. So, that is exactly where we are putting energy in accelerating some of those assets now back in the country.

Moderator: Thank you. Participants who wish to ask questions, you may please press the ' Ask a Video Question ' tab or click on ' Join as Attendee or as Panelist '. Ladies and gentlemen, we have no further questions. I would now like to hand the conference over to the management for any closing remarks.

Bhushan Akshikar: Once again, thank you very much to each one of you for your continued interest in this company. As we said, we are extremely proud of the past 100 years, but we are completely focused on the future. Obviously, the second century has already started for us and we are the first year of the second century is something where we intend to

step change, but more importantly continue to sustain the performance and continue to focus on top

ine as well as sustain the margins. Thank you very much for your time this evening. Once again, thank you and see you at the next quarterly call. Thank you.

Moderator: Thank you. On behalf of GlaxoSmithKline Pharmaceuticals Limited, we conclude this conference. Thank you for joining us. You may now disconnect your lines.

Call: May 2025

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15th May 2025

To,
BSE LIMITED THE NATIONAL STOCK EXCHANGE OF INDIA LIMITED
Phiroze Jeejeebhoy Towers Exchange Plaza, 5
th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
Mumbai - 400001 Mumbai - 400051

Dear Sirs,

Subject: Transcript of Analyst / Institutional Investor Meetings

We wish to inform you that pursuant to Regulation 30(6) of Schedule III, of the Listing Obligations and Disclosure requirements (LODR), Regulations, 2015, please find enclosed the link for video recording & transcript link uploaded on the website at <https://india-pharma.gsk.com/en-in/investors/analyst-meets/> of the Analyst / Institutional Meeting held on 13th May 2025 .
Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

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“GlaxoSmithKline Pharmaceuticals Limited
Q4 FY 2025 and Full Year Earnings Call”
May 13, 2025

MANAGEMENT : MR. BHUSHAN AKSHIKAR – MANAGING DIRECTOR –
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED
MR. JUBY CHANDY – CHIEF FINANCIAL OFFICER –
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED

Moderator: Hi, good evening, everyone. This is Zico Pereira, your moderator from Chorus Call. Welcome to the GlaxoSmithKline Pharmaceuticals Limited Q4 FY 2025 and Full Year Earnings Call. From the management at GlaxoSmithKline Pharmaceuticals Limited, we have Mr. Bhushan Akshikar, Managing Director, GlaxoSmithKline Pharmaceuticals Limited and Mr. Juby Chandy, Chief Financial Officer, GlaxoSmithKline Pharmaceuticals Limited. By participating in this event, you consent to the recording, distribution, and publication of this event. Kindly note that this call is meant for investors and analysts only.

All participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation from the management concludes. I now hand the conference over to Mr. Bhushan Akshikar. Thank you and over to you, sir.

Bhushan Akshikar: Thank you very much once again for setting that context. A very good afternoon to all the investors and the analyst community. As we commit every quarter, we just concluded our board meeting and here we are to share the results of Q4 and the full year financial year 2025.

Before I begin, I think I just want to start off from where we left last time. We continue to be one of the most broadly diversified healthcare companies operating in India. We are in our first year of our second century of commercial operations, so indeed a privilege and honor to share the full year financial results.

As we had shared, the progress continues to be broad-based as a fairly diversified commercial operation. We operate at both ends of the spectrum and if you see the general medicines portfolio, which really is the mainstay of touching millions of lives almost on a daily basis, we have had a fairly stable performance in Q4. Contrary to our expectations, the external landscape did not evolve as well as we had expected, especially in comparison to Q4 of the previous financial year.

Nevertheless, in a market that saw stunted growth, especially around acute segments of the Indian pharmaceutical market, our brands continue to hold sway across the portfolio. If you have seen our evolution index, as well as our growth, held their numbers. If you see our EIs (Evolution Index), which are the best reflection of our prescription support, continue to be ahead of 100, which means you've done better than the market, and the same story played out for our pediatric vaccine business.

Something that I had talked to you a couple of quarters back in terms of really becoming a frame of reference for accelerating our digital ways of working, we continue to move strength to strength and with this quarter, we had a significant contribution in terms of our digital strategy apart from our frontline sales reps who meet and detail and promote our products to healthcare practitioners.

We've also been able to continue to unlock value by having more than 400,000 unique HCP touch points through our omni-channel and digital strategy. So, in terms of the sheer number of touch points, we continue to move quarter to quarter and really unlock value through our digital ways.

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So, from that perspective, we clearly move to some of the main brand performances. You've seen the mainstay of our brand. So, if you look at a majority of the period between April 2024 and March 2025, Augmentin has continued to be holding the market leadership within the broad market as well as the anti-infectives segment.

Not only that, we have improved our market shares and our evolution index. So, I think the same applies to Cefixime, the same applies to T-Bact. So, our mainstay, which is the anti-infectives portfolio, in spite of a suppressed external growth of just about 3%, if you look at our volume growth, we

have really held the share as well as grown our market growth.

An area of importance and increasing relevance continues to be our investments in our respiratory portfolio. One of the things that I've been talking about is how do we continue to build our growth platforms and the new assets that we have in the form of Trelegy Ellipta, as well as Nucala, and continue to move from strength to strength every single month. We have done well and especially in Q4, we have created new benchmarks in terms of our evolution index as well as the number of sheer patients that we were able to touch.

As we all know, the single inhalation triple therapy continues to be the biggest growth driver within the respiratory space and Trelegy, which is our biggest global asset in the general medicines portfolio, continues to be a cornerstone of our future respiratory strategy, even in India. Something that we've been betting on and I've been sharing with you the progress. So, for

the quarter four, we've tweaked our strategies after really spending energy and building -- and building shingles awareness in the country over the last several quarters.

We've continued to hold our awareness at a certain level, but I think the good news is with every passing month, especially if you see on that bar graph, for quarter four, we've been able to have almost 20,000 doses [of Shingrix], touching 10,000 doses in a single month of March alone to really have new patients being empowered to take charge of preventive medicine, especially to prevent a dreadfully painful disease like shingles and herpes.

We not only continue to build our energy around the adult vaccination ecosystem, but we also on a monthly basis, we have new wins in the form of not only HCP [Healthcare Practitioners] recommendations, but examples of really creating those adult vaccination ecosystems, especially around joint business plans that we're putting together in the hospital setting where patients have easy access.

And we can complete that golden journey from the time that a patient hears about prevention and actually gets a jab in his or her arm. So, we continue to invest, we continue to remain absolutely confident about the way forward in building the vaccination ecosystem for adults, led by Shingrix in the coming quarters.

So, as you would see in the financials, the second century of our operations becomes even more exciting. As we had said in the last quarter, in this coming financial year, we will also have the launch of new growth areas, especially new therapies, like oncology, and it's an exciting year ahead. But for now, based on the strategic elements that I just shared with you in the broad-based portfolio of products and businesses that we have, we'll now spend some time looking at GlaxoSmithKline Pharmaceuticals Limited

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the financials, and we'll spend more time on the question-answers in the coming few minutes.

Juby, over to you.

Juby Chandy: Thank you, Bhushan. Good evening to all of you. I'll take some time to share with you, I'll give you a bit more flavor on the financials, which you might have already seen, which we released to the market. Firstly, we made strong progress both on every level of financials during the year, and I'll take you through a bit more detail in the coming slides.

But if you see the revenue block, INR 3723 crore, which is a 9% growth, and this is predominantly led by volume growth. Our general medicine business, more of the established brands in that portfolio, volume growth of 8% we have seen for the full year.

Specialty segment, which we have taken a bet to invest more during the year. We've seen a growth of 35%. Pediatric vaccines remain stable with 12% growth. Shingrix, Bhushan just spoke about that and shared more details. There also we made a great start during the year. All our key brands have been gaining market share and out-

performing the market.

EBITDA growth, this is the highlight of this financial year. It's INR 1169 crore, which is a growth of 30%, and we have significantly improved our EBITDA margins from 26% last year to 31% this year. EBITDA margins have improved predominantly on gross margin and operating leverage, which we had.

Gross margins have improved both on pricing side, as well as better pricing on the raw materials. On SG&A, we've seen cost optimization and cost consciousness have driven us to help to be on track on SG&A. SG&A as a percentage of sales have decreased close to 4 percentage and our field productivity has also improved 9 percentage.

PAT is keeping the same trend, 32% growth and 25 percentage as a margin point. Again, the highlight here is all the profits have been converted to cash. We are sitting at a balance sheet date estimated close to INR 2,500 crore cash, very healthy balance sheet. We are a zero debt company and EPS growth of 32 percentage. Again, all the return metrics have improved during the year. And you would have also noticed the Board has declared a final dividend of 42 per share. This is in addition to the interim dividend, which we had declared in the last quarter.

I'll take a moment to showcase the progress we have made. If you remember 2 years back during the NLEM time, me and Bhushan came and shared a strategy and articulated how we are moving up with the volume led strategy and some of the new launches we have. We were planning then. So we made significant progress since then. If you see over the last 2 years, we made improvements in the gross margin. Our EBITDA profile has improved from 23% to 31% over the last few years. We were able to contain our employment cost. As we speak, we are at 15.7 as a percentage of sales. This has come down significantly. That's why operating leverage is at the end of the place.

And if you see the return on capital employed and return on net worth, both are tracking upwards and high double digit percentages. Again, market cap is a reflection of all these things and we have seen significant improvement in the market cap over the last few years. So that's a short summary of the full year, as well as the improvements we have made in the last couple of years.

Now move on to the quarter. You could see the similar trend in the quarter. The revenue growth has been 6%, predominantly led by volume growth. It's been a soft quarter for antibiotics. That's why it's a bit muted compared to the full year, but we've been gaining market share across our portfolio. Specialty portfolio has been doing quite well. It's been growing double digit. Pediatric vaccines 10% growth and Shingrix again driving performance on the vaccine portfolio. EBITDA margin, you could see the same trend, the growth and the margin profile keeping up. 30% growth on EBITDA for the quarter and PAT, 36% growth. Our cash position is very strong and that's some context in which we have declared a strong dividend as well. So that's it. A quick summary on the financials. We can open the floor for any questions.

Bhushan Akshikar: Thank you, Juby. I think the whole idea is to spend more time answering questions from our investors.

Moderator: Thank you. We will now begin the question -and-answer session. Our first question comes from the line of Pritesh Chheda. May I request you to accept the prompt on your screen? Introduce yourself and go ahead with your question, please.

Pritesh Chheda: Yes, I wanted to ask you on the margin side. So, you know, we see a newer margin in quarter four now at about 34%. And last full year, we have gone through a large round of margin upgrade via the SGA line leverage. Any comments on the incremental margin expansion possibility over the full year of 31% which you reported? And obviously, last year, there were fall

outs by the

management on the reasons for the margin upgrade. So, should 34% exit is a reference one can refer to, or you have any other comments on the margin upside?

Juby Chandy: So, let me take that question, Bhushan. So, firstly, I think a business is seasonal. So, different quarters have different margins. So, for guidance, you might have to take the full year margin, which is a bit of margin of 31.4. This is what we are tracking as a sustainable margin. Now, you might have noticed the margin profile has gone up significantly, a lot of low hanging fruit. So, lots of changes we have made and we are at a perhaps at a stabilization stage. So, at this stage, you should not expect a big change in the margin profile upwards.

Pritesh Chheda: Okay. My second question is on the new launches. So, we had a lot of products for approval in India, especially on the onco [oncology] and the respiratory side. So, any update you want to share on the possible launches, if any, this year and next year?

Bhushan Akshikar: So, I will take that question, Mr. Pritesh. So, first of all, thank you very much for asking around the margin and I think I echo Juby's sentiment. Our endeavour will be to sustain that margin, annual margin as opposed to seeing any opportunities for further expansion.

Having said that, I think, which also means our ability to invest in the new business line, especially in the high growth areas like oncology. As I had shared in the last investor call, we have the marketing authorization for two of our global assets; Dostarlimab which is the brand - the trademark is Jemperli and Niraparib, which is Zejula, respectively, in gynecological malignancy. So, those are the first two ones, which will go off the block in terms of a long sequence in this coming financial year.

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We will continue to update the investor community because, as I have said, in the previous investor meetings, we have global clinical trials ongoing for a section of Indian patients as well and that would help us accelerate some of the regulatory timelines in terms of rules. But to begin with, our launch sequence is first focused on these two assets within oncology, namely Jemperli in endometrial cancer and Zejula in ovarian cancer.

Pritesh Chheda: Okay. So, most products are in onco [Oncology] ?

Bhushan Akshikar: The first two ones, the first two innovative assets will be in oncology. That's how I would answer that.

Pritesh Chheda: So, the two launches this year, you are referring to is the onco launch, right?

Bhushan Akshikar: As I said, the first one is that will go off the block next month. As I said, we have clinical trials ongoing. As you would expect, there are regulatory timelines of which quarter of which we would love to launch some of these assets sooner than later. Rule 101 is yet to play out in terms of the finer details. So, we have a slew of products in oncology as well as vaccines. And to begin with, these are the two assets where we have full visibility and that's what we are sharing with you.

Pritesh Chheda: My last question, sir, is on the general medicines performance this year, which is a fairly robust performance considering the market. So, an 8% volume growth for a base portfolio is a fairly

strong number. So, what went into this growth and what is the sustainable growth you think in the general medicines portfolio? That will be my last question. So, the reason for this stronger growth, what exactly we need to do it? What efforts have we put? And what should be a sustainable?

Bhushan Akshikar: Probably, Mr. Pritesh, we will require two hours to answer your first question, but I will definitely give you an abridged version of it. Clearly, what defines the success of some of these iconic assets that we have in the general medicines portfolio is the science behind why some of these brands

continue to be, as I keep saying, not trademarks, but these are trust marks. And if you have to dial up that trust with healthcare practitioners, patients at large, we need to keep investing in science.

So, I think we've unlocked some significant value, especially in the area of antibiotics, for example, in creating something called India Infection Index, which really allows physicians to take real-time decisions in choosing the appropriate antibacterial, even the major issues of antimicrobial resistance that we have in our country. So, there are areas which have really helped us unlock value, and I think we will continue to invest in these areas moving forward.

The second question that you had was in terms of how do we see? So, obviously, many of these brands will continue to be the frames of reference within their respective therapy areas. And our endeavor as a team will be to continue to beat the market growth so that we continue to leverage and deliver similar performances in the coming quarters for the gen med portfolio.

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Moderator: Our next question comes from Viraj Mithani. May I request you to accept the prompt on your screen? Introduce yourself and go ahead with your question, please. Mr. Viraj Mithani, may I request you to accept the prompt on your screen and go ahead with your question, please?

Viraj Mithani: My question is in terms of the launches...

Moderator: Sorry to interrupt you, Mr. Viraj. May I request you to use the handset, please? We are unable to hear you clearly, sir.

Viraj Mithani: So, my question is, there are two launches. One is in the oncological segment, which you said and one is in the respiratory. So, can you give some color in terms of the profitability, growth profile and the profitability of the company? Would it change the margins and any color is useful for us? I know you don't give guidance, but you can at least give some color. That's my first question.

Bhushan Akshikar: So, clearly, I talked about assets in oncology. Now, as we all are aware, oncology is a highly specialized, very focused area. So, you don't require armies of thousand reps on the ground.

Given the nature of the landscape, these are very nimble-footed teams led by high science, evolving data readouts. So, that's where most of the investments will happen.

I've also said in one of the earlier calls that it will be our endeavor to build the next growth platforms at GSK India to not only sustain the performance of our base businesses in gen med and periodic vaccines, but also bolt-on these new growth levers led by, for example, Shingrix and then now oncology.

We also talked about products like Trelegy and Nucala in the past. And I think if you look at the freshness index in terms of the contribution of some of these assets over the last couple of years, this business didn't exist for us. Today, it contributes about INR120 crores -INR130 crores annual revenues. And the idea is to continue to build.

And you can imagine, in spite of those investments that we have made, the margin has only improved. You've seen in Juby's slides where we've shown how the margins have evolved. So, our endeavor will be to sustain the margins and the top line. We've made a very clear aspiration in terms of what will it take for us to continue to touch millions of lives with our gen med [general medicines] portfolio and our pediatric vaccine portfolio but also transform the operating model with the launch of innovative assets.

So, both would happen concurrently. I don't think I can hazard a response in terms of giving you guidance on the margin, but it will be our endeavor to hold the margins is the best I can tell you.

Viraj Mithani: So, my next question is...

Bhushan Akshikar: I'm sorry, Viraj, but there's some problem with the audio. Your voice is coming and going. I can't hear you clearly.

ly.

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Viraj Mithani: My next question is, if I'm supposed to visualize Glaxo [GlaxoSmithKline Pharmaceuticals] five years from now, what kind of company should I be visualizing as? Gen med plus the specialty, plus the vaccine, or plus something else?

Bhushan Akshikar: Well, I mean, for a company that has endured and endured for more than 100 years, a five -year period would probably , wouldn't do justice to the heritage that we've created, the trust that we have gained, but more importantly, the next century to which we are committed in India. So, I think, as I said, our ambition will be to continue to bring energy because many of these trust marks that I talked of will remain relevant in the Indian setting.

But how do you really accelerate the launch of our innovative assets and continue to be the frame of reference of everything around innovation in this country, which we've done for the last 100, 101 years now? That will be the way I will see it. So, I'm not going to focus only on the next five years. I think the organization has outlived generations based on the trust. So, that's where our focus will be.

Of course, in doing so, delivering the fair share of returns to our shareholders who remain committed over this 55 , 60 years journey since the time you were listed. So, I think that remains the overarching principle for us.

Viraj Mithani: Sir, my last question is, any launches on the parents' [GSK Plc] table this year?

Bhushan Akshikar: I just talked about it. I think the previous question was on the same lines. I talked about two assets that will get launched in the next few months in the area of oncology.

Moderator: Our next question is a text question from Swathi from Zen Securities. Can you please provide revenue breakup in percentage from patented drugs, OTC, and prescription drug sales?

Bhushan Akshikar: You want to take that, Juby ? But I can give a quick... So first of all, we don't... I mean, typically, we don't give segment -wise analysis because it's a broad portfolio of products. As I said, we operate at both ends of the spectrum. But if I were to give you a thumb rule, we right now have about 80%, 70% or 80% of our business coming from the pharmaceutical business.

We don't have any OTC products. So, all our products are ethically prescribed. We have feet on the street. We have reps who go and promote our products. And the balance is the vaccine s portfolio. So, that's how we are structured right now. I hope I answered you, Swati.

Moderator: Thank you. Our next question is a text question from Deepak Malik from Carnelian Capital.

Please share your guidance for FY26 in terms of revenue and EBITDA margins.

Bhushan Akshikar: Do you want to take that , Juby ? We don't give guidance...

Juby Chandy: We don't give forward guidance on statements and EBITDA margin. So, that's it.

Moderator: Thank you. We have another question from Mr. Deepak Malik from Carnelian Capital. Please share new product launches for FY26.

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Bhushan Akshikar: I think I just answered this earlier. So, we have... Apart from focusing on contemporizing our GenMed portfolio, so we're always looking at opportunities to launch line extensions for some of our big brands. And that will be continued. So, we have big brands like Calpol, Augmentin, Eltroxin . And it will be our endeavor to continue to breathe new life by way of extending the life cycles of many of these trust marks.

But apart from that, from the innovative abstract portfolio, we have Zejula and Jemperli from the oncology portfolio, which will get launched in the immediate next couple of months.

Moderator: Thank you. Our next question comes from Gokul Maheshwari from Awriga Capital . In the press release, you have stated that you intend to grow a

bove market growth in FY2026. In the past, there was an aspiration to grow in double digit. Does that aspiration still hold?

Bhushan Akshikar: Yes, absolutely. Our aspiration continues to hold. And that's something...So, if you've seen the IQVIA prognosis, the market is supposed to grow anywhere between 8% to 9%. And that's why you said we want to ensure that we deliver above market growth. So, the answer is yes.

Moderator: Thank you. We have a text question from Deepak Malik from Carnelian Capital. Please share the opportunity size, competitive landscape, and revenue potential for Jemperli and Zejula .

Bhushan Akshikar: As you would imagine, Mr. Malik, every single indication has a different opportunity size. The going in position, as you may be aware, Zejula is indicated as a first -line maintenance treatment in ovarian cancer. Amongst gynecological malignancies, ovarian cancer, endometrial cancer, they are among the top five.

So, if you look at the prevalence, if you look at the incidence, we have almost 6.6 per lakh population in terms of ovarian cancer. That's the prevalence right now. So, the idea is to really capitalize on the opportunity that Zejula offers as a first -line maintenance in ovarian.

And we are, of course, launching as a second -line indication for endometrial for Jemperli . Again, a significant opportunity. Concurrently, in line with our global strategy, we'll be pursuing first -line. And of course, there are ongoing trials for this asset in several other indications. So, each indication will open up new opportunities. So, I think we can always spend time looking at how

each of these opportunities will pan out.

But as you may be aware, the oncology therapy area is one of the fastest growing areas. And as a clear opportunity that we have in the form of the first immunotherapy that GSK will launch, it will be our endeavor to ensure that maximum patients benefit within the approved indications for which we will be launching this to begin with.

Moderator: Thank you. Our next text question comes from Ravi Purohit from Securities Investment Management Pvt. Ltd. While we have done well on the antibiotic portfolio and vaccines, we have some very strong brands in the derma space. That's one space with a lot of potential. Yet, we have not seen GSK talking, discussing much on these. It will be great if you could share some of the initiatives we might be taking in that space. Thanks.

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Bhushan Akshikar: Always good to hear from you, Mr. Purohit. Thanks for attending, like every time. Yes, I think I didn't give a flavor, a specific color on our derma portfolio.

But if you look at the specific quarter, in spite of the sluggish growth that we had in anti-infectives and pain, where we primarily have some of our big antibiotic brands, and of course, the big paracetamol brand sitting. In spite of our very sluggish growth there, we were able to deliver, you saw the 6% growth for the quarter. And it was all volume-led.

But I think the good news was the silver lining in that performance was the strong performance of the derms business, which grew in double digits for the quarter. Most of the brands have done exceedingly well throughout the year. Yes, I have not calling out derms, but we continue to be the number one player in the derms segment, again, going around science.

So clearly, I think derms is one area where we continue to invest and look for opportunities to even grow further. Thanks for asking that, Mr. Purohit.

Moderator: Thank you. Our next text question comes from Ravi Purohit from Securities Investment Management Pvt. Ltd. Is there an opportunity for us to be able to export some of the medicines to our associates, especially in Southeast Asia and African regions? A lot of our brands are well known, not just in India, but globally. If India is able to make it

cheaper, can this be one of the growth opportunities?

Bhushan Akshikar: Well, Mr. Purohit, first and foremost, it's a great question. Thanks for asking that. In the past, we've done that. We used to be a sourcing base. Some of our factories used to cater to the external demand. Today, as we speak, we have still our manufacturing operations. We still have a state-of-the-art site, which is operational in the Nashik area, which caters to almost one-third, more than one-third of our top line revenues of our general medicines business, all manufactured locally within our factory.

To begin with, we do manufacture for WHO from that factory, something that we supply as a part of our commitment to eliminate lymphatic filariasis. We do manufacture products here, which are supplied not just to the Indian government and the health ministry here, but the idea is to take it to the next level.

Will we be able to commercialize this and take it to... That's not a part of the remit right now. I think the focus is clearly to use all our expertise in accelerating the innovation pipeline, and that's where we would probably spend more energy and time.

I think the supply chain strategy continues to be an important element when I look at the overall GSK India strategy. I think right now the whole emphasis is on accelerating new launches from our global pipeline, because as I said, we have some exciting opportunities lined up, including some of the trials that are happening in India. So that's where the focus will be.

Moderator: Thank you. Our next text question comes from Mahesh Vyas from UTI Mutual Funds. How much can be TAM for next two onco products, which will be launched and also in terms of usage, in terms of stages of patients, in terms of range-bound market size?

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Bhushan Akshikar: Well, I mean, it's a great question. And as you would expect for any oncology asset, I said that in one of the earlier questions, the going in position for both the assets is the approved indications that we have. And as I said, Zejula, which is Niraparib, is indicated as the first-line maintenance in ovarian cancer.

There are trials going on for the same asset in different other malignancies right now, including in India. And each of these presents significant commercial opportunities. But as you would expect, we all have to wait for the data readouts, for the data to be presented in oncology congresses.

And that's where we probably are able to give you color in terms of the size. Going in position, again, might sound repetitive, but we have approval for second-line endometrial, which is still a significant opportunity for Jemperli, dostarlimab. The same asset is being evaluated by way of clinical trials, even happening in India, for colorectal, which is colon cancer, for head and neck, for non-small lung cells.

So there are different trials ongoing. And as I said, each of these presents a completely unique opportunity. But to begin with, we have the going in position for these two assets in the approval of indications that we have.

Moderator: Thank you. We have a text question from Ravi Purohit from Securities Investment Management, Private Limited. Given that we are now doing parallel clinical trials for global as well as India market, should we expect future new launches to come with shorter intervals compared to their global launches in India? Thanks. And I would like to congratulate the entire team led by Mr. Akshikar to getting growth back for GSK in India.

Bhushan Akshikar: Mr. Purohit, thank you very much for the reassurance and confidence one more time. Yes, I think the whole idea is to get the drug lag, the launch lag reduced. And that's the whole intent of having almost 19 global clinical trials happening in India as we speak, in the areas of oncology, in the areas of hepatology. The

re's a global asset that we have, which might be the first functional cure for chronic hepatitis B infection.

And so assets like that, which are completely innovative are undergoing trials here. With this data, we can always engage with the regulatory agencies, especially in the context of Rule 101 to see accelerated launch timelines. And that's the real intent of ensuring that patients in India can benefit without having to have long lag times between global launch and within India. So the answer is yes to your first question. And thanks once again for your second point.

Moderator: Thank you. Our next question comes from the line of Mr. Deepak Malik.

Deepak Malik: Yes, hi, this is Deepak from Carnelian. So thanks for taking my question. So my first question is, so next year, you said that you intend to deliver growth more than the IPM growth, which is around 8%, 9%. So in this, how do you see the pricing growth, especially in the portfolio, which is under the NLEM and in the rest of the portfolio? Because last year, I believe that pricing was an issue where we were not growing. Volume growth was very healthy. So next year, so how do you see the pricing growth in your overall portfolio?

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Bhushan Akshikar: Thank you very much, Mr. Deepak. I'll take the first part and probably hand it over to Juby if he wants to add anything. Yes, if you look at the last two years, we've had our performance underpinned by strong volume growth. I mean, right from the time we had inclusion of some of our big assets in the NLEM list and therefore becoming a part of the pricing control order end of 22, the last two years have largely been volume led. And I think that will continue for us.

To your specific question around pricing, yes, we couldn't dial in any price hikes last year. More than half of our portfolio in general medicines is under price control. So that obviously leverage is not available for us. We've seen this year, the government has already announced a WPI high curve, I think 1.75. So that's something that we can dial in at appropriate times for the portfolio, which is under price control.

And the rest as and when we have opportunities, we will ensure without, of course, ensuring the right balance between what the price is for the patients and consumers. So the answer is yes, that's a lever. But again, compared to the past, we don't see this as the main driver. I think our strategy will still continue to be driven largely by volumes because that really is the best metric to tell you the health of the business, because that's the closest unit to your prescription generation that you're doing as a company. Mr. Juby if you want to add something.

Juby Chandy: I think I would just add a couple more points. One is, if you understand our portfolio close to 38% of our portfolio is under price control, we call it an NLEM. So that 38% of our portfolio, whatever WPI price increase comes from the government, we'll take it. Okay, so that part, depending on what comes out, that will be playing out as it goes. The remaining 60%, wherever there are opportunities at the end of the day, most of our products are generalized, there is a volume play.

So wherever there is an opportunity compared to competition, it doesn't make it less competitive for us, we'll take a price increase. So basically, we are very active on the pricing, wherever there's an opportunity we'll take, particularly on the NLEM part based on the WPI, non NLEM part based on the competitiveness.

Deepak Malik: Thanks. So here, if I see the comparison, like many of the Indian companies which are like quite big in the domestic pharma, like Torrent and the other companies. So there I see their volume growth is hardly 1% or 2%, 3% and like 7%, 8% kind of price increase they take. But here GSK like even in the 60% of the portfolio, we have

ve not seen much of the price increase. So any reason

that we refrain from taking price increases for the portfolio under the NLEM, I can understand, but even in the rest of the portfolio, there's hardly any price increases over the last 2 years. And going forward also, we are not that keen. So any reason for that?

Juby Chandy: The rest of the portfolio, we take price increases, just get blended and you are not seeing that. This year what happened, the NLEM portfolio, there was an increased discount strategy to drive the volume that's getting blended there. At the same time, vaccine portfolio, we are not able to take price increase to manage the competitiveness. Rest of the portfolio, we take on price increase.

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Bhushan Akshikar: So Mr. Deepak, I think I will also jump in. Clearly, we do not have an allergy for price hikes and ensuring that we are not leaving any money on the table, especially from a shareholder value standpoint. But I think having said that, we are also mindful of the elasticity where we can take, we are definitely not losing, we are not leaving any money on the table.

Moving forward, I don't know whether I conveyed, it's not that we don't want to take, but the point I made was, it's largely a volume-led strategy because compared to many other companies as an organization, we have at least 11, 12 assets which are significant in size when you have an annual revenue of more than INR100 crores, INR 150 crores of annual sales and you have 11 brands, 12 brands which are more than that sales value. I mean, every incremental prescription that you generate or unit that you generate is only going to multiply.

I think that's the going and driving strategy. On top of that, price hikes are like a cherry on the cake. I mean, we will not leave anything, but that's not the main driver for our business.

Deepak Malik: Fair enough. And I appreciate that we prefer the volume growth instead of price growth because that is the way you get the small market share. Second, I have an observation here. So, in IQVIA, whenever people then share the growth numbers for the month and for the quarter, for GSK it's always a very low number is being shared like 1% or 2% kind of growth or maybe decline also. Our numbers which we have reported are always very superior to the one which is shown by the IQVIA. Any reason why our reported primary sale number and secondary sale numbers doesn't match? The difference in our case is quite high if I compare with the other companies where the difference is 1% or 2% kind of, but here the difference is very high?

Bhushan Akshikar: Absolutely. That's a great question. So, if you look at some of our key brands, the reflection is very, very similar. So, if you look at the top assets, our top key brands, big size brands, the reflection as you would see in the external syndicated research data, whether it is IQVIA or any other is very similar. I think where we have a sizable non-promoted portfolio which operates. So these are brands which are not promoted. We either have a distribution model and that's a sizable portfolio about INR300 crores. That's where the reflections are completely differing. Plus, the vaccine market is usually under-reported if you see what we report in vaccines because that doesn't go through the normal channels of pharmacies and medical stores and general distributors. So, clearly there is an under-reflection in some of those therapy areas. But if you look at, if you stick to the big brands, the reflections are very similar. But that's really the underlying.

Deepak Malik: But then base is also the same. So, in the base also, then vaccine will not be there. So, on a base number, if there is such a strong volume growth in like general medicine category, so at least that should be reflective?

Bhushan Akshikar: So, you can take, as I

said, when you look at, when you splice and dice, when you look at some of the big brands, the growths are absolutely, that's why you would see some of our brands which continue to be at the top of the pharma industry rankings. We have at least three or four brands which are in the top 20, 30, including the number one brand. So, those are brands where we don't see any differing views because those are exactly aligned with what we do in the market.

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Juby Chandy: I think just to add the IQVIA reporting on the innovative effects and the vaccines, particularly both for the sake of growth as well as for the base, it is not aligned and it's not correct. So, that is where we have this disconnect is coming.

Deepak Malik: Thank you. Thanks for taking my question.

Moderator: Thank you. Our next question is a text question from Nikhil Upadhyay from SIMPL. For Zejula and Jemperli, we have been investing in market development for last 2 quarters. So, how many doctors have we touched base and what is the total doctor population we would need to reach?

Bhushan Akshikar: So, it's a good question and we are putting, as we speak, obviously we have the new oncology

business head in place. The recruitments are going, as we speak, the recruitments are going full swing. We've also invested largely on the medical side. So, that's where the investments are going.

As I've said earlier also, medical oncology or Hematology are areas which are very tightly, you don't have more than 1500 medical oncologists in this country. So, you don't need an army of 1000 people on the ground to cover. These are small nimble-footed teams and that's exactly where we are putting energy.

So, we've been investing around creating knowledge around science. The actual visits will start from our commercial team members only in the coming days. So, that's where we are as of now.

Moderator: Thank you. The next question from Nikhil Upadhyay from SIMPL. For focused brands and vaccine portfolio, how has our doctor coverage increased in last 3 years versus 3 years back?

How has our prescription doctor increased? From here on, do you see more opportunities to increase our doctor coverage or will the higher growth be led by increased prescription?

Bhushan Akshikar: There are 3-4 questions there but I'll try and answer the first one. I think 2 years ago when I presented to the investor community what we were doing on the digital side, I did mention that although our absolute face-to-face coverage had gone down, because we had optimized our selling field force on the ground but we had dialed up a significant Omni-channel digital strategy. What do I mean by that?

We have healthcare practitioners who have consented to us to reach out to them through different channels, through different touch points and that becomes an important area for us. To answer your question, today we cover almost 2,75,000 distinct unique healthcare practitioners in the country.

But on top of these doctors whom we meet physically face-to-face through our reps on the ground, we also have a significant number of doctors whom we don't cover face-to-face but have also consented to reach out via Emailers, via WhatsApp or different touch points and that's where we create.

Our reach has actually gone up by more than 40% on top of what we had. That's what I presented. I think if you remember the numbers that I presented and we also uploaded that, the 4 lakh plus touch points that we talk of, our reach and coverage has in fact increased over the last couple of GlaxoSmithKline Pharmaceuticals Limited

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years and we look at efficient ways to continue to do that apart from of course the sales team. These additional touch points will only supplement and complement the great work done by the field force on the ground.

Moderator:

Thank you. Our last question comes from the line of Mahesh Vyas from UTI Mutual Funds.

Breakup of CMO and in-house manufacturing. For Shingrix, how do you see in terms of years 3, 5, 7, 9 that the vaccine will be fully accessible, available also in terms of awareness to the masses? How Salesforce plans working for this?

Bhushan Akshikar: Great question. Thank you very much. The first question, we have our own site in Nashik as I said. Plus we work with 20 CMOs who cater to our so almost 99% of all that we do in our general medicines portfolio is locally manufactured except for maybe an inhaler that comes from outside.

Otherwise everything is locally manufactured. One-third coming from our own site, two-thirds coming from the balance 20 CMOs. That's the breakup. That's the first question.

On the Shingrix front, I think it's a great opportunity to do some market shaping because as you can imagine, when you create a new category, it is I always say it's not a 100-meter sprint. It's an endurance sport where every quarter, every year you're learning, you're creating something different. Today nobody worries about where to get a pediatric vaccine. We all know the pediatrician is the default vaccination point. She or he will keep and store vaccines and when a baby or infant needs and that was created by GSK more than two decades ago. They are putting a similar energy in creating the adult vaccination ecosystem to really dial up this whole importance of preventive medicine. And to begin with, of course, our focus is on shingles prevention.

As many of you may be aware, we have a flu vaccine. That's another one where we continue to build the yeoman service to create the adult vaccination ecosystem. Moving forward, we have trials ongoing even in India for our RSV, respiratory syncytial virus vaccine, which will be the next adult vaccine.

It's a long drawn approach and when changes happen, as I said, if you see that slide also, from a point where we were selling 2,000 and 3,000 and 4,000 doses, we started touching 10,000 doses on a monthly basis. At a certain point, it doesn't follow a linear curve. It follows a different pattern.

I wouldn't call it exponential as yet, but it follows a different curve completely because more

and more people are getting aware. More and more people are coming and asking how to prevent this painful and debilitating condition and that's where we pivoted. You asked me a question about the sales team.

We always say that these are not sales reps, these are adult vaccination ecosystem creators. The idea is to really partner with hospitals, health HCOs, with clinics, to really understand how to create that ecosystem where when a patient comes, he or she is not only coming for treatment, but also talks about prevention of some of these diseases.

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I think that's the approach we've taken and that's why we've pivoted our strategy now towards creating that ecosystem in about 4,000 clinics and some of the biggest hospitals in the country. That's the approach and your question around 3 years, 5 years, 7 years. The idea is very simple. We have more than 10 to 12 million Indians in this country who are more than 50 years in age who are not only eligible to prevent shingles as a disease, but also have concomitant diseases or indications where they're at a greater risk from getting this painful disease. That's the focus. With every passing month, we're getting more leverage, more acceptance, more awareness and that's something that will continue and we'll see continued traction from GSK in this area.

Moderator: Thank you. Please note our last text question comes from Swathi from Zen Securities. Hi sir, your view on changing regulations, government interest towards prescribing generic medicines instead of brand name medication and GSK's preparations to deal with this?

Bhushan Akshikar: So again, we have to see this is, I won't call it sub diseases, but there are hearings going on in the Supreme Court based on a case that was filed by an association. So we have to watch this space. This is not the first time there have been individual states that have come up with strictures on similar lines.

But as you can imagine, health being a state subject across the Board, although there is no reimbursement of any kind in our country. Eventually, when a healthcare practitioner prescribes a medicine, he or she is prescribing it because of the evidence, body of evidence, the trust that the healthcare practitioner has.

And that's why some of the assets or some of the brands that you see, they're made around science. It's not just about what a brand does or what a molecule does, what a product does. It's also about where that brand should not be used in terms of the adverse events or the side effect profile. And that's what companies bring to the table.

I think it's a long drawn conversation. In our industry associations, I'm sure all of us are deeply engaged to watch this space and also shape this external landscape in terms of a better understanding of why it is important that the patients have the choice, healthcare practitioners have the choice.

Anyway, on the other hand, you have a very strong code in the form of, and you see PMP already drafted, already gathered it. So the Uniform Code of Pharmaceutical Marketing Practices is already in place, which serves as a great guardrail for pharmaceutical companies to operate in. When it comes to trust, I think we'll have to watch this space closely, continue to engage with the right stakeholders across the group, including advocacy groups, so that patients continue to get the benefit of the best medicine and not compromise quality. Because even one patient's life cannot be taken at risk with.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments.

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Bhushan Akshikar: Thank you very much. I think once again, as we've committed, I think almost 2.5 years ago, I and Juby made a commitment that every quarter, we'll come and share with you the progress that we're making and update you on the journey that we are on.

As I said in my first opening slide, we are remarkably proud of the 100-year journey, but even more proud that as we started the second century of our operations in India, we will continue to focus not only in bringing innovative assets, but also protecting the interests of all our shareholders and the investor community.

Thank you very much for your interest this evening. Thank you once again, and look forward to seeing you in the next quarter. Bye for now.

Moderator: Thank you. On behalf of GlaxoSmithKline Pharmaceuticals Limited, we conclude this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2025

(no extractable text — likely a scanned image PDF)

Call: Feb 2026

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11th February 2026

To,
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th Floor, Plot No. C/1, G Block
Dalal Street Bandra -Kurla Complex, Bandra (East)
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Dear Sirs,

Subject: Outcome of Schedule of Analyst / Institutional Investor Meetings

Pursuant to Regulation 30 read with Part A of Schedule III of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript link uploaded on the website at <https://india-pharma.gsk.com/en-in/investors/analyst-meets/> of the Analyst / Institutional Meeting held on 9th February 2026.

Thanking you,

Yours faithfully
For GlaxoSmithKline Pharmaceuticals Limited

Ajay Nadkarni
Vice President – Administration, Real Estate
& Company Secretary

CIN: L24239MH1924PLC001151

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“GlaxoSmithKline Pharmaceuticals Limited
Q3 FY 2026 Earnings Conference Call”
February 09, 2026

MANAGEMENT : MR. BHUSHAN AKSHIKAR – MANAGING DIRECTOR –
GLAXOSMITHKLINE PHARMACEUTICALS LIMITED

MR. JUBY CHANDY – CHIEF FINANCIAL OFFICER –
GLAXOSMITHKLINE PHARMACEUTICALS LIMITED

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Moderator: Hi, good evening, everyone. This is Dorwin Dias, your moderator from Chorus Call. Welcome to the GlaxoSmithKline Pharmaceuticals Limited Q3FY 2026 Conference. From the management at GlaxoSmithKline Pharmaceuticals Limited, we have Mr. Bhushan Akshikar, Managing Director, and Mr. Juby Chandy, Chief Financial Officer.

By participating in this event, you consent to the recording, distribution, and publication of this event. Kindly note that this call is meant for investors and analysts only. All participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation from the management concludes.

I now hand the conference over to Mr. Bhushan Akshikar. Thank you and over to you, sir.

Bhushan Akshikar: Thank you very much, Dorwin. A very good evening to everyone. Thanks once again for joining us this evening for the Q3 conference call. Starting off from where we left last time when we announced the Q2 results, at GSK India we continue to be anchored around the commercial ambition which is around focus, innovation, and agility.

We continue to be a broadly diversified company operating at both ends of the spectrum with a significant general medicines portfolio with leadership positions in therapy areas like anti-infectives, dermatology, pain, and most importantly, vitamins. In the prevention space, we continue to move month after month over the last several quarters with the vaccines business, both pediatric as well as the adult vaccination space.

What's new and relevant is our foray in the specialty side. Over the last couple of years, we've clearly stated our intent of significantly improving our presence beginning first with our respiratory specialty portfolio, and now over the last couple of quarters, our foray in the oncology space.

And that's where we will continue to build our energies in the coming three to five years. Can we move to the next slide. I'll start off first with the performance of the general medicines business, which is still a significant portion of our top line revenues, just to give a frame of reference in terms of how the market continued to behave.

After a couple of soft quarters, the last quarter, Q3, we saw even the external market bumping up. And when you look at our represented market for GSK, we did see a fillip where the external market grew almost in the range of 9%, in line with the acute market where we primarily participate. So on the left-hand side of the chart, if you look at where our majority of portfolio operates, the acute market roughly stood at about 8.6% growth.

That was after a muted Q2, driven by all the variables, both volumes -- after a couple of quarters we've seen volume growth coming back into the positive area -- and a good smattering of price and new introductions. If you look at the middle table, which is really the space where most of our products participate, the therapy areas, anti-infectives after a significantly suppressed Q2, in Q3 we saw anti-infectives coming back to almost a 4% growth, and then coupled with this, we did see areas like vitamins, pain, and most importantly, vaccines growing in significant numbers.

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The last chart on this slide is on the right-hand side where compared to the represented market, the specific therapy areas where we operate and the brands in those represented markets. So that's where overall we delivered a growth. Next slide, Farah. A growth of 10%, which was slightly above the market.

And as I said, this quarter after the last two soft quarters was an area for us to come back, led by two things: one, the interruption that we saw on the supply front for the last two quarters is now done. We've clearly seen from Q4 onwards we will definitely have no more supply constraints.

straints.

We did have some specific areas and we'll talk about that.

But in the GenMeds portfolio, if you look at our performance externally, we have evolution index more than 100, which means we did better than the market for majority of our brands. On the vaccine front, we've continued to grow share as well as improve our ability to get more benefits with our both pediatric vaccines as well as the adult vaccination space.

Over the last two and a half, three years, all our efforts around creating the adult immunization space continues to move from strength to strength. And that's where we've now pivoted towards

the cardiovascular metabolic disease opportunity to further talk about the bi-directional nature of Shingles prevention and the CVMD space. Last but not least is the specialty area. Trelegy Ellipta continues to occupy the central space in the triple closed single inhalation therapy. And Nucala had, again, a significant quarter with almost 100 patients month after month, every single month in Q3. So that's where we were. I think, as I said, the most significant for us from a portfolio standpoint was the, this was the first full quarter where we had our oncology portfolio available, both Zejula and Jemperli indicated for ovarian and endometrial cancer respectively. I think the bigger news for us was the regulatory approval in the second half of December where now we have approval with the RUBY-1 trial of first line endometrial cancer. So from almost 800 eligible patients for second line, we now have almost 6,000 patients as eligible patients for endometrial cancer in first line. So that's the most significant impact we will see moving forward as well.

On the digital pathways that we've really articulated over the last couple of years now, we continue to do cutting edge work on our Omnichannel strategy. So if you look at beyond our sales reps time in the clinics, you continue to have both face-to-face as well as digital connections and touch points. We do reach out to a significant number of HCPs digitally. And for the quarter, we had almost 4 million touch points, which are above, including the face-to-face interactions.

Next slide. Yes, Juby do you think you can go?

Juby Chandy: Yes. So good evening everyone. I'll begin with the top line. This is a milestone quarter as you would have noticed. This is the first time for a quarter we are crossing INR1,000 crores with a growth of 8.1% standalone basis, and on consolidated basis, we are talking close to 10% growth.

Now what's driving this growth? If you see, the general medicine business is back to growth. All the key brands, Augmentin, Ceftum, T-Bact growing double digits.

Specialty portfolio, this is the first quarter of the oncology launch. In addition to oncology launch, we have Nucala as well as Trelegy driving the growth in specialty. Pediatric vaccines

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portfolio continued to deliver strong growth, 11% growth for the quarter, led by Boostrix, Varilrix, and Havrix. These three brands are growing double digits.

So broadly you're seeing across the portfolio you're seeing very strong growth and market share gains as we've ramped up the activities and supplies are pretty much back to pre-supply level.

Shingrix continued to gain the market share like Bhushan explained, especially we are engaging with more HCOs as well as HCPs with our CVMD strategy.

EBITDA front, it's a fantastic quarter with 35.9% margin, which is a 520 basis point

improvement, 5.2%, and a growth of 26.7%. If you see the levers, we continue to improve on the gross margin front. Secondly, we've been containing our cost, disciplined cost control has been, again, another good quarter on productivity improvement.

And this growth is also on the backdrop of one-off labour cost impact of INR11.8 crores. Profit after tax, you can see the same trend continuing on the EBITDA in the PA

T. There is interest

and depreciation which is helping us to even go to 27.3 margin with 290 basis point improvement. EPS growth of 9% in the quarter and the cash position is quite healthy with INR2,426 crores.

So this is one slide we've been trying to share on the last two or last three meetings. Where are we heading towards? Our portfolio is from established business to Shingrix, Nucala Specialty, and now onto the oncology. What does it mean? We are getting into the high category growth. And if you see on the profitability, our profitability has been improving steadily over the last many quarters.

We are at 35.9 (%) and two years back it used to be at 24.3 (%). And the highlight again on this is the competitive performance. We continue to maintain competitive performance, improving our market share. And with this portfolio transformation, profitability improvement, and market share gains, we are at a good place for the quarter.

Maybe we can open up for the questions, Bhushan.

Bhushan Akshikar: Sure.

Moderator: Thank you very much. We will now begin the question-and-answer session. You can choose to ask your questions in two ways. The first is on video. To ask a video question, please press the Ask A Video question tab and follow the instructions to join the queue.

By clicking on join as attendee, you will be on audio only. By clicking on join as panelist, you will be on audio and video. Before asking the question to the management, please introduce yourself, providing your name and your organization's name.

The second way is by typing your question directly in the chat box. Please limit yourself to a maximum of two questions so we can accommodate as many questions as possible. Ladies and gentlemen, we will now wait for a moment while the question queue assembles.

Our first question is from the line of Mahesh Hemchand Purohit from H.J. Securities. How do you see the innovative pharmaceutical industry growing over the next five to seven years? Also, what role do you think biopharmaceuticals will play in that?

Bhushan Akshikar: Thank you very much, Mr. Purohit, for, as always, wonderful questions. Clearly there is a stated, explicitly stated intent for the pharmaceutical industry to move from being a pharmacy of the world to a research powerhouse. And that's where I think the evolution will continue to move from established medicines to specialty and super specialty.

Clearly, given the number of global clinical trials happening in India as well as discovery from some of the Indian players, I clearly see the biopharma space only expanding. If you look at a data point of the announcement in the budget of the biotech fund, clearly most of the energy will be spent by players across the board in this area.

And that's where, if you saw Juby's last slide, even at GSK we have a very strong and a robust base of our general medicines and our established vaccines business which will continue to be relevant, but on top of that I think the growth will certainly come from biopharmaceuticals. And that's where even our company will move in the same direction to bring innovation at pace and at scale.

So clearly, I think that's an explicitly stated intent as well as action for companies like ours so that we reduce the launch lag that our patients typically see in terms of those innovative launches happening globally and when they get launched here. So that's the first part. I think even the second part of your question I think I've answered partly.

But clearly I certainly see, given the ecosystem that all providers, including the government, are trying to establish, a large part of the growth will happen on the other side.

Moderator: Thank you. Our next question is a text question from Mehul Savla from RW Equity. Are current EBITDA margins sustainable? What is the impact of recent budget announcement relating

to

duty-free import of cancer meds? And what is the total employee strength and how much is the field force? What are the expansion or addition plans?

Bhushan Akshikar: Thank you very much for all three questions. I think the first question is around the EBITDA. As we all know, we typically don't give forward-looking guidance. As I've always said in all our meetings, our endeavor will be to hold the margins and stay competitive and sustain these margins as opposed to spending any time to improve.

If that happens, it's an outcome. But I think our first priority will be to maintain the margins that we've now been able to sustain over the last three years. So that's the first one. The second one is in terms of the budget. Yes, there were some announcements around reduction of basic customs duty.

We already had some customs duty reduction for some of our assets. This time around we've not had any of our products getting included, for which we will definitely be making representation to the relevant authorities. But that's where we are. We didn't get anything extra on this announcement. That's the second question.

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Your third question was around the productivity and the number of employees. We have roughly about 3,000 employees across the enterprise. And within the sales organization, we have about 2,000 employees on the ground. And that's something that has remained over the last two years. As I said, we have unlocked significant value with consents from healthcare practitioners where we reach out not only face-to-face by a face-to-face interaction but also unlocking value through the omni-channel stroke digital ways of working. So that's where we are currently. We have about 2,000 people on the ground as our sales reps. I hope I've answered all three.

Moderator: Thank you. Our next question is from Finnacle Institute. Please go ahead. Participant who has joined us from Finnacle Institute, please accept the prompt on your screen, unmute your audio and video and proceed with your question.

Ahmed: Am I audible?

Moderator: Yes, you are audible. Sir you may proceed.

Ahmed: I have three questions. Firstly on the Augmentin part, we had some disruption because of the CMO partner having plant on fire. So how is the status now and have we seen normalization fully restored? Secondly, in terms of MIP for, I'm assuming, key raw material for one of the, it is Augmentin's combination drug and the key raw material is amoxicillin. So because of the MIP, do we sort of see any impact in terms of our margins?

Number three, in terms of our vaccine business, in the presentation, we have spoken about 80% growth in Shingrix. So what sort of annual run rate we are at as of now? And lastly, in one of your, I think, news articles there was a mention of INR8,000 crores top line in a few years, four, five years. So can you give some sort of, in case you are willing to speak about it, can you give some sense how are we planning to achieve it, what sort of product pipeline and are we expecting to launch?

Bhushan Akshikar: Sure. Thank you very much. So your first question was about the fire at the CMO, our contract manufacturing organization. Just a small correction, it was not the Augmentin site at all. It was a site which was manufacturing products like Calpol, Cobadex CZS, and a few others. And those are the products that were impacted for almost three quarters, including Q3. So we did normalize largely by mid-November, December, but we still lost sales, there were supply constraints to the tune of roughly about INR25 crores, INR30 crores, shaving off almost 3% of our growth on our top line revenue.

So if you look at our underlying growth, it should have been in the range of 11%. But the good news is we have now resolved those issues. And from a supply continuity standpoint, all the required remediation that was required at

the site is done. We worked very closely with the CMO

to ensure that in Q4 and beyond, we have -- we are now building up inventories. So that's the first part to your question. I think I made a note of all three.

Ahmed: The second one was on the MIP on the raw material.

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Bhushan Akshikar: Yes, so MIP will not have any impact on our, you know, the Amoxiclav has been included, but there is no impact for GSK in terms of the margin and the EBITDA and the gross margin. So we don't see any impact of that second one. I'll take that last question first before I come back. So I think yes, we did articulate a clearly drawn-out strategy of doubling the business. And I think the thinking is very simple.

This organization has to grow in the range of 12% to 13% annually over the next five years so that we can double the business from where we were in '23-'24 to the point of becoming an INR8,000 crores business. So I think two or three elements are very important. I think our base business, as you've seen, has continued to get that kind of tailwind, both the GenMeds and the established vaccines business.

But I think the growth platforms, namely Shingrix which we launched, and to your earlier question, the growth came from a pivot that we've made towards a cardiovascular metabolic strategy, as I said, where there is a clear bidirectional nature and linkage of shingles prevention and cardiovascular events. So that's something that we are now focused on.

I can give you a rough estimate, but I think we are touching almost 9,000 to 10,000 patients every month. So in terms of volumes, it was almost 100,000 doses that we sold for the whole calendar year of last year, and we estimate that number will only go up as we close out our financial year.

In terms of values, it roughly translates around INR70 crores-INR75 crores. So that's where we are on a calendar basis. The last question around the growth strategy, yes, as I said, the base business will continue to sustain with the things that we are doing, but I think the significant impact will come from our new portfolio.

This year, we also have a couple of oncology assets lined up, including belantamab, which is Blenrep, which is for multiple myeloma. So that's another asset that we will get activated in the next financial year, this calendar year, but the next financial year. We have a couple of assets coming up in oncology. We also have assets coming in the liver disease area. Again, first of its kind potential functional cure in liver diseases like chronic hepatitis B infection.

We just had the first readout globally in the month of December. So, clearly, these are the two areas which will set us up in terms of the growth platform for this company over the next three to five years. So that's where our strategic intent of continuing to grow the business by the double digit number remains intact.

Ahmed: I have two follow-ups. Just to clarify, did you mean that the INR75 crores is the CY '25 Shingrix top line? Am I correct?

Bhushan Akshikar: That's right.

Ahmed: Okay. And secondly, can you give us some sense with the launch of Zejula and Jemperli, what sort of run rate is it practical to achieve with the kind of traction you've seen? I mean, it is probably a little early, but I think we are, I think in nearly I think six months of launch since

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August. So can you give some sense what sort of run rate is practical to achieve a couple of years down the line in these two products?

Bhushan Akshikar: Sure, but I think it's a -- it would be a premature assumption on my part to give you. But I think, clearly, the first three months since we launched it on Independence Day, the whole idea of getting freedom from gynecological malignancies with the pivot with these two innovation-led molecules, b

oth Jemperli and Zejula, respectively, in endometrial and ovarian, got launched.

As I said, the first full quarter that we were available, especially Jemperli, which is dostarlimab, was indicated only for second-line patients in endometrial cancer. And that eligibility pool was just about 800 patients. As I said, the second half of December, which is the end of Q3, we got the approval for the first-line in endometrial cancer. And that pool of eligible patients is almost 6,000-7,000 patients available. So it's not going to be a like-to-like comparison.

What we did in the first quarter is a great start, but I think it'll only get better as we move forward.

I think our intent will be to ensure that as many patients of this pool of 6,000 available for first-line endometrial can get the benefit with this remarkable asset, which incidentally has the RUBY 1 trial clearly talks about this being the only monoclonal antibody which has got progression-free survival as well as overall survival benefit. So that's where our intent will be.

Moving forward, we also have indications in rectal, so that readout should happen in 2026 calendar year, not in the financial year. So each of these indications will open up new opportunities for us as we move forward. So I think, as I said, it'll be a comparison of apples-to-oranges if I were to hazard a number and guess it here.

But I think the intent is to ensure that as you would be aware, oncology is a high-growth area, one of the highest growth is in oncology, almost 20%-22% is the therapy area growth annually, crossing almost INR10,000 crores, INR11,000 crores now. And our ability, therefore, to make the most with these innovative assets with the current indications as well as the trials that are ongoing, unlocking each of these indications would be intact. So I think that's where we are with these two assets.

Moderator: Sir, would you like to share your name with us, please?

Ahmed: My name is Ahmed.

Moderator: Thank you. Our next question is a text question from Julie Mehta from B&K Securities. She has three questions for us. The first is, what is our current PCPM?

Bhushan Akshikar: Are you going to give all three questions or?

Moderator: Sir, you can proceed with the first. I can follow with the next two later.

Bhushan Akshikar: Yes, so clearly if you see where we stand, it's in the -- we have a seasonality component because obviously a part of our portfolio is acute. So especially around Q3 our PCPM or per capita per month productivity goes up significantly because that's the season really. But we have one of the highest productivities per rep. So if you just look at our top line revenues and the 2,000 reps GlaxoSmithKline Pharmaceuticals Limited

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that we have on the ground, it's a substantially high number compared to the peer companies or other players.

Moderator: Our second question from Julie is, how many MRs do we plan on adding in the near future?

Bhushan Akshikar: So Ms. Julie, I think over the last two years I've been talking about our ability to therefore do a lot more unlocking digital. So it doesn't mean that our reps are any less relevant, our reps remain absolutely relevant, but we are supplementing and complementing everything that our reps do with digital touch points. And that's where this field force of 2,000 that we have largely in general medicines remains intact.

I think where we will keep expanding is the specialty areas. So we just launched our oncology business with a team of key account specialists as well as product specialists. We will be -- we are now working to launch our haematology team in the coming few weeks and months. So that's where some of the additions will happen on the specialty and the super specialty front.

But currently what we have in general medicines and the established vaccines business is, I think, we've -- we are really reaching out in terms of coverage,

the potential that we have. I think

we are at the right sweet spot. So I don't see any need to expand right now.

Moderator: Our third question from Julie is, can you also throw some light on how is the acceptance this time around with new onco launches among oncologists and what is our strategy to see greater acceptance?

Bhushan Akshikar: So I think it's a great question and within oncology if you look at clinical outcomes and the biggest one in that is the overall survival benefit. If you were to ask any oncologist at the end of the day if you are able to add, you know, extra number of years to that n=1 patient, I think that's

the holy grail in oncology. And I think when you look at the readouts that we have, when you look at the data published even for Dostarlimab in the Ruby-1 trial, in the intent-to-treat population.

It's one of the few or probably the only one, which has got well-articulated and established documented overall survival benefit. So I think that's where we are making a significant impact. Both healthcare practitioners, caregivers, and patients with every passing month are gaining confidence with this therapy. And I think that's what will separate us in the coming time. We will remain focused on creating evidence generation both at the local level as well as using some of the global trials that are happening in India.

So that's what will further create the impact that you just talked of. So I think the first four-five months that we've seen, every month we've been growing from strength-to-strength. We have almost 250 patients who are currently on treatment with Zejula and Jemperli as assets. So the idea is to move it even further in the coming months.

Moderator: Thank you. Our next question is from Vamsi Hota from ASK Investment Managers. The first of three questions is, on the pipeline, what are some of the key assets under trials that you plan to launch over the next three years and in which therapy areas?

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Bhushan Akshikar: You can give all the questions together, Dorwin. Yes, give all the questions, please.

Moderator: Certainly, sir. We have the second question. The second one is, is it a fair assumption to assume that the gross and EBITDA margin profiles of our injectable assets are higher than the regular portfolio? And the final question is, as in the case with some of the MNC peers, is there a risk that the parent entity of GSK could launch any assets directly in India instead of via GSK Pharma India? Thanks and all.

Bhushan Akshikar: So I think first of all I'll take the last question first. I mean if you see the demonstrated evidence, we've had both Zejula and Jemperli being launched through the listed entity, so I don't see -- we've got data points to prove our intent and that will stay. I'll connect that with the first question on the pipeline. So clearly if you look at the global strategy is really to strengthen our expertise in the area of immuno-oncology and that's where there's a lot of ongoing work.

We have the next generation of ADCs as they're called, antibody drug conjugates. So those are clearly a part of the remit that we have for India. So there's a compound called B3H7 that's slated for trials in India across indications. So over the next five years there are at least 12 to 15 indications for this antibody drug conjugate that we have. As I said, the immediate here and now would be Belantamab which is Blenrep, which is indicated for multiple myeloma.

India was a part of the global clinical trial, so as and when the readout happens, the first readout happens and there's approval, marketing authorization in some of the established markets, we will go concurrently. The intent is to launch it in the coming financial year, probably Q2 or Q3 latest. Apart from that, we also have trials ongoing in the vaccine business. So we have a trial for RSV, respiratory syncytial v

irus, which we've completed.

So that's another asset that will complement our efforts in the adult vaccination space. On top of that, as I said, one of the focal areas for us as a company is this whole area of liver disease. So whether it is Bepirovirsen which is the first functional cure for chronic Hep B infection -- the first readout happened in December, as I said -- so that's another one where again India was a part of the global clinical trial.

So we will be using the Indian sub-section of data to submit to the regulators. So that's another one. Plus we have a couple of other compounds in the liver disease area including a molecule called Efimosfermin. India was again -- India is again a part of the global clinical trial. So these are the three big areas where we will continue to unlock value by getting these innovative assets into India through the same entity. I hope I've answered all three. I think the second one was around gross margin and EBITDA for injectables.

Juby Chandy: The base business margins are slightly more than the injectables as the cost to launch the assets are heavier. But as of now if you're thinking about diluting the cost or changing the cost profile, the margin profile is not going to -- you're not going to see any impact because the size of the base business is pretty huge to make any impact on the margin profile.

Moderator: Thank you. Our next question is an audio question from Monica Bhaskar. Please accept the prompt on your screen, unmute your audio and video and proceed with your question.

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Monica Bhaskar: Yes, hi, sir. I think I just want to indicatively tie up the comments here and appreciate a lot of

new launches specifically from the India listed entity that has helped us to kind of boost the growth numbers for us? Now, when I understand that we have an aspiration to hit a INR8,000 crores topline and when I just tie up with your comments that we want to double this revenue from where we started or left at FY '24 ending or so.

Now, to my mind, reaching a INR7,000 crores, INR8,000 crores topline from here on would indicate north of 20% CAGR in next couple of years. While we do have that as one of the targets, we also see that maybe 12% to 13% is the growth aspiration we want to execute on.

Just help me understand where truly is the realistic scenario here? I know we'll obviously target for much more, but given all the moving parts, specialty business, vaccine launches, where do we kind of look to take this business forward from here on?

Bhushan Akshikar: Sure, it's a very good question, Miss Monica. So Miss Monica, I've been saying this for the last more than a year now and the context was simple. If you remember, we are in the second year of our second century of commercial operations in India.

GSK was established in 1924 in India and when we were completing our century of trust in 2024, that's when I said that, if our base of '23 was a certain turnover, should we be waiting for the next 100 years? Obviously no. What will it take for us to fast-track? And that's where the intent of doubling the business comes from.

So when you look at where we were end of '23-'24 and then scope it out over the next 5 to 7 years, that's the -- that's where you frame the reference and talk about the 12% to 14% growth. Now, if you look at the base business, which is still substantial for us, whether you look at the GenMed or the established vaccines business, that market will still -- if you will look at the prognosis, that market will still continue to deliver a high single-digit growth for us even if you were to scope it out over the next 5-year period.

I think the growth platforms that I talked of and I just articulated and you've got demonstrated evidence of the kind of opportunities that exist in the specialty, super-specialty space, especially in oncology, especially in u

nmet needs like liver disease.

That's where we clearly see, a breakaway in terms of not only holding the growth numbers in the range of 8% to 10% for our base business but really having those boosters, so to speak, for this enterprise with those new launches. So that's where the context is of moving from the number that we were at '23-'24.

Monica Bhaskar: Fair, just a follow-up here. When we say that GenMed and the base portfolio would be growing at high single digit. Now when I Shingrix, for example, today has become INR75 crores brand as we speak of if as we highlighted.

Now can we have amongst maybe the next let's say we aspire to launch 5 new launches to 6 new launches over next 7 to 8 years? Do you fairly believe that with the paying potential in India, we can have a INR500 crores plus brands of these newer launches? Because that is what it will take to reach the aspiration where -- what are we hitting for?

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Bhushan Akshikar: Sure, first of all, Miss Monica, you make a good point. The Indian consumer or Indian patient is not just cost-conscious, I think she is value-conscious. So if you are able to get innovative therapies which bring the right value for those patients and those healthcare practitioners and we've seen loads of examples including of Shingrix that we have in our portfolio where patients are willing to pay that -- pay that price.

I think secondly, we've always had in India -- we've always had an India-centric pricing approach including of the assets that we talked of. So I think that sweet spot is something that we've -- that we've always focused on in terms of pricing, including with the oncology portfolio.

I think the patient assistance programs that we have -- we talked about this last year when I had my media briefing. So I think these when you triangulate these three, we remain absolutely confident of what we've said in terms of the aspiration and the objective that we've set up.

Monica Bhaskar: Okay, fair enough. Thanks a lot, those were my questions.

Moderator: Thank you. Monica, may we please request you to announce your organization name as well?

Monica Bhaskar: Yes, SBI Mutual Fund.

Moderator: Thank you. Our next question is from Aejas. Please accept the prompt on your screen, unmute your audio and video and proceed with your question.

Aejas: Thank you. My name is Aejas, I'm calling from -- I'm calling from Unifi AMC. Sir, I have one very specific question, which is that if you look at GSK in a block of, 3 to 4 years, in the last, block of this 3 to 4 years, you've been able to benefit from API prices being rather benign, some of the restructuring and employee productivity measures that you have taken have helped the bottom line.

Additionally, SG&A and your adoption to the digital channels has helped. And that has led to a situation where the growth in profitability has been materially higher or ahead of the growth in

revenues.

But given where we stand today, my question is really that if you were to take the block of the next 3 years or 4 years, could you give some directional sense that, will profitability be ahead of revenue numbers?

And the reason I ask this is because you've alluded to the base portfolio growing at a certain rate and the innovative asset portfolio will continue to grow ahead albeit a low base, but the margin profile of that and given the launch expenses with the newer assets will be always heavier and they're coming in blocks, right? So is it fair to understand that the block of the next 3-4 years, the topline growth will mirror the bottom line growth? That's my question.

Bhushan Akshikar: Sure, it's a very good question, Aejas. So one of the things that you probably didn't miss out, I mean you made a good reflection of the last 3-4 years, but I just want to remind all of us, 3 years ago when I and Juby stood up 3.5 years back for the fi

rst time, I talked about the impact that we had with the NLEM then.

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And if you remember, we were the most significantly impacted with the NLEM with some of our biggest brands getting included in the then NLEM list of 2022. When I scope out the three and in spite of that, we drove volumes and we are today if you look at where Ceftum and T-Bact are as two brands. We are in value terms, in spite of having a price cut of more than 55%, in value terms Ceftum is in the -- for a calendar year also it's above INR200 crores.

So in we've more than recovered in terms of the value, that's because we've grown the volumes by 3% to 4% over the last four years. But I think coming to the next three to five years when I were to again look at the prognosis, this time around when the NLEM happens, one the impact on our top line, bottom line for the GenMed business because most of our brands are already even if you to re-average we are in a certain band so we there are no out of the—no outliers so to speak in terms of what we underwent the last two times around.

So I think that's one of the most significant changes. I think to your point around growing the innovative portfolio, we are in that sweet spot. I think you talked about everything that we've done in the last three years to ensure that this machine is now working at a certain pace in terms of really maximizing our field force productivity, really dialling up our omni-channel integration. And that's where we are.

So I don't think even if you to scope out of the next three to five years, even with the new launches -- because remember these new launches, especially the areas that I talked of, don't require field forces of 1,000 and 2,000 people. I mean, if you even if we look at the oncology space or hepatology space, given the number of healthcare practitioners in these specific areas, the field force strength is in -- you can count them probably on your fingertips or probably multiple of fingertips. So that's where we are.

But as I said in the beginning, one of the other questions, our objective will be to hold and sustain these margins which we've worked to deliver over the last three years. So that would be the objective rather than expanding gross margin any more or expanding the EBITDA. I don't know, Juby, if you want to add something there.

Juby Chandy: So I think Bhushan you explained well. So the idea is we are in a very unique spot to invest to grow. So that is where our P&L is very strong, we can invest more and sustain the margins. That's the kind of strategy. So to long story short on your on your question, next three-four years our focus is to bring all these new assets bolt on the growth what we are seeing in addition to the base asset, at the same time use this good opportunity of the nice margin level to invest and grow the strategy.

Bhushan Akshikar: Understood. So it's a fair conclusion that it's going to result in a far more accelerated sales growth GSK for the next block of three to five years and profitability to grow broadly in line with revenue growth.

Moderator: Thank you. Our next question is a text question from Nitin Agarwal from DAM Capital. In the next three years, what proportion of our revs can be contributed by vaccines and specialty products? Also, is it fair to assume that vaccines and specialty products typically generate margins?

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Juby Chandy: Yes, on the margin level since there's lot more question comes actually. So the vaccines as well as specialty generates the vaccine margins because these are imported products. So that's one thing. The only caveat on the margin side is our base business, 80% of the business is the raw

material product, it's hugely dependent on the raw material prices.
As we speak, the raw material prices what we

are seeing in the market are pretty stable, at least for the last two years. So if any change in the raw material prices comes, there might be up and down on the margins. If the raw material prices soften, we might improve the margin level. Other way around also could happen. That's on the margins. On the field force, Bhushan, you want to take?

Bhushan Akshikar: I'll take it. Yes, I think again going back to -- I said in my first slide, what makes us unique at GSK is we continue to be broadly diversified. And when I -- when you look at the uniqueness of GSK in India, we still have a substantially significant base business, an established business, which allows us to therefore invest to what Juby said even in the earlier question.

And I think that's the uniqueness of this operating model. It allows us to absorb shocks as well as invest freely so that we can create the new growth platforms. That's how I see the next three to five years continuing to evolve.

Juby Chandy: Yes, but majority of the headcount is still focused on general medicines because we don't need too many headcounts in the specialty and vaccines because it's a small cohort of doctors we are targeting.

Moderator: Thank you. Our next question is a text question from Mahesh Hemchand Purohit from H.J. Securities Private Limited. What role do you see for our company in manufacturing and exporting to other markets?

Bhushan Akshikar: Well, so I think thank you very much Mr. Purohit for that question again. We continue to have our manufacturing operations in-house in our Nashik factory along with, of course, more than 20 contract manufacturers. At Nashik we do manufacture something which goes out to the WHO. So that's an area. Obviously as of now, given where we are in the supply chain, we are in for India, in India model.

We don't really manufacture anything for the global organization unless there are some specific cases where we've done something in the past 12 months, sending a national emergencies or some such situations where couple of countries have asked us to send. But otherwise our model is largely India for India. But that's where we are currently.

Moderator: Thank you. Our next question is a text question from Vishal Advani from Bandhan Life Insurance. Hello, sir, could you please share how many total MRs are currently dedicated to the oncology segment and where we stand today in terms of oncologist coverage? Thank you.

Bhushan Akshikar: Thank you very much for that question. So we have about 25 key account specialists in the oncology team. They've all come with collective experience of oncology. They've been in the oncology space for a substantial number of years now. So that's where we have. We cover about GlaxoSmithKline Pharmaceuticals Limited
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1,000 oncologists on the ground, anywhere between 850 to 1,000. Because that's—so that's the whole universe that we have currently.

So we cover medical oncologists and the and the fraternity both in surgical and radiation to some extent, but medical oncology is where we focus completely. As I said, the next one for us is the Hematology business. So that's where we will be investing in launching the Hematology team focusing on various types of blood cancers, beginning with multiple myeloma.

Moderator: Thank you. Our next question is an audio question from Abdul Abdulkader Puranwala.

Abdulkader Puranwala: Yes, hi, sir. Thank you for the opportunity. It's Abdul Qadir Puranwala from ICICI Securities. So my first question is with pertaining to your three verticals. So just wanted to get some sense from you as to, you know, in the last, say, couple of years and this quarter, how would the growth across your general medicines and respiratory portfolio specifically would be?

And on the vaccine side as well, we've talked about Shingrix doing well, but then on the residual side, if we look beyond Shingrix, how the

volume growth in the residual portfolio also if you could throw some light on?

Bhushan Akshikar: Sure. As I said, if you look at the correlation to the Q3 results, we have almost an 8% topline growth, 8.1%. And I did talk about supply constraints. So when you ask that question Mr.

Puranwala about the last few quarters, we've been quite objective in sharing with you the unfortunate incident at the CMO which took a toll for two subsequent and successive quarters for us in terms of stock availability and constraints.

So the comparison might not be correct because we were shaving off almost 4% on our growth because of non-availability of supplies, this even including Q3. So I think moving forward, that's where we are. This whole objective of driving a double-digit ambition remains intact. So that's where we are as an organization.

When you look at the question around vaccines, our vaccine business has been delivering a

double-digit growth for, I think, now six successive quarters. As I said, the GenMed business, which still accounts for more than 75%-78% of our topline revenue, underwent a significant challenge for two quarters bringing pulling that growth down to low single-digit, but that's where we are back now and I think that's where we will sustain our business.

Your question around Shingrix, I think even when you were to separate Shingrix from the overall vaccine business and look at our pediatric vaccine business, I think Juby showed that in one of his slides there in the presentation where we've talked about a double-digit unit growth for most of our pediatric vaccine business assets including Varilrix, Havrix. So, it's a fairly well-rounded growth that we've seen, especially in this quarter.

Abdulkader Puranwala: Got it, sir. And sir, secondly on just a clarification on, the close to INR12 crores of impact on account of the new Labour Code. So this is a part of your employee expense or we have adjusted that as exceptional item here?

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Juby Chandy: So we've put it as part of employee expenses given it's not material in terms of the numbers. So if you see the notes on the printed financials, we put it to that extent it's in the employee cost and it's one-off.

Abdulkader Puranwala: Okay. Okay. So just on that then, what explains the Q-on-Q dip of nearly say close to 10%, if I strip off, close to INR16 crores of one-time benefit again what we had in last quarter? So, I think employee cost is down on Q-on-Q basis, so if you could help me understand the reason for this dip?

Juby Chandy: Last year we had some one-off provision reversals in the last year, but employee cost as an inflationary base for the future, it will be growing close to 7% to 8%. That's the kind of targets or the underlying growth we have.

Abdulkader Puranwala: Understood. Thank you.

Moderator: Thank you. Our next question is an audio question from Viraj Mithani.

Viraj Mithani: Okay, I'm Viraj Mithani from Jupiter Financial. Am I audible?

Bhushan Akshikar: Yes, Mr. Mithani.

Viraj Mithani: Okay, good evening, sir, and thank you for the opportunity. Sir, my first question is the recent acquisition by the Glaxo parent, how is it going to benefit India and in what time frame will it benefit India? The biopharma and some two-three acquisitions have happened recently?

Bhushan Akshikar: Sure. So I think one of the -- it's a great question again. One of the stated intent for us is to launch and have early access to our innovation portfolio. So these acquisitions that globally we've made is for assets in the area of food allergies. That's a growing area where you see significant number of people developing allergies to certain types of food.

So, as and when appropriate, the good news is for most of these molecules, India will get pitched as a place to do clinical trials and as and when that happens, our ability to therefore reduce the la

g in terms of the launch timelines and therefore use the subset of data for Indian patients with the regulators is what will allow us to launch these innovative assets much earlier. So that's where we fit in. I think increasingly, we are seeing India locked into the global model of early access of innovation. So that's how I would see it.

Viraj Mithani: Okay. Sir, my second question is given the momentum your parent is, 12% to 13% growth seems too low actually, you know?

Bhushan Akshikar: No, we would love to grow much higher. But as I said, if you look at where we've come from, I think our ability to therefore hold and deliver consistent performance quarter after quarter is more important than having a flash in the pan. So that's how we see it.

Viraj Mithani: Okay. And sir, my last question is with supply constraints getting over and other things behind.

Now, can we expect double-digit growth coming in next quarters only? It's fair to assume that?

In topline?

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Bhushan Akshikar: So, I think it's a very good question again because we've always said that that's what we should be aiming for. As I said, the last two quarters were soft and we shaved off almost -- and I just said, we were shaving off almost 4% of our topline growth, our underlying growth was at least 4% higher. That's the intent, so I think I think that's where we will remain focused.

Viraj Mithani: Okay. Thank you and all the best, sir.

Bhushan Akshikar: Thank you very much.

Moderator: Thank you. Our next question is a text question from Vamsi Hota from ASK Investment Managers. What percentage of portfolio is currently under NLEM? Would our recent and

upcoming specialty launches follow the usual supply chain distribution led by stockists, or is there an increasing share of direct procurement via bigger hospital chains?

Bhushan Akshikar: So, our current NLEM, the portfolio that we have in the covered by the NLEM is to the tune of almost one-third.

Vamsi Hota: What percentage of portfolio is currently under NLEM? Would our recent and upcoming specialty launches follow the usual supply chain distribution led by stockists or is there an increasing share of direct procurement via bigger hospital chains?

Bhushan Akshikar: So, our current NLEM, the portfolio that we have in the covered by the NLEM is to the tune of almost one-third. Over 35% of our overall portfolio is under price control, 35%-38%. So that's the response to your first question. For the specialty and super-specialty portfolio, we still have, as you would expect given the agility required in the supply chain, to get these innovative assets as quickly as you can, whether these are infusions in a hospital setting or in day care centers, especially in oncology.

You don't have the same distribution setup that you would expect in traded products in general medicines kind of business. So we have a super distributor format, where we get, the whole focus is on getting fast service to the patients and the hospitals and the healthcare practitioners. So it's an agile supply chain model that we follow for the specialty portfolio.

Moderator: Thank you. Ladies and gentlemen, we have no further questions. Pardon me, ladies and gentlemen, we have a late entry to the queue. We have a question from Viraj Mithani. Please go ahead with your question. Viraj Mithani, you may unmute your audio and video and proceed with your question.

Viraj Mithani: Yes, am I audible now?

Moderator: Yes, you're audible.

Viraj Mithani: Sir, my thank you for the opportunity again. My question was will we benefit from the parent launch globally as well as the India would be simultaneous or would be some time lag?

Bhushan Akshikar: So it really depends on how our regulator, so look at the data that we submit. So if you've seen in the recent budget also, there is a whole focus on improving the regulatory landscape and r

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fast-tracking approvals. So that's clearly stated as one intent in the recently announced budget for the pharma sector.

And the reason is simple because although you may have approvals in the Western world, it still takes anywhere between six months sometimes to 18 months to get approval from our local regulator. So I think the regulator also have stated their intent to fast-track and accelerate, but it's a function of therefore how things evolve in the coming months.

But hypothetically speaking, the whole idea of launching concurrently given that we have now Rule 101, which means the six countries where you have already got the marketing authorization and the approval from the regulators there, you should ideally be getting the same in with data you should be able to get simultaneous approval.

So that's the eventual holy grail if I see it that way. But I think we are in that direction. We are increasingly seeing reduced timelines. But I think in the next 12 months we'll see how this space pans out.

Viraj Mithani: And sir, if I may squeeze in one more question. At what stage in say next seven years' journey will the share of new medicines will be broadly close to our traditional medicines?

Bhushan Akshikar: Sorry, I didn't understand the question. Is it in terms of the contribution to the topline revenue or...?

Viraj Mithani: In terms of contribution to topline revenue in terms of like say share of cancer drug, cancer drugs plus our vaccines would be more or less equal to our traditional medicines which have been there like, you know, T-Bact and other things like those.

Bhushan Akshikar: So Mr. Mithani, it'll so as I keep saying, I think we are blessed and fortunate to have a robust established business, both for GenMeds and vaccines. Now if you look at this INR3,500 crores plus business that we already have, will something replace overnight? Chances are no. But I think I've always talked of the concept of freshness index, which means what is what are the new launches really contributing in terms of a topline.

We had three years ago we said that our intent is to have at least a 10% freshness index, which means products that are launched in the last two to three years should contribute 10% of our topline. Today when you look at the coming financial year, that's a clearly stated intent that our oncology business, Shingrix which is about less than three years old, the RSV product that we will launch, Blenrep, all these products put together should be in the range of that 10%-15% mark.

So I think as you with every passing year that number will only increase as I see it. But even after if I were to scope it over a 5 to 7 year period, given the importance of our base business,

that base business will still be important. I would probably safely hazard a number of around 20%-25% of the freshness index.

Viraj Mithani: Okay, thank you and all the best, sir. Thank you for the opportunity. Very much.
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Moderator: Thank you. Ladies and gentlemen, that was our last question. On behalf of GlaxoSmithKline Pharmaceuticals Limited, we conclude this conference. Thank you for joining us. You may now disconnect.

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"GlaxoSmithKline Pharmaceuticals Limited
Q4 FY2026 and Full Year Earnings Call "
May 13, 2026

MANAGEMENT : MR. BHUSHAN AKSHIKAR – MANAGING DIRECTOR –
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED
MR. RONOJIT BISWAS – CHIEF FINANCIAL OFFICER –
GLAXO SMITH KLINE PHARMACEUTICALS LIMITED

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Moderator: Hi, good evening. This is Dorwin Dias , your moderator from Chorus Call. Welcome to the GlaxoSmithKline Pharmaceuticals Limited Q4 FY 2026 and Full Year Earnings Call . From the management at GlaxoSmithKline Pharmaceuticals Limited, we have Mr. Bhushan Akshikar, Managing Director, GlaxoSmithKline Pharmaceuticals Limited and Mr. Ronojit Biswas, Chief Financial Officer, GlaxoSmithKline Pharmaceuticals Limited.

By participating in this event, you consent to the recording, distribution, and publication of this event. Kindly note that this call is meant for investors and analysts only. All participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation from the management concludes.

I now hand the conference over to Mr. Bhushan Akshikar. Thank you, and over to you, sir.

Bhushan Akshikar: Thank you very much, Dorwin . Once again, a very warm welcome to the earnings call for our Q4 performance as well as the full year. I am also incredibly happy and excited that we have our new CFO, Ronojit , joining us on this call with me .

Over the last 3 years, we have been very objective about engaging with the investor community as soon as we have our quarter results Board meeting. So here we are again, this time sharing with you some of the key highlights. But before that, just wanted to re-contextualize our priorities.

So over the last, as I said, 2 or 3 years, we have been clearly calling out the roadmap that we have put for ourselves. As a broadly and a very well-diversified healthcare company in India, we continue to operate at both ends of the spectrum.

At one end, in the area of prevention with both pediatric and adult vaccines, with a large GenMed business that impacts more than 270 million Indians annually with products that impact lives at scale through our primary care portfolio. And for the last 2 or 3 years, we are building our new growth platforms on the specialty side.

So that model continues to evolve. I just wanted to also restate, aligned with the global priorities, our focus remains relentless on three priorities, which is to drive top-line growth, and that is something that you will keep hearing over the next few quarters as well as we start this upcoming financial year.

The second priority aligned with our global strategy is also unlocking value of our innovation portfolio, especially in key markets like India, to accelerate the launch of innovative medicines and vaccines with accelerated timelines as well as reducing the launch lag that we have seen in the past.

So that is something that is going to continue to be one of the mainstays. And last but not least is continue to embed GenAI and digital ways of working so that we can continue to pull out inefficiencies and focus on improving value across the value chain.

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Next slide, please. A quick highlight on where our quarter was for us. If you all seen, the quarter delivered almost INR 64,000 crores to INR 65,000 crores for the industry externally. These are all numbers from IQVIA, the syndicated research agency. As many of you know, we operate in select therapy areas, so the therapies where we participate, the represented market grew by about 7% for the quarter for Q4.

Within that, we did not see -- we typically are mindful that this quarter is not the peak season for us given the dependence on the acute portfolio. But if you see that table, we have the Advanced Oral Antibiotics, that is the acronym for AOA, there was muted growth there. In fact, that therapy area declined by about 2 % for that specific quarter.

And then other areas where we operate, primarily paracetamol market, grew by about 7 %. So those are all the external data points in terms of how the

market evolved. In context of that

growth, how did we deliver our performance? So we will move to the next slide to give you a quick snapshot of how our brands delivered.

So broadly speaking, we delivered a competitive performance across our three pillars. We did see some good tailwind for the promoted brands that we have, especially brands like Augmentin, Calpol continued to grow ahead of the market. So if you see the top two or three brands that we have in our promoted portfolio, they grew at least 3 to 4 percentage points higher than the market as evident in the EIs.

Where we saw some soft demand was on brands like Eltroxin as well as the distributed portfolio. So there was clearly a mixed bag on the general medicines portfolio, which kind of diluted our growth story for the quarter.

While I am very happy to report that vaccines continued the double -digit performance for the quarter as well, both the pediatric vaccines business as well as the adult vaccines business led by the shingles prevention vaccine continued to deliver and continue to create newer models, operating models for us, especially in the adult space.

So that part of the business has continued to grow from strength to strength. For the specific quarter in isolation, it was a great quarter for our adult vaccine, Shingrix. We not only been able to consolidate our prescription base but also pivot to a new cardiovascular metabolic strategy, which we have been working on for the last few months, clearly harnessing the bidirectional linkage between cardiovascular metabolic disease and herpes prevention. So that is an area which we have been working on the last few months.

The new growth platform for us, clearly specialty business, which was almost non-existent, led primarily by the respiratory portfolio with Trelegy Ellipta as well as Nucala, both have delivered performances in line with our expectations, both sustaining market share in spite of generic onslaught, especially for Trelegy Ellipta. And Nucala has continued to build on the base in severe asthma patients month after month.

Every month, we have been clocking more than 100, 120 patients. So that part of the business has really grown well. On the new growth platform of oncology, if you recall 2 quarters ago, I shared with you the roadmap for us to really unlock value with our oncology portfolio globally.

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We launched two assets, Zejula for ovarian cancer and Jemperli in endometrial cancer. So both of them have continued to grow. The big news for us as we began Q4 was we got the approval for the Ruby -1 trial, essentially unlocking value in first -line treatment of patients with endometrial cancer.

So I think that is a significant win for us, and both these assets have continued to build a rock-solid base for us as we step into the coming financial year. We have continued to develop and build on our omnichannel digital model. So apart from the 2,000 plus reps that we have in the GenMed team for face -to-face interactions, we have continued to improve our share of voice with digital ways of working, really unlocking a seamless customer experience through different touch points, including digital ways.

So that has continued to be one of the focal areas for us as we closed out the last financial year.

The next slide, just one slide on a story that I have been talking about for the last couple of years. If you remember, I have been talking about the freshness index for our company in terms of what our new products will contribute. And one of the stated intents that we had talked about was having at least a 10 % top-line contribution coming from our innovative portfolio. Happy to report that as we closed out Q4, we have almost 6 % of our top -line coming from the new portfolio led by innovation -driven medicines and vaccines, starting with Jemperli and Zejula.

a. I

just talked about these two.

I just talked about what we have been able to accomplish with Shingrix month after month. And last but not the least, the respiratory portfolio led by Trelegy Ellipta and Nucala. So, all in all, the performance on the broad-based portfolio that we have has been in line with our expectations. The growth story has been slightly muted on account of two factors. As I said, the first one was the distributed portfolio, the tail-end brands did not give us the kind of tailwind that we expected, some significant headwinds there.

But more importantly, we continue to have the last of what we saw on the supply constraints, especially with one of the CMOs that we had the fire. I think this was the last quarter where we had some residual impact, and those are the two principal reasons why we were not able to have the growth in line with expectations. The underlying growth would be in the range of about 5% to 6%. So that is the story for Q4.

I will just hand over to Ronojit to give you a quick snapshot of the financials before we open it up for questions. Over to you, Ron.

Ronojit Biswas: Thanks, Bhushan. Good evening, everyone. This is my first earnings call in this role, and I want to briefly outline how we are looking at the business both for the quarter and the full year. I will begin with the full year financials before we get onto the quarter. On a full year basis, revenues were up 2%. EBITDA grew double digit at 11%. PAT growth was also double digit at 10.0%. So, in summary, in the year, we continued our trajectory of profitable growth and margin expansion.

As Bhushan already called out, our top-line growth was muted because of the disruption in a key CMO, and that impacted largely our general medicines portfolio. But that said, our promoted GlaxoSmithKline Pharmaceuticals Limited
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brands delivered competitive external performance and we gained market share. The other point to emphasize is we are building momentum with our innovative portfolio and our new launches. Both Zejula, Jemperli have delivered a great showing in including the final quarter of the year. Both products with now first-line indication alongside our new respiratory portfolio, which is Nucala and Trelegy. Shingrix in adult vaccine was actually a key growth driver. As Bhushan has touched on this earlier, we have delivered strong prescription growth and we continue to build on this new data we have in cardiovascular and metabolic disease. And that said, even our pediatric vaccines portfolio was up 9% where we maintain our lead in the private vaccines market in the segments we operate.

Gross margins for the year were up 190 basis points, and combined with the SG&A efficiencies we have driven, our EBITDA ratios improved sharply to 34% for the full year, which is an improvement of 290 basis points versus last year. Now our SG&A ratios reduced by 1% point, and this is in the backdrop of continued investment in our new launches and innovative therapies because this is a key driver to our future growth. The main gains on the main efficiency gains were from field force productivity and AI-led optimization, which helped with our disciplined cost management.

We did cross a milestone for the year. We delivered our first-ever PAT excluding exceptionals for more than INR1,000 crores. Earnings per share was at INR59.6 per share. That is up 10% versus last year. We had a healthy cash generation, and our year-end cash position is now at INR2,745 crores. Our return on capital employed was at 61.0%. And if you have seen our announcement, the board earlier today declared a final dividend of INR57 per share.

Next slide, please. So, we have consistently improved our profitability metrics and the return ratios are stable. So, as you can see, both EBITDA and gross margins show meaningful improvement over time and versus the pre

vious year. Next slide, please. Right. So, moving to the quarter standalone, sales were up 2%. EBITDA grew at 5%, and PAT growth was at 6%. Our headline sales were muted, as we alluded to, because we continue to experience some lingering impact of supply constraints. And this was also as a result of some delayed vaccines shipments, which will result in some shifting of sales into the June quarter. Notably, we continue to see great momentum on our new launches.

The oncology portfolio and the new respiratory portfolio, which are now a meaningful portion of overall sales at 6%. EBITDA for the quarter improved 1% point up to 35% on better gross margins and cost management. We maintained our focus on cash generation and as we strengthen and optimize the balance sheet.

Next slide, please. This slide summarizes the journey we have been on. On the left is the transition to an innovative and specialty-led business. And this is important why? Because these are higher growth, higher quality of sales, strongly differentiated IP-protected assets, and this is where we see our future growth coming from. The second chart on the right is the constant profitability improvement, which we have delivered all the way up from 24% four years ago up

And to highlight again, in terms of competitive performance, underpinning all of this is competitive performance on our general medicines brands where we continue to gain share. So, to sum up, in combination, the market share gains, the portfolio transformation, and the profitability gains puts us in a strong position as we enter the next year.

Thank you. And with that, we will move on to questions.

Bhushan Akshikar: I will just spend a minute because I think I skipped that slide and thanks a lot, Ron, for that. As we segue into the next financial year, and of course, we will open it up for questions on Q4 as well as the last financial year, I talked about the need to really build that Freshness Index, and I am happy to share with all of you that we have currently 26 ongoing clinical trials in specific areas of our interest, essentially for unmet medical needs.

So, we have 14 studies going on. Happy to report that we just have the approval, we got the marketing authorization for Belantamab, which is for relapsed refractory cases of multiple myeloma, an antibody-drug conjugate that we will be launching very soon. So that is also a part of those 14 clinical trials. We also have a new compound, Velzatinib, which is for a type of gastric cancer. The trials of that are ongoing here.

And of course, another antibody-drug conjugate, B7-H4, for which we also got India to be included in the Phase 3, 3B trials. So, this part continues to be one of the biggest building blocks for us. On top of that, of course, we will continue to build our strength in the respiratory and immunology areas with the compound mentioned here.

Liver disease continues to be a focal area for us, and in the next 12 to 18 months, we should see more news in the areas of chronic hepatitis B with specifically Bepirovirsen and the compound that is indicated for fatty liver disease, which is now called metabolic-associated steatoh epatitis, that is Efruxifermin.

So those two compounds will also help us build new expertise in the area of liver disease. So, I think I just wanted to sum that up because all these will play out in the coming financial year on a solid foundation of our established business of both GenMed and pediatric vaccines. So that is the slide that I just wanted to sum up before we open it to questions.

So, Dorwin, we can open it up for questions.

Moderator: Certainly. Thank you. We will now begin the question-and-answer session. Our first question is a text question from Jinal Sheth from Awriga Capital. Hello. Somewhere you had

given an

indication that this supply issues should be out of the way, but it was a bit negative to see the impact continued. When do we expect these to rationalize as in a way that this is lost sales? Is that correct?

Bhushan Akshikar: Is that the end of the questions, Dorwin?

Moderator: Yes, that is the end of the question from Jinal.

Bhushan Akshikar: Sure. Thank you very much, Jinal, for that question. Yes, I do own up that in the last call, we had clearly articulated that the supply issues are behind us. And I think when it comes to products like Calpol, we were able to mitigate a large part of the impact that we saw in the first three quarters. As you heard from Ron, we had some residual issues on our tail-end brands, which we still could not address given some of the quality issues, and that is why it has taken longer.

To answer your question, I think we have taken some robust steps to ensure that from a business continuity planning, all the learnings that we have had with this incident are factored in so that moving forward, right from the beginning of the first quarter of this financial year, we are not caught by surprise, especially with supply constraints. So that is where we are. You also heard from Ronjit that a part of that supply constraint.

So, we shaved off almost 3 %, 3.5% of Our top-line growth even for the quarter because of the supply constraints.

Part of it was phasing because of the late arrival of some vaccines consignments. But that is -- as I said, it is a phasing issue, so it should get reflected in the Q1 of this financial -- coming financial year. So that is where we are. I must reassure you that from a Board standpoint, from a management standpoint, we are putting in some robust mechanism to ensure that the business continuity planning as well as alternative contingencies are in place, especially to avoid any such issues in the future. Thanks for that question, Jinal.

Moderator: Thank you. Our next question is from Ahmed Madha. Ahmed, please accept the prompt on your

screen, unmute your audio and video, and proceed with your question.

Ahmed Madha: Yes. Thank you so much. This is Ahmed from Unifi Capital. I have a few questions. Firstly, just to reiterate on the point you mentioned regarding CMO supply challenges. Is it fair to assume - I mean, I had a similar question last quarter and you had mentioned 3 % to 4% revenue impact and things normalizing maybe from November, but obviously Q4 we had a similar impact. So is it fair to assume from Q1 things should normalize and there shouldn't be any supply -side challenges?

Bhushan Akshikar: So Mr. Ahmed, thank you very much for that question. And I can only reassure you, given the visibility and the oversight that I along with the management team have, beginning the month of April and Q1 of our new financial year, we don't have any supply constraints. As I said, including the shipments of vaccines that were delayed, for Q1 of this financial year, we are behind -- we are over that curve.

Ahmed Madha: And you also spoke about vaccines -related supply challenges. What would that be? Will it be importing from the parent entity and delays in that case? If you can expand a bit on that ?

Bhushan Akshikar: Sure. So if you recall, our vaccines have to be cleared by central labs. So it was just a timing issue. Although the vaccines arrived in the month of March, we couldn't get the clearances in the right time from the regulatory agencies. We did that in a matter of a few days, but then it had spilled over into the Q1 of the financial year. So it is not -- it is just a phasing issue. There are no supply constraints there. It is largely a phasing from Q4, which has got spilled over into the Q1 of this financial year, the next coming year.

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Ahmed Madha: Does that mean the primary consumption and growth and market share, all of that is intact and is in a sense the -- you putting inventory in the channel is delayed? Is that the way to look at it?

Bhushan Akshikar: So if you -- that is a good question, Mr. Ahmed. If you look at our off -take or the secondary sales analysis as captured even by the syndicated research agency, our growth continues to be a healthy high -single-digit or double -digit. So that is where the underlying fundamentals and the health of the business is still intact. These phasing issues are just off from one month to the other. So the answer is yes.

Ahmed Madha: Coming to the Trelegy product and SITT market in general, you have spoken about fluticasone going off -patent with the loss of exclusivity and that gaining market share. How should one look at the product comparison between -- different like, a lot of similar sort of products coming in the same therapy? Do you see you gaining more market share moving forward if you can explain a bit on that?

Bhushan Akshikar: Sure. So Mr. Ahmed, as you know, there are country with almost 70 million to 80 million COPD patients. So the market is -- market continues to grow, and within that growing market with patients who continue to need treatments which will really get them relief, the single inhalation triple therapy continues to be the fastest growing. So that is not changed.

Yes, it is not fluticasone, we lost the patent on umeclidinium, which was one of the ingredients in the triple combination. So you have a lot of similars or generic versions of our product, but nothing like the original Trelegy Ellipta. So in spite of having 8 to 10 generic versions of the combination, of the triple combination, I don't think we have the -- still anyone which has a device or the quality of ease of usage as Trelegy Ellipta.

So in spite of having those 8 to 10 generic versions being launched over the last now almost 10 months, we have not only continued to hold, but the last couple of months, including the last month of the last quarter and the first month of this quarter, we have grown our patient share and market share as well. So I think in the coming quarter, I should be able to give you more color, but our continued emphasis on Trelegy Ellipta remains intact.

Ahmed Madha: In the vaccines business, what sort of growth rate one should think about? I mean, the prescription data you mentioned in the presentation is pretty good for Shingrix and you had some supply challenges right now. So if I have to look at just Shingrix as a product, is the sort of growth we did in FY26 sustainable for a couple more years considering we still have a large market to penetrate? What will be your thinking in terms of scaling up this brand?

Bhushan Akshikar: So Mr. Ahmed, as you probably know, in every investor call, I always say we don't generally give forward -looking guidance. But in terms of ensuring that we are maximizing every single opportunity, our ability to defend our market leadership position in our private self -paid pediatric market continues.

So that is a space where all the antigens where we operate, be it the hexavalent category, be it the Tdap category, be it the hepatitis A or the Havrix, so all the antigens where we have in the pediatric space, we are distinct leaders. And I think that continues. We have grown that business

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in double -digit for the last two successive financial years, so the intent will be to maintain that momentum.

I think your question around adult vaccination, with every passing quarter, we are only improving our ability. And mind you, this is all self -paid patients. We are just about with 2 years of experience behind us, now we are getting into some of the institutional accounts, getting into public accounts.

So I think the next 12

months, this coming financial year, especially for shingles prevention vaccine or Shingrix, will be path -breaking. And that is how we see the coming year. So I think in terms of giving you a blended range, it should be strong high -double -digit numbers for the vaccines business.

Ahmed Madha: Sure. One more question around the specialty portfolio. I mean, you have defined it as Shingrix and oncology product and new respiratory products. And the number disclosed in the presentation is 6% of the sales in Q4. Will it be possible to give the same number comparable for same products in last year Q4? And as a whole, is there any sort of a sense or management thinking what percentage we need to scale this number to for the innovation portfolio?

Bhushan Akshikar: I think Mr. Ahmed, I am sorry to interrupt you, but I think I answered some parts of that question even when I was presenting. Our stated intent is always to say -- I have always said this that our Freshness Index should be around 10%, which means can we have the innovative portfolio contributing about 10% of our top -line.

On a like -to-like basis, that number was about 2%. You heard it in my CFO's pages, we have brought it to 6% for the last quarter, and that is the journey that we have been on. So I think we will be relentless to ensure that this growth platform really kick s in for us. Watch this space. I think it is still early days, so watch for the next couple of quarters.

Ahmed Madha: I have a few more questions, so please allow me. Two questions on margins.

Bhushan Akshikar: Go ahead.

Ahmed Madha: Firstly, for the specialty portfolio, how should one think of the margin profile considering obviously these are not manufactured in India and you will have imports? So how should one differentiate the margin profile of specialty portfolio versus your base business?

Secondly, just considering we have significant or rather material NLEM exposure and there is a lot of inflation in RM prices and it is showing up in API prices and it will come to our P&L as well. So how should we navigate or how are we thinking to navigate it in because there will be some limitations to how much price hikes you can take? There will be some RM volatility. So if you can just spell out how are you looking at margins in two different buckets ?

Bhushan Akshikar: So two things there, Mr. Ahmed. Hopefully, this is the last question. Others also get a chance to ask questions. But more importantly, if you remember, we have always said that it is our stated intent to remain relentlessly focused on the top -line growth. So that is our first objective. I think GlaxoSmithKline Pharmaceuticals Limited

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anything that comes in the way of our top -line growth is secondary. The primary objective is to grow the business.

Yes, in the process, we will be also focused in expanding the gross margin. So I think that is also worked out you saw in one of those pages that Ronojit was presenting, the significant improvement that we have had, therefore leading up to the EBITDA margins that we have and the operating margins as well.

But that is not the end in itself. I think wherever possible, whether it is cost optimization, whether it is locking in with agreements which will kind of protect us for the next 12 months , 18 months, those agreements are in place. We are constantly looking at the evolving situation externally and given the handle that we have on the end -to-end value chain including raw materials, packaging material.

I think we are watching this space with every passing month. I think at this stage, I can't really crystal ball gaze and tell you, but I think our focus will be on to hold on to the margins that we have been able to accomplish till now. That is how I would answer you. Your second question on the NLEM, yes, we continue to have almost 40% of our business covered in NLEM.

But I think t

his time around, even when NLEM is announced, we don't have any surprise like last time. I am sure you recall last time we had two or three of our big brands being included. Our engagement along with the industry associations are in the right direction. So I think on that front also, we are -- we anticipate that we are pretty much well ring -fenced as of now and we

have all the scenarios planned out well as of now.

Moderator: Thank you. Our next question is from Gokul Maheshwari of Awriga Capital Advisors LLP .

Bhushan sir, in the past annual reports and interactions, you have expressed an aim to grow the business in double digits. In FY26, we grew 2%, in FY25 we grew 8% and FY24, 7%. How do we bridge this gap to achieve the aspiration double-digit growth?

Bhushan Akshikar: Thank you very much, Mr. Gokul. I think it is always good to hear from you. I think our ambition to grow double digit remains undiluted. So that is the first and it is not only an intent, and that is the reason why I talked about the innovation portfolio. We are mindful that our established business is significantly substantial given our operating model.

As I said in the beginning, we operate at both ends of the spectrum with a large established business and now on top of that bolting on all these growth platforms. So if you recall 3 years ago, we didn't have this kind of launches happening. I just talked about we getting the market authorization for multiple myeloma in the area of multiple myeloma for one of our key global assets called Blenrep, Belantamab.

So this is on top of what we already have for ovarian and endometrial cancer respectively for Zejula and Jemperli. We are also -- we have also got the Subject Expert Committee approval, we are awaiting the market authorization for one more compound, which is a vaccine to prevent respiratory syncytial virus, which is the RSV virus.

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So that is another one. So I think between now and the next four quarters, we have at least one significant launch happening for an unmet need and that too with significant value for each asset. So that is how we remain pretty confident about bridging this gap. Yes, our stated intent and the actual there was a delta and I just talked about the underlying growth being in the range of 6% even this year if we had not had this unfortunate headwind of the CMO incident. But that said, as I said in the beginning, the ambition of a double-digit growth remains intact. That is the roadmap that we are very clearly focused on.

Moderator: The second question from Gokul Maheshwari is what is the growth in general medicines, vaccines and specialty in FY 26?

Bhushan Akshikar: So Mr. Maheshwari, as it is we generally don't give segment-wise breakup, but just to give you a broad view, we remained more or less flat for FY 2026 on GenMed and that was largely because of losing out almost more than INR100 crores worth of supply constraint. So if you adjust that, the underlying growth would be in the range of at least 4% -- 3% to 4%.

And that has been a significant headwind for us. On the vaccines portfolio, as I said, we have grown double digit with a blended growth which is in excess of 11%, 12% for the vaccines portfolio. And specialty, I think on a smaller base, the growths have been significantly higher. So that is the -- that is the blend for us and that is why we ended up at 2%.

Obviously, moving forward, our assumption as we see the market playing out with all our supply issues behind us and of course our flagship brands led by Augmentin, Calpol, T-Bact in place, we do estimate that the GenMed business should be back to what we consistently delivered in the range of anywhere between 6% to 8% on the top-line growth. So that is what we estimate and the large part of the growths will be unlocked by the new assets plus the existing business

So that is how we see the composite.

Moderator: Thank you. Our next is a text question is from Vishal Manchanda from Systematix. Hi, good evening everyone. On your oncology assets, how long can it take for you to ramp up these to peak potential and also what is the size of our field force that you have deployed in oncology?

Bhushan Akshikar: Thank you very much. Good to hear from you, Mr. Manchanda, as always. So as you probably would be aware, oncology business is because for a country of our size, we barely have a thousand oncologists, so you don't need armies of several thousand people. In oncology, we have a team of about 25 people on the ground already for solid tumors, which is where we have Zejula and Jemperli.

We have just started the recruitment for a hematology team and that is where we will put another team to start working on Blenrep for which we have just got the market authorization. So that is that is the field force size as of now. But more than the commercial teams, I think a large part of the focus will be on the medical education and the engagement because this is about cutting-edge science.

It is about the new readouts that happen almost on a daily, weekly basis with the trial data. So that is where our energy is right now in bringing evidence and new science to our healthcare

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practitioners, especially the medical oncologists and the hematologists in the coming weeks and months.

To answer your first question, as you would be again aware, for oncology assets to get to peak sales, you need to have both arms unlocked. One is all those patients who are in the private setup who pay out of pocket, but more importantly, also unlock the public accounts and both are working hand in hand.

As of now, we have been very successful in unlocking value in the private setup. With every month, we have patients now for almost on the eighth cycle. We have some remarkable successes being shared by healthcare practitioners where patients are in complete remission and they have had complete response rates, especially in endometrial cancer with our dostarlimab compound, which is Jemperi.

So I think we are moving from strength to strength with each month. The answer to your question whether it takes 12 months, 24 months, I think we are putting now a new structure even to unlock our access strategy. So that is where we are trying to see how we can accelerate and get to peak sales in the next 12, 18, 24 months for these assets.

Moderator: Thank you. Our next text question is from Julie Mehta of 360 ONE Capital. How much revenue contribution comes from specialty and innovation portfolio on an annual basis? Secondly, it is great to see our pipeline under clinical trials expanding and even at parent level, we are seeing several assets progressing well. Just to understand, what is the kind of market opportunity do we anticipate from Blenrep?

Bhushan Akshikar: Two parts of the question. First is again, as I said, the big change you seen in the last 3 years and I have talked about it in every investor call, in every earnings call is our ability to therefore unlock the significant value from our global pipeline. And that is why if you recall that number was 16 global trials two quarters ago, we now have 24 global trials happening in India. And that is a significant milestone, especially when you get Phase 3A, 3B trials happening in India. We can use the same data and use it with the regulators to help accelerate the launch. A case in point is Blenrep. I just talked about Belantamab in multiple myeloma. As many of you may be following, even in Western markets like the US, this product was launched just a few months ago.

So you can imagine the launch lag is barely reduced to 6, 8 months now between the first launch that happens typically in the US market and India. So that is going to be

the focus for us, how

do you really build the regulatory agility to keep this building block of accelerated launches as the central theme of everything we do moving forward.

To answer your next question, multiple myeloma is a significant area. It is the third most reported blood cancer reported, hematological malignancy reported even globally. So if you see the number of cases that are reported globally, very significant huge numbers. Coming to India specifically, if you look at multiple myeloma, we have about 18,000 new cases that are reported in India annually.

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So that is the incidence. The prevalence is in the area of 40,000, which means there are always, at any given point of time at least 40,000 patients who have multiple myeloma. And the reason I am mentioning those two numbers is as you may be aware, multiple myeloma is essentially a refractory disease, which means any patient who gets response in the first line will still need because there is a relapse. He or she will still need a second line. And I think the good news for us and patients who need this innovative treatment is the approval that we have got is for second line. So, the advantage of using Belantamab in early lines and therefore having significant overall survival, progression-free survival is -- will be the key aspect for us.

I think in terms of sizing the opportunity, I won't put it in values, but I think our objective will be to have as many patients of the 17,000 or the prevalence of 40,000 that are available at any given point of time to benefit from Blenrep in early lines, especially in second line as we keep hearing from healthcare practitioners. So that will be the focus for us. I think watch this space as we launch it and share the outcomes and the progress.

Moderator: Thank you. Our next text question is from Udhaya Prakash from Value Research. Can you expand a bit more on the supply issues? What actually started it? Because if we look at the growth over the past three years, except for two, three quarters, we have had multiple quarters with single-digit revenue. Of course, we had a healthy expansion in margins, which supported profits, but is this the level of growth we can expect going forward too?

Bhushan Akshikar: Thanks again for that question. I think I answered this question in part in one of the earlier questions. As I said, the stated objective, not just the intent, continues to be a double-digit growth. So, I think that is something that is undiluted in terms of the roadmap that we have set

for ourselves. Yes, we were -- we were dealt with an unfortunate incident at one of our most significant CMOs with a fire that started exactly a year ago in April.

I mean, the event happened in April, but the follow-through in terms of remediation, getting the site re-validated, ensuring that every single pill that comes out of that site has the same standards that GSK expects so that no single patient is exposed to any risks and safety issues, that took longer than expected. And that is really the story for us.

As I said, it is unfortunate, but it has also made us work harder to look at business continuity plans in place and look at alternatives as well so that we move forward in the coming times. So I think that is where we were. A large part of the portfolio that got impacted was led by Calpol in the first six months.

We got the products out by the end of Q3 for products like Calpol, but there were still some tail-end brands, some brands which were distributed, which still are material for us in terms of losing out those INR28 crores, INR30 crores that we did for Q4. So, I think beginning the first quarter of this financial year, that supply chain is now remediated and we don't have any more supply constraints from that side. So that is the long story to give you a quick summary of what happened

there.

Moderator: Thank you. Our next text question is from Vishal Manchanda from Systematix. Any broad guidance on how operating margins would work out for your oncology portfolio versus rest of GlaxoSmithKline Pharmaceuticals Limited

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the business? Also to get a sense on affordability of your oncology portfolio, can you give a range as to how it is priced on a per-patient per-year perspective? Can you also guide whether these treatments get covered under health insurance plans?

Bhushan Akshikar: Mr. Manchanda, thanks for the questions again. Yes, as you probably are aware, I will take the pricing question first. We have always had an India-centered, India-focus tiered pricing model.

So obviously, if you look at the cost of treatment for products that we have both in ovarian cancer and endometrial cancer, namely Zejula and Jemperli, we do have patient assistance programs in place.

So, I would say the cost of treatment is still in line with the class of treatments. Remember Jemperli is an immunotherapy, it is among the leading immunotherapies globally in terms of the kind of indications that we are pursuing. So given the kind of evidence, given the overall benefit that we are seeing both for progression-free survival and overall survival, the cost of treatment is pretty much in line with what you would see with the standard of care right now in the market. I wouldn't hazard a guess in terms of the exact prices because every patient needs a unique dosage regimen. So anywhere between INR10 to INR16 lakhs is what patient ends up paying in some of these types of cancer. But again, as I said, it is a function of the dose required, it is a function of how many cycles are required. But to especially overcome that challenge, we have always instituted specific patient assistance programs under the Project Phoenix for patients who need that benefit.

Your second part of the question, what will it take to unlock? I think I answered that. Our objective is also to unlock value in the public accounts. We are working very closely to get on the GTE exemption list so that we can start participating, especially for the proprietary innovative medicines to get on some of these formularies. That is an equal emphasis that we are placing.

Probably in the next six to eight months, we should have more progress on the public account.

But as of now, we are moving from strength to strength every month within the private setup, private segment. Your question around the insurance schemes, I think again, every insurance policy, every insurance scheme has its nuances.

Not all insurance policies cover cancer coverage, especially for medicines, although hospitalization is covered. But there are certain policies which do cover, and that is where we are just about scratching the surface as of now. So there is no real impact that we have seen in terms of unlocking the insurance segment. I hope I have answered your questions, Mr.

Manchanda.

Moderator: Thank you. Ladies and gentlemen, if you wish to ask questions, you may click on the ask a question tab. Alternatively, you may enter your question into the chat box below. Our next text question is from Rajat Srivastava from Tata Asset Management Company. With top line expected to grow in double digits, do you expect margins to see further expansion or would you be reinvesting back for growth?

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Bhushan Akshikar: Mr. Rajat, as I said in the beginning, our objective is always to grow the top line. In the process, if you are able to get a favorable tailwind on the EBITDA, that's an -- its and outcome that not we work as a primary endpoint. Our objective will be to maintain the margins and not to -- we would love to reinvest in the business.

As I said, the amount of launches that we are having in the next six months for this locally

entity listed

entity are significant. We have at least two significant launches in the area of oncology, one in the area of adult vaccine, and that would all require investment. So, we would rather invest that money in growing the business with the expectation that we are able to hold the margins. So that's my response. I don't know if Ron, if you want to add something there.

Ronojit Biswas: No, I think you've summed it up very well.

Moderator: Our next question is from Vishal Manchanda. Vishal, you may accept the prompt on your screen to unmute your audio video and go ahead with your question.

Vishal Manchanda: Yes, hi. Thanks for the opportunity again. So on Bepirovirsen, which is the hepatitis product, when do you expect an approval there?

Bhushan Akshikar: So, we are still working very closely with the subject expert committee. The trial was completed. The first readout, if you saw the global readout, we are expecting the launch in several key markets, including Japan. We just signed a deal in China for a partnership to unlock, given the huge prevalence that you see of chronic hepatitis B.

Given the fact that it's the first potential functional cure for chronic hepatitis B, we do estimate there's a huge amount of work that we'll have to do along with regulatory agencies. More importantly, the health ministry, both at the state level as well as the federal, at the national level. So, we want to ensure that we have got everything right. To your question, we estimate the market authorization to be with us probably in the next six months. I estimate it will happen in this financial year.

Vishal Manchanda: Okay. And in the trials, how many patients got completely cured on account of the trial?

Bhushan Akshikar: So, if you've seen the comparator, Mr. Manchanda, there is no functional cure for chronic hepatitis B. So if you see the trial readouts, for the first time you have, I think, 25% of the patients getting complete functional cure. So I think that's from a baseline of almost nothing existing, you have something which is for the first time actually talking of a potential functional cure.

And that itself is a significant step change. So that's where the first readout is.

Vishal Manchanda: Right. Okay. And any sense on what is the current market size, like how many patients would be on a treatment for hepatitis B and some sense on the pricing strategy so that we can understand the potential opportunity here?

Bhushan Akshikar: Mr. Manchanda, at this stage, probably I won't hazard a guess in terms of pricing, because as I said, we're still launching this asset in some of the key markets. This requires a well-orchestrated effort, especially in some of the big markets like ours, where there's a huge need, there's a huge GlaxoSmithKline Pharmaceuticals Limited

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unmet need, but at the right price point. So I think we want to get this right. What's the space?

We'll definitely answer this transparently with you. But at this stage, it's still a work in progress.

Vishal Manchanda: And on oncology, you've been talking about opening a public account. So is that government patients or government hospitals that you're talking about when you say public accounts?

Bhushan Akshikar: So, if you see the speciality model, especially in oncology, what is known as CARE accounts, as a term across the industry, which is CARE is an acronym for CGHS, Army, Railways and ESIC patients. So those are the ones where you primarily start getting into ensuring that beneficiaries in all these institutions get access to your proprietary innovative medicine. So those are the accounts that we're working on in terms of inclusion.

Vishal Manchanda: And they kind of fetch you the same thing. So you have similar realizations across private markets and these public accounts?

Bhushan Akshikar: Absolutely, yes.

Vishal Manchanda: Okay. And on the patient

assistance program, is that funded from the GSK P&L or the parent P&L, the patient assistance program?

Bhushan Akshikar: So, it's all a blended, the way we work is obviously we look at the P&L per business unit. And I think it's all, we ensure that the margins remain undiluted for locally listed entities.

Vishal Manchanda: Okay. Right. And so this patient assistance program is usually higher when the product is launched and it comes, you kind of lower this as the product matures? Is that right?

Bhushan Akshikar: No, no, Mr. Manchanda. I think clearly every patient, regardless of where you are in your life cycle need, because disease doesn't differentiate whether you are a just launched product or in the market for five years, the assistance programs don't differentiate. So, I don't think that's the

criterion. I think it's about what's the unlock that we can have, especially given the huge cost that patients have to bear in our country. So, the answer would be no. Regardless of what stage in your life cycle you are, they would be consistent.

Moderator: Thank you. Our next text question is a text from Udhaya Prakash from Value Research. Would it be possible for you to give an overview on the margin expansion that has occurred over the last four years? Our top-line growth has been in single digits and a good chunk of the portfolio is in NLEM. Exactly what happened and what drove these margins?

Bhushan Akshikar: So, if you have seen the last four years, I will before I hand it over to Ron to give his flavor. You know, four years, three and a half years ago when I started doing these Investor meetings, my first Investor meeting I remember we were just included, a large part of our portfolio was included in the NLEM that was announced end of '22, around October, November, December 2022.

And at that time in that first Investor meeting, I had clearly articulated along with the then CFO our ability as well as our clear intention to grow volumes, and that has been the real underlying story for us. If you see each of the assets that was included, we have grown volumes at least two

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times over these four years, and that has been the story for us. So, there has been a relentless focus on pulling out inefficiencies.

We have been working on cost optimization. We have also looked at unlocking digital ways, embedding IA and AI both, as I keep saying, it is not just artificial intelligence but also seeing which are the areas where we can have intelligent automation or IA to help our frontline sales reps really help unlock and create that seamless experience for our healthcare practitioners. So, I think it is a mix of these three, but I would also request Ron to add to that.

Ronojit Biswas: Sure, happy to contribute. So, the way I see it, there are three levers we have used, and I will speak more to the gross margin and then I will add on. One is we have consciously focused on shifting our mix away from the price controlled to the other part of the portfolio. So that is mix.

Secondly, we've been quite competitive in terms of our pricing and our ability to, you know, keep in step with peer group pricing on portions of the portfolio. So pricing has helped, and we are maintaining the same price-led growth as the industry.

And then the third is we have actively worked very hard at reducing the cost of goods for some of our NLEM products through active sourcing changes, you know, forward contracts, etc. So that in -- those three have helped our gross margins, plus of course the SG&A efficiencies driven by field force productivity, AI-led initiatives has also compressed our SG&A. So you are right that all-in-all, that adds up to a nearly 10 percentage point EBITDA improvement that you saw on the slides that we presented over the last four years.

Moderator: Thank you. Our

next question is a message from Mehul Savla from Ripple wave Equity. No question, but would like to congratulate the GSK team for great focus, effort, and excellent communication regarding business and strategy. Best wishes.

Bhushan Akshikar: Thank you very much, Mr. Mehul. Thank you. Truly appreciate it.

Moderator: We have time for one last question. We have a question from Vishal Manchanda. Vishal, you may accept the prompt on your screen, unmute your audio and video, and proceed.

Vishal Manchanda: Yes, hi. So on the multiple myeloma drug where you got approval, does that compete with Darzalex? And if you could -- if you have the market opportunity size for Darzalex, how large is that?

Bhushan Akshikar: Mr. Manchanda, thanks a lot for that question. Yes, we do operate in the same market, but I wouldn't comment on a competitive brand. Blenrep or belantamab is the first antibody drug conjugate that has been approved for relapsed refractory multiple myeloma. I think the advantage, as I said, is it's one of the, in terms of the approval and given the nature of the refractory cases that we see, which is the nature of multiple myeloma as a disease, early approval and early usage in early lines is really the focus for us.

As I said in the beginning, at any given point of time, you have 40,000 -45,000 patients of multiple myeloma in India. Every year, you are adding about 17,000 to 18,000 new patients.

And you can imagine, there is a clear first-line treatment protocol that's used across by

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hematologists, and typically all patients will still relapse on that and will need a second line. And that's where you have the role of Blenrep or belantamab coming in.

Given the fact that we have data that clearly shows overall survival as well as progression-free

survival, you know, leading to several months of survival benefit. So, that's something that is unique to belantamab, and that's what we will focus on. In terms of, as I said, the values are an outcome. I think our ability to get as many patients with the benefit of having this treatment in early lines, in second line, will be the focus for us.

Vishal Manchanda: Got it, sir. Thank you very much.

Bhushan Akshikar: Thank you.

Moderator: Thank you. We have no further questions, ladies and gentlemen. On behalf of GlaxoSmithKline Pharmaceuticals Limited, we conclude this conference. Thank you all for joining us. You may now disconnect.

Bhushan Akshikar: Thank you very much.